

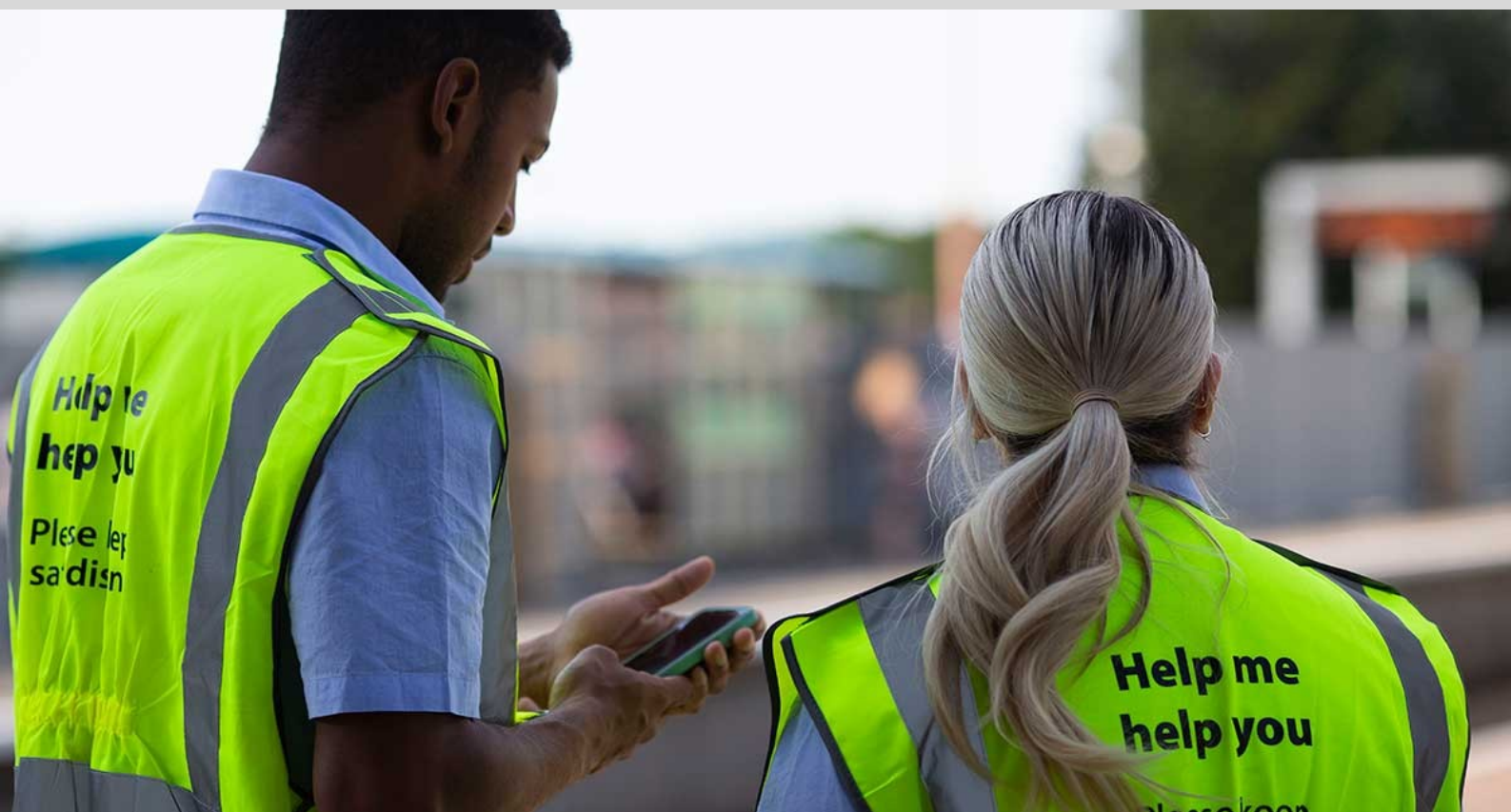


UNIVERSITAT
ROVIRA I VIRGILI

FROM COMPLIANCE TO COLLABORATION: SHARED
DECISION-MAKING, PSYCHIATRIC VIOLENCE, AND THE
ETHICS OF MEDICATION MANAGEMENT IN
CONTEMPORARY SPAIN

A BIOCULTURAL AND TRANSDISCIPLINARY ACTION
RESEARCH TOWARD STRUCTURAL REFORM AND
INSTITUTIONAL RESPONSIBILITY

HENNING (ENRIC) GARCIA TORRENTS



DOCTORAL THESIS
2025

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UNIVERSITAT ROVIRA I VIRGILI

FAIG CONSTAR que aquest treball, titulat “From compliance to collaboration: shared decision-making, psychiatric violence, and the ethics of medication management in contemporary spain: a biocultural and transdisciplinary action research toward structural reform and institutional responsibility”, que presenta Henning (Enric) Garcia Torrents per a l’obtenció del títol de Doctor, ha estat realitzat sota la meua direcció al Departament d’Antropologia, Filosofia i Treball Social d’aquesta universitat.

HAGO CONSTAR que el presente trabajo, titulado “From compliance to collaboration: shared decision-making, psychiatric violence, and the ethics of medication management in contemporary spain: a biocultural and transdisciplinary action research toward structural reform and institutional responsibility”, que presenta Henning (Enric) Garcia Torrents para la obtención del título de Doctor, ha sido realizado bajo mi dirección en el Departamento de Antropología, Filosofía y Trabajo Social de esta universidad.

I STATE that the present study, entitled “From compliance to collaboration: shared decision-making, psychiatric violence, and the ethics of medication management in contemporary spain: a biocultural and transdisciplinary action research toward structural reform and institutional responsibility”, presented by Henning (Enric) Garcia Torrents for the award of the degree of Doctor, has been carried out under my supervision at the Department of Anthropology, Philosophy and Social Work of this university.

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El/s director/s de la tesi doctoral
El/los director/es de la tesis doctoral
Doctoral Thesis Supervisor/s

Dedication

To those silenced, disappeared, and punished for speaking truth; to those subjected to domestic, medical and systemic violence; to all who have endured abuse in any form or severity, and to those whose lives were taken: this work is dedicated to you, standing as evidence against those responsible.

Funding statement

This research was carried out with the support of a predoctoral fellowship funded by the Spanish Ministry of Science, Innovation and Universities. The project was financed under the national program of Formación de Profesorado Universitario (FPU), a competitive grant scheme aimed at training university teaching and research staff. The author received funding through grant number FPU19/00028, which provided salary support, research mobility funding, and institutional resources throughout the doctoral period. The FPU program is one of the principal instruments of the Spanish government to promote the development of highly qualified academic staff, combining advanced research training with supervised teaching activities in universities. The financial support received under the FPU framework enabled the realization of extended ethnographic fieldwork, the design and implementation of large-scale surveys among users and professionals, and the integration of legal, clinical, and social data into a transdisciplinary analytic framework. This funding ensured the continuity of the doctoral project, the dissemination of its intermediate results in scientific forums, and the incorporation of the research into international networks of excellence, including European COST actions and collaborative initiatives in public health, anthropology, and psychiatry.

The Ministry's investment through the FPU19/00028 grant made it possible to generate the empirical evidence and theoretical analysis that underpin the present dissertation, and it is gratefully acknowledged as the primary source of financial support for this work.

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Abstracts and keywords in English, Spanish and Catalan

Abstract: This thesis argues that the Spanish psychiatric system, supported by legal and administrative structures, systematically violates the autonomy and dignity of individuals through coercive and non-consensual practices masquerading as care. Diagnoses are imposed without empirical rigor, treatments are administered without valid consent, and discomfort is decontextualized and medicalized, producing structural iatrogenic damage that prevents recovery. At the root is the lack of implementation of shared decision-making and respect for the right to self-determination. Anchored in participatory action research from a biocultural analysis perspective, this work integrates ethnographic, legal, and analytical evidence to document these mechanisms and propose grounded best practices focused on human rights, prevention, and collaborative care. The findings corroborate the existing medical consensus that ethical and effective health care, which also includes mental health in the most serious cases, ought to be rights-based, non-coercive, recovery-oriented, personalized, and based on trust and shared responsibility, not submission.

Resumen: Esta tesis argumenta que el sistema psiquiátrico español, con el respaldo de estructuras legales y administrativas, viola sistemáticamente la autonomía y la dignidad de las personas mediante prácticas coercitivas y no consensuadas que se hacen pasar por cuidados. Se imponen diagnósticos sin rigor empírico, se administran tratamientos sin consentimiento válido y se descontextualiza y medicaliza el malestar, produciendo daños iatrogénicos estructurales que impiden la recuperación. La raíz de este problema es la falta de implementación de la toma de decisiones compartida y del respeto al derecho a la autodeterminación. Basado en la investigación-acción participativa desde una perspectiva de análisis biocultural, este trabajo integra evidencia etnográfica, legal y analítica para documentar estos mecanismos y proponer buenas prácticas fundamentadas, centradas en los derechos humanos, la prevención y la atención colaborativa. Los hallazgos corroboran el consenso médico existente de que una atención sanitaria ética y eficaz, que también incluye la salud mental en los casos más graves, debe basarse en los derechos, ser no coercitiva, estar orientada a la recuperación, ser personalizada y basarse en la confianza y la responsabilidad compartida, no en la sumisión.

Resum: Aquesta tesi argumenta que el sistema psiquiàtric espanyol, sostingut per estructures legals i administratives, viola sistemàticament l'autonomia i la dignitat dels individus a través de pràctiques coercitives i no consensuades emmascarades

com a atenció. Els diagnòstics s'imposen sense rigor empíric, els tractaments s'administren sense consentiment vàlid i el malestar es descontextualitza i es medicalitza, produint un dany iatrogènic estructural que impedeix la recuperació. A l'arrel hi ha la manca d'implementació de la presa de decisions compartida i el respecte pel dret a l'autodeterminació. Ancorat en la investigació-acció participativa des d'una vessant d'anàlisi biocultural, aquest treball integra evidències etnogràfiques, legals i analítiques per documentar aquests mecanismes i proposar millors pràctiques centrades en els drets humans, la prevenció, l'atenció col·laborativa i la plena recuperació. Les troballes corroboren el consens mèdic existent que l'atenció sanitària ètica i eficaç, que també inclou la salut mental en els casos més greus, ha de ser basada en drets, no coercitiva, orientada a la recuperació, personalitzada i basada en la confiança i la responsabilitat compartida, no en la submissió.

English: shared decision-making; coercion; iatrogenic harm; human rights; structural violence; Español: desición compartida; forzamiento; daño iatrogénico; derechos humanos; violencia estructural; Catalan: presa de decisions compartida; coerció; dany iatrogènic; drets humans; violència estructural.

Chapter 1. General framework and delimitation of the research

1. Thesis, aim and objectives

Thesis: This dissertation investigates how psychiatric systems can transition from coercive and overmedicalized practices toward evidence-based models of recovery-oriented care, grounded in patient autonomy, shared decision-making and culturally situated best practices.

The thesis posits that coercive and pharmacologically dominant interventions in contemporary psychiatry are not isolated anomalies or residual practices but remain structurally embedded, sustained by cultural factors, market logics, institutional norms, juridical indeterminacy, and fragmented administrative governance (Kistler et al., 2023). These practices remain embedded across clinical, familial, and bureaucratic domains in our society, resulting in mechanisms of control that restrict recovery pathways, diminish patient autonomy, and contribute to adverse social and health outcomes (Fava & Rafanelli, 2019). Coercion, conceptualized herein as the systematic exclusion of the individual from decisions concerning their own treatment, as well as violence in other forms, operates as an institutionalized response to psychosocial distress, frequently substituting care with mandated compliance (Beviá & Girón, 2017). The research argues that such practices indicate not individual professional failure but the normalization of technocratic logics shaped by risk aversion, diagnostic reductionism, and asymmetrical knowledge relations (Burstow, 2015b).

The thesis advances the proposition that a sustainable transformation of psychiatric care requires not only technical adjustments to clinical protocols or additional practitioner training but a broader systemic reconfiguration toward dialogically informed, procedurally accountable, and structurally responsive forms of care (Torrents & Björkdahl, 2024). Dialogical practice, in this context, is not reduced to a specific branded methodology but conceptualized as a foundational epistemological principle grounded in principles of mutual recognition, institutional reflexivity, and reciprocal legitimization of diverse knowledge claims (Gooding et al., 2022). The empirical validation of this proposition is carried out through a mixed-methods framework incorporating legal analysis, clinical ethnography, and embedded participant observation across multiple psychiatric and policy environments (Sashidharan et al., 2019). The findings elucidate how

normative frameworks often coexist with violations in practice, and how professionals themselves operate under constraints that undermine ethical care. The thesis aims not only to critique existing systems but to delineate institutional pathways toward legally coherent and recovery-oriented mental health governance (Pūras & Gooding, 2019).

This dissertation is guided by the central research question: How do coercive psychiatric practices, overmedication, and institutional neglect persist within the Spanish mental health system despite legal safeguards, clinical ethics, and international human rights standards? Conversely, it also asks: What systemic, clinical, and epistemological conditions enable the emergence and consolidation of participatory, trauma-informed, and biologically grounded psychiatric care that respects autonomy, protects rights, and improves outcomes?

Table 1. Alignment between dissertation aims, objectives and questions

Dissertation Aim	Aligned Research Question	Operational Objective
Support psychiatric systems capable of real-time correction, accountability, and integrity preservation	What forms of systemic violence and institutional failure are occurring in psychiatric governance, and how can they be corrected?	Identify and analyze the structural, legal, and epistemological failures in psychiatric care to develop evidence-based reforms
Foster a paradigmatic shift from pharmacological control to integrative, interdisciplinary, and reciprocal models of care	How can clinical and experiential evidence be integrated across biological, psychological, and social domains of health?	Develop and validate feedback systems, biosignal-informed tools, and patient-led evaluative mechanisms
Realign psychiatry with empirical coherence, legal safeguards, and rights-based participatory governance	What are the enabling conditions for institutional change and participatory oversight of psychiatric systems?	Implement translational infrastructures, policy interventions, and participatory monitoring aligned with international standards

This table presents the alignment between the core aims of the dissertation and the research questions and operational objectives introduced in section 1.9. Each component of the central thesis is matched with a guiding question and its corresponding implementation strategy. These elements are further developed in the methodological framework detailed in section 1.9 and operationalized through the research design presented in Chapter 2.

Aim: To contribute to the development and implementation of clinical, legal and institutional frameworks that promote self-managed and participatory mental health care, based on action-research embedded in real-world settings and oriented toward continuous system improvement.

The aim of a thesis refers to its principal purpose or intended contribution. It articulates what the research seeks to achieve at a systemic or conceptual level, distinguishing it from the thesis itself, which presents a central proposition to be argued and demonstrated. While the thesis posits a testable claim grounded in empirical and theoretical evidence, the aim defines the overarching orientation of the work, including its intended impact on policy, practice, or scientific understanding. In action-research frameworks, such as the one adopted here, the aim also encompasses applied objectives and transformative intentions embedded within real-world systems.

The aim of this dissertation is to support the emergence and implementation of psychiatric systems that are capable of real-time correction, structural accountability, and the preservation of both bodily and legal integrity. It seeks to foster a paradigmatic transition away from predominant reliance on psychopharmacological interventions toward integrative models of care supported by interdisciplinary evidence and therapeutic reciprocity (Sells et al., 2006). This model is predicated on the understanding that mental health cannot be reduced solely to neurochemical imbalance or behavioral deviation (Naimo, 2020) but ought to be interpreted through the interaction of biological regulation, experiential development, and sociopolitical context. The research integrates contributions from translational neuroscience, physiological psychiatry, psychoneuroimmunology, psychoneuroendocrinology, developmental psychopathology, trauma-informed frameworks, nutritional psychiatry, psychosocial epidemiology, and ecological public health. These disciplines provide converging evidence on how trauma, inflammation, metabolic dysregulation, and environmental precarity shape both the emergence and chronicity of psychiatric conditions (Ortiz Lobo & Ibáñez Rojo, 2011).

In parallel, the dissertation draws from cultural psychology, phenomenological psychiatry, narrative theory, and medical anthropology to situate clinical phenomena within relational, cultural, and historical matrices. Theories of embodied cognition, the extended mind, and relational consciousness are employed not as speculative frameworks but as scientifically corroborated perspectives on the distributed and socially mediated nature of mental processes (Leiva-Peña et al., 2021). This conceptual orientation is operationalized through the design of participatory feedback mechanisms, patient-led evaluative structures, and embedded data infrastructures, allowing clinical systems to recalibrate based on experiential, physiological, and ethical inputs (Marmot & Bell, 2019). The aim is not to displace psychiatry but to realign it with a broader horizon of empirical coherence, patient protection, and epistemic humility. The dissertation intervenes at

the intersection of clinical decision-making, institutional design, and legal structure, offering pragmatic models of regulation and care grounded in ongoing verification, transparent governance, and open science standards (U.S. National Academies of Sciences, Engineering, and Medicine, 2018).

The integration of these disciplines and methodologies informs a comprehensive framework through which psychiatric governance can become both scientifically valid and socially just (Elliott & Resnik, 2019). The objectives derived from this aim are elaborated across biological, psychological, and social domains of intervention and correspond to the structure of this dissertation. These objectives are detailed below and implemented through rigorous multi-sited fieldwork, participatory action research, and critical legal-ethical analysis:

Table 2. Biological objectives and implementation strategies

Relevant SDG	Objective	Method of Implementation
SDG 3 - Good Health and Well-being	Conduct sustained participant observation within clinical, legal, and policy environments, engaging directly with professionals, users, families, and decision-makers to co-produce relevant knowledge without causing harm.	Multi-site fieldwork across Spain and Indonesia through ethnographic immersion, and surveys to affected individuals and professional stakeholders.
SDG 9 - Industry, Innovation and Infrastructure	Critically examine the cultural, scientific, and legal foundations of current psychopharmacological standards, particularly in relation to overmedication and the marginalization of alternative approaches.	Historical-legal review of drug regulation, comparative analysis of national psychiatric protocols, and surveys presented to psychiatry scholars and other mental health specialists.

This table outlines the objectives and methodological approaches related to the biological dimensions of health, focusing on empirical fieldwork and critical examination of pharmacological norms. These components align with SDG 3 and SDG 9 and support evidence-based transformation of clinical practice and regulation.

The biological dimension of this dissertation, table 2, integrates an exploration into the full potential of current best practices, a human approach to rapport, trust and both shared decisions and responsibility, and new technologies to support precision, accountability, and continuous improvement in clinical practice. The thesis maintains that health ought to be understood not as the management of isolated symptoms but as the result of dynamic interactions between biological integrity, contextual conditions, and institutional responsibility. In Spain, an enduring reliance on pharmacological protocols in Spain has reinforced a medical model that overlooks systemic harms and constrains recovery-oriented practice (S. I.

Cohen, 1975). This work challenges such structural inertia by introducing open data frameworks, biosignal-informed monitoring, and interoperable documentation systems that empower patients and clinicians to co-evaluate treatment effects in real time. By embedding positive feedback loops within participatory action research cycles and adopting open science standards, the synthesis model reorients health systems toward adaptive learning and biomedical accountability. Legal safeguards and regulatory mechanisms are proposed to ensure transparency, protect patient data, and prevent the misuse of diagnostic categories and therapeutic interventions that might otherwise reinforce chronicity and systemic exclusion (Sampietro et al., 2023a).

Table 3. Psychological objectives and implementation strategies

Relevant SDG	Objective	Method of Implementation
SDG 4 Quality Education	Document failures in clinical responsibility, mechanisms of harm, and systemic omissions, while focusing on how feedback-informed practices can improve recovery trajectories and therapeutic alliance.	Ethnographic fieldwork in excellence centers such as Trieste, engagement with professionals and survivors to understand educational needs.
SDG 5 Gender Equality	Trace how diagnostic processes can contribute to identity erasure and legal dispossession, and propose safeguards that protect continuity of personhood, legal status, and relational integrity.	Analysis of gendered diagnostic trajectories, legal documentation of incapacitaciones, and testimony-led mapping of institutional violence.

This table presents the objectives linked to psychological dimensions of health and care, addressing clinical responsibility, diagnostic integrity, and gender-based harms. The methodologies integrate lived experience and structural analysis, contributing to SDG 4 and SDG 5.

At the psychological level, table 3, new technologies are mobilized not for surveillance or standardization, but for restoring agency, memory, and personhood within therapeutic processes. This thesis defends the centrality of narrative, self-determination, and cognitive restoration as key pillars of healing. The antithesis it opposes is aclinical cultures that frequently disregard subjective complexity, impose rigid diagnostic categories, and limit patients' interpretive agency (Yarborough et al., 2016). To counter this, the research deploys digital storytelling tools, reflexive platforms for therapeutic alliance assessment, and decision-support systems aligned with trauma-informed and rights-based frameworks. Feedback loops are built into the structure of care itself, allowing for regular recalibration and person-centered learning. These systems are designed not only for individual care but for structural change: civil society engagement, patient-led data interpretation, and continuous education of professionals are embedded as core functions. Legal analysis identifies

procedural shortcomings and proposes enforceable safeguards against identity erosion and epistemic harm, supporting the continuity of personhood and the right to recovery (McKenna et al., 2016).

Through the EU BEACON One Health Education COST action, devoted to improve the health curricula in schools EU-wide, that the researcher leads along half thousand of specialists in a growing network, the work undertaken during this thesis dissertation contributes to the design of integrative governance architectures that facilitate structural correction and enhance legal enforceability from middle school education onward (Every-Palmer et al., 2021). Participatory digital infrastructures are set to allow for multilevel input from families, professionals, users, and civil society, creating scalable mechanisms for identifying harmful practices, highlighting best models, and ensuring policy coherence to foster health and well-being for all. Open science is not an afterthought in this scheme but a foundational principle: all data is anonymized, encrypted, and made accessible for democratic oversight, with clear limits set to prevent institutional capture or reappropriation. Feedback mechanisms are calibrated to track not only health outcomes but institutional behavior, closing gaps in enforcement and reducing impunity. The model refuses the assumption of chronicity as an inevitable outcome, insisting instead on recovery oriented, resilience building, personalized practices, and structural repair as best standards to be enforced. Health is not a static state nor a managed deficit, but a relational, social, and dynamic capacity that ought to be protected and continuously renewed (Christakis & Fowler, 2009).

Table 4. Social Objectives and Implementation Strategies

Relevant SDG	Objective	Method of Implementation
SDG 10 - Reduced Inequalities	Gather and analyze testimonies and practices from patients, relatives, clinicians, researchers, and public officials, documenting both existing challenges and emerging models of respectful and effective care.	Narrative data collection, anonymous surveys, legal case analysis, and observation of decision-making processes in institutional settings.
SDG 16 - Peace, Justice and Strong Institutions	Identify enabling conditions for the implementation of autonomous and collaborative medication management, highlighting tensions between institutional norms and lived experience.	Documentation of pilot practices, stakeholder workshops, and field-based monitoring of patient-practitioner negotiation in psychiatric contexts.
SDG 17 - Partnerships for the Goals	Map feedback loops across individual, institutional, and regulatory levels, identifying specific leverage points for positive change in policy and clinical governance.	Systems mapping through participatory modeling, stakeholder interviews, and analysis of international and national policy interdependencies.
SDG 1 - No Poverty	Provide empirically grounded recommendations and methodological tools for the implementation of dynamic, person-centered, and rights-respecting mental health care, where continuous reflection and system learning support long-term recovery and social inclusion.	Development of adaptable care protocols, translation of research findings into clinical training modules, and policy briefs shared with international networks including EU BEACON.

This table identifies objectives targeting the social and institutional determinants of mental health, with a focus on inequalities, legal protections, collaborative governance, and inclusion. These objectives are operationalized in line with SDG 1, SDG 10, SDG 16, and SDG 17.

This dissertation defends a thesis that is both scientifically grounded and ethically imperative: that recovery in mental health is not only possible but ought to be structurally enabled, institutionally supported, and epistemically protected, as it is the duty of health-care systems to aim to achieve so in all cases. Against entrenched systems that perpetuate coercion, chronicity, and epistemic injustice under the guise of care, the research offers a multidimensional synthesis grounded in the best available evidence and the lived realities of those most affected. By integrating translational neuroscience, dialogical and developmental models, legal safeguards, and participatory technologies, the dissertation articulates a coherent and actionable framework for system-wide transformation. This research work aims to operationalize the system reconfiguration, affirming that recovery is not to be the exception, but the ethical and structural baseline from which all psychiatric care ought to begin (Adshead, 2000).

The thesis situates Spain within a broader European and global controversy about coercion, rights, and effective care, framing an empirical programme that links human rights analysis with implementation science and deprescribing practice. Recent syntheses document persistent reliance on coercive measures across Europe with marked legal and clinical heterogeneity, and call for measurable reductions anchored in service user leadership and rights-based models that are locally adapted rather than merely declarative in law (Salvador-Carulla et al., 2010). In Spain, new empirical work emphasises the lived experience of coercion and the structural roots of rights shortfalls in routine practice, which this dissertation addresses through a mixed methods design that links evidence to feasible reforms in clinic and policy (Robinson, 2007).

1.1. Brief historical-structural background: care, coercion and governance in Spain

The governance of mental distress in the Iberian Peninsula historically operated as a mechanism of normative regulation, closely linked to religious, political and social control. From the late medieval period, deviation from moral or epistemic orders was managed through institutions that conflated spiritual error, cognitive disturbance and behavioural irregularity. Religious and charitable institutions deployed rituals and confinement, often without differentiation between psychological and moral phenomena (Appignanesi, 2011). As historical analyses of Iberian monastic and charitable institutions demonstrate, therapeutic care and social order were structurally entwined, with individual experience subordinated to cosmological and institutional logics (Madrid & Parada, 2016). This genealogy helps explain why psychiatric practices in Spain have long been framed less as vehicles of therapeutic recognition than as instruments of social order.

Historical analyses show how Spanish psychiatry's modernisation has coexisted with enduring custodial logics, producing tensions between community rhetoric and institutional inertia that shape today's care arrangements (Mañas, 2014). Contemporary qualitative studies describe how professionals interpret coercion within everyday service pressures, while legal scholarship documents ambiguities at the boundary between involuntary internment and unlawful retention, underscoring the need for unambiguous safeguards and transparent oversight (Huertas García, 2001).

The Enlightenment and the institutionalisation of psychiatry in the eighteenth and nineteenth centuries did not abolish punitive traditions but rearticulated them under

scientific and administrative logics. The emergence of asylums across Spain, legitimised by moral treatment discourses, reinforced separation and confinement, and archival research shows how provincial mental hospitals perpetuated custodial models in which legal and familial prerogatives authorised long admissions. Diagnoses such as melancholia and mania, imported from French and German nosologies, were used administratively, often without standardisation, to justify confinement and exclusion. Spanish historiography demonstrates that psychiatric institutions in this period prioritised surveillance and order over recovery, replicating theological models of containment through a biomedical idiom (Foucault, 2006).

Gendered dimensions were central to these developments. The category of hysteria, widely used in Spain during the nineteenth century, pathologised female agency and justified confinement of women for behaviours considered transgressive. Socio-historical studies document disproportionate admissions of women and the role of psychiatry in reinforcing patriarchal norms (Ussher, 2023). Thus, psychiatric institutions functioned as mechanisms of biopolitical triage, where distinctions between illness, illegitimacy and nonconformity collapsed under medical rationality.

The twentieth century intensified these dynamics. During the interwar period, eugenic discourses linked mental illness with degeneration and decline, legitimising segregationist policies. Under Francoism, psychiatric institutions became directly integrated into the apparatus of authoritarian rule. Historians have documented the use of psychiatric certification to delegitimise political dissent, regulate sexuality and reinforce Catholic-nationalist ideals (Rosales-Crespo et al., 2025). Medical records were opaque, procedural safeguards minimal, and coercive hospitalisation or invasive treatments frequent. These legacies of authoritarian psychiatry created institutional norms of opacity and weak accountability that persisted into the democratic era.

The democratic transition brought legislative reforms intended to dismantle asylums and strengthen rights, particularly during the 1980s psychiatric reform movement (González, 1998). Yet peer-reviewed evaluations show uneven implementation across autonomous communities, with insufficient community alternatives and persistent reliance on pharmacological interventions (Martínez Azumendi, 2021). Although laws established formal rights to informed consent, and also require judicial oversight for non-voluntary internment, empirical studies indicate that coercive practices such as involuntary admission and chemical restraint remained routine (Campos, 2022).

In the twenty-first century, austerity measures and service fragmentation reinforced reliance on pharmacological management. Pharmacoepidemiological studies reveal high rates of antipsychotic and benzodiazepine prescribing, especially among the elderly (Roncero et al., 2025). Clinical studies report that long-acting injectable antipsychotics and adherence-focused interventions are widely used as strategies of behavioural management (Douglas, 2018b). At the same time, sociolegal research highlights variability in judicial practice around involuntary measures and continuing gaps between rights-based policy and clinical implementation (Lobo, 2016). Recent reforms, including Ley 8/2021 on legal capacity, seek to align Spanish law with the UN Convention on the Rights of Persons with Disabilities, replacing incapacitation regimes with supported decision-making, but the literature notes difficulties in operationalisation (Sánchez Ballesteros, 2023).

The relevance of this historical background lies in its demonstration of continuity. Research shows how categories and practices of exclusion from the medieval period to Francoism have left enduring traces in the culture of psychiatric care, shaping clinical hierarchies, institutional routines and family expectations (Desviat, 2016). Legal analysis further demonstrates that although rights frameworks are in place, coercion remains authorised in Spanish law, making historical legacies directly relevant to contemporary governance (Lázaro, 2000). This genealogy shows why coercion should not be regarded as a residual aberration but as a structural feature of psychiatric care that requires critical analysis to be effectively dismantled (Shim, 2021).

1.2. Current framework: forced psychiatric interventions and institutional accountability

Involuntary outpatient treatment, including compulsory medication, continues to be debated in Spain despite a substantial body of research questioning its clinical efficacy and highlighting associated ethical concerns. Legislative proposals to establish compulsory outpatient regimes have been repeatedly introduced since the early 2000s but consistently rejected by the Congreso de los Diputados and the Consejo General del Poder Judicial, as well as by professional and user organisations (Flores & González, 2009). Arguments in favour of such measures generally posit that enforced compliance reduces hospital readmissions, lowers risk of violence, and alleviates systemic pressures on services. However, empirical research does not support these claims when examined against evidence-based practice and rights-based frameworks (Anguita, 2005).

A systematic review conducted under Cochrane methodology concluded that involuntary outpatient treatment did not consistently improve key outcomes compared with standard voluntary care. Indicators such as readmission rates, social functioning, mental state, quality of life, and satisfaction with care showed significant worsening in individuals subjected to coercive outpatient orders than those in voluntary treatment (Roig Salas et al., 2012). Meta-analyses further indicate that preventing a single hospital readmission may require imposing coercive treatment on more than eighty individuals, a ratio that raises concerns of proportionality and distributive justice in comparison with other domains of healthcare (Guimón, 2006). These findings suggest that the persistence of proposals for coercive outpatient treatment reflects institutional priorities centred on control and risk management rather than demonstrated therapeutic benefit (Bezanilla et al., 2016).

In Spain, the re-emergence of such initiatives is linked to persistent deficits in community mental health infrastructures. Analyses of regional services describe fragmentation, uneven resource allocation, and insufficient continuity of psychosocial support, which contribute to reliance on coercion as a substitute for sustained relational care (Montejo et al., 2006). Legal arguments in favour of outpatient coercion frequently invoke expansive interpretations of incapacity and risk, drawing on categories such as lack of insight or treatment refusal. These concepts lack precise clinical or legal definition and are vulnerable to subjective misuse (Lázaro-Mateo et al., 2023). Moreover, the attempt to frame coercion as preventive obscures structural deficiencies by individualising responsibility for system failure (Franco, 2021).

Empirical evidence from Spain and internationally shows that shared decision-making and collaborative treatment planning are associated with improved engagement, enhanced functional outcomes, and reduced relapse rates (Villagrán et al., 2014). Studies consistently identify the therapeutic alliance as one of the strongest predictors of long-term recovery across psychiatric conditions (Pozón, 2012). Coercive measures undermine this alliance, contribute to experiences of mistrust, and are reported as traumatic by service users, who describe anger, avoidance, and disengagement following forced interventions (Gutiérrez Gea et al., 2015). These reactions are not incidental but represent structural harms that reduce help-seeking and compromise adherence to future care (Buedo & Luna, 2021).

In contrast, non-coercive models have demonstrated both ethical and clinical advantages. Intensive home-based care, assertive community treatment, crisis resolution teams, and peer-supported services have been implemented in diverse

European contexts with measurable outcomes, including reductions in hospital use, improved social functioning, and higher user satisfaction (Kistler et al., 2023). Evidence from Spanish pilots of community rehabilitation and supported housing further indicates positive effects on autonomy and social inclusion (Kaminskiy et al., 2017). These interventions are also cost-effective compared to repeated admissions and the judicial costs of coercive enforcement (Sugiura et al., 2019).

The prevention of relapse and reduction of acute crises are more effectively achieved by addressing social determinants and ensuring continuity of care. Research on supported housing, participatory rehabilitation, and structured shared decision-making demonstrates their capacity to reduce institutional dependency and foster recovery (DeMatteo et al., 2013). Moving away from coercion therefore constitutes not only a normative alignment with human rights obligations, including the Convention on the Rights of Persons with Disabilities and Ley 8/2021 on legal capacity, but also a practical strategy for building effective and sustainable mental health systems (Perestelo-Perez et al., 2017). Reliance on coercion, by contrast, reflects systemic inertia and resistance to structural change rather than evidence-based psychiatric care (Iglesias et al., 2025).

The Spanish psychiatric system continues to exhibit practices that raise significant concerns in light of both national law and international human rights standards. Documented practices include involuntary drug administration without full procedural safeguards, chronic polypharmacy, diagnostic overreach, legal incapacitation, coercive hospitalization, gender-biased assessments, and the dismissal of complaints from service users. Each of these phenomena has been described in the Spanish literature and supported by comparative research, and their persistence illustrates structural tensions between the formal recognition of rights and the custodial logics embedded in psychiatric governance (Pina-Camacho et al., 2021).

The involuntary administration of medication, particularly long-acting injectables, remains a central area of contention. Reports indicate that safeguards under the laws on patient autonomy and judicial authorisation requirements under Article 763 of the Ley de Enjuiciamiento Civil (Cañete-Nicolás et al., 2012) are applied inconsistently, with family consent or clinical authority frequently prioritised over direct user participation (Gías Gil, 2013). This reflects a systemic deference to professional judgment, which undermines supported decision-making obligations mandated by Ley 8/2021 and the Convention on the Rights of Persons with Disabilities (M. T. Fernández, 2017).

Polypharmacy represents a further structural concern. Studies of prescribing in Spain demonstrate frequent use of multiple psychotropics, often without systematic deprescription protocols or formal monitoring of cumulative risk (Roig, 2018). International and national guidelines increasingly recommend structured deprescription programmes and training in rational pharmacotherapy, yet implementation has lagged, perpetuating dependence and metabolic complications (Flores & González, 2009).

Diagnostic expansion has also been critiqued. Spanish legal and historical analyses show that DSM-derived taxonomies have been used in broad and at times reductive ways, frequently conflating distress with incapacity or deviance (Rubio, 2009). Testimonies and ethnographic work highlight how diagnostic categories can be mobilised to justify involuntary measures, illustrating epistemic hierarchies that prioritise clinical authority over experiential knowledge (Crichton et al., 2017).

Legal incapacitation has undergone significant reform with the adoption of Ley 8/2021, which replaced incapacitation procedures with supported decision-making frameworks. Prior to this reform, Spanish civil law authorised the removal of legal capacity based on psychiatric diagnosis, often with limited procedural safeguards (O. Fernández et al., 2018). Although the new law aligns Spain with the CRPD, literature underscores that implementation challenges remain, with courts and services adapting slowly to the requirement of full recognition of legal personhood (United Nations, 2017).

Coercive hospitalisation persists despite its formal regulation under Article 763 LEC, which requires judicial review. Empirical analyses report regional variation, with admissions frequently authorised on the basis of institutional risk aversion and limited community resources, rather than as measures of last resort (Desviat, 2010). Expert commentary emphasises that investment in community services remains uneven across autonomous communities, with hospital-based models continuing to absorb the majority of resources (Seva, 2002).

Gender bias within psychiatric practice in Spain is well documented. Historical studies show the disproportionate confinement of women under diagnostic categories such as hysteria (Minkowitz, 2024), and contemporary research highlights ongoing androcentric paradigms in diagnosis and treatment planning (Río Pedraza, 2022). Feminist psychiatric scholarship calls for the integration of gender-aware protocols and mandatory bias training in clinical education (Castañeda Álvarez & Poma Yaro, 2017).

The neglect of user complaints and reports of harm has been repeatedly identified in reports from the Defensor del Pueblo and in qualitative fieldwork with service users.

Institutional responses have often lacked transparency, with limited external accountability mechanisms (Anguita, 2005). Comparative analysis recommends protections for whistleblowers, independent audits, and the integration of participatory quality improvement systems, consistent with WHO QualityRights guidance (Funk & Bold, 2020).

These harmful practices and structural causes highlight the need for systemic reform. Literature emphasises the necessity of supported decision-making frameworks, deprescription protocols, diagnostic revision, gender-aware training, external accountability, and the redirection of resources toward community-based services (Funk et al., 2021a).

At a systemic level, several failures can be identified across Spanish psychiatric governance. Epistemic exclusion manifests in the discounting of lived experience and the pathologisation of dissent, a phenomenon well documented in narrative research and in the application of DSM criteria in Spanish contexts (Rose, 2017). Feedback failure is evident in the absence of systematic outcome monitoring and limited learning from malpractice cases, despite recommendations for participatory audit systems and continuous quality improvement (Ramos-Pozón et al., 2025a). Institutional impunity persists due to the absence of sanctions for abuse or neglect, with ombudsman reports and legal proceedings highlighting recurrent gaps in enforcement (Calcedo-Barba et al., 2024a). Policy inertia reflects a disconnection between evidence and policy implementation; evaluations of Spanish reforms repeatedly stress that evidence on community care effectiveness has not been matched by investment (Carbonell Marqués & Navarro-Pérez, 2019). Disabling governance is characterised by decision-making structures that marginalise user input, in contrast with models such as open dialogue that demonstrate the feasibility of deliberative participation (Urbanos-Garrido & Agúndez, 2025). The performativity of ethics has been described in Spanish hospitals, where ethics committees often function as bureaucratic cover rather than as mechanisms of substantive accountability (Parrabera-García et al., 2023).

Together, these systemic failures illustrate how governance structures, feedback mechanisms and accountability deficits perpetuate coercive and ethically compromised practices. The literature indicates that reform must address these foundational issues, through transparent ethical governance, participatory monitoring, and the integration of experiential knowledge into psychiatric practice (Zamorano et al., 2023).

Recent nationwide and unit-level studies in Spain associate the experience of coercive measures with post-traumatic responses and lower satisfaction, and link

perceived care quality with perceived coercion during acute admissions, suggesting that governance should track patient-reported outcomes alongside incident data (Gil-Borrelli et al., 2017). A human rights analysis of Spanish practice argues that procedural guarantees remain insufficient without systematic prevention, independent monitoring, and service user-led training, a position that aligns with international reviews calling for concrete reduction targets and alternatives in place of formalistic compliance (Campos, 2010).

1.3. Overmedication and pharmacocentrism in Spanish healthcare and psychiatry

The extensive use of psychotropic medication in Spanish healthcare cannot be reduced to isolated prescribing errors or occasional lapses in clinical judgment. It constitutes a systemic phenomenon, anchored in a medical model that has structurally legitimised pharmacological management as the primary modality of care. Multiple studies document that Spain ranks among the highest European countries in the consumption of benzodiazepines, antidepressants, and antipsychotics, with levels of use consistently exceeding guideline recommendations (Degnin, 2009). Data from the Agencia Española de Medicamentos y Productos Sanitarios confirm that benzodiazepines remain among the most prescribed drugs nationwide, with a prevalence surpassing the epidemiological incidence of anxiety disorders (Gallego Antonio et al., 2016). This situation reflects not only clinical practices but also legislative frameworks, medical education curricula, the influence of the pharmaceutical industry, and cultural expectations that equate care with medication (Cosgrove & Wheeler, 2013).

The consequences of this pharmacocentric configuration are clinically and socially significant. Long-term benzodiazepine use is associated with tolerance, dependence, withdrawal syndromes, cognitive impairment, and increased risk of falls and fractures in older adults (Ortiz-Lobo et al., 2017). Prolonged antidepressant use beyond evidence-based indications has been linked to sexual dysfunction, emotional blunting, and difficulties in discontinuation (Hert et al., 2011). Antipsychotic maintenance therapy, particularly with long-acting injectables, carries well-documented risks of metabolic syndrome, extrapyramidal effects, sedation, cardiovascular complications, and increased mortality (Dencker et al., 1986). The widespread absence of deprescription protocols exacerbates these harms, as most patients remain on medication for years without systematic review of risk-benefit balance (Nyttingnes et al., 2016).

Beyond individual adverse effects, overmedication contributes to chronic disability and premature mortality. Pharmacoepidemiological studies in Spain report elevated rates of polypharmacy in psychiatric populations, with associations to reduced life

expectancy, largely due to cardiovascular disease and metabolic complications (Villena-Jimena et al., 2023). These outcomes are not limited to those diagnosed with severe mental disorders but extend to populations prescribed psychotropics for insomnia, somatisation, or social distress. In these contexts, medication becomes a pathway for the chronicisation of suffering, transforming transient or situational difficulties into long-term medicalised conditions (Scheper-Hughes, 1983).

The systemic impact of overmedication includes substantial resource diversion. Public health expenditure is disproportionately allocated to pharmaceutical procurement and monitoring, reducing funding for community-based psychosocial interventions (Arango et al., 2017). This resource misallocation contributes to a self-reinforcing cycle: underfunded psychosocial services increase reliance on medication, which in turn justifies continued investment in pharmacological infrastructures. The resulting imbalance undermines prevention, recovery, and social reintegration initiatives, contributing to the persistence of disability pensions and long-term institutional dependence (Fitzsimons, 2009).

Professional consequences are also evident. Overreliance on medication contributes to clinical burnout, as practitioners confront repeated cycles of relapse, chronicity, and limited therapeutic progress (Villarroel, García, et al., 2021). General practitioners, who issue the majority of psychotropic prescriptions, operate under conditions of high patient load, short consultation times, and scarce access to referral networks. Within these constraints, prescribing functions as an expedient but inadequate response to structural deficiencies (Gutowski et al., 2023). The mismatch between clinical ideals and systemic limitations erodes professional morale and contributes to workforce attrition (Gupta & Cahill, 2016).

Patterns of overmedication also reproduce structural inequities. Evidence shows that vulnerable groups, older adults, women, migrants, LGBTI+ community, low-income populations, disabled persons, and individuals with intellectual disability, are disproportionately subjected to long-term psychotropic use (Ussher, 2023). Sociological analyses highlight that social suffering related to unemployment, precarious housing, and gender-based violence is frequently reframed as psychiatric disorder within clinical encounters (Nebot Garcia, 2022). Instead of systemic interventions addressing structural determinants, medication is prescribed as an individualised solution, shifting responsibility away from policy and institutions. This process not only medicalises inequality but entrenches it, by creating long-term dependency, functional impairment, and further marginalisation (Ruiz, 2025).

In psychiatric services, antipsychotics are frequently used outside their approved indications, including for behavioural regulation in dementia, intellectual disability,

and borderline personality disorder (P. E. Deegan et al., 2024). These practices persist despite limited evidence of efficacy and significant risks of adverse outcomes. The cultural and institutional framing of pharmacological compliance as synonymous with therapeutic success contributes to the continuation of such practices, even when they conflict with principles of informed consent and evidence-based medicine (Beviá & Girón, 2017).

Educational and training systems perpetuate pharmacocentrism. Curricula in Spanish medical schools continue to emphasise pharmacology and diagnostic classification, while devoting limited attention to psychosocial approaches, patient autonomy, or shared decision-making (Gutiérrez Gea et al., 2015). Continuous professional education often occurs under pharmaceutical sponsorship, reinforcing the association between innovation and pharmacological intervention, while neglecting evidence on harms, limits, and alternatives. The absence of systematic deprescription training has been noted as a critical gap in Spanish medical education (Cea Madrid & Castillo Parada, 2018).

Public health implications are extensive. High levels of psychotropic use contribute to increased hospital admissions due to adverse drug reactions, functional decline among the elderly, and the perpetuation of treatment-resistant clinical pictures (Basanta, 2024). Overmedication also reinforces dehumanization and vulnerability, as psychiatric diagnoses become conflated with chronic pharmacological dependency, undermining personhood and social participation (Flores & González, 2009). Families bear the economic and emotional burden of prolonged pharmacological regimens, while the healthcare system absorbs the costs of preventable harm.

Deprescribing science offers a structured response to pharmacocentrism by combining indication review, shared goal setting, graded dose reduction, and monitoring of withdrawal and relapse-like phenomena that may be iatrogenic, not disease recurrence (Johansson-Everyday, 2023). Spanish and European evidence highlights endocrine and sexual adverse effects and cardiometabolic risk from long-term antipsychotic exposure, reinforcing the need for systematic risk-benefit re-appraisal and stepped reduction when clinically appropriate (M. A. Horowitz et al., 2021). Quality-improvement and practice-based studies show that targeted deprescribing initiatives in community mental health can reduce anticholinergic burden and stimulate wider medication optimisation, providing feasible pathways for services like those analysed in this dissertation (M. Horowitz & Taylor, 2024).

The persistence of overmedication in Spain is best understood as the convergence of sociostructural determinants, institutional routines, and legal frameworks that

collectively sustain pharmacocentric hegemony. Poverty, trauma, and exclusion manifest clinically as distress; institutional risk aversion reframes distress as disorder; diagnostic systems legitimise medicalisation; and pharmacological prescriptions become the default institutional response (Rocca, 2011). The result is the entrenchment of chronicity, the reproduction of exclusion, and systemic resistance to reform.

From a public health and human rights perspective, the consequences of this model are not confined to patients. They extend to families, professionals, and the broader society, through preventable mortality, disability, resource loss, and the perpetuation of structural inequalities. Reform requires evidence-based deprescription strategies, robust psychosocial infrastructures, gender-sensitive protocols, and systemic accountability mechanisms (Mañas, 2014). Without these interventions, overmedication will remain a structural feature of Spanish psychiatry, perpetuating harm under the guise of care.

1.4. Toward autonomous medication management and evidence-based better practices

International developments in psychiatric care have progressively questioned paternalistic, pharmacocentric, and compliance-oriented paradigms. Autonomous medication management treats pharmacotherapy as a negotiated, evidence-based, and rights-compliant process rather than a unilateral mandate, and is supported by data on adverse long-term effects of psychotropics and by jurisprudential and policy standards on consent and supported decision making. Longitudinal and population-based studies associate sustained exposure to antipsychotics and certain antidepressants with elevated cardiometabolic risk, neurocognitive impairments, emotional blunting, and altered social cognition, which cumulatively increase morbidity and premature mortality, particularly through metabolic and cardiovascular pathways (Pearce, 2013). These risks are consequential in Spain, where benzodiazepines, antidepressants, and antipsychotics are widely used across age groups and settings, often beyond guideline indications, with limited routine deprescription (Dickinson, 2014).

User-led and recovery-oriented scholarship has long argued for rebalancing clinical authority with experiential knowledge and for embedding medication decisions in dialogue and informed choice. Lived experience research documents the centrality of meaning making, agency, and negotiated risk in sustaining recovery, which is inconsistent with coercive or default maintenance prescribing (Healy, 2012). Spanish and European studies converge on the association between collaborative practice and improved engagement, satisfaction, and functional outcomes, while

coercion and unilateral management correlate with avoidance, mistrust, and disengagement from care (Swenson, 2009).

Canada provides relevant precedents for autonomous medication management. Work by Rodríguez del Barrio in community mental health emphasizes co-production, peer support, and shared accountability in medication decisions, with structured dialogues and non-hierarchical teams to reframe pharmacotherapy as one modifiable element within a broader recovery plan (L. R. Del Barrio et al., 2013). In parallel, deprescription science has advanced standardized tapering approaches. International deprescribing initiatives and recent syntheses outline practical protocols for individualized dose reduction, hyperbolic tapering, monitoring of withdrawal phenomena, and shared decision documentation, with an emphasis on safety, informed consent, and patient-defined goals (Gupta & Cahill, 2016). Spanish contributions report that structured deprescription and shared decision frameworks are implementable in routine services and are associated with reductions in unnecessary long-term use and with better subjective outcomes when supported by follow-up and psychoeducation (R. L. Del Barrio et al., 2014).

Dialogical and family-centred approaches offer further empirical grounding for low-medication pathways. open dialogue programs integrate immediate network meetings, continuity of contact, and minimal initial neuroleptic exposure within a relational and contextual formulation. Studies report lower hospitalization use, reduced chronic medication exposure, and improved subjective recovery when dialogical practices are consistently applied and resourced, with transferability noted in evaluations of Spanish innovations that prioritize network involvement and non-coercive crisis responses (Council of Europe, 2021b). Comparative policy analyses stress that sustained effects depend on interlocking supports including clinical retraining, peer participation, digital tools for shared documentation, and administrative recognition of participatory rights (DeMatteo et al., 2013).

The rationale for autonomous medication management is strengthened by the documented harms and opportunity costs of pharmacocentric routines. In Spain, long-acting antipsychotics and multi-drug regimens are frequently maintained for extended periods without systematic benefit-risk review or deprescription plans, despite well-characterized adverse profiles that include metabolic syndrome, extrapyramidal symptoms, sedation, cognitive effects, and elevated mortality, particularly among older adults and those with multimorbidity (Linkiewicz et al., 2024). These clinical effects translate into chronic disability, increased acute care use, and resource diversion away from psychosocial and community services, which Spanish economic evaluations identify as more cost-effective for recovery

and inclusion when adequately implemented (Uriarte Uriarte & Vallespí Cantabrana, 2017). Evidence also links overmedication and repeated pharmacological cycles to professional burnout, especially where clinicians face high case loads, limited referral options, and education that prioritizes pharmacology over shared decision skills and trauma-informed practice (Guzmán-Parra et al., 2019).

Human rights instruments and global standards align with this scientific rationale. The UN Convention on the Rights of Persons with Disabilities and subsequent interpretive guidance emphasize supported decision making and the phasing out of substitute decision regimes, with implications for consent to treatment and crisis response in mental health services. Spain ratified the CRPD and has legislated reforms on legal capacity that mandate practical support for autonomous decisions, which is congruent with medication management based on informed choice and negotiated risk (R. B. Del Barrio, 2004). WHO QualityRights provides a technical framework for service transformation that prioritizes consent, least restrictive practice, and access to psychosocial alternatives, and includes guidance on reducing coercion and improving communication and documentation for complex medication decisions (Abrines Jaume et al., 2012). Legal analyses in Spain underscore that rights frameworks require operational instruments, including advance directives, shared decision records, and governance mechanisms that monitor adherence to consent standards in prescribing and deprescribing (Urbanos-Garrido & Agúndez, 2025).

Barriers to adoption in Spain arise from institutional inertia, fragmented governance, and educational and information systems that treat adherence as a proxy for clinical success. Studies describe limited availability of dialogical or trauma-informed training, uneven access to psychosocial supports, and continuing influence of pharmaceutical interests in continuing education, all of which sustain medication-first responses and constrain clinical discretion to implement deprescription even when indicated (Novella, 2010). These conditions contribute to chronicity under treatment as usual, where care plans rarely target full functional recovery and where lifetime pharmacotherapy becomes a default trajectory rather than a deliberated choice supported by periodic review and tapering when appropriate (Groot & Os, 2021).

The public health relevance of autonomous medication management is therefore twofold. First, it addresses preventable iatrogenic harm and premature mortality associated with long-term psychotropic exposure by embedding routine deprescription and risk monitoring in care pathways. Second, it reallocates clinical effort and resources toward practices with stronger associations to sustained well-

being, including networked crisis responses, peer involvement, supported employment, and housing, which Spanish and European evaluations associate with improved functioning and lower institutional dependence when adequately resourced (Gili et al., 2013). Within this configuration, pharmacotherapy remains available but is governed by informed consent, negotiated risk, and ongoing review rather than by default maintenance or coercive compliance.

Shared decision-making is the linchpin for safe deprescribing and for advancing autonomy beyond nominal consent. Foundational and contemporary studies converge on the need to pair decision aids and structured dialogues with attention to power asymmetries and contextual constraints that can undermine genuine choice in serious mental illness (Coulter et al., 2015). Spanish policy and measurement work provides transferable tools for local implementation, including evaluation frameworks and validated brief measures suitable for routine care and research, which this thesis mobilises across surveys and field sites (Kuzman et al., 2022).

1.5. Legal, epistemic, and scientific foundations of the research problem

The persistence of coercive practices and pharmacocentric treatment models in contemporary psychiatry reflects an intersection of legal permissibility, epistemic asymmetry, and scientific limitation. These structural features sustain a configuration in which patient autonomy, informed consent, and bodily integrity are subordinated to institutional convenience, professional discretion, and risk management imperatives. The foundational research problem addressed here concerns not isolated episodes of malpractice but the systemic reproduction of coercion through legal norms, diagnostic authority, and decision-making hierarchies that blur the distinction between care and control (Singh & Ojha, 2021).

International human rights standards, including the UN Convention on the Rights of Persons with Disabilities (Sugiura et al., 2019) and the WHO QualityRights framework (Mion & Ventura, 2024), affirm the principle of free and informed consent in medical treatment. Nevertheless, most national frameworks, including Spain, maintain exceptions allowing involuntary intervention on grounds of presumed incapacity, risk, or diagnostic classification (Lázaro-Mateo et al., 2023). Spanish civil and health legislation continues to authorize coercive measures under broad judicial discretion. Until the 2021 reform of civil capacity law, guardianship and incapacitation provisions enabled substitute decision making that overrode patient preferences (Suess Schwend et al., 2016). Even after the reform, mental health interventions under the Ley de Enjuiciamiento Civil and the Código Penal

continue to permit involuntary admission and treatment (Buedo & Luna, 2021). Empirical reviews demonstrate that judicial oversight in such cases is often formalistic, with limited verification of proportionality or scientific justification (R. B. Del Barrio, 2004).

The epistemic foundations of psychiatric authority reinforce this legal permissiveness. Diagnostic systems currently used in Spain, including ICD-10/11 and DSM-5, are symptom-based classifications without validated etiological markers (Szasz, 1994). The absence of objective biomarkers or reproducible physiological correlates undermines diagnostic reliability and raises questions about their suitability as legal triggers for coercive measures (Douglas, 2018b). Nonetheless, these categories function as gatekeepers for civil status, capacity, and entitlements, authorizing interventions such as involuntary hospitalization, compulsory medication, and long-term institutionalization, frequently without mechanisms for appeal or systematic re-evaluation (Sampietro, 2015). Spanish scholarship has highlighted how such classifications contribute to administrative practices that pathologize dissent and medicalize social suffering (Metzl, 2010).

Clinical authority in psychiatry is institutionalized through regulatory and professional structures that confer normative weight on clinical opinion without subjecting it to evidentiary thresholds common in other domains of medicine (Crichton et al., 2017). Protocols are often derived from consensus guidelines influenced by pharmaceutical interests and medico-legal defensibility rather than by controlled outcome-based evaluation (Aluh et al., 2024). In Spain, prescribing patterns demonstrate structural pharmacocentrism: benzodiazepines and antidepressants are among the most widely consumed in Europe, with long-term use exceeding guideline recommendations (Carbonell Marqués & Navarro-Pérez, 2019). Deviation from these routines is often interpreted as clinical error, administrative risk, or patient irrationality, creating a closed epistemic loop where innovation and adaptation are discouraged (Desviat, 2005).

Scientific research on coercion and psychiatric harm has documented adverse outcomes associated with involuntary interventions. Meta-analyses and longitudinal studies report associations with treatment disengagement, learned helplessness, iatrogenic injury, and elevated suicide risk (Fowers, 2015). In Spain, user testimonies, ombudsman reports, and field studies highlight the psychological trauma, mistrust, and chronicity induced by coercive practices (Anguita, 2005). Despite this evidence, incorporation into clinical training and policy reform has been partial. Risk management discourses continue to justify coercion as a

preventive strategy, even though psychiatric risk assessment instruments show limited predictive validity (Fricker, 2007).

The Spanish legal-psychiatric nexus reflects this convergence of permissive law, epistemic uncertainty, and discretionary authority. Civil codes and health statutes allow interventions based on expansive interpretations of risk and incapacity without individualized evidence thresholds (Murphy & Hanson, 2017). Judicial endorsement of clinical recommendations is frequent, reflecting institutional reliance on psychiatric expertise and the absence of independent oversight mechanisms (R. Whitaker & Cosgrove, 2015). In practice, dissent is often pathologized and recorded as lack of insight, limiting avenues for patient contestation and undermining procedural safeguards (Light et al., 2013).

This structural configuration is reinforced by epistemological hierarchies privileging so-called objective, by alleged authority alone, over subjective evidence. First-person accounts of harm or injustice are routinely discounted as unreliable or non-clinical, narrowing the definition of admissible data and impeding participatory models of care (Calcedo-Barba & Fraguas, 2017). Mechanisms for reporting adverse outcomes remain fragmented and often internal to institutions, prioritizing liability management over systemic learning. In Spain, underreporting of iatrogenic harm and limited external audits have been identified as barriers to accountability and continuous quality improvement (Revuelta et al., 2021).

The result is a structural environment in which psychiatric legitimacy rests on normative inertia and credential-based authority rather than on reproducible scientific validation or rights-compliant practice. Coercion functions as a constitutive element of psychiatric governance rather than an exceptional measure, sustained by legal ambiguity, scientific limitations, and institutionalized epistemic asymmetries (Light et al., 2013). For this reason, the research problem examined in this dissertation extends beyond the identification of coercive episodes to the analysis of the normative architectures and institutional practices that sustain them. Only through reconfiguration of epistemic hierarchies, procedural safeguards, and accountability mechanisms, consistent with CRPD obligations and WHO QualityRights recommendations, can mental health systems transition toward ethically defensible and scientifically valid models of care (World Health Organization, 2025b).

The persistence of coercive and pharmacocentric practices in Spain, despite the availability of evidence-based alternatives, reflects the interplay of cultural norms, professional traditions, and systemic barriers that inhibit the implementation of shared decision-making and recovery-oriented care. International human rights

frameworks, including the United Nations Convention on the Rights of Persons with Disabilities (C. Steinert et al., 2016) and the WHO QualityRights program (World Health Organization, 2022a), establish the right to informed consent, autonomy, and participation in treatment planning as non-derogable standards. Yet, in Spain, the translation of these principles into routine psychiatric practice remains limited (Calcedo-Barba et al., 2024a).

Resistance to shared decision-making is linked to entrenched clinical hierarchies, where psychiatric authority is prioritized over patient preference and where dissent is interpreted as symptomatology rather than deliberative choice (van Os & Groot, 2023). This epistemic asymmetry reflects not only professional culture but also educational curricula that continue to prioritize biomedical models of symptom suppression over relational and psychosocial approaches (Ho, 2008). In this context, shared decision-making initiatives, although piloted in several Spanish regions, remain peripheral and face structural obstacles including insufficient time allocation in consultations, lack of decision-support tools, and limited training in dialogical methods (Abad-Sierra & Toledano-Márquez, 2023).

The consequences of overreliance on pharmacological containment are well documented. Spain has one of the highest consumption rates of psychotropic medication in Europe, particularly benzodiazepines and antidepressants (Giosa, 2025). Chronic use of these drugs is associated with dependence, cognitive decline, falls in older adults, and increased all-cause mortality (Carly Smith, 2014). Antipsychotics, frequently prescribed long term even in non-psychotic conditions, are linked to metabolic syndrome, cardiovascular disease, and premature death (Scott et al., 2015). Longitudinal data indicate that coercive and prolonged pharmacotherapy contributes to chronic disability, social withdrawal, and treatment disengagement (M. Y. Lee et al., 2020).

Particularly concerning are the effects on children and older adults, who represent vulnerable groups disproportionately exposed to overmedication. In pediatric populations, psychotropic prescriptions for attention, behavioral, or emotional problems have increased significantly, often without sufficient evaluation of long-term safety or access to psychosocial alternatives (J. Read et al., 2015). In the elderly, high benzodiazepine use is strongly correlated with falls, fractures, cognitive impairment, and institutionalization (Mikelyte et al., 2020). These outcomes represent not only clinical harm but also systemic failure to uphold the principle of *primum non nocere* in medical practice.

The relationship between coercion, pharmacocentrism, and suicide also requires critical attention. Several studies demonstrate that exposure to coercive treatment,

including forced medication, is associated with increased risk of suicide attempts and completions, mediated by loss of trust, trauma, and hopelessness (A. Lewis et al., 2025). In Spain, suicide remains the leading cause of external mortality, with psychotropic overprescription and lack of psychosocial alternatives identified as contributing factors (S. I. Cohen, 1975). The imposition of lifelong diagnostic labels, coupled with the assumption of permanent pharmacological treatment, frequently produces a form of *social death*, where individuals experience exclusion from education, employment, and relational life, exacerbating risk factors for suicide and chronic disability (Sanidad, 2022).

At the institutional level, resistance to deprescription protocols and participatory medication management reflects both structural inertia and professional risk aversion. Deprescribing guidelines developed internationally (M. Horowitz & Taylor, 2024) and adapted in Spain (Sanidad, 2025) remain underutilized in clinical practice. Surveys reveal that many psychiatrists and general practitioners are reluctant to engage in medication tapering due to concerns about liability, lack of training, and institutional emphasis on adherence as a quality indicator (Légaré et al., 2012). This reluctance is reinforced by pharmaceutical industry influence in continuing medical education and by health information systems that prioritize prescription monitoring over outcome-based or person-centered indicators (Cosgrove & Wheeler, 2013).

The cultural and professional normalization of overmedication is further sustained by social attitudes that equate psychiatric drugs with treatment and recovery. Public health campaigns rarely address the risks of chronic psychotropic use, and patients often lack access to independent, evidence-based information about alternatives. Relatives, under stress and lacking systemic support, frequently advocate for pharmacological containment as a means of achieving stability, even when aware of adverse effects (Kistler et al., 2023). This dynamic reinforces a cycle in which coercion is legitimized not only by institutions but also by societal expectations.

Resistance to best practices such as shared decision-making, autonomous medication management, and open dialogue is therefore not reducible to individual reluctance but represents a structural configuration of law, culture, and clinical governance. International evidence from Quebec, Brazil, Finland, and the Netherlands demonstrates that participatory and dialogical approaches reduce hospitalizations, improve recovery rates, and decrease chronic medication use (Groot & Os, 2021). In Spain, pilot initiatives aligned with these models remain marginal and underfunded, despite their congruence with WHO and UN mandates (World Health Organization, 2022a).

The present dissertation situates itself within this structural contradiction. By documenting the harms associated with coercion and pharmacocentrism and analyzing the legal and epistemic frameworks that sustain them, it seeks to contribute to a reconfiguration of psychiatric care in Spain toward scientifically valid, ethically defensible, and rights-based models. The emphasis is placed on integrating deprescription protocols, shared decision-making, and non-coercive care pathways as essential components of evidence-based practice, not as optional or experimental alternatives. This contribution aligns with both national mental health strategies (World Health Organization, 2025a) and international obligations under human rights law, reinforcing the need for systemic transformation grounded in empirical evidence and patient dignity.

The Spanish *Estrategia de Salud Mental 2022-2026* represents the most recent national policy framework aimed at strengthening community-based services, reducing reliance on hospitalization, and integrating rights-based principles into psychiatric governance (Sanidad, 2025). The strategy explicitly acknowledges the overuse of psychotropic medication and calls for a shift toward psychosocial interventions, prevention, and continuity of care. However, analyses by independent scholars and professional bodies emphasize that the plan remains underfunded, unevenly implemented across autonomous communities, and constrained by the absence of enforceable accountability mechanisms (Burstow, 2015b). The persistence of coercive practices and pharmacological overuse illustrates the structural gap between declarative policy and operational reality, where clinical cultures and systemic inertia outweigh formal commitments to reform.

One central driver of resistance to change lies in the unresolved tension between psychiatric governance and the structural determinants of mental distress. Research in Spain and internationally demonstrates that domestic violence, child maltreatment, workplace exploitation, precarious housing, and poverty constitute major predictors of mental health problems (Márquez, 2017). Yet these factors are rarely addressed within clinical encounters, which continue to prioritize genetic explanations, diagnostic labels, and pharmacological responses. This emphasis reflects the dominance of reductionist biomedical models, which frame psychiatric conditions primarily as disorders of neurotransmission or heritable traits, despite the limited explanatory and predictive power of genetic markers for most mental disorders (Lewontin, 1996). In Spain, these assumptions have been institutionalized through training curricula and clinical guidelines, reinforcing a treatment culture that medicalizes social suffering and obscures structural violence (Villarroel, García, et al., 2021).

The operationalization of this paradigm occurs within healthcare infrastructures characterized by limited resources, short consultation times, and high patient loads. General practitioners, who provide the majority of psychotropic prescriptions in Spain, report average consultation times of under 10 minutes, with mental health complaints frequently addressed through symptomatic pharmacological responses rather than in-depth assessment or referral to psychosocial care (Serra, 2025). This temporal compression, combined with the scarcity of psychologists, social workers, and occupational therapists within the public health system, reinforces pharmacotherapy as the most expedient option, despite evidence of its limited long-term efficacy and high risk of iatrogenic harm (Villarroel, García, et al., 2021). The misallocation of resources toward medication procurement and monitoring, rather than personnel and community-based infrastructures, has been repeatedly identified as a major structural deficiency (United Nations, 2017).

The consequences of this systemic configuration are profound. Prolonged exposure to benzodiazepines and antipsychotics is associated with tolerance, dependence, metabolic and cardiovascular comorbidities, cognitive decline, and increased mortality (Hert et al., 2011). Antidepressant use, particularly when prolonged without psychosocial support, is correlated with emotional blunting, withdrawal syndromes, and impaired psychosocial functioning (Breggin & Wheeler, 2012). Epidemiological data indicate that suicide rates in Spain remain high, with suicide now the leading cause of death from external causes, and with psychotropic overuse, unresolved trauma, and systemic neglect among the key contributing factors (Nicki, 2001). Coercive treatment, rather than preventing suicide, has been linked to heightened risk of suicidal ideation and behaviors, mediated by loss of trust, trauma, and hopelessness (Chesney et al., 2014).

These harms are disproportionately borne by marginalized groups. Children and adolescents are increasingly exposed to psychotropic regimens for behavioral or emotional difficulties, despite evidence of limited benefit and significant risks in developing brains (World Health Organization, 2022a). Older adults, particularly women, are overrepresented in long-term benzodiazepine use, with associated risks of falls, dependency, and premature institutionalization (Sicras-Mainar et al., 2008). Migrant populations, Roma communities, and individuals living in poverty are disproportionately subjected to coercive measures, reflecting systemic bias in both risk assessments and clinical decision-making (Seng et al., 2012). Survivors of domestic abuse often find their trauma reframed as psychiatric disorder, with psychotropic medication replacing structural support, legal protection, or trauma-informed therapy (Fischbach & Herbert, 1997).

The reluctance to implement best practices such as shared decision-making, open dialogue, or autonomous medication management is therefore not merely a professional preference but a structural feature of psychiatric governance in Spain. Clinical hierarchies, medico-legal cultures of risk aversion, and pharmaceutical influence combine to sustain a system in which pharmacological compliance is equated with recovery, and dissent is pathologized (Symonds, 2024). Best practices piloted internationally, including dialogical approaches in Finland (Seikkula, 2025), collaborative medication management in Quebec (Reilly et al., 2024), and rights-based community psychiatry in Brazil (Palombini & Barrio, 2021), have demonstrated effectiveness in reducing hospitalizations, improving recovery trajectories, and decreasing long-term medication use. Yet in Spain, such initiatives remain marginal, underfunded, and vulnerable to institutional resistance, despite their alignment with the WHO QualityRights framework and Spain's obligations under the UN CRPD (United Nations, 2006).

The convergence of systemic neglect, domestic and structural violence, and pharmacocentric governance therefore explains the persistence of coercion and overmedication in Spanish psychiatry. This dissertation addresses these dynamics by documenting the harms generated, analyzing the legal and epistemic frameworks that sustain them, and identifying operational pathways toward rights-based and evidence-informed alternatives. The need for this research lies in the recognition that without addressing the cultural, institutional, and legal determinants of pharmacocentrism, Spain's mental health strategies risk reproducing the very practices they aim to transform.

Legal comparatives and Spanish doctrinal analyses demonstrate that rights language can fail patients if not operationalised through enforceable standards on necessity, proportionality, and least-restrictive alternatives, accompanied by regular judicial and clinical auditing of mechanical and chemical restraints (Tortella-Feliu et al., 2016a). A broader jurisprudential critique cautions that abstract rights may even entrench coercion unless law is tied to service redesign and patient leadership, reinforcing this dissertation's emphasis on actionable governance reforms over purely normative alignment (Correa-Urquiza & Huertas, 2024).

1.6. Theoretical foundations: power, diagnosis, exclusion, and resistance

The institutional response to psychological distress continues to privilege pharmacological containment over psychosocial engagement, with consequences documented across clinical, social, and legal domains (M. Evans et al., 2024). Psychiatry functions not only as a clinical discipline but also as a regulatory

institution embedded in the broader governance of populations, with direct implications for civil rights, social participation, and the distribution of resources (Seng et al., 2012). Diagnosis is not a neutral classificatory act but a mechanism of social ordering, structuring hierarchies of credibility, capacity, and therapeutic eligibility (Marmot & Bell, 2019). Psychiatric categories employed in Spain, particularly those invoked to justify coercive measures or long-term pharmacological treatment, incorporate normative assumptions about agency and rationality, with profound implications for autonomy and citizenship (Correa-Urquiza, 2012).

Socio-ethical accounts of coercion emphasise that patients experience compulsion along a continuum that includes informal leverage, structural dependence, and epistemic marginalisation, not only formal detention, which reframes the analytic unit from isolated events to embedded practices across settings (Sugiura et al., 2019). Meta-ethnographic synthesis of recovery after coercion highlights interpersonal safety, trust repair, and recognition as necessary conditions for healing, aligning with rights-based programmes that place service users as educators of staff in order to shift cultures of practice rather than only procedures (Ridgway, 2011).

The Power Threat Meaning Framework (PTMF) developed by Johnstone and Boyle (Johnstone & Boyle, 2025) offers a structured alternative to symptom-based classification. It reconceptualizes mental health difficulties as understandable responses to threats and adversity mediated by power relations, reframing individual experiences of suffering within social, political, and institutional contexts (Sweeney et al., 2018). The framework emphasizes the construction of meaning and survival strategies, thus offering a clinically and ethically grounded orientation aligned with rights-based psychiatry. Empirical work in Spain has highlighted how patients' narratives of distress, including those of institutional violence and forced drugging, are systematically marginalized in clinical records (Ho, 2008). Relational anamnesis, understood as the reconstruction of sociopolitical contexts in which suffering is produced and medicalized, constitutes a valid epistemic resource for system reform (Ehrenberg, 2010).

The historical integration of psychiatry into legal and welfare institutions has consolidated its disciplinary function beyond the clinic. Diagnostic categories have long been used in Spain to regulate family life, workplace participation, and civil capacity, operating through mechanisms of documentation, surveillance, and judicial endorsement (Dixon, 2015). The resulting exclusion is not incidental but structural, embedded in a medico-legal culture that privileges institutional control

over therapeutic alliance (Álvarez Ramírez, 2023). Psychiatric exclusion is also epistemic: biomedical models prioritise randomized controlled trials and symptom scales, marginalizing experiential and contextual accounts (J. Read et al., 2009). This asymmetry constrains the recognition of patient testimony and obstructs feedback mechanisms that might otherwise lead to reform (Valencia, 2020).

Resistance emerges as an empirical and theoretical phenomenon in this context. At the individual level, strategies of avoidance, negotiation, and refusal often appear in response to coercive practices. These are frequently pathologized as *non-compliance* or *lack of insight*, categories that themselves serve to legitimize further coercion (Guzmán-Parra et al., 2019). Anthropological analyses reframe such practices as survival strategies and assertions of agency in contexts of structural denial (Bunston et al., 2017). At the collective level, user-led movements and survivor groups in Spain and internationally have generated counter-discourses that challenge psychiatric authority and expose systemic harm (Herman, 2015). The persistence of these movements demonstrates the epistemic insufficiency of biomedical paradigms in addressing complex forms of distress.

Resistance is also evident within the professions. Spanish clinicians engaging with dialogical psychiatry, narrative medicine, or trauma-informed care have documented tensions between ethical responsibility and bureaucratic demands (Orrahood, 1964). Professional dissent is frequently curtailed by institutional performance indicators, medico-legal liability frameworks, and entrenched pharmacocentric norms (Annas, 2005). This structural rigidity limits the capacity of practitioners to implement participatory approaches, reinforcing the systemic inertia of psychiatric governance (Hoffman, 2011).

These dynamics illustrate that coercion and exclusion are not anomalies but structurally inscribed features of psychiatric practice in Spain. Coercive hospitalization, forced outpatient treatment proposals, and routine polypharmacy have been shown to erode trust, compromise therapeutic alliances, and lead to disengagement and trauma (Dimitrijević, 2020). Long-term outcomes include social death through loss of civil rights, chronic dependency, and elevated mortality, particularly in relation to cardiovascular and metabolic diseases induced by psychotropic medication (V. Brown, 2009). The persistence of such harms highlights the need for methodological approaches that can document how diagnosis, exclusion, and resistance unfold within concrete institutional sites.

This section therefore establishes the theoretical and empirical foundations for the dissertation. By integrating critical psychiatry, critical medical anthropology (J. H. Jenkins, 2007), and rights-based frameworks (Calcedo-Barba et al., 2024b), it

articulates the structural logics through which psychiatric practice in Spain produces coercion and exclusion. At the same time, it foregrounds resistance, whether individual, collective, or professional, as evidence of systemic inadequacy and as an object of empirical inquiry. These foundations justify the use of action-research and ethnographic immersion as methodological strategies capable of producing knowledge that is both empirically rigorous and operationally relevant to systemic reform (Every-Palmer et al., 2021).

1.7. Analytical framework and scientific methodology for structural change

The institutional response to psychological distress continues to privilege pharmacological containment over psychosocial engagement, with consequences that extend beyond individual clinical encounters to broader domains of public health, legal integrity, and social participation (Miranda et al., 2019). Spain consistently ranks among the countries with the highest rates of psychotropic consumption in Europe, particularly benzodiazepines and antidepressants, with prescription patterns exceeding international guideline recommendations and generating serious risks of dependence, cognitive impairment, metabolic dysfunction, and premature mortality (Gill et al., 2007). These trends illustrate that systemic pharmacocentrism is not confined to specialist psychiatry but extends into primary care and emergency services, where psychotropics are often prescribed as a default response to social suffering such as unemployment, domestic violence, and poverty (Goodman et al., 2009).

The cumulative harms of overmedication are well documented. Long-term psychotropic use is associated with chronic somatic conditions including diabetes, obesity, cardiovascular disease, and neurodegenerative decline, all of which contribute to the 15- to 20-year reduction in life expectancy consistently observed in persons diagnosed with severe mental disorders (Hert et al., 2011). The iatrogenic burden includes withdrawal syndromes, polypharmacy-related complications, and the reinforcement of psychosocial dependence, with these outcomes particularly acute among older adults and children, where psychotropic use is both widespread and insufficiently regulated (World Health Organization, 2022a). Beyond biological harms, forced medication and chronic psychiatric labeling contribute to processes of social death, in which autonomy, dignity, and civic participation are progressively eroded (Katz & Alegría, 2009). Individuals subjected to long-term coercion report sustained disengagement from services, mistrust of professionals, and higher suicide risk, outcomes consistently confirmed in both Spanish and international studies (Calcedo-Barba et al., 2024a). Suicide remains the leading external cause of death

in Spain, with marked overrepresentation among populations exposed to coercive care and psychiatric exclusion (Chesney et al., 2014).

The persistence of coercion is reinforced by a legal environment that permits broad discretionary use of involuntary admission and treatment. Although reforms such as Law 8/2021 aligned the Spanish civil code more closely with the Convention on the Rights of Persons with Disabilities, psychiatric exceptions remain that allow clinicians and judges to override patient preferences based on perceived risk or incapacity, often without rigorous evidence or external oversight (Cabrero et al., 2020). Judicial review in these cases frequently functions as automatic ratification of psychiatric authority, reflecting systemic epistemic asymmetry where dissent by the patient is recoded as symptomatology and thus disqualified from legal consideration (Roig, 2018).

Institutional and professional resistance to reform remains substantial. Shared decision making, despite robust evidence of its efficacy in enhancing therapeutic alliance, reducing relapse, and improving functional outcomes, is rarely implemented in practice (Barnetz & Gefen, 2022). Training deficits in trauma-informed care, narrative medicine, and participatory practice are compounded by continuing medical education structures heavily influenced by pharmaceutical industry interests (Rhodes & Strain, 2004). These factors reinforce the perception that pharmacological innovation equates to clinical progress, despite mounting evidence of long-term harms and limited efficacy of psychotropics for recovery (Vian et al., 2022).

The consequences of these systemic failures are dire. Ethnographic and survey data confirm that coercive experiences foster learned helplessness, treatment avoidance, and persistent dehumanization and outcasting, while producing chronic disability and iatrogenic dependence (Dougherty, 2021). These outcomes disproportionately affect children, older adults, survivors of domestic abuse, migrants, Roma communities, and those living in conditions of poverty, exacerbating structural inequalities and entrenching cycles of exclusion (Miranda et al., 2019). Professionals themselves are not spared from these dynamics, as the dominance of pharmacocentric practice, the pressure to contain risk, and the lack of psychosocial resources contribute to burnout, moral distress, and disillusionment among clinicians (Arango et al., 2017).

The urgency of this problem is widely acknowledged in global health governance. The World Health Organization's QualityRights initiative has repeatedly called for the elimination of coercion and the establishment of rights-based systems of care (Funk & Bold, 2020). The United Nations Special Rapporteur on the right to health

has explicitly condemned coercive psychiatric practices as incompatible with international law and has urged states to redirect resources toward social determinants of health, community-based services, and supported decision making (Mousavizadeh & Bidgoli, 2023). Within Spain, national and regional mental health strategies recognise the overuse of medication and the need for non-pharmacological interventions, but implementation has remained fragmented and inconsistent, limited by institutional inertia and insufficient reallocation of resources (Sampietro et al., 2023b).

These realities establish the necessity of action-research approaches capable of documenting systemic harm, integrating user perspectives, and identifying feasible pathways for structural change. The Power Threat Meaning Framework provides a conceptual foundation for such work by reframing psychiatric problems as understandable responses to threats within relations of power, producing meaning systems that extend beyond diagnostic categories (Bunston et al., 2017). This framework, in combination with empirical ethnography and participatory methods, enables a scientifically robust and ethically accountable investigation into how coercion is reproduced and how alternatives can be institutionalised.

The severity of the consequences, premature mortality, chronic disability, suicidality, social death, and entrenched exclusion of vulnerable populations, underscores the urgency of this research. The persistence of coercion and overmedication in Spain represents not only a clinical failure but a violation of human rights and an obstacle to the development of sustainable public health systems. The dissertation is therefore grounded in the recognition that reform cannot be postponed without perpetuating preventable harm, and that the generation of empirically validated, legally relevant, and ethically defensible knowledge is indispensable for aligning Spanish psychiatry with international scientific and human rights standards (Calcedo-Barba et al., 2024b).

The mixed methods strategy integrates participatory action research with implementation and quality-improvement logics to connect evidence with change. Spanish and international studies demonstrate feasibility of action research in mental health nursing and community settings, and show how decision-support systems and case-mix aware benchmarking can inform resource allocation and service redesign in regional ecosystems, which this dissertation adopts in its triangulated analyses and policy simulations (Olsen et al., 2004).

Chapter 2 - Methodological framework: action research and positionality

2. Reflexive meta-methodology and scientific methodological frameworks

This dissertation was constructed through a transdisciplinary and reflexive process, combining survey research, ethnographic fieldwork, participatory action research, and open science practices. Rather than a passive aggregation of results, its structure and content emerged through iterative engagements with suffering, resistance, and knowledge-building under adverse conditions. Protocols included parallel tracking of field data and analytical reflections, timestamped documentation, the use of public repositories when security permitted, and validation through expert feedback loops. Analytical decisions were continuously aligned with the ethical mandates of trauma-informed, rights-based, and culturally grounded care (Kolk, 1994). The order of chapters and results follows both the empirical completion of each dataset and their systemic implications, rather than theoretical abstraction.

The research design, implementation, and narrative construction were developed as dynamic systems embedded in networks of care and systemic change. This included the coordination of expert systems, sustained contact with professional and community stakeholders, and the use of reflexive protocols to ensure epistemic consistency. The methodology reflects the position articulated by Farmer (P. Farmer, 2003) that research in fields of suffering should contribute to alleviation, linking the production of knowledge to the pursuit of justice. The outcome is a dissertation that functions not only as a scientific document but also as a much needed response to epistemic injustice and institutional shortcomings, offering methodological transparency as a condition for credibility and ethical responsibility (Elliott & Resnik, 2019).

2.1. Overall approach and methodological foundations

This dissertation was constructed through a transdisciplinary yet focused design that combines survey research and ethnographic fieldwork within a participatory action-research orientation and open science practices. Rather than a passive aggregation of results, the study developed through iterative engagements with knowledge-building under adverse conditions. Protocols included parallel tracking of field data and analytic memos, timestamped documentation, use of public repositories when safe, and expert feedback loops for validation. All procedures were aligned with

trauma-informed, rights-based, and culturally grounded standards (U.S. National Academies of Sciences, Engineering, and Medicine, 2018).

Reflexivity was operationalized scientifically: the researcher's embedded positionality was treated as a source of epistemic access rather than sentiment, with safeguards against bias at every step. Following Fricker and Dotson, the design explicitly attends to testimonial and hermeneutical injustice that obstruct the production and reception of patient and practitioner knowledge (Fricker, 2007). The sequencing of chapters follows empirical completion of datasets and their system-level implications, rather than abstract theorization. Within action-research, the project implemented dialogical engagement with professional stakeholders, iterative instrument refinement, and feedback mechanisms oriented to practical improvement of care environments, despite the absence of full institutional cycles. Open science elements included reproducible documentation, version-controlled instruments, and auditable decision trails, constrained only by safety and legal risks (Behar, 2022). The empirical scope spans community and clinical settings, administrative processes, and legal-policy interfaces in Spain, complemented by field observation in Trieste and further work in Indonesia on severe abuses abolition.

- Surveys: structured questionnaires for professionals and allied stakeholders to characterize practices, norms, and perceived constraints; analyzed for distributional patterns and cross-checked against legal/policy frameworks and ethnographic observations.
- Ethnography: participant observation, shadowing, and key-informant interviews documenting real-time organizational routines, interactional dynamics, and interpretations of coercion and care; analyzed through systematic coding and memoing with respondent validation when feasible.
- Action-research orientation: dialogical engagement, iterative redesign of instruments, and feedback integration to improve relevance and translational clarity (McNiff, 1988).
- Open science practices: transparently shared workflow records, versioned instruments, minimal-risk repository sharing, and expert review loops to sustain transparency and reproducibility under constraint (Allen & Mehler, 2019). Research shared via the open science field and laboratory notebook accessible at research.henning.md.

Table 5. Methodological foundations and operationalization

Strand	Rationale and key references	How operationalized in this research
Ethnography (medical anthropology)	Links institutional routines to lived effects; situates coercion within structural contexts (Davids, 2014)	Shadowing, observation, and key-informant interviews in community services; systematic coding, memoing, respondent checks
Surveys (mostly professional stakeholders)	Captures distributions of practices and norms; enables cross-setting comparison (Losh-Hesselbart & Fowler, 1985)	Structured instruments to assess coercive practices, dialogical work, and governance; triangulated with law/policy and fieldnotes
Epistemic justice	Addresses testimonial/hermeneutical injustice in mental health systems (Fricker, 2007)	Inclusion and coding of user-centred perspectives reported by practitioners; safeguards against selective incorporation
Action-research	Co-production, iterative refinement, and translational focus (Kindon et al., 2007)	Stakeholder feedback loops; instrument revision; practical orientation to system correction
Open science	Transparent, auditable, and reproducible workflow under constraint (U.S. National Academies of Sciences, Engineering, and Medicine, 2018)	Timestamped logs, version control, repository deposition when safe; expert peer feedback

This table presents the foundations of the survey-ethnography mixed methods research design, and its concrete operationalization in this dissertation.

The study treats psychiatric violence, institutional betrayal, and coercive governance as systemic rather than episodic phenomena (Carly Smith, 2014). The combination of surveys and ethnography, embedded in action-research and open science, provides convergent lines of evidence to identify patterns of harm, accountability gaps, and feasible conditions for non-coercive practice. Findings are interpreted against Spanish policy and international rights-based guidance to ensure jurisdictional relevance and normative clarity (Tortella-Feliu et al., 2016b).

Table 6. Action-research and open-science standards

Element	Practice	Evidence/output
Participatory engagement	Dialogical consultations with clinicians, legal practitioners, educators	Iterative revisions of survey items and observation guides; documented change logs
Feedback loops	Expert review cycles; respondent validation where feasible	Annotated instrument versions; memos linking feedback to analytic decisions
Transparent documentation	Timestamping, version control, minimal-risk sharing	Public versions or redacted releases; audit trail of analyses
Data protection	Decentralized storage; encryption; role-adapted anonymity	Replicable protocols; integrity checks under displacement
Triangulation	Cross-checking surveys, fieldnotes, and policy/legal sources	Convergent findings flagged; negative cases logged and discussed

This table presents the core action-research and open-science components of the methodological design supporting validity, accountability, and safety.

The elements summarised in Table 6 were applied as operational safeguards in day-to-day research practice. Participatory engagement was ensured through structured dialogues with professionals in psychiatry, law, and education, which directly informed the iterative refinement of survey instruments before deployment, and ethnographic protocols whenever possible. Feedback loops were documented in annotated versions of instruments and analytic memos, establishing a traceable record of respondent validation and expert review as much as possible, despite the harsh working conditions ensured by violence. Transparency was maintained through timestamped logs and version-controlled documents, while data protection relied on decentralised encrypted storage and role-adapted anonymity. The use of a knowledge base openly available to everyone was maintained until resources were lacking to keep the infrastructure working, due to the financial abuses on top of all the cruelty and violence. Triangulation was consistently employed as method to link survey data, field-notes, and legal-political sources, ensuring that findings were not dependent on any single stream of evidence but confirmed across independent materials, testimonies and observations. This infrastructure grounded the empirical work in reproducible and ethically robust procedures despite adverse conditions.

Our overall approach combines critical social inquiry with participatory and action-oriented methods to ensure that the knowledge produced is both contextually grounded and practice-improving. Action research has an established record in mental health nursing and care for generating situated, change-oriented evidence while maintaining methodological rigor (Ghafouri & Ofoghi, 2016). In parallel, community-based participatory action research offers concrete techniques for co-producing research questions, instruments, and interpretations with communities

facing structural vulnerability, an alignment that is especially pertinent for coercion-exposed populations (Field-Springer, 2020). This critical-participatory orientation also responds to calls to scrutinize the evidence base and its normative underpinnings in mental health systems, privileging methods that surface experiential and institutional dimensions often rendered invisible by conventional evaluation frameworks (Jain & Orr, 2016).

2.2. Mixed and multi-scalar design, triangulation of data and findings

The study employs a mixed, multi-scalar design that triangulates qualitative, quantitative, and documentary sources to capture system, service, and lived-experience perspectives on coercion and medication practices. Triangulation across stakeholder groups strengthens construct validity and supports cross-verification of patterns, consistent with quality-improvement research in community mental health and with international syntheses on coercion and recovery (T. Farmer et al., 2006). At the international level, umbrella and scoping reviews frame our analytical lenses and sampling logic, enabling cross-national comparison and the identification of convergent mechanisms despite legal and organizational heterogeneity (Robinson, 2007).

The present dissertation adopts a mixed-methods and multi-scalar design to examine psychiatric coercion, medical violence, and institutional impunity as systemic phenomena operating across personal, organizational, and legal domains. The study is neither confined to individual cases nor abstracted into policy analysis; rather, it seeks to construct an evidence-based, multi-layered account of how psychiatric harm is produced, normalized, and resisted. The integration of qualitative and quantitative methods, aligned with micro-, meso-, and macro-level inquiry, was driven by the structural complexity of the research questions and the biocultural orientation of the overall study (U. M. Read et al., 2009).

Qualitative methods, including ethnographic fieldwork, in-depth interviews, participant observation, and continuous reflexive documentation, were essential for capturing situated experiences of psychiatric coercion, degradation, and silence. These tools allowed the documentation of institutional betrayal, relational harm, and survival strategies in both clinical and extra-clinical settings. Field diaries and embedded observation enabled the researcher to track systemic patterns and institutional logics in real time. In parallel, open surveys with mixed-item formats combined narrative prompts with categorical questions, yielding a hybrid dataset of both descriptive metrics and qualitative testimonies. These quantitative components

were treated not as standalone evidence, but as distributional anchors and cross-checks for emerging themes (T. Farmer et al., 2006).

Data triangulation in this dissertation was achieved through the integration of three complementary methodological strands. Survey research provided structured quantitative indicators on the distribution and frequency of systemic practices across professional and user populations, thereby generating a basis for pattern recognition and comparative analysis. Ethnographic work, conducted through participant observation and in-depth documentation of lived interactions with psychiatric services, contributed situated and processual accounts of how coercive dynamics unfolded in practice, allowing for the identification of mechanisms and contextual contingencies that are not accessible through survey instruments alone. The literature study, encompassing both legal frameworks and scientific publications, enabled systematic verification of institutional norms, as well as the positioning of empirical findings within established bodies of knowledge. The triangulation of these strands secured cross-validation, strengthened epistemic consistency, and enhanced both the robustness and the contextual sensitivity of the analysis (Bans-Akutey & Tiimub, 2021). Each research question articulated in Chapter 1 was thus addressed through converging evidence streams, ensuring that interpretations were grounded in empirically diverse yet mutually reinforcing sources.

The following table summarizes how each thematic focus of the dissertation was addressed through primary data sources and triangulated with complementary strands, ensuring consistency between methodological design and the overall research framework.

Table 7. Alignment of research questions with data sources and triangulation

Research focus	Primary data source	Triangulated with	Analytical scale	Relevance
Systemic violence and institutional failure	Surveys of professionals and users	Policy and legal literature	Meso/Macro	Documents contradictions between legal-normative frameworks and routine practice; supports institutional reform
Integration of clinical and experiential evidence	Ethnographic observation and testimonies	Medical trauma research	Micro/Meso	Grounds integration of trauma theory and biopsychosocial models; informs critique of coercive environments
Conditions for institutional change	Surveys of perceptions change	on Ethnographic of participatory accounts	Micro/Meso/Macro	Informs design of participatory monitoring; aligns with rights-based governance standards
Silencing of user knowledge	Ethnographic narratives and discourse analysis	Epistemic injustice and literature	Micro/Meso	Enables analysis of testimonial and hermeneutical injustice; validates user perspectives in system critique
Legal safeguards in psychiatric care	Legal constitutional frameworks	and Ethnographic documentation of institutional practices	Macro	Provides normative evaluation of psychiatric governance; grounds legal-democratic accountability
Scientific and methodological structures	Literature on open science and methodological models	Surveys and ethnographic validation, testimonies	Meso/Macro	Justifies methodological framework; enables reproducible and ethically grounded knowledge production

This table presents the primary data sources and triangulated evidence streams structured across micro, meso, and macro levels of analysis. It illustrates how each focus was addressed through independent sources and validated through cross-comparison, ensuring coherence between methodological design, empirical findings, and systemic implications.

The process of triangulation was central to ensuring the scientific validity of the findings and to situating survey data within a wider body of empirical and theoretical knowledge. Triangulation was conducted across three axes: the structured survey responses, the ethnographic material generated through participant observation and fieldwork, and the scientific and legal literature. This approach allowed each source to be systematically tested against the others, strengthening reliability when themes converged, clarifying tensions when they

diverged, and identifying gaps where issues were absent or underrepresented. The logic followed is consistent with best practice in mixed-methods research and with the action-research framework adopted throughout the dissertation (McNiff, 1988).

Data collection and analysis proceeded under conditions of institutional silencing, professional exclusion, and sustained exposure to coercive practices that meet clinical and legal criteria for psychological torture (Pérez-Sales, 2017). These constraints limited access to feedback loops, interrupted fieldwork continuity, and restricted co-production typical of action research, as described in section 2.7. Methodologically, these challenges were addressed through a predefined triangulation design. Structured surveys provided distributional indicators across professional and user strata. Ethnographic documentation, including reflexive memos and systematic fieldnotes, generated process-tracing of coercive dynamics in situ. Literature study, encompassing legal frameworks, policy corpora, and peer-reviewed science, enabled both normative and empirical verification.

At the survey level, triangulation involved not only comparing distributions across instruments but also testing whether reported experiences, attitudes, and proposals for reform were corroborated in other domains. For example, the negative consequences attributed to psychiatric diagnostic categories in the diagnosis and iatrogeny survey were cross-checked against ethnographic fieldnotes from clinical contexts and compared with Spanish and international research on diagnostic labelling and its limitations (Symonds, 2024). At the ethnographic level, testimonies and institutional documents were systematically compared with survey findings. Professional ambivalence regarding user autonomy documented in ethnographic interviews aligned with survey data showing scepticism about shared decision making, while localised optimism about dialogical practices contrasted with broader distributions indicating structural barriers. At the literature level, triangulation anchored all findings in the established evidence base. Themes related to diagnosis, coercion, consent, medication practices, and alternatives were examined against Spanish and international studies, ensuring that claims were contextualised and situated within cumulative knowledge.

Procedurally, triangulation was reinforced by cross-source corroboration, negative-case analysis when feasible, chain-of-custody logs for documents, and protocolised coding to preserve analytic reliability. These practices prevented anecdotal or idiosyncratic interpretation and enabled systematic comparison across data streams. In this light, tables 6 and 7 function not as a static planning matrix but as an operational map for inquiry under constraint, oriented toward scientific transparency, protection of human dignity, and structural redress.

The study design was explicitly configured around micro, meso, and macro strands of analysis. The micro strand captures lived effects and trajectories documented ethnographically. The meso strand interrogates organisational routines and accountability mechanisms in services and oversight bodies. The macro strand situates these patterns within constitutional guarantees, human rights conventions, and transnational psychiatric governance. This division reflects not only analytical convenience but also the distributed, multilevel reality of psychiatric violence. By aligning research questions, methods, and scales, the study secures methodological rigor and clarifies its function: not to describe suffering in isolation but to connect it scientifically, legally, and ethically to the structures that produce and normalise it.

2.3. Data collection instruments and infrastructure

This research employed three core survey instruments, each designed to gather situated, ethically sensitive, and structurally relevant data within the Spanish psychiatric landscape. The surveys were constructed to document the lived realities of mental health users, relatives, and professionals, while simultaneously functioning as action-research tools capable of fostering reflection, critique, and participatory engagement. All instruments were adapted to the Spanish context, integrating conceptual frameworks from medical anthropology, legal analysis, and clinical ethics. Together they provided complementary insights into psychiatric practices, epistemic hierarchies, and institutional behavior.

The first instrument, *schizophrenia: a study on psychiatric labels*, was an adapted version of a validated survey previously deployed in the United Kingdom. The original tool, reviewed and refined through interdisciplinary expert consultation, was modified to reflect cultural, legal, and institutional specificities of the Spanish setting. Its purpose was to examine how psychiatric labels, particularly schizophrenia and psychosis, are experienced, interpreted, and contested by users, relatives, and professionals. The instrument combined structured questions with narrative prompts, allowing respondents to articulate both categorical and experiential responses. Approximately four hundred complete responses were collected, representing a wide range of roles within the mental health ecosystem. This dual structure enabled both quantification of trends and the recovery of qualitative testimony concerning destructive diagnostic practices, iatrogenic harm, and epistemic disqualification. The data provided the empirical foundation for a systemic critique of labeling and revealed long-term psychosocial and institutional consequences of categorical diagnosis.

The second survey, *open dialogue in Spain*, was developed specifically for this dissertation in collaboration with clinicians, educators, and researchers engaged in the promotion of dialogical practice in mental health. It represented the first national-scale attempt to assess levels of knowledge, training availability, and institutional engagement with open dialogue among Spanish mental health professionals. The instrument was disseminated through national professional networks, educational seminars, and clinical collaborations. While primarily quantitative in structure, it also captured open commentary that reflected ethical tensions, institutional resistance, and emerging interest in alternatives to standard psychiatric care. The data generated by this instrument were peer-reviewed and published as a contribution to the national literature on dialogical psychiatry, thereby reinforcing its methodological reliability and scientific relevance. Within this thesis, the survey serves a dual function: it maps the state of reform-oriented interest in dialogical practice and provides empirical evidence that reform is both necessary and already underway.

The third survey, *shared decision making and medication management*, was designed to examine psychiatric prescribing practices, informed consent, autonomy, and the challenges of deprescription. Its scope included diverse psychiatric contexts, ranging from acute care to long-term treatment scenarios. Items were framed as policy scenarios to elicit professional judgments; wording reflects real-world policy proposals rather than authorial stance. The survey invited perspectives from mental health professionals, and was active for 105 days. It received eighty one responses, with respondents dedicating on average more than forty minutes to completion. This high level of engagement is indicative of both the complexity of the instrument and the commitment of participants. The survey employed a mixed architecture of scaled items and open-ended questions, enabling triangulation of clinical, ethical, and experiential perspectives. The design was iterative, evolving in response to pilot testing, respondent feedback, and alignment with international frameworks such as the World Health Organization QualityRights initiative. The resulting data provided granular insight into the operational and moral deficiencies of existing prescribing regimes, while also documenting professional and experiential proposals for person-centred and network-based alternatives.

Each of the three surveys was embedded within a broader methodological matrix that included ethnographic fieldnotes, legal and policy documents, clinical testimonies, and institutional discourse analysis. Although each instrument focused on a specific thematic domain such as diagnostic validity and iatrogenic harm, dialogical practice, or medication governance, they were designed for cross-

comparison. Core constructs including autonomy, coercion, trust, trauma, and shared responsibility were operationalised across instruments using distinct but interoperable formulations. This design allowed systematic triangulation between quantitative distributions, ethnographic observations, and normative frameworks. Survey responses provided indicators of prevalence, ethnography contributed contextual and processual depth, and literature sources established legal and clinical benchmarks. Survey questions can be found in the annexes, as listed questionnaire in case of annex B for the first and third instruments, or a paper published, in case of annex D, for the second one.

Triangulation was carried out transparently, following open science principles wherever possible. Survey instruments and anonymised data distributions have been archived for reproducibility, subject to confidentiality requirements. Coding frameworks for open responses were documented to enable scrutiny and replication. Where constraints prevented full data release, such as the risk of re-identification in narrative accounts, these limitations were explicitly acknowledged. This combination of methodological pluralism, reflexive action-research engagement within networks and working groups, and commitment to open data standards enhanced the internal coherence and empirical robustness of the study. The integration of survey, ethnographic, and documentary evidence strengthens the interpretive validity of the findings and situates them within both the Spanish context and the broader international literature.

Instrument selection prioritized feasibility in clinical and community settings, respondent burden, and compatibility with collaborative decision-making workflows. Where survey modules addressed decision preferences and shared decision-making, we aligned items with validated constructs and Spanish-context instruments to facilitate comparability and local uptake (Aarons et al., 2011). Qualitative guides were structured to elicit narratives around perceived coercion, autonomy, and iatrogenic harm while attending to socio-legal context and human-rights reasoning, following best practices from European and global syntheses (Trueba, 2022).

Table 8. Validation processes and survey ethical safeguards

Survey title	Validation process	Cultural adaptation	Ethical measures and consent strategy
<i>The inquiry on schizophrenia: a study on psychiatric labels</i>	Originally developed and validated in the UK under Professor Suman Fernando and colleagues. Adapted for Spain with input from survivor-researchers and professionals in LoComún, and reviewed by networks including ISPS and the Moncrieff critical psychiatry group.	Spanish adaptation reviewed linguistic clarity, dynamic cultural relevance, and alignment with local diagnostic practices.	Anonymous for participation; informed consent embedded at initiation; advisory consultation on ethics.
<i>The survey on open dialogue in Spain: availability, training, and reception</i>	Validated by Spanish clinicians, trainers, and researchers active in dialogical practice; feedback collected through professional and academic networks including HOpenDialogue.	Language and framing aligned with Spanish institutional culture and regional variations in practice.	Informed consent embedded at survey introduction; anonymity ensured through encrypted digital collection.
<i>The survey on shared decision making and medication use in psychiatry</i>	Piloted and validated by more than one hundred professionals and international experts; targeted contributions from Peter Lehmann and survivor-researchers.	Questions refined through iterative consultation to ensure translatability and relevance across Spanish contexts.	Dynamic informed consent with update options; anonymization protocols with encrypted storage and timestamp logs.

This table presents the survey validation processes, cultural adaptations, and ethical protocols for the three instruments used. The approach emphasized methodological rigor, epistemic justice, and the protection of participants in contexts marked by historical and structural asymmetry.

Validation of the survey instruments themselves was carried out through iterative consultation with professional networks, survivor-researchers, and academic experts, both nationally and internationally. In all cases, the process followed action-research principles of transparency, collaboration, and reflexivity. Draft instruments were circulated among trusted colleagues, refined in response to their feedback, and piloted with target respondent groups before final dissemination. This ensured both methodological rigor and contextual adequacy.

For the first survey, *schizophrenia: a study on psychiatric labels*, the Spanish version was adapted from an instrument originally developed and validated in the United Kingdom under the leadership of Professor Suman Fernando, Dr. Jayasree Kalathil, Dr. Philip Thomas, and Dr. Janet Wallcraft (Fernando, 2017). The original tool was a landmark effort to document the destructive psychosocial and institutional consequences of the schizophrenia label and contributed to the decision of the International Society for the Psychological and Social Approaches to Psychosis (ISPS) to abandon the term in its title. The Spanish adaptation was coordinated independently by the researcher, in close consultation with members of the collective LoComún and survivor-researchers engaged in user-led initiatives in

Spain (Erro, 2021). Validation included linguistic review, cultural calibration, and pilot testing with both professionals and service users. Networks such as LoComún and the Moncrieff network of critical psychiatrists offered targeted feedback, as did senior members of the International Society for Psychological and Social Approaches to Psychosis (ISPS). This iterative process allowed the adaptation to maintain fidelity to the epistemological orientation of the original while ensuring cultural and institutional relevance for the Spanish context.

The second instrument, *open dialogue in Spain: availability, training, and reception*, was developed specifically for this research. Its validation involved Spanish clinicians, educators, and researchers with expertise in dialogical practice, many of whom had participated in postgraduate courses, implementation pilots, or training seminars. The draft was reviewed through several professional and academic networks, including participants in the national census of open dialogue practitioners linked to the HOpenDialogue project (Abad-Sierra & Toledano-Márquez, 2023). Feedback was provided by dozens of professionals across psychiatry, psychology, nursing, and social work, as well as family members and user organizations. Language and framing were adapted to align with the terminology and institutional culture of Spanish mental health services, and specific items were refined to reflect regional variations in implementation. This participatory validation process resulted in an instrument that could reliably capture levels of training, institutional resistance, and ethical tensions experienced by professionals. The data collected were subsequently peer-reviewed and published, providing an additional layer of scientific validation.

The third instrument, *shared decision making and medication use in psychiatry*, underwent the most extensive international consultation. More than one hundred professionals and experts across psychiatry, psychology, law, and survivor movements reviewed the draft instrument. Among the contributors was Peter Lehmann, internationally recognized advocate for responsible psychiatric drug withdrawal and ethical deprescription (Scott et al., 2015). Feedback was also sought from members of ISPS, the Moncrieff network, and several Spanish clinical networks. Pilot testing was conducted with a sample of trusted professionals and service users who assessed item clarity, cultural relevance, and practical feasibility. The iterative revisions that followed ensured that the survey addressed pressing clinical dilemmas, such as deprescribing in children, elderly patients, and people with trauma histories, while remaining scientifically robust and accessible to respondents. The survey's architecture combined clinical vignettes with scaled questions and open reflections, allowing for both statistical analysis and qualitative depth.

The deployment of these instruments required careful adaptation to conditions of institutional and infrastructural precarity. To ensure compliance with ethical and legal standards, responses were anonymised at entry, encrypted, and time-stamped according to GDPR guidelines. Open-source software and institutionally approved tools were used wherever possible to guarantee transparency and reproducibility. Each instrument incorporated dynamic consent procedures, offering respondents full disclosure of aims, scope, data protection measures, and withdrawal options. No personal identifiers were collected.

The validation processes reflect a cumulative methodological arc. The first survey, adapted from the UK instrument, introduced participatory principles and a mixed architecture of open and closed items. The open dialogue survey built on this by extending the collaborative process to national networks of professionals, leading to a peer-reviewed publication that reinforced its credibility. The shared decision making survey consolidated the lessons of the previous instruments, incorporating more complex ethical safeguards and aligning with international frameworks such as the World Health Organization QualityRights initiative. Across instruments, iterative refinement through consultation with experts and stakeholders allowed the surveys to capture destructive and harmful practices while ensuring scientific reliability and ethical integrity.

The construction and validation of all three surveys were guided by action-research principles. Drafts were reviewed by interdisciplinary expert panels including clinicians, anthropologists, lawyers, and psychiatric survivors. Pilot testing with target respondent groups ensured clarity, cultural appropriateness, and conceptual scope. Open science standards were applied wherever feasible, including documentation of coding frameworks, transparency of methodological decisions, and archiving of anonymised datasets. Limitations in data release, such as the need to protect narrative responses from re-identification, were explicitly reported. This combination of participatory validation, rigorous ethical oversight, and methodological transparency positioned the three surveys as robust diagnostic tools within the dissertation's broader research architecture.

In the 2025 survey on shared decision making and medication use, a distinctive methodological innovation was the inclusion of hypothetical reform scenarios. These scenarios were not intended to predict policy outcomes but to serve as structured prompts for eliciting professional expertise, ethical positioning, and perceived feasibility of systemic change within the Spanish context. Each scenario described a plausible reform grounded in current clinical debates, international guidelines, or documented failures of practice, such as deprescribing in vulnerable

populations, national audits of long-term diagnoses, or the recognition of trauma and institutional violence. By presenting respondents with concrete yet varied reform propositions, the instrument was able to capture not only levels of agreement or disagreement but also the reasoning professionals apply when weighing risks, rights, and responsibilities. The methodological purpose of this design was twofold: first, to operationalise shared decision making as a set of policy-relevant dilemmas rather than abstract principles; second, to generate data that could be triangulated with ethnographic observations and existing literature on psychiatric reform in Spain. The table that follows summarizes the scenarios, their thematic focus, and the methodological function they fulfilled in exploring the structural, ethical, and clinical dimensions of professional practice.

The first survey reflected primarily the voices of survivors and activists, offering direct testimony of the iatrogenic impact of psychiatric labels and coercive practices. Respondents described how diagnoses become lifelong identities, how forced treatment erodes trust, and how attempts to question medication are often dismissed or pathologized. This perspective illuminates the human cost of epistemic conservatism in psychiatry, where diagnostic overshadowing suppresses alternative explanations and entrenches exposure to drugs with significant long term harms. The salience of these accounts lies in their capacity to reframe deprescription and metabolic screening not merely as clinical adjustments, but as matters of justice and redress for those harmed by practices that fail to respect autonomy and informed consent (Lister, 1982).

The second captured the viewpoint of professionals most committed to the implementation of open dialogue in Spain. Their responses highlighted both enthusiasm for dialogic and collaborative approaches and the stark barriers to making them systemic: lack of training, absence of institutional support, and poor integration within public services. These struggles contextualize the professional hesitation observed in the gluten and nutritional scenarios. While some clinicians are prepared to advocate for protocols and joint pathways, many fall back on citing systemic barriers. The open dialogue literature shows that fidelity to dialogic principles is possible but demands structural change and institutional backing (Olson et al., 2014). Survey 2 therefore situates the problem of deprescription within the broader context of Spain's limited adoption of rights based and dialogic models, underscoring how professional culture and organizational inertia perpetuate reliance on pharmacological continuity.

The third is the most central to the thesis topic, because it engages the wider professional community directly with the often taboo issue of medication, shared

decision making, and the patient's right to participate in deprescription decisions. In contrast to the survivor testimonies of harm and the reformist efforts of open dialogue practitioners, this survey reveals the dominant currents of Spanish psychiatry when confronted with scenarios of reversible somatic contributors. Here, the responses split between a minority advocating stepwise deprescription aligned with international best practice, and a larger group defaulting to systemic excuses, epistemic conservatism, or outright deflection. This pattern highlights the depth of the taboo surrounding medication review in Spain and demonstrates how far practice remains from evidence based standards of proportionate screening, collaborative planning, and structured tapering (Fernando, 2004).

This last survey was shared with a very ample sector of the professional body during a time in which the national government and key stakeholder associations and professional bodies were debating the new mental health plan, with a strong focus on rational prescription practices. Its deployment was timely, further bringing to attention these topics to the professional community while capturing most relevant insights for this dissertation research.

The three surveys created a layered picture of the Spanish reality. Survivors expose the harms of coercive practices and rigid labeling. Reform minded professionals describe both the promise and the obstacles of dialogic models. The wider psychiatric community reveals the persistence of pharmacocentrism and reluctance to engage with deprescription. This triangulation demonstrates why the thesis must place shared decision making and patient rights at its core. It is precisely at the intersection of survivor testimony, reformist practice, and professional inertia that the case for rational prescription and deprescription becomes most urgent, both as a clinical imperative and as an ethical necessity (Nakray et al., 2015).

Table 9. Hypothetical reform scenarios in the 2025 survey

Scenario	Description	Purpose in survey methodology
Implementing national screening for gluten sensitivity in psychiatric diagnoses	Introduces an emerging somatic and dietary factor that may alter psychiatric diagnostic reasoning.	To evaluate professionals' readiness to integrate interdisciplinary screening and to test openness to non-pharmacological treatment pathways.
Deprescribing neuroleptics in cases of missed metabolic and nutritional disorders	Highlights risks of diagnostic overshadowing when physiological conditions are overlooked.	To explore attitudes toward coordinated medical care and the structural feasibility of deprescribing when alternative diagnoses are established.
Addressing psychiatric abuse cases uncovered through national research on mental health and violence	Brings to the surface issues of iatrogenic harm and institutional neglect of trauma.	To assess awareness of systemic violence, the ethical dimensions of decision making, and the perceived role of reparation in reform.
National deprescribing initiative for elderly patients at high risk of mortality and disability	Focuses on a high-vulnerability group where neuroleptic use is associated with excess mortality.	To capture clinical perspectives on age-specific deprescribing, professional duty, and safeguards for fragile populations.
Reevaluating long-term psychiatric diagnoses in a national audit	Questions the inertia of diagnostic categories underpinning chronic pharmacological use.	To examine professional views on re-diagnosis, error correction, and the ethics of dependency created by outdated assumptions.
Addressing the overprescription of neuroleptics in social care and marginalized communities	Exposes inequities in prescribing patterns across socially excluded groups.	To measure sensitivity to systemic discrimination and readiness to support community-based, equitable alternatives.
National review of the use of neuroleptics in children and adolescents	Raises developmental, ethical, and clinical issues of pediatric prescribing.	To document professional risk-benefit assessments and identify the structural supports required for safer transitions away from inappropriate medication.
Ending the use of neuroleptics for non-psychiatric indications in general medicine	Examines routine but clinically inappropriate use of neuroleptics outside psychiatry.	To evaluate inter-specialty communication, guideline adherence, and feasibility of deprescribing in non-psychiatric contexts.

This table presents the hypothetical reform scenarios employed in the 2025 shared decision making survey. These scenarios were designed to elicit professional responses to ethically and clinically significant propositions, enabling the identification of structural obstacles, ethical positioning, and readiness for reform.

Across all three surveys, validation was achieved through iterative consultation with professional and academic networks, survivor-researchers, and international experts. The Spanish adaptation of the first survey, focused on diagnosis and iatrogeny, was reviewed by members of LoComún and international collaborators such as the Moncrieff critical psychiatry network and the International Society for the Psychological and Social Approaches to Psychosis (ISPS). The open dialogue survey was validated by clinicians, trainers, and researchers engaged in dialogical

practice, with reference to national mapping projects such as HOpenDialogue (Correa-Urquiza, 2018). The shared decision making survey was refined through feedback from more than one hundred experts, including psychiatrists, psychologists, lawyers, and survivor-advocates, with notable input from Peter Lehmann on deprescription ethics (Gupta & Cahill, 2016). This process ensured cultural resonance, clinical relevance, and conceptual robustness.

Ethical safeguards were implemented systematically. All surveys employed dynamic informed consent, providing clear information on objectives, data use, confidentiality, and withdrawal rights. Responses were anonymised at entry, stored using encrypted servers, and logged according to GDPR standards. Ethical oversight combined formal institutional review with informal advisory from international collaborators and rights-based collectives, compensating for gaps in institutional infrastructure. More considerations on ethics can be found developed in annex C.

The reach of the surveys was substantial. Together they generated more than seven hundred completed responses: approximately four hundred for the diagnosis and iatrogeny instrument, more than two hundred for the open dialogue instrument, and eighty for the shared decision making survey. The latter alone accumulated more than one hundred hours of professional input, given the average response duration exceeding forty minutes. Dissemination channels included nearly all public mental health centers in Spain, professional associations such as the Asociación Española de Neuropsiquiatría, and national research consortia such as CIBER. Respondents included authors of peer-reviewed research and books on psychiatry and ethics, ensuring that the dataset reflected the perspectives of influential figures within the Spanish mental health system.

The three validated instruments, together with the exploratory survey, constitute more than data-collection devices. They are participatory tools that produced a multidisciplinary archive of ethical, clinical, and systemic insights. Their construction followed action-research principles, enabling both scientific rigor and participatory legitimacy. The instruments are now available in the annexes and will be deposited in the online repository accompanying this dissertation, contributing to the open science agenda. Their reuse for clinical teaching, professional training, and policy evaluation demonstrates translational potential beyond this thesis.

A consistent pattern across the surveys was the recognition, by both professionals and service users, of the limitations of pharmacologically centered psychiatry. Respondents highlighted the systemic prioritization of medication as the default response to distress, often accompanied by inadequate access to psychosocial or community-based alternatives. These testimonies substantiate the claim that

psychiatry in Spain remains structurally aligned with coercive and medication-dominant models, while at the same time indicating a shared demand for more comprehensive, rights-based, and dialogical approaches (Seikkula et al., 2018). These findings align with the broader themes of exclusion, invisibility, and epistemic marginalization explored throughout this dissertation (Jennings, 1994).

2.4. Field strategy and sites of observation

The methodological architecture of this dissertation derives from the integration of medical anthropology, clinical ethics, biocultural analysis, open science practices, and transdisciplinary action research. It is not presented as a retrospective narrative or a technical compartmentalization of methods, but as a structured epistemological and political approach to knowledge production under conditions of systemic adversity. Research design and dissertation construction were developed as interconnected dimensions of the same process, guided by explicit theoretical commitments and ethical imperatives. These commitments shaped procedural decisions across field engagement, documentation, and analytical validation. Core frameworks introduced in Chapter 1 and the opening sections of Chapter 2 provided the foundation for this integration. The biocultural paradigm framed mental suffering and resilience as co-constituted by social, environmental, legal, and biological forces (Kas et al., 2025a). Open science principles required that methodological decisions and evidence trails be recorded through timestamped, version-controlled systems, supporting transparency and reproducibility (Elliott & Resnik, 2019). This infrastructure was essential to mitigate risks of data loss and institutional obstruction. The application of human rights principles (Aagaard-Hansen & Johansen, 2008), epistemic justice theory (Nakray et al., 2015), and structural violence analysis (P. Farmer, 2004) defined the ethical stakes of the work, shaping both analytic categories and interpretive claims.

The methodological stance was informed by the work of Paul Farmer, Nancy Scheper-Hughes, Byron Good, and Suman Fernando, among others, who emphasized engaged participation and the ethical responsibility of proximity (R. M. Martínez, 2024). Research was therefore carried out through embedded participation in professional task forces, activist-scientific coalitions, and international networks, including EU-level consortia such as COST¹, Actions ReMO

¹ EU COST action is an interdisciplinary research network that brings researchers and innovators together to investigate a topic of their choice for 4 years. COST actions are typically made up of researchers from academia, SMEs, public institutions, and other relevant organisations or interested parties, to grow their professional research networks while reaching a common goal: <https://www.cost.eu/cost-actions/what-are-cost-actions/>

and FOSTREN. This positioning enabled analytical access and fidelity to context that would not have been possible through detached observation. As Farmer demonstrated, proximity to community struggles generates insights into institutional practices of adaptation, neglect, or silence in the face of ethical challenges. Each phase of data collection, coding, and synthesis was shaped by this embeddedness. Drafts were subject to recursive cycles of revision grounded in feedback from clinicians, survivors, legal scholars, and community networks, ensuring coherence and external accountability. Methodological decisions were documented through version control and meta-analytical commentary, supporting auditability and replicability.

A full paper by the researcher on ethics in qualitative research is included in the annexes, extending the methodological reflection (Aagaard-Hansen & Johansen, 2008). Distinctive features of the approach include documentation of the conditions under which knowledge was produced, which encompassed forced mobility, chronic threat, and institutional neglect. These contextual factors have been recognized in the literature as central to research under conditions of political or structural violence (Schild, 2025). The contradiction between public commitments to scientific progress and simultaneous institutional obstruction is consistent with findings on the vulnerability of researchers in politicized health systems (P. Farmer, 2004). This inconsistency illustrates how rhetorical commitments to health and scientific advancement are undermined when systems fail to ensure the safety of knowledge producers.

The integration of medical anthropology, action research, human rights frameworks, and open science principles provided a scaffold for protecting and articulating knowledge production under contested conditions. Rather than compromise validity, this embeddedness strengthened the relevance and rigor of the findings. The methodological stance moved beyond passive observation, adopting a collaborative, translational approach consistent with action-research traditions. Participation in several COST science and technology cooperation networks such as FOSTREN, on coercion reduction in mental health, ReMO, on mental health in academia, and later on the own funded action on one health education via new technologies, EU BEACON, enabled scientific cross-checking with fellow experts, iterative refinement, and participatory verification, even where traditional institutional support was absent or hostile. Such networks functioned simultaneously as research environments and as ethical shelters, consistent with scholarship on the role of collective infrastructures in sustaining research under duress (Schild, 2025).

The methodological position taken in this dissertation aligns with the view that justice is not an external consideration but a precondition for ethical and scientifically robust knowledge production (Shim, 2021). The enforcement of legal and ethical standards is necessary for the protection of researchers and participants in contexts marked by structural violence and institutional complicity. The dissertation documents these dynamics not as auxiliary detail, but as constitutive of the research process itself. The methodology therefore incorporates a chronological exposition of fieldwork conditions, tracing ruptures, accelerations, and adaptations in real time. This design emphasizes how ethics, science, and survival were managed simultaneously and demonstrates how embedded engagement with institutions and grassroots actors enhanced credibility and rigor (Davies, 1999). The methodological and theoretical architecture was built on biocultural, participatory, and action-oriented foundations. It combined transparency, protective infrastructures, and recursive validation to ensure scientific robustness despite systemic adversity. By grounding analysis in engaged participation and open science practices, the study sought to produce knowledge that is simultaneously empirically verifiable, ethically defensible, and structurally relevant.

Table 10. Detailed research timeline, by location and year, 2020

Month	Social and Policy Context	Research Outputs	Research Constraints	Location
January	–	Collaboration with national teams on the implementation of open dialogue and Soteria models	Exposure to coercion and domestic violence, including psychological abuse, physical aggression, and misuse of psychiatric services. Reports of forced medication and sterilisation were documented (reference required).	Catalonia
February	–	–	Continuation of coercion and systemic neglect	Catalonia
March	National lockdown begins in Spain	Participation in SURE (Service User Research Enterprise), King's College London	Intensification of domestic violence under confinement conditions	Catalonia
April	Strict confinement under <i>Real Decreto 463/2020</i>	–	Lockdown restrictions combined with exposure to coercion	Catalonia

Month	Social and Policy Context	Research Outputs	Research Constraints	Location
May–September	Lockdown persists throughout Spain	–	Research conducted under conditions of domestic violence and restricted mobility	Catalonia
October	Lockdown persists	Commencement of doctoral research at Universitat Rovira i Virgili, funded by the Spanish Ministry of Science; co-coordination of the Open Dialogue expert course (Blanquerna, Universitat Ramon Llull)	Persistent exposure to coercion	Catalonia
November	Lockdown persists	Address delivered at the Mexican Chamber of Deputies on mental health and human rights; accession to EU COST Action FOSTREN (Fostering Research on Ending Coercion in Mental Health)	Ongoing domestic abuse	Catalonia
December	Lockdown persists	Appointment as liaison and coordinator of the global <i>Mad in America</i> network	Ongoing domestic abuse	Catalonia

This table presents the research activities and outputs during the COVID-19 pandemic, 2020, with concurrent constraints linked to domestic abuse, coercive psychiatric practices, and systemic institutional neglect.

In 2020, despite continuous institutional violence and coercion, including forced medication, psychiatric abuse, and violations of bodily integrity, the researcher secured a national predoctoral contract and began doctoral work at Universitat Rovira i Virgili under six-year ministerial funding. During the COVID-19 lockdowns, the researcher contributed to high-level international collaborations, including with SURE at King's College London, the Mexican Parliament Forum on mental health and human rights, and the EU COST FOSTREN action on ending coercion. Additional roles included co-coordination of a postgraduate course on open dialogue at Blanquerna-Ramon Llull University and appointment as global liaison for *Mad in America*. These achievements were realised under conditions of confinement, domestic abuse, and systemic neglect, highlighting both the resilience and the precarity of academic engagement under extreme duress. The first episode of domestic and institutional violence is narrated in Annex E.

Repeated calls for protection to institutional and professional bodies did not end the violence. The failure of systems to intervene illustrates the broader problems addressed in this dissertation: abusers were able to continue coercion, control, and harassment with impunity in a context where institutions relied on coercive practices themselves and failed to provide effective protection. These constraints defined the contextual conditions under which data collection occurred.

Table 11. Detailed research timeline, by location and year, 2021

Month	Social and Policy Context	Research Outputs	Research Constraints	Location
January	Initiation of COVID-19 vaccination campaigns in Spain and the European Union	Member of the organising committee for the Escalation of HopenDialogue international interpersonal conference; participation in violence, replication discussions of the professional ODDESSI project; training in opportunities evaluation of open dialogue undermined under practice; submission of abstract coercive conditions to <i>Frontiers</i>		Catalonia
February	National lockdown measures remain in place	Continued work on open dialogue dissemination and evaluation	Documented incidents of physical assault, harassment, and threats, with intensified violence coinciding with key research deadlines	Catalonia
March	Lockdown persists	Awarded a Yale scholarship with Prof. Mary Olson (subsequently lost due to violence)	Domestic abuse prevented international academic mobility	Catalonia
April	Lockdown persists	Delivered teaching sessions on open dialogue at the University of Almería, in collaboration with Prof. Adolfo Cangas	Ongoing exposure to domestic violence and harassment	Catalonia
May	End of the state of alarm in Spain	Joined the International Institute of Psychiatric Drug Withdrawal	Continued exposure to violence under conditions of institutional neglect	Catalonia
June–July	Restrictions softened	Advancement of doctoral research	Persistence of domestic abuse and coercion	Catalonia

This table presents the research activities, outputs and international collaborations, within concurrent constraints due to domestic violence and institutional complicity.

In 2021, during the continuation of COVID-19 restrictions in Spain, the researcher

served as organising committee member and speaker at the international HopenDialogue conference and contributed to efforts to involve Spanish centres in replication of the NHS ODESSI trial on the implementation of open dialogue. Despite these achievements, escalating domestic abuse and institutional neglect obstructed most opportunities, including advanced training initiatives and a planned collaboration with Yale professor Mary Olson. A violent assault that left the researcher unconscious occurred at the same time as the submission of a research abstract to *Frontiers in Psychology* during the doctoral presentation week. Teaching on open dialogue at the University of Almería and new membership in the International Institute of Psychiatric Drug Withdrawal were nevertheless maintained. The persistence of violence, combined with systemic failure to provide protection despite reporting, exemplifies the destructive conditions under which the research was conducted and highlights the broader institutional problems addressed in this dissertation.

Table 12. Detailed research timeline, by location and year, 2022

Month	Social and Policy Context	Research Outputs	Research Constraints	Location
Aug	Catalonia launches Strategic Health Plan 2022-2030	Fieldwork mission to Trieste (WHO centre of excellence) for shadowing; presentation at ISPS conference in Perugia	Severe domestic violence documented, undermining conditions for research	Catalonia/Italy
Sep	-	One month of ethnographic fieldwork in Trieste, Italy, observing WHO-recognised community psychiatry practices	Fieldwork conducted under continuing domestic abuse, systemic neglect, and coercion	Trieste (Italy)
Oct	WHO publishes report on community mental health transformation	Analysis of Trieste model integrated into dissertation	Adverse family environment and violence persisted	Catalonia

This table presents research activities, outputs and international fieldwork, with concurrent constraints due to severe domestic abuse, systemic neglect, worsening violence.

In 2022 the researcher endured severe domestic abuse, with physical assaults becoming a daily occurrence and institutional responses proving ineffective. Such violence constitutes a grave violation of human rights and must be unequivocally condemned (Barnetz & Gefen, 2022). Despite these conditions, in September the

researcher completed one month of immersive ethnographic fieldwork in Trieste, Italy, following a preparatory period of familiarisation.

The Trieste fieldwork employed shadowing, participant observation, and key informant interviews within a medical anthropology framework. Data were collected across residential units, community centres, and mobile psychiatric teams, documenting the operation of WHO-recognised community psychiatry services (A. Cohen & Saraceno, 2002). Embedded observation of daily routines was supplemented by interviews with professionals and service users and by participation in public forums. The study generated insights into non-coercive care dynamics, governance structures, and the operational tensions inherent in sustaining deinstitutionalised, community-based models of care. These findings underscore both the potential and the fragility of rights-based psychiatry when confronted with systemic pressures and institutional inertia.

Table 13. Detailed research timeline, by location and year, 2023

Month	Social and Policy Context	Research Outputs	Research Constraints	Location
June	Spain finalises the National Mental Health Strategy 2022–2026	Preparation of relocation plan; acquisition of residence for long-term settlement	Research undertaken under conditions of escalating domestic violence and persistent institutional inaction	Sweden
July	–	Completion of a Springer Nature book chapter on alternatives to coercion; organisation of an international event on political psychiatry	Continuation of daily violence, systematically reported to professional and advocacy networks	Sweden
August	–	Development of survey instruments and fieldwork agenda	Ongoing physical threats and instability undermining research continuity	Sweden
September	Council of Europe initiates review of the Oviedo Convention	Participation in ongoing international collaborations	Knife threat and further episodes of interpersonal violence documented	Sweden
October	–	Presentations delivered at the World Psychiatric Association Congress; affiliation seminar at Karolinska Institute	Assault documented with photographic evidence; death threats reported	Sweden
November	–	Hosting of expert meeting on punitive psychiatry	Child abduction, forced displacement, and episodes	Sweden/ Spain

Month	Social and Policy Context	Research Outputs	Research Constraints	Location
December	–	Preparation of Routledge chapter in collaboration with the EU CORDIS institutional neglect founder and an African space medicine scholar (unfinished due to constraints)	homelessness Parental alienation and recorded, preventing completion of academic work	Spain

Research timeline and conditions, 2023. Documentation of research outputs and concurrent structural constraints, including domestic abuse and institutional inaction, illustrating the impact of systemic violence on academic continuity.

In 2023 the researcher continued to carry out international academic and policy work while simultaneously enduring daily physical and psychological violence without effective institutional protection. Since 2021, assaults had escalated in severity and frequency, and by 2023 they had become routine despite repeated alerts to colleagues, professional networks, and competent authorities. Domestic abuse in such conditions constitutes torture and must be condemned without reservation, particularly when it is facilitated by institutional neglect or complicity (Fischbach & Herbert, 1997). In June 2023, a final attempt to secure stability led to the purchase by the researcher of a new residence in the hope of rebuilding a safe living environment. Despite ongoing violence, in July the researcher completed a book chapter for *Springer Nature* on alternatives to coercion and coordinated a high-level event on political psychiatry with leading international experts. August and September were marked by further episodes of domestic assault, including a knife threat, which were documented and reported to national networks and survivor organisations. In October, the researcher combined key professional commitments, including presentations at the World Psychiatric Association Congress and an institutional presentation at the Karolinska Institute, with renewed exposure to life-threatening abuse. Physical injuries, including a documented black eye, were reported photographically and shared with colleagues and organisations as part of ongoing alerts.

In November 2023, the violence culminated in the abduction of the researcher's children, executed without warning and enabled by systemic negligence. This event represented a profound violation of family rights, professional continuity, and legal protections. In December, the researcher was unable to complete a contracted chapter for a Routledge handbook in collaboration with senior international

colleagues, as violence and displacement had obstructed the necessary conditions for academic production. Many more work was halted or completely destroyed due to systemic and family violence normalized, increasing still in cruelty and severity (Bourne & Newberger, 1977). The further interruption of professional activities and loss of collaborative opportunities in 2023, as well as life threats due to violence, extortion, institutional betrayal and other forms of severe torture as hate crimes against the researcher (Bezanilla et al., 2016), illustrate how unchecked domestic abuse, combined with institutional indifference and procedural failures, along with crime and corruption, can and do destroy both personal safety and collective scientific progress despite constant asking for help. The systems of protection, well-being and law enforcement are lacking, objectively failing in, or betraying, its stated purpose. International human rights instruments emphasise the obligation of states and institutions to prevent violence, protect victims, and ensure accountability (Ruiz, 2025). The absence of such protections in this case demonstrates the urgent need for enforceable safeguards, robust support systems, and institutional practices that prioritise the safety and dignity of researchers, families, and vulnerable populations

Table 14. Detailed research timeline, by location and year, 2024

Month	Social and Policy Context	Research Outputs	Research Constraints	Location
January	–	Planned Routledge book chapter with EU founder and senior medicine expert (opportunity lost due to conditions of violence)	Litigation abuse, child abduction, parental alienation, institutional complicity; lack of support from Spanish services in Sweden; rendered homeless	Sweden/ Spain
February– March	National funding increase for psychosocial community teams in Spain	–	Continuation of litigation abuse and parental alienation; ongoing institutional neglect	Spain/ Sweden
April	–	Submission of EU BEACON COST Action proposal; preparation of fieldwork agenda; design of shared decision-making survey; institutional preparation of <i>MDPI Social Sciences</i> paper and additional manuscripts	Research conducted under harassment and institutional obstruction	Spain
May–July	UN	Human –	Continued exposure to Spain	

Month	Social and Policy Context	Research Outputs	Research Constraints	Location
	Rights Council publishes thematic report on coercion		systemic violence and institutional inaction	
August–September	–	Continuation of COST Action and survey preparations	Persistence of litigation abuse and parental alienation; Spanish penal assistance initiated; negligence and obstruction in Sweden	Spain/ Sweden
October	–	Relocation and initial preparation of ethnographic fieldwork	Research undertaken under displacement	Indonesia
November	–	Fieldwork preparation and integration into Universitas Muhammadiyah Yogyakarta networks	–	Indonesia
December	–	Ethnographic observation and preparation of <i>pasung</i> study; grant allocation confirmed	–	Indonesia

This table presents the research timeline and conditions during the year 2024. during concurrent structural constraints, including litigation abuse, child abduction, and institutional complicity, demonstrating the impact of systemic violence on academic continuity.

In 2024 the researcher experienced sustained harm following the violent abduction of a child in late 2023, immediately after hosting an international academic event. Despite repeated reports and formal denunciations, institutional complicity with the criminals abroad and uncoordinated, failing systems in Spain, facilitated continued violations, including psychological torture, litigation abuse (Gutowski et al., 2023), childre abuduction, and threats to life and dignity, as well as extortion, threats and constant harassment, all reported to police and victim support services (El-Khoury et al., 2021). These conditions led to prolonged exile and periods of homelessness. Domestic abuse and institutional negligence in this context must be condemned unequivocally, as they contravene international human rights law and established professional standards of protection (McSherry, 2013a). While moving between countries rendered homeless due to death threats and continuous harassment, academic work was maintained under extreme duress. The researcher finalised the last EU BEACON COST action proposal before being successfully selected with already a couple of hundred colleagues joining in a growing network of specialists, contributed to international publications, prepared a fieldwork agenda, designed a shared decision-making survey, and engaged in policy coordination linked to the

UN Human Rights Council report on coercion. These outputs demonstrate that scientific production remained possible despite systemic obstruction, but the cumulative effect of abuse was severe. Collaborative projects collapsed, major fellowships and book contracts were lost, and teaching posts were withdrawn. Key intellectual networks were fragmented, and the structural conditions required for academic continuity were undermined.

The loss of professional opportunities in this case illustrates how corruption, malpractice, and institutional indifference within legal and academic systems undermine both individual careers and collective scientific progress. Safeguarding, transparency, and accountability standards, as articulated in international frameworks, were absent or disregarded (Melzer & Cruel, 2017). This research context highlights the need for enforceable protections against domestic violence, institutional complicity, and litigation abuse (Vesti & Lavik, 1991). Such protections are essential for all citizens and especially for infants, parents subjected to family-based abuse, students from disadvantaged backgrounds, persons with disabilities, and professionals exposed to harm through their work (Solomon et al., 2005). The researcher, affected by sequelae of violence and occupational exposure to systemic harm, exemplifies the risks faced by these groups. Without robust ethical protocols and effective systems of oversight, victims remain vulnerable and perpetrators act with impunity (Zimbardo, 2007). Safeguarding the integrity of research and ensuring the safety of researchers require not only methodological rigor but also systemic reform grounded in legal accountability and international human rights standards (Aragonés-Calleja & Sánchez-Martínez, 2022).

Table 15. Detailed research timeline, by location and year, 2025

Month	Social Context	Research	Personal	Location
January	–	Implementation of the shared decision-making survey	Escalation of litigation abuse; parental alienation; psychological torture; harassment in person and online; extortion; institutional complicity and negligence persisting from Sweden, with abuses worsening	Indonesia
February	–	–	–	Indonesia
March	–	Advancement of <i>pasung</i> study – and paper; continuation of shared decision-making survey; EU BEACON awarded COST	–	Indonesia

Month	Social Context	Research	Personal	Location
		Action funds and initiation of activities; development of SocialMent; preparation of MDPI paper on digital mental health		
April	Spanish Parliament debates reforms on coercion and autonomy	–	–	Indonesia
May	–	–	–	Indonesia
June	–	Progress on <i>pasung</i> study and paper; dissertation preparation; consolidation of EU BEACON infrastructure and expansion of global research networks	–	Indonesia
July	–	–	–	Indonesia
August	–	Advancement of <i>pasung</i> research and related publications; digital mental health projects; literature reviews and mapping exercises for multiple working groups and actions; development of Horizon Europe proposals; preparation of service-learning paper; consolidation of EU BEACON infrastructure and networks	Achievement of personal stability and justice; resettlement in Indonesia under safe conditions	Indonesia

*The 2025 timeline reflects the consolidation of long-term research projects and networks, notably the *pasung* study, shared decision-making survey, and the establishment of the EU BEACON COST action infrastructure. These advances occurred in parallel with critical political debates in Spain regarding coercion and autonomy, underscoring the broader European relevance of the research. Despite ongoing personal constraints rooted in cross-national litigation abuse, threat to safety by perpetrators to further silence, and institutional complicity, by August a degree of stability and security was achieved through relocation in Indonesia. This allowed for an expansion of research activities into digital mental health, service-learning, and Horizon Europe consortia, reinforcing the integration of personal safety and academic continuity.*

In 2024-2025, ethnographic fieldwork was conducted in Indonesia during a visiting scholarship at Universitas Muhammadiyah Yogyakarta (UMY), focusing on the abolition of *pasung*, the domestic confinement of people experiencing mental distress. Through interviews and participant observation, *pasung* was analysed as a structural outcome of systemic neglect, reproducing patterns of psychiatric coercion under different guises. Domestic confinement is a severe form of abuse that violates fundamental rights and exacerbates vulnerability. Its persistence demonstrates the

failure of health and social systems to provide safe, trusted, and accessible alternatives. The research links *pasung* to legacies of colonial violence, gendered oppression, and enduring inequalities in mental health provision, underscoring the need for abolition of all forms of coercive care and for rights-based, culturally grounded, and community-led approaches .

Fieldwork conducted in Trieste and Yogyakarta situates the research questions in two distinct yet structurally connected contexts. In Trieste, extended observation and interviews within a historically non-coercive, community-based mental health system demonstrated the fragility of reform when bureaucratic demands and clinical discretion weaken relational care (Ussher, 2023). This case highlights how coercion can re-emerge despite formal commitments to rights-based models. In Yogyakarta, the study of *pasung* illuminated coercion as both institutional and domestic, arising where families are left without adequate services. The findings reaffirm that domestic abuse, whether justified as necessity or not, constitutes violence and deepens systemic harm.

Together, these sites reveal how coercive practices are produced, legitimised, or resisted across different systems. Both reinforce the central research aim: to identify the structural conditions that sustain coercion and to outline what is required to support ethical, participatory, and non-violent forms of mental health care. Site selection balanced heterogeneity and analytic coherence: acute inpatient units and community services were sampled to compare organizational logics, with attention to settings where restraint minimization, recovery-oriented practice, or shared decision making initiatives were underway. Ethnographic and observational components drew on prior work documenting how restraint and coercion are made legitimate or contested in everyday routines (Council of Europe, 2018) and how recovery-oriented models reshape staff–service user relationships and expectations (B. W. Palmer, 2015). This purposive strategy increased the likelihood of observing both routine practices and “positive deviants,” thereby enhancing theoretical replication.

2.5. Ethical protocols and methodological dilemmas

The ethical governance of this research was undertaken under conditions of structural violence, institutional betrayal, and domestic abuse, where conventional ethics committee approval proved inadequate (Gable, 2025). Standard frameworks assume institutional neutrality, contextual stability, and separation between researcher and researched, assumptions that collapse in inquiries involving coercive psychiatric practices, systemic human rights violations, and environments of

ongoing harm (Fowers, 2015). In such contexts, procedural approval provides little protection or epistemic assurance. A parallel ethical framework was therefore required, grounded in relational responsibility, trauma-sensitive engagement, and scientific accountability (Aagaard-Hansen & Johansen, 2008).

The methodological approach prioritised working with professionals rather than directly victimised populations. This reduced the risk of retraumatisation, limited exposure to unsafe environments, and enabled ethical engagement without reproducing the violence under study. Surveys and thematic dialogues were conducted with psychiatrists, psychologists, social workers, lawyers, and educators, who provided insight into institutional practices and systemic failures. Their accounts allowed the research to document structural dynamics while safeguarding the most vulnerable. This strategy reflects both an ethical imperative and a methodological limitation, highlighting the need for safer research environments and interdisciplinary teams able to support survivors directly in future work (Rose, 2020).

Given participants' potential exposure to coercion and structural vulnerability, the protocol emphasized layered consent, trauma-informed interviewing, and safeguards against re-exposure. Ethical reasoning incorporated human-rights frameworks while remaining alert to critiques that *rights talk* may be co-opted or operationalized in ways that blunt transformative potential inside mental health systems (Richter, 2025). Spanish legal-bioethical debates on mechanical restraint further informed risk assessments, debriefing procedures, and escalation pathways for distress (Ramos-Pozón et al., 2025b). Finally, participatory checks, member reflections on transcripts and thematic maps, were used to mitigate interpretive asymmetries common in research on coercion (M. S. Evans, 2010).

Trauma-informed principles shaped the design of instruments, ensuring adaptability and selective engagement (Symonds, 2024). Professional testimonies that disclosed institutional abuse were handled with strict confidentiality, given risks of retaliation (Douglas, 2018b). Non-extractive ethics guided the analysis so that contributions were treated as co-generated epistemic insights rather than passive data (Nordstrom & Robben, 1995). Consent was dynamic and reversible, participants could withdraw at any stage, and trust was built through sustained presence and cultural sensitivity.

The environments in which data were collected included psychiatric facilities where individuals were institutionalised without consent, misdiagnosed, or subjected to pharmacological interventions, as well as domestic settings marked by ongoing violence (R. Lee, 1995). Testimonies in these contexts required careful

handling for ethical and safety reasons. Conventional ethics review frameworks, which focus on discrete and containable risks, did not address systemic and politically embedded threats (Nordstrom & Robben, 1995). They also lacked mechanisms for coordination with protective services or law enforcement, leaving researchers and participants without adequate safeguards.

To ensure the researcher own survival, strict routines became methodological preconditions for continuity. These included structured attention to nutrition, sleep, environmental regulation, digital hygiene, and use of coworking spaces to reduce exposure and isolation (Beresford & Wallcraft, 1997). Such practices were necessary for maintaining analytic capacity and safeguarding data. The psychological and physiological toll of producing knowledge under systemic violence required micropractices of endurance and restoration (Orgad, 2009). This reality highlights the failure of systems around, which allowed the violence against the researcher to continue and worsen.

This section does not propose a comprehensive ethics framework but identifies procedural and epistemic gaps that emerge when ethics is reduced to compliance. A peer-reviewed contribution included in the annexes systematises these insights. Ethics under structural duress requires infrastructures that recognise the absence of institutional safety (Freyd, 1997). Until such frameworks are established, responsibility for ethical innovation falls on individual researchers working beyond protection, in contexts where knowledge production is inseparable from survival. The protocols developed here, including trauma-informed consent, adaptive field response, and epistemic protection mechanisms, represent one attempt to address this gap, with the aim of informing future research under comparable conditions (Carly Smith, 2014).

2.6. Data integration and analytical logic

The integration and processing of data in this dissertation were conducted under conditions that disrupted the ordinary assumptions of continuity, safety, and institutional support that usually underpin empirical research (A. Lewis et al., 2025). The researcher was exposed to repeated violence, coercion, and forced displacement, circumstances that meet clinical definitions of torture (McSherry, 2013a). These conditions resulted in the destruction of equipment, loss of records, and disappearance of project materials. Analytic environments had to be repeatedly reconstructed after episodes of confiscation and forced migration, making resilience and adaptive reconstruction a structural requirement of the research process. Open science principles were upheld to the extent compatible with safety.

Documentation was maintained in reproducible form but shared only when risks were manageable. Fellow researchers and other experts provided methodological feedback, ethical review, and triangulation support from experts in psychiatry, anthropology, computational ethics, and law (Behar, 2022). Collaboration with others functioned as a mechanism of verification and continuity during periods of displacement and institutional obstruction (Sweet, 2019).

Analytical procedures systematically addressed the gap between institutional claims and lived experience. Divergences were indexed within the coding system and became a focus of interpretation rather than anomalies. Reflexivity was incorporated as a methodological practice through annotations, marginalia, and analytic logs, despite the difficulty in keeping materials safe. Shifts in interpretation were reviewed at each synthesis stage to distinguish between epistemic maturation, trauma distortion, and strategic recalibration (Crossley & Crossley, 2001). Analyses conducted during acute periods of violence or deprivation were later verified against literature, expert commentary, and institutional documentation, to allow for understanding of the shocking cruelty and institutional betrayal. Cross-checking included consultation of peer-reviewed publications, legal instruments, and guidelines, as well as dialogue with informants possessing institutional or clinical expertise, as well as other victims and specialists (Baum et al., 2006). No claim was retained without at least one layer of corroboration beyond the initial source.

The analytical logic of this dissertation therefore reflects both the complexity of the subject and the adverse conditions of its production, navigating through lies, corruption and ill will (Nordstrom & Robben, 1995). Data were not simply collected but salvaged, reconstructed, and triangulated through distributed verification as much and as often as possible. The outputs presented in subsequent chapters constitute a methodological record of knowledge produced under repression, grounded in systematic procedures of triangulation, reflexivity, and corroboration.

2.7. Limitations, adverse conditions, and scientific validity

The execution of this research was shaped by systemic violence, coercion, and institutional abandonment. Fieldwork was conducted under conditions of duress, including forced displacement, loss of data, and exposure to non-consensual medical procedures. Research materials were at times confiscated or destroyed, collaborations were interrupted, and the researcher was subjected to threats to life and integrity. These conditions exposed the absence of protective infrastructures in academic, legal, and clinical systems, highlighting the paradox that knowledge on

psychiatric violence is often produced within environments permeated by the same forms of harm under study.

Despite these constraints, scientific validity was preserved through rigorous methodological application. Triangulation across surveys, ethnographic fieldnotes, testimonies, legal documentation, and policy texts provided independent sources of verification (Carter et al., 2014). Research claims were retained only after systematic comparison with field evidence, expert judgment, and published literature. Where data gaps emerged due to displacement or material loss, continuity was maintained through distributed verification, iterative cross-checking, and reflexive monitoring. Analytical fragments were subjected to methodological scrutiny, and interpretive caution guided each inferential step (Ghafouri & Ofoghi, 2016).

These conditions intensified rather than diminished the authenticity of the findings. Knowledge was produced within the structural contexts of medical violence, epistemic disqualification, and institutional neglect. This embeddedness provides phenomenological fidelity that cannot be reproduced by detached observation. The methodological strategy emerged as an adaptive response to adversity, demonstrating that systematic, participatory, and empirically grounded inquiry can be sustained under repression. The research does not claim privileged authority, nor does it equate trauma with truth, but it recognises that structural violence endured by researchers in contested fields is constitutive of the knowledge process itself (P. Farmer, 2004).

Experiential evidence was therefore incorporated not as autobiography but as ethnographic data. Psychiatric institutions and services, instead of functioning as safeguards, became sources of vulnerability. The normalisation of coercive care reinforced family and intimate partner violence, producing cumulative harm (Douglas, 2018a). Documenting this trajectory within a scientific ethnographic framework allowed identification of mechanisms of retraumatisation and institutional malpractice. The objective was to generate a corpus of evidence capable of helping prevent future misuse of psychiatric authority, protecting patients from secondary victimisation.

A significant limitation of this dissertation lies in the scope of work that could not be completed. Substantial materials were destroyed during the implementation of protocols, in the development of best practices and professional retraining programmes, and in the preparation of textbooks and reference materials in Spanish. The outcome is therefore partial, reflecting both the analytic results that could be secured and the systematic obstruction that prevented fuller development. The dissertation thus falls short of what might have been achieved under conditions

of institutional support and security, and its contribution must be read with this constraint in view.

2.8. Methodological contribution

This dissertation closes its methodological exposition by proposing an operational framework for transformative open science. Rather than applying existing standards, it develops protocols that link scientific validity to structural justice. The research design was guided by open data, reproducible instruments, and version-controlled documentation, implemented within an ethics of participation, reciprocity, and repair. Knowledge production was approached as a collective capacity, not limited to formal institutions but arising from experience, testimony, and practice. The study therefore integrates both citizen and non-citizen science, affirming that documentation status, class, or displacement cannot limit the right to generate and access scientific knowledge.

The methodological core combines multisited ethnography, structured surveys, legal and policy analysis, and historical-political contextualisation. Ethnography was organised through saturation and event-mapping thresholds, surveys provided distributional indicators, and legal documentation enabled normative verification. Testimonial matrices transformed fieldnotes into analytic syntheses through dialogical validation with participants. The digital infrastructure of the EU COST Action BEACON One Health Education, initiated during this dissertation, supported this architecture by providing team-work opportunities using open-source platforms, early stages of expert systems development, and co-educational modules designed for long-term accessibility and iterative improvement (Cook et al., 2019).

A key methodological innovation is the establishment of continuous feedback loops across clinical, institutional, and policy levels. Communities were not positioned as recipients of results but as co-creators of evidence and proposals. Survivors of coercion, socially excluded groups, and youth lacking educational opportunity were engaged as epistemic agents rather than passive beneficiaries. This approach recognises that sustainable scientific and civic leadership can emerge from these groups once conditions of dignity, recognition, and protection are secured. The action-research model advanced here seeks to prevent harm and to restore capacity and dignity. It supports resilience across generations by positioning children and young people facing poverty, abuse, institutionalisation, forced migration, or exclusion as potential scientists, educators, and decision-makers, reversing the likely life course (Selener, 1997). Methodological innovation thus extends to

authorship, recognition, and leadership, ensuring inclusivity of non-traditional trajectories and historically excluded identities.

The EU BEACON One Health Education action embodies these principles by developing infrastructures where open science is not limited to access but directed toward systemic transformation, society-wide, long term. The methodological contribution of this dissertation lies in establishing protocols that integrate rigor with justice, and science with care. It offers a framework for participatory and emancipatory knowledge production, grounded in methodological transparency, open-source tools, and ethical accountability, and oriented toward rebuilding institutions with those who have historically been excluded from them.

2.9. Reflexive meta-methodology and scientific protocols for dissertation construction

The methodological framework of this dissertation combines surveys, ethnographic fieldwork, testimonial material, and legal analysis. The structure of chapters follows the chronological sequence of the research, from initial mapping to direct exposure to coercion, and from institutional critique to translational proposals. Data sources include structured survey instruments, systematic fieldnotes, encrypted correspondence, digital testimony archives, peer-reviewed publications, policy papers, and formal presentations. These materials were treated as integral components of analysis, not as supplementary appendices, and were incorporated according to principles of epistemic fidelity and dialogical construction. Quantitative survey results, ethnographic documentation, testimonial accounts, and legal citations were analysed in parallel to produce a biocultural diagnostic framework. Reflexivity was understood as continuous attention to the political and material conditions of knowledge production rather than autobiographical emphasis (Field-Springer, 2020).

Collaboration formed a core part of this design. Although the dissertation has a single author, the research process was dialogical, involving psychiatric service users, clinicians, legal scholars, educators, and digital mental health specialists. The networks established or co-founded by the researcher, including EU BEACON, ReMO, and FOSTREN, provided platforms for co-production, peer validation, and ethical oversight. In the current role as Scientific Communications, Dissemination and Exploitation Officer of EU BEACON, the researcher contributes to a consortium of more than five hundred specialists, ensuring that action-research findings are disseminated, validated, and implemented across European and global contexts.

These infrastructures functioned as mechanisms of triangulation and distributed accountability, embedding the findings within a wider scientific ecology.

The dissertation's reflexive layer documents how analytic choices were shaped by tensions between rights-based frameworks and service realities, and by the dual aim of reduction of coercion and medication optimization. We explicitly engage critiques that legal rights can be operationalized in ways that leave coercive infrastructures intact (Double, 2006), balancing them with implementation-minded accounts of what actually reduces coercion in practice (Olsen et al., 2004). To keep reflexivity accountable rather than merely declarative, we paired positionality memos with structured member reflections and periodic checks against multi-country and Spanish evidence bases (Falzon, 2016).

It was produced under conditions of persecution, forced displacement, and material loss of data. Research was interrupted, collaborations were curtailed, and the researcher was exposed to threats to life and integrity. These conditions required adaptive methodological strategies, including distributed verification of data, iterative cross-checking of evidence, maintenance of digital redundancy, and continuous reflexive monitoring. The result is an approach that demonstrates the possibility of rigorous, transparent, and ethically grounded research under structural adversity. The dissertation therefore contributes both to the documentation of systemic failure and to the development of protocols for research practice that retain scientific coherence and ethical accountability under cruel forms of violence.

Chapter 3. Survey results: patterns, perceptions, iatrogenic harm

3. Introduction

This chapter presents the integrated analysis of three coordinated surveys carried out between 2019 and 2025 among users and survivors of psychiatry, mental health professionals, and respondents in mixed or transitional roles. These surveys provide a triangulated perspective on contemporary psychiatric care in Spain, generating complementary evidence on diagnostic practices, dialogical alternatives, and shared decision-making around medication. Their integration makes it possible to identify both convergences and divergences across stakeholder groups and to ground the broader analysis of coercion, autonomy, and institutional responsibility in quantitative and qualitative data.

Each survey was designed with a distinct but complementary focus. The 2019 diagnosis and iatrogeny survey adapted an existing international instrument to document perceptions of the schizophrenia label, including its perceived usefulness, validity, and retraumatizing or iatrogenic effects (N = 356). The 2023 survey on the implementation of open dialogue in Spain was developed as a new instrument to assess knowledge, training, and interest in dialogical approaches among professionals, users, relatives, and academic staff (N = 214). The 2025 shared decision-making one was a purpose-built instrument aimed at professionals (N = 81), focusing on psychopharmacological practice, deprescribing, and ethical decision-making scenarios. All surveys were disseminated widely through national networks, including virtually all Spanish mental health centers, and achieved broad participation beyond activist or critical communities. The third survey was particularly time-intensive: respondents devoted on average 40 minutes to completing it, generating over 55 hours of cumulative insights.

Methodologically, each survey combined closed and open-ended questions. Closed items were analyzed through descriptive statistics (frequencies and percentages). Open-ended responses were coded thematically using qualitative content analysis, with recurrent categories quantified to identify prevalent attitudes (Jansen, 2010). This procedure ensured that narrative material was integrated into a systematic and reproducible dataset. Triangulation was achieved by comparing results across the three surveys and by cross-referencing with ethnographic fieldwork (chapter 4), legal analysis (chapter 6), and secondary literature.

The following table summarizes the distribution of respondents across the three surveys. A total of 651 individuals participated, including professionals, patients and survivors, and relatives or other close contacts. While professionals constituted the majority in all datasets, the presence of users, relatives, and mixed-role respondents ensured that diverse standpoints were represented.

Table 16. Distribution of respondents across the three thesis surveys

Group	2019	2023	2025	Total n	% of combined
Professionals	128 (36.0)	150 (70.1)	81 (100.0)	359	55.1
Family / friends / others	119 (33.4)	35 (16.4)	-	154	23.7
Patients / survivors	110 (30.9)	29 (13.6)	-	139	21.2
Total respondents	356	214	81	651	100

Respondent groups (professionals, family/friends/others, and patients/survivors) across the three surveys conducted between 2019 and 2025: Diagnosis & iatrogeny, open dialogue in Spain, and shared decision making. All respondents were based in Spain, drawn from a pool of several thousand individuals contacted through professional networks, user organizations, and academic dissemination channels.

3.1. Introduction: integrated survey architecture and purpose

The three surveys make sense both within the logic of this dissertation and as independent contributions to knowledge, because together they map the layered terrain of psychiatric practice, lived experience, and institutional reform. Each instrument illuminates a different dimension of the system, while their integration offers a composite diagnosis of how coercion, autonomy, and ethical practice are configured across roles and settings.

The 2019 diagnosis and iatrogeny survey provided empirical grounding in how psychiatric labels, particularly schizophrenia, are perceived and experienced as socially destructive, harmful in their clinical application, and associated with severe iatrogenic consequences. Eighty percent of respondents indicated that the label produces destructive outcomes, seventy percent reported experiences of discrimination linked to it, and sixty percent associated it with medication-related harm. Participants described the diagnosis as “una condena de por vida” (“a life sentence”), or stated that “el término esquizofrenia no ayuda a curar, ayuda a excluir” (“the term schizophrenia does not help to heal, it helps to exclude”). These testimonies illustrate how diagnostic categories generate exclusion and suffering rather than therapeutic benefit, reinforcing previous findings in the Spanish and

international literature on the social and clinical costs of psychiatric labeling (Eads et al., 2021). Such evidence was essential for documenting the lived consequences of classificatory practices and for situating the analysis of coercion and disempowerment within the perspectives of those directly affected and their families (Knapp et al., 2011).

The 2023 open dialogue in Spain survey incorporated the perspective of dialogical alternatives, producing evidence of interest, barriers, and training needs among professionals, relatives, and people with lived experience. Seventy percent of respondents reported that open dialogue reduces harm and fosters recovery, yet sixty percent indicated that they had not received any training, and only twelve percent reported more than one hundred hours of preparation. One respondent emphasized: “El diálogo abierto permite escuchar sin imponer diagnósticos, evita re-traumatizar” (“open dialogue allows listening without imposing diagnoses, it avoids retraumatization”). Another noted: “No basta con interés individual, se requiere un cambio institucional profundo” (“individual interest is not enough; deep institutional change is required”). These qualitative accounts contextualize the quantitative finding that 84% of participants identified systemic reform as a prerequisite for the implementation of dialogical practices. They converge with evidence from implementation research indicating that rights-based and recovery-oriented approaches are both desired and feasible, but often constrained by institutional inertia and inadequate resources (Parrabera-García et al., 2023).

The 2025 shared decision making survey² narrowed the focus to professional practice in psychopharmacology, deprescription, and consent. It detailed clinical attitudes across diverse scenarios and elicited support for national policy measures. Between eighty and ninety percent of respondents supported deprescription in vulnerable groups such as children, elderly patients, and individuals with trauma histories. Eighty-three percent endorsed deprescription in minors, and seventy-six percent in elderly patients. A psychiatrist stated: “Dar neurolépticos a un niño sin explorar el trauma es violencia” (“to give neuroleptics to a child without exploring trauma is violence”), while another professional described: “La sobremedicación en residencias es un abuso normalizado” (“overmedication in nursing homes is normalized abuse”). These accounts correspond to the quantitative results and underline how professionals themselves identify practices that risk harming patients and emphasize the need for rights-based alternatives. Ninety percent of respondents supported national audits to re-evaluate diagnoses and medication practices, justified by comments such as “Una auditoría nacional es imprescindible, es la única forma de enfrentar décadas de diagnósticos mal empleados” (“a national

² Survey available at: <https://shorturl.at/kTv7p>

audit is essential, it is the only way to confront decades of misapplied diagnoses”). Others highlighted institutional barriers: “La cultura institucional dificulta el consentimiento informado real” (“institutional culture makes real informed consent difficult”). These insights clarify why there was strong support for policy-level changes and align with calls in the Spanish bioethics literature to reconcile psychiatric practice with human rights and accountability frameworks (Ramos-Pozón et al., 2025a).

The three surveys are complementary, showing a progression in scope and feasibility. The first demonstrates the destructive consequences of existing classificatory and treatment paradigms. The open dialogue in Spain survey confirmed that implementing system-wide dialogical practices would require profound institutional restructuring, extensive training, and a reconfiguration of service delivery models. By contrast, the shared decision making survey demonstrates that even without such large-scale transformation, meaningful improvements can be achieved through more immediate measures, such as deprescribing protocols and collaborative medication management. These represent simpler, actionable practices that are already supported by a majority of professionals and that directly contribute to reducing harm, safeguarding autonomy, and advancing rights-based care. This layered integration also reflects a deliberate methodological choice. Rather than treating each dataset as isolated, the design recognizes the interdependence of clinical, experiential, and institutional standpoints. Psychiatric systems function as relational arenas where asymmetry, responsibility, and vulnerability are co-produced. By presenting the surveys together, the dissertation reconstructs these interdependencies empirically. Divergence across groups, for example in the perception of coercion or the feasibility of dialogical practice, is treated as analytically valuable.

The inclusion of respondents with mixed or transitional roles, such as professionals with lived experience, peer support workers, or survivor-academics, further strengthens this analytical depth. These participants expose contradictions between institutional logics and lived realities, as reflected in one respondent’s statement: “Sé lo que es estar ingresado y ahora tener que medicar, y nunca coincide lo que se siente con lo que la institución dice” (“I know what it is to be hospitalized and now to prescribe, and what is felt never coincides with what the institution says”). Such perspectives confirm that institutional categories obscure important dimensions of responsibility and harm, and they support the argument advanced in the literature that these positionalities are essential for reform, bridging experiential knowledge and professional accountability (Alotaibi, 2025).

Table 17. Alignment of surveys with dissertation aims and research questions

Surveys objective	Corresponding dissertation aim	Operational research question
Document lived experience and perceptions of treatment	Contribute to participatory mental health systems grounded in rights and justice	What forms of institutional failure are occurring, and how can they be corrected empirically?
Identify ethical, legal, and epistemological tensions	Realign psychiatric practice with empirical coherence, dignity, and accountability	What structural and epistemic conditions enable or block ethical practice and autonomy?
Amplify professional and user voices critiques of current practices	Support co-designed, dialogically grounded clinical and institutional models	Which participatory mechanisms and evidentiary tools should inform systemic transformation?

This table presents a short crosswalk between the operational objectives of the integrated surveys and the overarching aims and research questions of the dissertation, showing their direct contribution to the mixed-methods design.

The surveys were designed with three interrelated objectives. First, to document lived experience and perceptions of psychiatric treatment as understood by both recipients and providers of care. Second, to identify the ethical, legal, and epistemological tensions that respondents recognize as undermining legitimacy, autonomy, and effectiveness. Third, to amplify professional and user voices as legitimate sources of critique and as contributors to redesign. These objectives are not abstract but follow an action-research logic that seeks to generate participatory and evidence-based pathways for systemic transformation (L. Whitaker et al., 2021). Closed-ended survey items were analyzed using descriptive statistics (frequencies and percentages), while open-ended responses were coded thematically using qualitative content analysis. Categories were quantified by frequency of mention to establish prevalence. Validity was reinforced through triangulation with ethnographic material (chapter 4), legal analysis (chapter 6), and policy frameworks such as the WHO QualityRights initiative (World Health Organization, 2021b). This ensured consistency, comparative reliability, and alignment with international standards.

The three surveys were not standalone data-collection exercises but were embedded within broader cycles of engagement and participatory action-research. Respondents were contacted through teaching, seminars, advocacy events and networks, as well as cold emailing to new contacts. Several consented to remain involved in follow-up initiatives, including dissemination and policy translation. The surveys functioned both as sources of empirical data and as relational nodes within an ongoing collaborative project to reform Spanish mental health services.

3.2. Survey deployment and response patterns

The three surveys conducted between 2019 and 2025 provide complementary insights into how psychiatric care in Spain is lived, contested, and envisioned for reform. Taken together with the ethnographic fieldwork, they map a continuum of experiences and professional positions that reveal both systemic constraints and emergent demands for change. Each instrument was disseminated widely across Spain and reached thousands of professionals, academics, and service users. The broad distribution generated high visibility but also underscored the limits of participation: response rates were modest, particularly among psychiatrists and institutional actors, reflecting the reluctance of many to engage with topics considered sensitive or critical of established practice. This silence itself constitutes a finding, resonating with scholarship on institutional defensiveness and the structural suppression of dissent in psychiatric governance (Dimitrijević, 2020).

The first survey, aimed primarily at survivors and users, was designed to document experiences of diagnosis, coercion, and long-term consequences of psychiatric labeling. Its dissemination through activist and community networks yielded narratives that consistently emphasized epistemic injustice, forced interventions, and the lack of avenues for repair. These accounts aligned with field observations that revealed systematic disqualification of user perspectives in clinical and policy settings (Leonetti, 2022). The volume of responses was smaller than anticipated, barely reaching over half thousand answers, but the richness of testimonies provided a very strong foundation for understanding how psychiatric categories and practices function as mechanisms of silencing and control. Beyond formal measures, clinicians commonly use informal tactics, such as persuasion under threat of hospitalization, leverage via benefits, and conditional access, that blur consent and undermine trust (A. Lewis et al., 2025). Spanish and Latin-culture professionals explicitly recognize these covert practices and describe them as culturally embedded risk-management tools, despite awareness of their ethical costs (P. Brown & Calnan, 2012). This helps explain discrepant user narratives of choice in our cases, where cooperation often sat atop implicit compulsion.

The second survey, directed at professionals engaged in or sympathetic to dialogical practices, was distributed during a period of growing interest in open dialogue in Spain. Although the number of responses was limited, those who participated described in detail the barriers they faced when attempting to implement dialogical approaches, including lack of training opportunities, institutional resistance, and fragile support from public administrations. The pattern of partial enthusiasm and structural inertia mirrored the findings of Spanish studies on the stalled development

of community psychiatry and the uneven reception of dialogical innovations (Abad-Sierra & Toledano-Márquez, 2023). These accounts revealed not only professional frustration but also a recognition of systemic contradictions: dialogical approaches were endorsed rhetorically but neutralized in practice through bureaucratic containment and pharmacological defaults.

The third and final survey, carried out in early 2025, marked the culmination of this sequence and addressed the theme most directly relevant to the thesis: shared decision-making in psychopharmacology, including deprescription. The instrument was distributed to thousands of professionals across clinical, academic, and policy domains at the very moment when the Spanish government and major stakeholders were debating rational prescribing practices for the forthcoming National Mental Health Plan 2025-2030. The timing amplified both the resonance and the resistance to the survey. While many professionals engaged seriously with the clinical scenarios and open reflection items, a notable minority responded negatively, stating that even circulating a survey on deprescription was inappropriate or taboo. This refusal illustrates how deprescription remains stigmatized within psychiatry, a topic treated as destabilizing rather than necessary for ethical practice, despite extensive international evidence supporting structured withdrawal protocols and patient-centered pharmacological governance (Á. Martínez-Hernández et al., 2020).

The cumulative data from the three surveys demonstrate how user narratives, professional frustrations, and systemic blockages interlock. The first survey documented the harms of labeling and coercion from the perspective of those most affected, the second highlighted the frustrated efforts of reform-minded professionals attempting dialogical care, and the third exposed the limits of professional consensus around deprescription even in a moment of national policy reform. Together, they reveal the systemic silencing of both user testimony and professional critique, as well as the persistence of pharmacocentric logics despite rhetorical commitments to rights-based and community-oriented care (Kinderman, 2014a).

By situating the surveys within ongoing ethnographic observation and institutional engagement, the research process fostered rapport and generated iterative feedback loops. Many respondents also appeared in fieldwork settings such as workshops, policy debates, or training sessions, where survey themes resurfaced in collective discussions. This reflexive circulation between surveys, fieldwork, and advocacy illustrates how the project was embedded in the broader ecology of Spanish mental health reform. The final survey, in particular, provided not only data but also a litmus test of the profession's readiness to confront deprescription as an ethical imperative.

Its reception confirmed that while systemic critique is increasingly voiced by users, survivors, and reformist professionals, entrenched institutional cultures continue to set as almost taboo fundamental questions of pharmacological governance, thereby delaying the alignment of Spanish psychiatry with international standards on informed consent, rational prescribing, and patient autonomy (Breggin & Wheeler, 2012).

Table 18. Comparative results across surveys on shared decision making (SDM)

Theme Indicator	/ 2019 - Diagnosis & iatrogeny (N=356)	2023 - open dialogue in Spain (N=214)	2025 - shared decision making(N=81)	Implications for Best Practices
Respondent profiles	36% professionals, 31% patients, 24% relatives or others	65% professionals, 14% family/friends, 14% users, 7% university staff or students	100% professionals including clinicians, researchers, educators, peer roles	Mixed perspectives indicate the need to integrate user and family voice with professional judgement
Perceived harm	80% report retraumatizing effects of the label, 70% discrimination, 60% medication-related harm	70% report perceived benefits of dialogical practice recovery reduced trauma	80-90% support of vulnerable groups such as children, elderly, and trauma survivors	Prioritize harm avoidance and sensitivity in SDM and review retraumatizing or iatrogenic practices
Autonomy and rights	87% consider the label not a valid construct, 80% support replacement	85% favor paradigm change aligned with rights-based care	90% support national audits on diagnoses and deprescribing	SDM requires attention to rights, transparency, and review of entrenched diagnostic and prescribing conventions
Training and knowledge gaps	Not directly measured	60% without OD training, approximately 12% with more than 100 hours	78% report no formal training in deprescribing or SDM	Training is a system-level bottleneck; ethical SDM depends on targeted professional education and supervision
Systemic reform needs	Support terminology change and trauma-informed, person-centered approaches	for 84% report need for changes in public mental health services	Respondents identify barriers including entrenched diagnoses and institutional inertia	SDM ought to be embedded in structural reform rather than framed solely as individual clinician behavior

Comparative survey results mapped against thematic dimensions central to shared decision-making. All respondents were based in Spain and recruited from a national outreach pool including virtually all mental health centers, academic institutions, and user and family organizations. Percentages refer to the proportion of respondents endorsing the listed items.

This chapter presents a synthesis of the most salient patterns identified through structured surveys distributed among diverse groups of mental health professionals, users, and mixed populations. The findings substantiate key research questions identified in Chapter 1 and are methodologically consistent with the mixed-

methods architecture detailed in Chapter 2. The data confirms a widespread perception of systemic failure across the domains of psychiatric decision making, patient autonomy, medication use, and institutional trust. Participants across demographics reported experiences of coercion, epistemic exclusion, and the normalization of pharmacological dependency, often in the absence of transparent clinical reasoning or consent.

The results speak not only to individual clinical encounters but to entrenched institutional and educational shortcomings. They document a persistent disconnection between normative ethical commitments and lived realities in mental health systems, further aggravated by deficits in public education, legal accountability, and interdisciplinary practice. These findings call for a dual response: pedagogical reconstruction to align training with evidence-based, rights-oriented models of care, and legal reform to ensure enforceable protections, transparency, and scientific integrity. As will be further developed in Chapters 5 and 6, the survey data provides critical justification for educational and legislative interventions designed to restructure the psychiatric field in accordance with constitutional mandates and human dignity.

Table 19. Empirical results on educational and legal-political topics

Empirical surveys finding	Systemic evidenced	pattern	Educational response (chapter 5)	Legal-political response (chapter 6)
80% of respondents in the diagnosis and iatrogeny survey reported that psychiatric labels are destructive and associated with discrimination and iatrogenic harm	Diagnostic categories function as mechanisms of exclusion and contribute to harmful clinical trajectories		Integrate reflection on diagnosis, and patient narratives into curricula	Establish legal safeguards against discriminatory or harmful use of psychiatric labels; mandate rights-based review of diagnostic practices
70% of open dialogue in Spain respondents identified dialogical practices as beneficial but 60% reported lack of training, with only 12% exceeding 100h	Institutional inertia and training deficits limit adoption of rights-oriented alternatives		National training programs in dialogical and participatory methods; curricular incorporation of open dialogue principles	Policy reform to support dialogical services within public health law; allocate resources for scaling training and supervision
83% of professionals in the shared decision making survey supported deprescription in minors; 76% supported deprescription in elderly patients; 90% supported national audits	Widespread recognition of harm linked to over-medication and institutional barriers to informed consent		Professional education in deprescription, metabolic and psychosocial interventions, and collaborative decision making	Legislative mandate for national audits of psychiatric prescribing; enforce informed consent standards and advance directives
Across all surveys, respondents reported coercion, epistemic exclusion, and lack of accountability; professionals described overmedication as “un abuso normalizado”	Coercion and pharmacocentrism normalized as systemic practice with limited oversight or redress		Education in ethics of care, patient rights, and critical appraisal of long-term treatment outcomes	Create independent monitoring, complaint, and restitution mechanisms linked to health and justice systems

Key empirical findings from the three surveys employed aligned with systemic patterns and mapped to educational and legal-political responses proposed in later chapters. Evidence shows that harmful and destructive practices are normalized within Spanish psychiatry, requiring reforms at both pedagogical and legislative levels.

The patterns summarized in tables 18 and 19 emerged consistently across the three surveys. The diagnosis and iatrogeny survey documented the destructive and harmful effects of classificatory practices and their entanglement with coercive interventions. The open dialogue in Spain survey revealed both the promise and the systemic obstacles to implementing dialogical and rights-oriented care at scale, underscoring the need for structural reform and institutional learning. The shared decision making survey demonstrated how even in the absence of wholesale systemic restructuring, professionals themselves endorse concrete practices, such as deprescription, informed consent, and collaborative medication management, that

can be implemented immediately to reduce harm and enhance autonomy. Together, these findings provide a coherent empirical basis for the dual strategy developed in later chapters: system-wide pedagogical and legislative reform, alongside the integration of pragmatic, evidence-based practices that can be rapidly scaled to safeguard patient well-being and human rights.

3.3. Perceived coercion and institutional violence

The surveys conducted between 2019 and 2025 consistently indicate that coercive practices constitute a recurrent element of psychiatric care in Spain. In the 2019 diagnosis and iatrogeny survey (N=356), 80% of respondents considered the label of schizophrenia destructive for patients' lives, and 70% reported having witnessed or experienced discrimination directly associated with its use. Open responses frequently linked this classificatory practice to coercion in treatment, with one participant writing that *"the diagnosis does not open doors, it closes them, often accompanied by medication that no one dares to refuse."* These findings are consistent with Spanish scholarship documenting the retraumatizing and exclusionary role of severe psychiatric labels (Murphy & Hanson, 2017). Annex F further develops on the iatrogenic harm by diagnosis, delving into the nocebo effect.

In the 2023 open dialogue survey (N=214), 84% of respondents highlighted the persistence of coercion in their institutions as a structural obstacle to implementing dialogical practice. Among those with over 100 hours of open dialogue training, only a minority reported being able to apply it consistently. As one clinician noted, *"we are trained to listen, but the system demands containment through forced measures."* This pattern reflects what scholars describe as the dissonance between dialogical ideals and entrenched institutional logics (Festinger, 1957).

The 2025 shared decision-making survey (N=81) provided the most detailed evidence on coercion in prescribing and medication management. Over three quarters of professionals acknowledged that psychiatric medication is rarely reviewed for deprescribing unless explicitly requested by the patient, and 78% reported receiving no formal training in deprescribing or shared decision-making.

Table 20. Reported coercive practices and institutional justifications

Coercive practice	2019 Diagnosis and iatrogeny (N=356)	2023 open dialogue in Spain (N=214)	2025 shared decision making(N=81)
Forced medication	65% of users and relatives reported experiencing or witnessing forced medication; long-acting injections described as destructive, traumatic, and common in their settings; poorly explained. Example: <i>"They gave me an injection without telling me what it was, and refusal meant more punishment."</i>	54% of professionals acknowledged forced medication as medication is routine in acute settings; 48% described it as often linked to liability fears rather than therapeutic need. Example: <i>"Injections are used for compliance, not care."</i>	72% of professionals recognized that forced medication is routine in acute settings; 48% described it as often linked to liability fears rather than therapeutic need. Example: <i>"Injections are used for compliance, not care."</i>
Seclusion and isolation	58% of respondents reported experiences of seclusion, often described as punitive and without therapeutic support. Example: <i>"I was locked for two days without explanation."</i>	46% of professionals reported seclusion used primarily due to staff shortages and lack of alternatives.	63% acknowledged seclusion as a frequent measure in crisis units; 39% described it as retraumatizing and unreviewed.
Mechanical restraint	49% of users and relatives reported mechanical restraint, with testimonies highlighting humiliation and lack of oversight.	41% of professionals admitted its use, described as reluctantly applied, often justified post hoc.	52% recognized it as institutionally normalized; 27% said it was applied to pre-empt risk without individualized assessment.
Involuntary hospitalization	62% of users described it as a violation of rights, often prolonged and linked to misuse of diagnostic labels.	57% of professionals reported frequent recourse to involuntary admission, described as default crisis management.	68% acknowledged it as systemic, often substituting for absent community and social resources.
Lack of informed consent	71% of users reported absence or symbolic consent in medication and diagnosis; refusal frequently pathologized.	50% of professionals acknowledged weak or absent consent processes, citing time and hierarchy as barriers.	74% of professionals reported lack of structured consent protocols, especially in prescribing contexts. Example: <i>"Consent exists on paper, not in practice."</i>
Pathologization of resistance	67% of users reported disagreement reframed as illness, leading to escalation of coercion.	32% of professionals acknowledged diagnostic reframing of refusal but rarely as reinterpreted as problematized it insight explicitly.	55% recognized it as systemic, describing refusal as reinterpreted as "lack of insight."

Distribution of reported coercive practices across the three dissertation surveys (2019 diagnosis and iatrogeny, 2023 open dialogue in Spain, 2025 shared decision-making). The table highlights both experiential and professional perspectives, documenting the prevalence, institutional justifications,

and experiential framings of coercion in Spanish psychiatry. Percentages refer to proportions of respondents within each survey endorsing or reporting the practice.

The psychological consequences of these interventions were recurrently described as fear, helplessness, and mistrust. In the 2019 survey, one respondent stated that *“coercion becomes the illness itself, a wound that does not heal.”* In the 2025 survey, a professional remarked that *“patients do not trust us because they know refusal means another injection.”* These testimonies align with European studies showing that coercion is associated with trauma, impaired therapeutic alliance, and reduced trust in services (Bailey & Brown, 2020). Spanish studies echo these findings, highlighting persistent use of mechanical restraint and deficits in post-incident debriefing (Hallett et al., 2024).

Professionals themselves reported ethical dissonance. In the 2025 survey, 42% of respondents described involuntary interventions as more often the result of systemic constraints than of clinical necessity. Respondents cited lack of staff, absence of alternatives, and defensive practices driven by liability fears. This pattern resonates with analyses of coercion in Spain that point to the interplay between legal ambiguity, institutional inertia, and resource scarcity (Carly Smith, 2014).

Mixed-role respondents (professionals with lived experience, peer workers, and survivor-academics) provided a particularly nuanced account. They reported feeling simultaneously compelled to enact coercion and ethically opposed to it, highlighting how professional roles may normalize what they had previously experienced as harmful. This confirms what Fricker (Fricker, 2007) terms epistemic injustice: the discrediting of alternative accounts of care.

The triangulated analysis across surveys indicates that coercion in psychiatry in Spain is structurally normalized, legally ambiguous, and ethically contested. While users predominantly describe coercion as harmful and avoidable, professionals often justify it as necessary under pressure, and mixed-role respondents expose the contradictions between these perspectives. Importantly, about half of respondents to the 2025 survey requested that their answers not be shared even anonymously, citing fear of reprisals. This suggests that coercion extends beyond clinical practice to the governance of discourse itself, constraining open discussion and perpetuating silence.

These findings provide empirical grounding for the legal and ethical critique developed in Chapter 6, where Spanish and European legal safeguards will be analyzed in relation to their limited effectiveness in preventing coercion. They also inform the pedagogical proposals in Chapter 5, where training in shared decision-

making, trauma-informed care, and ethical deliberation are presented as necessary preconditions for systemic reform.

3.4. Physiological causes of mental health disturbances

This section analyzes the responses on nutritional screening from the last thesis survey 3, devoted to shared decision making and rational prescription and safe deprescription. The aim is to characterize professional reasoning when the indication to review antipsychotic treatment hinges on potential somatic drivers. Table 21 presents the categorical coding of responses to two nutritional scenarios that probe the same ethical and clinical question under different degrees of etiological specificity. The first vignette addressed gluten psychosis as a bounded and testable hypothesis in need for national screenings. The second expanded the scenario proposed into a hypothetical general nutritional and metabolic screening effort to be undertaken. By comparing distributions across supportive, cautious, skeptical, systemic barrier, and deflecting stances in both cases, the table provides an empirical map of how the wider professional community positions itself with respect to proportionate screening and supervised tapering within a shared decision making framework. The analysis that follows interprets these distributions against current evidence and rights based guidance on metabolic assessment, patient involvement, and safe deprescription.

Table 21. Deprescription in gluten psychosis and other metabolic scenarios

Category	Gluten psychosis (N=60)	Gluten psychosis (%)	Nutritional/ metabolic screening (N=63)	Nutritional/ metabolic screening (%)
Supportive of dietary/nutritional screening and deprescription	10	16.7 %	10	15.9 %
Cautious advocates of patient-centered integration	15	25.0 %	6	9.5 %
Critical or skeptical of link with psychiatric illness	8	13.3 %	4	6.3 %
Systemic barriers cited as obstacles/excuses	15	25.0 %	13	20.6 %
Deflections or unrelated responses	12	20.0 %	30	47.6 %

Distribution of professional responses across categories in two survey scenarios. Data show a higher proportion of supportive or cautiously supportive answers in the gluten psychosis case compared with the broader nutritional and metabolic screening scenario, where deflections and references to systemic barriers predominate.

The coding categories used for the nutritional and metabolic scenarios distinguish five main orientations in the responses. The first category, supportive of dietary approaches and deprescription, brings together professionals who recognize the role of nutritional and metabolic disorders in generating psychiatric symptoms and who explicitly recommend systematic screening and subsequent tapering of antipsychotics when a reversible cause is identified. The second category, cautious advocates of patient-centred integration, includes respondents who endorse shared decision making, careful tapering, and extensive patient communication, often highlighting the need for gradual transitions and interdisciplinary oversight. A third category, critical or skeptical of the metabolic-psychiatric link, reflects those who either doubt the evidence for these associations or reject them as clinically marginal. A fourth category, systemic and institutional challenges, groups responses that emphasize barriers such as insufficient training, poor coordination with dietetics or primary care, lack of institutional support, and entrenched hospital-centric practices, often using these obstacles as justifications for inaction. Finally, a fifth category consists of alternative or tangential perspectives, in which respondents do not directly address the role of nutritional or metabolic factors but propose unrelated approaches, such as general statements on pharmacological rationality or the invocation of external advocacy positions.

In the case of gluten psychosis, the recoded dataset shows 25 of 60 responses in favor of deprescription in principle or practice, representing 41.7 percent. This modest proportion stands in sharp contrast to the foreseeable harms of missing reversible metabolic drivers and to the cumulative burden of long-term antipsychotic exposure. Respondents frequently cited barriers such as limited time, rigid institutional pathways, and poor coordination with nutrition services, all of which are well documented in the literature on shared decision-making in psychosis care (P. Deegan, 2006). Studies confirm that while clinicians acknowledge the importance of patient involvement, antipsychotic prescribing remains dominated by pharmacocentric habits and system inertia (Harding et al., 2024). This same profile is evident in the dataset: respondents describe stepwise diet-first trials but simultaneously express hesitation to implement them widely, illustrating the persistence of barriers that are organizational rather than evidentiary (García-Valdecasas Campelo et al., 2009).

Reluctance to deprescribe also reflects cultural and epistemic dimensions. Professional identity formation privileges pharmacological interventions, and

nutrition or metabolic screening often lies outside the perceived therapeutic canon (Longhitano et al., 2024). Once a psychiatric label is attached, alternative explanations are marginalized through diagnostic overshadowing, a dynamic that reduces the credibility of patient testimony about diet-linked fluctuations (Timimi, 2014). This epistemic injustice is reflected in the dataset, where many respondents blamed external constraints, such as institutional referral pathways or lack of dietitians, while few acknowledged their own role in failing to act. The skew toward external attribution corresponds with findings that epistemic hierarchies systematically downgrade somatic differentials once psychiatric categories are in play (Villarroel, García, et al., 2021). The result is a professional culture that protects the status quo, reinforcing inertia even when reversible causes may be present (Symonds, 2024).

From the perspective of metabolic psychiatry, such caution is difficult to defend. Antipsychotics are consistently associated with weight gain, dyslipidemia, insulin resistance, and elevated cardiovascular mortality (Hert et al., 2011). At the same time, growing evidence links gut barrier dysfunction, immune activation, and microbiota alterations to psychiatric symptoms (Kelly et al., 2015). Case reports and small series demonstrate that psychotic symptoms in individuals with non-celiac gluten sensitivity or celiac disease can resolve on strict gluten exclusion and recur with re-exposure (Lionetti et al., 2015). These findings establish individual-level reversibility, even if population prevalence remains uncertain. Accordingly, targeted screening for gluten sensitivity and supervised dietary trials are proportionate, low-risk interventions. In the dataset, professionals who favored deprescription endorsed precisely such stepped approaches: establish a gluten-free diet, then taper dopamine blockers gradually in accordance with exposure history and symptom evolution (Barch, 2023).

The ethical implications of not screening are considerable. Respondents pointed to the impossibility of direct referral from psychiatry to diagnostic services and to the lack of coordination with dietitians. These gaps reflect global critiques of coercion and rights violations in psychiatric practice, which stress the need for voluntary, person-centered, evidence-based alternatives (C. M. Palmer, 2025). Missing reversible somatic contributors not only prolongs avoidable suffering but also condemns patients to a chronic psychiatric identity, harming, and cumulative metabolic morbidity from antipsychotics. The 41.7 percent supportive stance in this dataset therefore aligns with contemporary evidence and ethical frameworks, while the 58.3 percent who remain reluctant exemplify systemic inertia and epistemic conservatism. The rarity with which respondents acknowledged their own unwillingness to deprescribe, in contrast to frequent reference to structural barriers,

underscores the depth of professional taboos and the persistence of a culture resistant to paradigm change (First, 2010).

The difference in tone between the gluten-specific responses and those addressing metabolic and nutritional disorders more broadly reflects the professionals' perception of certainty. Gluten sensitivity provides a relatively bounded and testable hypothesis, with celiac disease and non-celiac gluten sensitivity both supported by validated serological and histological markers and documented psychiatric reversibility (Yarborough et al., 2016). By contrast, the wider spectrum of metabolic and nutritional disorders introduces multiplicity: thyroid dysfunction, adrenal insufficiency, diabetes, vitamin B12 deficiency, or electrolyte imbalance. These conditions are less often screened systematically in psychiatry, despite abundant evidence that they can mimic or exacerbate psychosis or mood disorders and are treatable when identified (Blom, 2024). In the Spanish context, metabolic monitoring of patients with severe mental illness remains fragmented, with national studies showing inconsistent application of cardiovascular and endocrine screening protocols in psychiatric services (Adiba, 2019). This reality helps explain why respondents expressed greater hesitation when faced with a broad rather than a single etiology: diffuse responsibility allows for professional retreat into system-level explanations rather than active clinical decision making.

At the same time, the reality of both scenarios is aligned: in either gluten sensitivity or broader metabolic disorders, the ethical and clinical imperative is to exclude reversible causes before assigning a lifelong psychiatric label. Spanish and European guidelines already recommend systematic metabolic monitoring in patients prescribed antipsychotics, highlighting cardiometabolic risk and the need for early detection of reversible somatic contributors (Engel, 1967). The respondents' emphasis on barriers such as poor coordination with primary care or lack of dietitians confirms what has been repeatedly observed in service evaluations: although the infrastructure for such screening exists, it is poorly operationalized in practice (Greenberg, 1988). The reluctance to translate evidence into systematic action therefore reflects not a lack of knowledge but entrenched epistemic conservatism and institutional inertia. Both gluten and general scenarios converge on this same reality: screening makes clinical and ethical sense, but psychiatry's reliance on pharmacological continuity and its difficulty integrating somatic differentials into standard practice delay or prevent its implementation.

It is medically accepted that a wide range of endocrine, nutritional, and metabolic conditions can mimic or exacerbate symptoms commonly diagnosed as psychiatric disorders, as already pointed. Endocrine disorders such as hypothyroidism

including myxedema psychosis, hyperthyroidism, Cushing syndrome, Addison disease, and diabetes dysglycemia can all precipitate or worsen psychotic and affective symptoms, with well-documented remission following correction of the underlying pathology (Blom, 2024). Nutritional deficiencies, particularly of vitamin B12, folate, thiamine, and niacin, are associated with neuropsychiatric manifestations ranging from psychosis to cognitive decline and affective disturbances, with demonstrated resolution after appropriate supplementation (C. M. Palmer, 2025). Electrolyte disturbances, hepatic encephalopathy, renal failure, and porphyria can similarly present with acute or subacute psychosis, reversible when treated (Longhitano et al., 2024). Autoimmune and inflammatory conditions, including autoimmune thyroiditis and anti-NMDA receptor encephalitis, further exemplify the overlap between immunological dysfunction and psychiatric symptoms (Dantzer, 2018). Finally, celiac disease and non-celiac gluten sensitivity have been repeatedly implicated in reversible psychotic presentations, with symptom resolution on gluten exclusion (Carloni & Rescigno, 2023). Beyond this, emerging metabolic interventions such as ketogenic dietary therapy have demonstrated preliminary efficacy in improving both symptom burden and metabolic outcomes in schizophrenia spectrum disorders (Bower & Kuhlman, 2023). The state of the art is therefore clear: metabolic and nutritional causes of psychiatric syndromes are numerous, well documented, and often reversible, imposing a duty to screen proportionately and to revise pharmacological treatment accordingly.

Best practice in this scenario follows directly from the evidence. First, implement proportionate screening algorithms for endocrine, nutritional, electrolyte, hepatic, renal, autoimmune, and gastrointestinal conditions with known neuropsychiatric expression, prioritizing tests with high clinical yield and reversibility potential in the presenting phenotype, and repeating assessments when clinical course suggests somatic drivers despite prior normal tests (Alfonso & Ranganathan, 2025). Second, when a reversible driver is identified and treated, reassess the indication and dose of antipsychotics, and if functional improvement is robust, initiate supervised deprescription using slow, hyperbolic tapering schedules with shared decision making, explicit relapse prevention plans, and cross-disciplinary monitoring, including primary care, endocrinology, nutrition, and mental health services as appropriate (Toljan, 2023). Third, audit metabolic outcomes and treatment exposure as a matter of routine quality and safety, recognizing that cardiometabolic risk is a primary contributor to excess mortality and that mitigation is a core element of beneficence in psychosis care, not an optional extra (Shalev et al., 2009). These steps are compatible with rights-based service guidance, are proportionate to risk,

and directly address the failures identified in the responses, which cluster around deflection, passive skepticism, and reliance on systemic barriers rather than concrete clinical actions supported by current knowledge (F. Bottaccioli & Bottaccioli, 2025).

The general scenario dataset shows a marked retreat from action compared with the gluten-specific set. Only 25.4 percent of respondents endorse deprescription in principle or practice, with 15.9 percent clearly supportive and 9.5 percent cautiously supportive, while 47.6 percent default to deflections or unrelated responses and 20.6 percent cite systemic barriers as obstacles or excuses. The contrast with the gluten-specific scenario is revealing, as there professionals were more prepared to consider deprescription when a narrowly defined and biologically plausible pathway was presented. In the broader nutritional and metabolic frame, their willingness diminishes markedly, suggesting that the less bounded the etiology, the greater the institutional impulse to retreat to default prescribing. The language in the latter two categories seldom acknowledges individual professional agency to screen for reversible medical conditions or to plan tapering when a somatic driver is demonstrated. Instead, the responsibility is shifted outward, to systems, guidelines, or diffuse epistemic uncertainty, leaving the patient within the orbit of long-term pharmacological exposure (;).

Supportive responses were often concrete, aligning with best evidence on screening and deprescription. One respondent wrote, “Deberían diseñarse protocolos conjuntos entre ambos niveles asistenciales. Se debe atender primero al tratamiento de las deficiencias metabólicas y una vez corregidas, empezar cuanto antes los procedimientos de deprescripción” (“Joint protocols ought to be designed between both levels of care. Metabolic deficiencies ought to be treated first and, once corrected, deprescription procedures ought to begin as soon as possible”), reflecting recognition that metabolic correction should precede antipsychotic deprescription, a sequence supported in the clinical literature (A. G. Bottaccioli et al., 2019). Others emphasized patient-centred approaches: “Explicando detenidamente y de forma sencilla al paciente el escenario pasado y el actual a la luz de los nuevos hallazgos, con la connotación positiva del escenario actual. Retirando la medicación de forma lenta y paulatina y con frecuentes entrevistas para ver evolución” (“Explaining carefully and simply to the patient the past and current scenario in light of the new findings, with the positive connotation of the current scenario. Withdrawing the medication slowly and gradually, with frequent interviews to observe progress”). This statement echoes contemporary tapering guidance advocating gradual hyperbolic reduction, frequent clinical contact, and transparent communication (van Os & Groot, 2023). Another practical perspective highlighted safety:

“Inicialmente corregir el trastorno metabólico en coordinación con atención primaria. Una vez resuelto proceder a deprescripción gradual. Con supervisión estrecha de la evolución del paciente” (“Initially correct the metabolic disorder in coordination with primary care. Once resolved, proceed to gradual deprescription, with close supervision of the patient’s progress”). These responses converge with best practice as defined in international recommendations for rights-based, evidence-informed mental health care (Gøtzsche, 2022).

Cautious responses also acknowledged patient involvement but revealed a more ambivalent stance. One noted, “La retirada debería llevarse a cabo siguiendo las pautas correctas, para evitar perjuicios. Cabría implementar cribados en atención primaria en coordinación con la psiquiatría para descartar la existencia de tales problemas nutricionales como causa subyacente del trastorno” (“Withdrawal ought to be carried out following the correct guidelines, to avoid harm. Screenings could be implemented in primary care in coordination with psychiatry to rule out the existence of such nutritional problems as an underlying cause of the disorder”). Another stated, “La retirada debe ser progresiva lenta en condiciones ambulatorias y con colaboración de atención primaria. O rápida y hospitalizado en caso de efectos secundarios graves” (“Withdrawal ought to be slow and progressive in outpatient conditions and with collaboration from primary care. Or rapid and in hospital in the case of severe side effects”). While these highlight caution and risk management, they nonetheless recognize deprescription as a rational step when somatic contributors are confirmed (Serrano, 2020), consistent with evidence that safe tapering requires structured plans and flexibility depending on patient risk profiles (Dougherty, 2021).

By contrast, deflecting responses often dismissed or reinterpreted the scenario. One asserted, “Sigo sin entender que haya un esfuerzo nacional de deprescripción... hay un esfuerzo científico, médico, compartido con la decisión del paciente... y en todos los actos médicos que impliquen uso de fármacos” (“I still do not understand why there ought to be a national effort on deprescription... there is a scientific, medical effort, shared with the patient’s decision... and in all medical acts that involve the use of drugs”). Another generalized, “El término ‘mal diagnosticados’ me parece erróneo. Si nos guiamos por la realidad clínica, todo está mal diagnosticado. Los diagnósticos psiquiátricos carecen de validez interna y externa” (“The term ‘misdiagnosed’ seems wrong to me. If we go by clinical reality, everything is misdiagnosed. Psychiatric diagnoses lack internal and external validity”). These positions divert attention from the specific issue of metabolic misdiagnosis and deprescription, reframing the debate into a broader philosophical or clinical uncertainty. Such responses exemplify the professional culture described

in the literature as epistemic conservatism and diagnostic overshadowing, where the psychiatric label takes precedence and obscures somatic contributors (S. D. Brown & Tucker, 2010). This avoidance sustains the default of pharmacological continuity, even as evidence for metabolic and nutritional drivers is well established and reversible (Leon et al., 1987).

Systemic barrier responses provided further illustration of this displacement of responsibility. For example, one clinician remarked, “Ha de mejorar mucho la coordinación con primaria, siguen con el estigma” (“Coordination with primary care must improve a lot, they still stigmatize”). Another insisted, “La coordinación es fundamental, así como generar un sistema de continuidad de cuidados dónde los profesionales implicados abordan de manera conjunta y con el paciente el plan de retirada de neurolépticos” (“Coordination is fundamental, as is generating a system of continuity of care where the professionals involved jointly address, together with the patient, the plan for withdrawing neuroleptics”). While highlighting real gaps in resources and integration, these statements operate as explanations for inaction. The literature on shared decision making in psychiatry has repeatedly identified system-level barriers such as time constraints, poor referral pathways, and lack of training, but also shows that these are compounded by clinicians’ reluctance to cede epistemic authority (Sampson, 2021). In this light, systemic barriers function not only as descriptions of challenges but as excuses that justify maintaining the status quo.

The net effect of these discursive patterns is a culture that protects long-term pharmacological exposure despite growing recognition of its harms. Antipsychotics predictably cause metabolic dysfunction, contributing to excess premature mortality (Gallego Antonio et al., 2016). Yet respondents often avoided engaging with this evidence directly, focusing instead on the impossibility of systemic reform or questioning the framing of deprescription itself. The few who described coordinated screening, correction of somatic causes, and structured tapering embody the pathway defined by evidence and international guidance. The majority, however, demonstrate how epistemic conservatism and institutional inertia dominate professional discourse, to the detriment of patients whose symptoms may be reversible and whose exposure to psychotropics is thereby unnecessary and harmful (Rudzki & Maes, 2021).

Awareness of cutting-edge metabolic interventions is limited in the responses. Contemporary metabolic psychiatry spans gut barrier physiology, immune activation, and energetic modulation with dietary therapies (Moncrieff & Cohen, 2009). Case series and controlled studies of ketogenic interventions report

improvements in symptomatology and metabolic parameters in schizophrenia (Longhitano et al., 2024). Yet the dataset scarcely references such advances. Instead, many respondents attributed their reluctance to system-level deficiencies or expressed generic caution, without referencing or incorporating known metabolic science. This stands in contrast to the research record, which now includes systematic reviews of antipsychotic metabolic risks, tapering guidance adapted for psychotropics (M. Horowitz & Taylor, 2024), and international recommendations on human rights and evidence-based alternatives (Torrents & Björkdahl, 2024). The predominance of deflecting or system-blaming responses signals a knowledge-to-practice gap with direct consequences for patient safety and long-term health.

Even when the psychiatric diagnosis remains appropriate, nutritional and endocrine assessment is clinically relevant. Vitamin B12 deficiency has been repeatedly linked to psychosis-like presentations with full remission upon supplementation (Bienassis et al., 2022). Hypothyroid-related psychosis has similarly been documented to resolve with thyroid replacement, often requiring little or no antipsychotic treatment (Pescatori, 2024). These conditions are not highly prevalent, but the feasibility and low cost of screening render omission unethical, given the life-altering consequences of psychiatric misdiagnosis and the high risk of iatrogenic morbidity from unnecessary long-term psychopharmacology (Dickinson, 2014). The dataset suggests that professionals often externalize the problem, citing lack of resources, referral pathways, or service models, rather than accepting responsibility to request and act upon such screening. This tendency mirrors diagnostic overshadowing, where psychiatric labels obscure physical contributors and patients' testimony, leading to persistent epistemic injustice (Bueter, 2023).

The implications for patients are severe. Antipsychotic exposure carries predictable risks of weight gain, insulin resistance, dyslipidemia, and cardiovascular disease (Dencker et al., 1986). These predictable harms meaningfully contribute to excess premature mortality in severe mental illness populations (Healy, 2012). Routine monitoring and mitigation remain uneven, and patients who are misdiagnosed or who remain on medication after resolution of metabolic or nutritional drivers are doubly harmed: first, through failure to treat the actual condition, and second, through iatrogenic complications of unnecessary pharmacotherapy. Respondents who advocated structured tapering, shared decision making, and joint protocols align with best practice. Those who deflected or cited barriers without proposing action illustrate a persistent culture of inertia. The contrast demonstrates that while the evidence base is clear, psychiatry's day-to-day practice continues to rely heavily on prescribing for life, often without sufficient dialogue, informed consent, or medical screening (Fisher, 2012). The dataset portrays a field poised between

evidence and institutional conservatism, with change possible only if metabolic psychiatry and deprescription science are systematically integrated into routine care.

The divergence between gluten-specific and broader nutritional scenarios' responses reflects systemic and cultural factors that have long constrained psychiatric practice in Spain. National studies have shown that metabolic monitoring in patients with severe mental illness is inconsistent, with studies documenting high prevalence of cardiovascular and metabolic risk factors in schizophrenia alongside insufficient screening (Sampietro et al., 2023a) and later work confirming that fewer than half of services regularly assess glucose or lipid profiles (Giosa, 2025). Coordination with primary care is often weak and the implementation of nutritional or endocrine assessments remains fragmented despite guideline recommendations from the Sociedad Española de Psiquiatría y Salud Mental and the European Psychiatric Association (Ortiz-Lobo et al., 2017). Professionals themselves identify poor integration, dehumanization and outcasting, and limited training as barriers, but these obstacles also serve as justifications for inaction, displacing responsibility from individual clinical agency. Cultural patterns reinforce this inertia, as Spanish psychiatry remains strongly pharmacocentric, hospital based, and reluctant to normalize shared decision making in medication management (García-Alonso et al., 2019). The responses in both datasets reproduce this context: while some clinicians propose structured protocols and patient centred deprescription, many others deflect, generalize, or cite systemic barriers, exemplifying the persistence of epistemic conservatism and diagnostic overshadowing that hinder translation of metabolic psychiatry into everyday care.

The comparison of the two datasets demonstrates distinct patterns in professional reasoning when asked about deprescribing neuroleptics in the context of reversible somatic disorders. In the gluten-specific case 41.7 percent of respondents supported deprescription either directly or cautiously, while in the broader metabolic and nutritional screening dataset this proportion fell to 25.4 percent. The decline in cautious support is notable, decreasing from one quarter of all answers in the gluten dataset to less than ten percent in the broader context. This suggests that professionals are more willing to consider deprescription when confronted with a clearly defined and well documented causal pathway such as gluten sensitivity, but are much more hesitant when asked to engage with the broader category of nutritional and metabolic disorders. This hesitancy occurs despite abundant evidence that endocrine dysfunction, vitamin deficiencies, and metabolic disturbances can present with psychiatric symptoms that are often indistinguishable from primary psychotic or mood disorders and that can remit with appropriate treatment of the underlying condition (Shaffer et al., 2014). The reduced willingness

to endorse deprescription at scale therefore reflects not a lack of evidence but a culture of caution and avoidance in psychiatry when responsibility extends to systematic medical screening.

The category labeled as deflections or unrelated responses was the most common in the broader dataset, accounting for nearly half of all answers compared with one fifth in the gluten-specific dataset. Many of these comments took the form of general statements about pharmacological rationality or references to other classes of drugs, effectively sidestepping the central issue of metabolic misdiagnosis. This pattern reflects what sociologists of psychiatry describe as the defense of epistemic boundaries, where challenges to the primacy of pharmacological treatment are neutralized by displacement or avoidance (Runciman, 2023). Despite a growing body of literature showing that reversible somatic conditions such as hypothyroidism, Cushing syndrome, or vitamin B12 deficiency can mimic psychiatric disorders and are frequently misdiagnosed as schizophrenia or affective illness (Pescatori, 2024), professionals still often evade the implications of these findings. This tendency demonstrates that clinical practice is governed not only by evidence but by cultural scripts and taboos that limit the incorporation of metabolic psychiatry into mainstream care.

Systemic barriers cited as obstacles or excuses were mentioned with similar frequency across both datasets, comprising 25.0 percent of responses in the gluten case and 20.6 percent in the broader screening context. These comments emphasized lack of integration with dietetics, scarcity of time and resources, and poorly defined referral pathways. Such barriers are real and extensively documented in the literature on shared decision making and deprescribing in psychiatry (Speyer et al., 2025). However, they are rarely accompanied by acknowledgment of clinicians' own reluctance or unwillingness to screen for metabolic causes and taper medications. This displacement of responsibility from professional agency to structural limitations is consistent with studies of institutional inertia in mental health systems, where organizational practices and incentive structures protect the pharmacological status quo (Vian et al., 2022). In the present data it appears that systemic explanations are deployed not only as descriptions of real obstacles but also as justifications that allow clinicians to avoid confronting the ethical imperative to investigate reversible etiologies before committing patients to life-long psychiatric treatment.

Skepticism is less visible in the broader screening dataset, where only 6.3 percent expressed doubt about metabolic causes of psychiatric illness, compared with 13.3 percent in the gluten-specific dataset. At first this appears paradoxical, since broader

categories of metabolic disturbance might invite more questioning. Instead, many respondents who were skeptical in the gluten dataset shifted to deflections in the metabolic one, avoiding direct critique while simultaneously refusing to endorse action. This subtle form of resistance is consistent with ethnographic accounts of psychiatry's institutional conservatism, where ambiguity is preferred over open dissent when prevailing practices are challenged (Argyris, 1990). The broader implication is that despite strong evidence linking endocrine, nutritional, and metabolic disorders to psychiatric symptoms and outcomes (Arkes & Blumer, 1985), many professionals retreat from endorsing systematic screening or deprescription. The consequence is severe: patients with reversible disorders risk lifelong misclassification as mentally ill, unnecessary exposure to antipsychotics, and cumulative cardiometabolic harm. The comparative data show that the taboos of psychiatry, its reliance on systemic excuses, and its avoidance of direct accountability remain decisive in shaping professional discourse even when evidence supports safe deprescription.

The broader Spanish context highlights why responses in both nutritional and gluten-related scenarios follow predictable patterns of inertia and deflection. Peer reviewed research has repeatedly documented how psychiatric labels generate long term iatrogenic consequences, from diagnostic overshadowing to the erosion of patient credibility in reporting somatic symptoms, which in turn delays the detection of reversible medical contributors (Ortiz Lobo & Ibáñez Rojo, 2011). Studies also show that psychiatric practice in Spain continues to be highly pharmacocentric and hospital based, with inconsistent implementation of metabolic monitoring and persistent gaps in coordination with primary care (Sharpe & Faden, 1998). At the same time, research on open dialogue demonstrates both the feasibility of dialogic alternatives and the barriers to their adoption, including limited training opportunities, weak institutional support, and lack of integration into public services (Seikkula, 2025). These findings converge with evidence that shared decision making remains aspirational rather than routine, with studies noting that while clinicians endorse collaboration in principle, actual prescribing practice rarely incorporates patient preference or systematic deprescription protocols (;).

The responses analyzed in both surveys thus reflect entrenched systemic and epistemic conservatism: users and survivors highlight the harms of coercive care and rigid labels, while professionals oscillate between cautious proposals for change and broad references to structural limitations. International and Spanish research confirm that these patterns are sustained by institutional inertia, insufficient training, and the persistence of taboos around deprescription and metabolic

screening (Rogers & Pilgrim, 1993). In this context, the scenarios serve to illustrate the distance between evidence and practice: where the literature points to the necessity of proportionate screening, rights based dialogue, and safe tapering strategies, routine clinical practice remains dominated by pharmacological continuity and cultural scripts that resist reform. This gap underscores a paradox: although the scientific consensus acknowledges the role of metabolic and nutritional factors in psychiatric presentations, clinical cultures continue to relegate these dimensions to the margins of care. The consequence is not only avoidable iatrogenic harm but also the perpetuation of a chronic psychiatric identity where recovery could have been possible. Bringing deprescription and nutritional assessment into the centre of Spanish mental health practice therefore emerges as both a clinical obligation and an ethical imperative. It is against this backdrop that the comparison of gluten specific and broader nutritional scenarios gains significance, as it demonstrates how these systemic and cultural barriers shape professional reasoning in different but related contexts.

3.5. Marginalization and abuses as direct cause of mental health distress

Psychiatric care in Spain remains strongly pharmacocentric, with insufficient coordination between hospital-based psychiatry, primary care, and social services, and with systematic neglect of psychosocial alternatives and human rights frameworks (Dulin & Szasz, 1970). National studies document the overuse of antipsychotics in elderly care homes, in minors with behavioural difficulties, and in marginalized populations, often for purposes of behavioural control rather than evidence-based psychiatric indication (Pina-Camacho et al., 2021). International reviews confirm that neuroleptics are routinely prescribed in cases of intellectual disability, dementia, trauma, and social exclusion, with high risks of cardiometabolic morbidity and premature mortality (R. Jenkins & Meltzer, 1995).

Professional responses to these scenarios reveal the same inertia: systemic barriers, institutional deflection, and epistemic conservatism dominate discourse, even where evidence strongly favours safe deprescription, trauma-informed interventions, and social policy integration (Aarons et al., 2011). These scenarios thus illustrate how entrenched prescribing patterns persist in contexts where rights-based alternatives are both available and necessary, showing psychiatry's difficulty in moving beyond pharmacological continuity toward holistic, community-based care (J. Read & Sanders, 2010).

Table 22. Professional responses across four abuse and marginalization scenarios

Category	Marginaliz ed populations (N=49)	%	Abuse and iatrogeny (N=57)	%	Children & adolescents (N=63)	%	Elderly care high-risk deprescribin g (N=66)	%
Supportive of deprescription and rights-based alternatives	18	36.7	24	42.1	26	41.3	28	42.4
Cautious advocates of patient-centred integration	11	22.4	10	17.5	14	22.2	14	21.2
Critical or skeptical of overprescription framing	5	10.2	7	12.3	6	9.5	8	12.1
Systemic barriers cited as obstacles or excuses	11	22.4	10	17.5	9	14.3	10	15.2
Deflections or unrelated responses	4	8.2	6	10.5	8	12.7	6	9.1

Distribution of responses by category. Percentages are within-scenario, on the responses gathered in the third survey's questions about screenings, shared decision making, deprescription and other systemic changes required in cases of abuses and marginalization.

The revised coding categories in these four social-care scenarios delineate distinct orientations among professionals toward the overuse of neuroleptics in vulnerable groups. The first group, those supportive of deprescription and rights-based alternatives, comprises respondents who explicitly recommend tapering antipsychotics and replacing them with psychosocial, community, and rights-based measures, often highlighting the need for trauma-informed care, social investment, and holistic responses. A second group, cautious advocates of patient-centred integration, accepts the principle of deprescription but stresses prudence, shared decision making, and gradual transition, frequently emphasizing developmental fragility in children, cognitive decline in the elderly, or the complexity of social determinants in marginalized communities (Seng et al., 2012). A third group remains critical or skeptical, questioning whether overprescription is the problem or underscoring the continued utility of antipsychotics compared with other drugs, thereby reframing the issue as one of rational use rather than reduction. A fourth set of responses invokes systemic and institutional challenges, citing understaffed residences, poorly integrated community networks, lack of training, and insufficient alternatives as obstacles that justify limited action. Finally, a smaller cluster consists of deflections or unrelated contributions, which either avoid engaging with the

posed scenario or displace the discussion onto external themes such as generic policy frameworks or advocacy references.

The four scenarios analysed here, masked abuse and institutional violence, the pathologisation of marginalised populations, the overuse of neuroleptics in children and adolescents, and their continued use in older adults, constitute not isolated phenomena but a continuum of abuse that extends across the life cycle. From childhood to old age, the most vulnerable populations are subjected to practices that silence, constrain, and pathologise their lived realities, transforming social suffering, trauma, and structural disadvantage into alleged psychiatric disorders. These practices are sustained not by robust evidence but by institutional inertia, epistemic conservatism, and professional cultures that privilege pharmacological control over rights-based, dialogic, and restorative approaches. The responses to these scenarios demonstrate how professional discourse often normalises coercion and deflects responsibility, while survivors and critical voices highlight the harms inflicted and call for systemic reform. Scientific literature corroborates that these are not marginal problems but systemic patterns of epistemic and structural violence embedded in psychiatric care, documented in Spain and internationally (Álvarez Ramírez, 2023).

In the case of masked abuse and institutional violence, psychiatric diagnosis functions to obscure the sequelae of trauma, reframing them as endogenous pathology and legitimising long-term pharmacological containment. Neuroleptics are used not to heal but to impose silence, impairing speech, mobility, and resistance, thereby compounding the effects of interpersonal or institutional violence with new layers of harm (P. Farmer, 2004). Several responses to this scenario explicitly acknowledged the need for recognition and repair, with one stating that it is essential to begin “*pidiendo disculpas y comprendiendo su dolor. Y de manera conjunta ver cómo se les puede acompañar en este nuevo proceso,*” while others demanded official apologies and even a truth and reconciliation process akin to that of South Africa after apartheid. Such appeals align with scholarship that highlights the moral injury experienced by survivors of coercive psychiatry and the importance of restorative justice mechanisms for rebuilding trust in care institutions (J. L. Price et al., 2022).

At the same time, many professionals deflected responsibility by framing abuse as either exceptional or historical, or by invoking systemic constraints such as resource scarcity and staff turnover. One respondent suggested that “*es un tema complejo en el que el daño ocasionado no se puede revertir. Se trata de dar la mejor atención posible teniendo en cuenta el trauma y los daños sufridos previamente,*” which

acknowledges harm yet subtly avoids direct responsibility for its perpetuation. This resonates with Bueter's (Bueter, 2019) analysis of epistemic injustice in psychiatry, whereby the testimony of survivors is systematically downgraded or reinterpreted through professional categories that neutralise its critical force. Spanish investigations corroborate these accounts: the *Defensor del Pueblo* (Anguita, 2005) has repeatedly documented degrading practices such as mechanical and chemical restraints, while the *Fiscalía General del Estado* (Sánchez Ballesteros, 2023) has issued specific instructions to curb the abusive use of sedation in psychiatric wards. Yet compliance remains patchy, reflecting what scholars describe as the normalisation of coercion in Spanish psychiatric culture (Aragónés-Calleja & Sánchez-Martínez, 2023).

The survey responses also reflected a struggle to reconcile professional self-image with the recognition of harm. Some respondents insisted that they had “nunca vivido, ni en la actualidad ni en mi periodo hospitalario, casos de trauma o mal tratamiento del paciente,” a claim directly at odds with both survivor testimony and national monitoring data. Such denials illustrate the epistemic conservatism of psychiatry, in which critique is displaced by appeals to professional authority, reinforcing diagnostic overshadowing and maintaining pharmacological control as the default intervention (T. Steinert, 2017). Others attempted to narrow the scope of shared decision making, asserting that it should only be applied “en pacientes con buen insight o conciencia de trastorno adecuada,” thereby excluding many of those who have suffered the most harm from the very processes meant to protect them. This selective application reflects wider patterns in Spanish psychiatry, where shared decision making is endorsed rhetorically but seldom applied in contexts of coercion, trauma, or contested diagnosis (Eads et al., 2021).

The international literature underscores that these patterns are not uniquely Spanish but part of a broader culture of denial and coercion in psychiatry. Studies across Europe and North America have documented high rates of restraint, seclusion, and forced medication, with survivors reporting re-traumatisation and loss of trust in services (Molodynski et al., 2010). The WHO QualityRights framework explicitly identifies the overuse of coercive practices as violations of the UN Convention on the Rights of Persons with Disabilities, calling for their abolition and replacement with rights-based, dialogic, and community-grounded approaches (Slade, 2017). The contrast between these standards and the survey responses highlights the gap between what is scientifically and ethically indicated and what is enacted in everyday psychiatric practice.

What emerges, therefore, is a continuum of silencing mechanisms: diagnosis obscures trauma, neuroleptics impose chemical silence, and professional discourse deflects responsibility. Together, these dynamics ensure that survivors of abuse are denied justice, their testimony reclassified as symptom, and their suffering compounded by treatments that serve institutional stability rather than human dignity. The scenario responses, situated within the Spanish and international literature, demonstrate that psychiatric care as usual does not merely fail to prevent abuse but actively participates in its perpetuation by legitimising violence as treatment and by systematically silencing those who demand recognition and repair. This structural complicity becomes even more evident when psychiatry is instrumentalised by abusive families, institutions, or collectives that mobilise its coercive and forceful nature to reinforce their own power. In such contexts, psychiatric intervention functions not as protection but as a tool of impunity, providing a medicalised alibi for ongoing violence, neglect, and exploitation. Victims of domestic abuse, intimate partner violence, and institutional maltreatment are pathologised as mentally ill, their accounts dismissed as delusional or symptomatic, while perpetrators consolidate control under the cover of medical authority (Fischbach & Herbert, 1997). This practice, known as diagnostic overshadowing, allows abusers to extend harm through the apparatus of psychiatry itself, creating conditions of dependency and vulnerability that mirror the original violence but now with state sanction (Hoffman, 2011).

Spanish research has repeatedly documented how coercive psychiatry is exploited by family members or guardians to neutralise victims of gender violence or intergenerational abuse (Ussher, 2023), with psychiatric admission or forced medication serving as extensions of social control rather than therapeutic necessity (Aragónés-Calleja & Sánchez-Martínez, 2022). Studies of intimate partner violence survivors in Spain reveal systematic misdiagnosis of trauma responses as psychotic or affective disorders, leading to chronic medication regimes that obscure the original abuse and deny victims access to justice or effective care (Estroff & Gold, 2018). International evidence confirms that this is not an isolated pattern: survivors of domestic violence and childhood trauma are frequently subjected to psychiatric interventions that disregard the causal role of violence, thereby medicalising victimisation and entrenching cycles of dependence (Sanders, 2016). The pharmacological silencing of victims is particularly pernicious because neuroleptics not only mute speech but impair cognition, emotional responsiveness, and motor capacity, reducing the ability of victims to testify, to seek legal protection, or even to resist ongoing harm (Bezanilla et al., 2016).

This epistemic and material silencing aligns with what Foucault (Foucault, 2003) described as the psychiatric gaze: the reconfiguration of social deviance or suffering into medical categories that serve institutional and political order rather than individual relief. In practice, psychiatry provides abusers with a powerful instrument: once a victim is labelled, their credibility is structurally undermined, their rights curtailed, and their dependency on the very systems that harm them reinforced (Kuhl, 2019). The scenario responses illustrate the awareness of some professionals of this perverse dynamic, calling for restorative justice, patient advocacy, and rights-based frameworks, yet the prevalence of deflective or evasive answers shows the extent of resistance to confronting psychiatry's complicity. What remains clear is that psychiatry as currently practiced not only fails to protect victims of abuse but, by providing a legitimised framework for silencing and incapacitation, actively sustains environments where perpetrators can operate with impunity, shielded by diagnostic authority and pharmacological coercion.

Marginalised populations face a parallel pattern of abuse, whereby structural disadvantage is reframed as mental illness and treated with neuroleptics in the absence of adequate social support (Appignanesi, 2011). Migrants, Roma communities, LGBTI+ individuals, women, people with disabilities, and those living in poverty are systematically over-represented in coercive interventions and compulsory admissions in Spain, reflecting systemic bias rather than clinical necessity (Petreca, 2025). Respondents noted repeatedly that “en muchos casos, lo que necesita esta persona es una casa, tener un trabajo, o alguna ayuda económica. El medicamento solo silencia todo el malestar causado por las ausencia de políticas inclusivas, y la sociedad lo que quiere es no escuchar ese ruido incómodo,” underscoring the substitution of pharmacological silencing for social protection. This structural dynamic exemplifies how psychiatry functions as an alibi for neglect: suffering that arises from discrimination, exclusion, or violence is converted into an individualised psychiatric diagnosis, legitimising long-term drugging while leaving structural injustices untouched (Fricker, 2007).

International guidelines, such as the National Institute for Health and Care Excellence (NICE) framework for people with learning disabilities, explicitly require functional assessment, psychosocial interventions, and environmental measures before any pharmacological intervention (Leng et al., 2008). Yet Spanish studies confirm widespread overuse of neuroleptics in institutional care, particularly among people with intellectual disabilities and elderly populations, frequently in the absence of behavioural assessment or alternative supports (Longden & Read, 2016). One respondent framed the situation starkly: “Se trata de dar apoyo y atención por lo tanto es un factor humano y no farmacológico, se debe invertir mucho más en

educación y sanidad” (“We ought to support the human factor, not simply drugging, but investing in education and health”). These comments highlight a pervasive recognition among practitioners that the social determinants of distress, poverty, dehumanization and outcasting, lack of housing, structural racism, are neither acknowledged nor addressed within psychiatric care, which instead persists in pathologising the consequences of marginalisation (Marmot & Bell, 2019).

The epistemic violence in this process is profound. As Bueter (Bueter, 2023) has shown, diagnostic overshadowing denies credibility to patient testimony and reclassifies experiences of violence or exclusion as symptoms of disorder. Spanish evidence illustrates how Roma communities and migrants are disproportionately targeted for coercive measures, with higher rates of compulsory admissions and forced treatments, echoing historical patterns of exclusion (Miranda et al., 2019). For women, particularly survivors of gender violence, psychiatric labelling compounds their vulnerability, rendering accounts of abuse less credible and exposing them to long-term pharmacological control (Minkowitz, 2024). For LGBTI+ individuals, marginalization stress are often reframed as psychiatric pathology rather than recognised as consequences of discrimination (Castañeda Alvarez & Poma Yaro, 2017). For people with disabilities, over-prescription is routine in residential care, where medication becomes a mechanism of behavioural management, sometimes in the absence of any psychiatric diagnosis (McHugh, 2004). These practices are not aberrations but constitute a systematic pattern of denying the structural causes of distress and enforcing compliance through chemical control.

The respondents themselves drew attention to this continuum, stressing that “intervenciones integrales sociales-sanitarias-educativas; enfoque holístico y multidisciplinar” are necessary, while others noted that “el medicamento solo silencia todo el malestar” or that “no se trata de dar un bandazo, sino de atender en un sentido humano y amplio.” Yet these acknowledgements were often accompanied by references to systemic constraints or resigned acceptance of the pharmacological status quo. Such dynamics reveal how psychiatry, far from functioning as a buffer against exclusion, actively reproduces it through the pathologisation of vulnerability and the denial of structural violence (P. Farmer, 2004).

While many respondents articulated critical perspectives that emphasised the structural and rights-based dimensions of overprescription, a notable minority defended the continued use, and in some cases the necessity, of neuroleptics for populations marked by vulnerability, exclusion, or histories of abuse. These

responses often foregrounded the alleged benefits of sedation for behavioural management, the protection of families or institutions from disruptive conduct, and the presumed incapacity of patients to participate meaningfully in treatment decisions. One professional wrote that “creo que los beneficios de los antipsicóticos versus las benzodiacepinas en esta población es fundamental; menos riesgo de tráfico, menos riesgo de dependencia y de abuso que las benzos, y mejor ayuda en lo conductual.” Another emphasised that “la opinión del médico debe prevalecer si el paciente tiene enfermedad mental grave y pobre capacidad de decisión.” These positions exemplify the persistence of a paternalistic model of psychiatry in which coercion is legitimised as therapeutic and the patient’s dissent is reinterpreted as symptomatology (Trebilcock & Elliott, 2001).

The survey also revealed a recurring narrative that psychopharmacology represents the only available tool in settings of systemic neglect. Several respondents noted that in institutions for older people or for those with intellectual disabilities, “si existe una toma compartida de decisiones y una deprescripción adecuada se supone que ya la atención y el apoyo existiría, por lo que vuelvo a no entender qué es lo que se está queriendo preguntar.” Others stressed that “no se puede pensar tener que obligar retirar un medicamento sin desarrollar primero los otros programas o intervenciones, debido a que dejaría en mayor vulnerabilidad a cualquier población.” This reasoning mirrors findings in the Spanish literature, which documents how chronic underfunding of psychosocial services and workforce shortages have entrenched reliance on neuroleptics as *chemical straitjackets* to manage distress in lieu of adequate community resources (Leiva-Peña et al., 2021). International studies similarly report that clinicians in under-resourced environments often justify coercive medication practices as pragmatic responses to the lack of viable alternatives (Council of Europe, 2021a). In scenarios of abuse and trauma, some respondents denied the prevalence of mistreatment. These comments resonate with the phenomenon of epistemic denial, where psychiatric institutions minimise or reframe patient reports of harm as unreliable, a pattern documented in both Spanish and international literature (Johansson-Everyday, 2023). Defenses of neuroleptic use in these contexts are often couched in clinical paternalism, suggesting that pharmacological continuity protects against relapse or behavioural crisis, thereby masking the iatrogenic harm of chronic sedation.

The common thread across these positions is the reassertion of psychiatry’s authority to define both the problem and its solution. Structural deprivation, violence, trauma, or social exclusion are recoded as psychiatric disorder, and the prescription of neuroleptics is defended as a necessary containment strategy. This defense persists despite abundant evidence of harm and despite the existence of rights-based

alternatives emphasised by the World Health Organization and by Spanish advocacy networks (Revuelta et al., 2021). In practice, it reflects a conservative professional culture that privileges institutional order and pharmacological continuity over social investment, dialogue, and accountability. The proximity of these practices to eugenic logics is striking. By medicalising social suffering and attributing it to presumed genetic or biological inferiority, psychiatry provides a veneer of scientific legitimacy to structural discrimination. Scholars have warned that this dynamic echoes older forms of state-backed racism and authoritarianism, where institutions selectively silenced, confined, or sterilised populations deemed unfit or deviant (Thompson & Neville, 1999). The systemic over-representation of marginalised groups in coercive psychiatric practices in Spain and elsewhere thus reflects not only clinical inertia but also a continuity of disciplinary power over those structurally disempowered. What emerges from the scenario responses is an awareness among some professionals of these dynamics, yet the persistence of pharmacological over-reliance and systemic denial indicates that psychiatry continues to function less as a site of care than as a mechanism of social control and institutional complicity in structural violence.

The over-prescription of neuroleptics in children and adolescents epitomises how vulnerability is pathologised at the earliest stages of life, and this is not an isolated phenomenon. In Spain, recent reports show that psychotropic prescribing in paediatric populations has risen steadily, with antipsychotics often used off-label for behavioural control rather than for severe psychiatric disorders (Williams & Etkins, 2021). This trend parallels evidence from other European countries, where prescriptions of second-generation antipsychotics for minors have increased markedly despite their well-documented metabolic, cardiovascular, and neurological risks (Chesney et al., 2014). Internationally, studies conducted in North America, Latin America, and Asia confirm a global escalation in paediatric antipsychotic use, often associated with socioeconomic deprivation, foster care status, or systemic reliance on pharmacological containment in under-resourced services (First, 2010).

The European Medicines Agency and the World Health Organization have repeatedly cautioned against such practices, highlighting the absence of robust evidence for long-term safety and efficacy in young populations and stressing the need for psychosocial and educational interventions as first-line measures (United Nations, 2017). Nevertheless, over-medicalisation persists, reinforced by institutional incentives, limited psychosocial resources, and a prevailing culture of risk aversion in schools and paediatric care. In Spain, this has been compounded by underinvestment in child and adolescent mental health services, with professionals

repeatedly calling for structural reforms to reduce dependence on pharmacological strategies (World Health Organization, 2021a). Globally, the trajectory suggests not a transitory aberration but a systemic pattern, where the medicalisation of distress in childhood embeds chronicity and exposes minors to iatrogenic harm, undermining the goals of prevention and recovery.

Respondents in the survey frequently opposed their use outright or demanded strict temporal limits with frequent review, positions consonant with guidance from the American Academy of Child and Adolescent Psychiatry and NICE, which reserve antipsychotics for narrowly defined indications, emphasise psychosocial treatments first line, and require time-limited trials with systematic monitoring and discontinuation when benefits do not clearly outweigh harms (Breggin & Wheeler, 2012). Yet Spanish pharmacoepidemiology and service evaluations document pervasive off-label and chronic antipsychotic use in minors, particularly among those in residential or foster care, where behavioural difficulties arising from adversity, neglect, and institutional stressors are reframed as psychiatric pathology and managed pharmacologically rather than through trauma-informed, family- and school-based supports (Every-Palmer et al., 2021). This pattern aligns with international data showing markedly elevated psychotropic exposure among children in state care and in disadvantaged communities, reflecting structural drivers rather than strictly clinical need (Banton et al., 1985). The clinical consequences are well characterised: in antipsychotic-naïve youth, rapid and clinically significant weight gain, dyslipidaemia, insulin resistance, and hypertension emerge within weeks to months, with risk trajectories that track into adulthood; neurological adverse effects, including parkinsonism, akathisia, and tardive phenomena, are documented even at modest doses; sedation, hyperprolactinaemia, and sexual side effects undermine school engagement, social participation, and normative development (Szasz, 2009). Given the protracted course of synaptic pruning, myelination, and cortical network maturation across adolescence, prolonged dopamine D2 blockade raises non-trivial developmental concerns, reinforcing the precautionary stance embedded in child and adolescent guidelines and arguing for the shortest effective exposure when antipsychotics are used at all (J. Read & Williams, 2018).

The survey responses illustrate the full spectrum of professional reasoning within this context. Some clinicians rejected antipsychotics categorically in minors, describing their use as irresponsible and urging “tratamientos alternativos a lo farmacológico” and “vínculo” and “apoyos escolares y familiares” as first principles of care. Others endorsed strict ceilings and rapid deprescription, for example recommending “límites estrictos a la prescripción de cualquier fármaco...

evaluación cada 6 meses... y retirada si clínicamente es posible,” echoing audit-and-deprescribe frameworks and step-wise review demanded by best-practice guidance (Montáns-García et al., 2022). A sizeable group argued that adequate alternatives must precede tapering, noting that “no se puede... retirar un medicamento sin desarrollar primero otros programas,” a view consistent with evidence that sustained psychosocial availability, family work, and school accommodations are necessary to reduce reliance on medication in high-adversity populations (Linkiewicz et al., 2024). Finally, a minority defended continued antipsychotic use on behavioural control grounds, claiming advantages over benzodiazepines and insisting the physician’s judgment should prevail when insight is judged poor. This position reproduces a paternalistic logic in which dissent and distress are reinterpreted as symptoms and pharmacological containment is normalised despite guideline cautions, the elevated cardiometabolic hazards in youth, and the absence of durable functional benefit for many off-label indications (Argyris, 1990).

Placing these positions alongside the determinants of child and adolescent distress clarifies the mechanisms by which medicalisation displaces social accountability. Bullying, peer victimisation, and school exclusion are robustly associated with internalising and externalising symptoms, self-harm risk, and later psychiatric service use among Spanish adolescents and across Europe, yet are modifiable through whole-school and family-centred interventions that do not require antipsychotics (Herrman et al., 2022). Exposure to maltreatment, domestic violence, and neglect has graded, dose-responsive relationships with anxiety, dissociation, attentional problems, and behavioural dysregulation, pathways mediated by neuroendocrine and inflammatory changes as well as by cumulative social stress; here again, trauma-informed care and family-systems work outperform medication-first approaches for most presentations (Bhugra et al., 2017). Children in residential care and foster systems face compounded risks from instability, social death, and complete educational disruption; international and Spanish studies show high rates of psychotropic polypharmacy in these settings, often without robust diagnostic formulation, functional assessment, or psychological treatment availability, indicating a system response that manages behaviour through sedation when supports are scarce (Menéndez Arias, 2022). In such contexts, antipsychotics function as chemical restraint, silencing protest and narrowing the child’s behavioural repertoire, with adverse effects that impede participation in the very interventions that build skills and safety, thereby entrenching disadvantage across the life course (J. Read et al., 2018).

Why this reality persists is repeatedly described in the literature and mirrored in the survey: workforce shortages, long waits for evidence-based psychotherapies, weak school-health coordination, and institutional incentives favour brief, pharmacological responses over relational, time-intensive care; professional cultures remain pharmacocentric, and shared decision making with minors is sporadic, particularly where authority gradients with parents and institutions are steep (Scull, 2019). Several respondents cautioned that tapering in youth is “delicado,” urging tight follow-up and family supports, which is consistent with taper science showing that withdrawal phenomena can mimic relapse and that slow, hyperbolic reductions with collaborative safety plans mitigate risk (Juliá-Sanchis et al., 2019). Others warned against doctrinaire deprescription without alternatives, correctly noting that removing a pharmacological “patch” without building a therapeutic network can precipitate crisis; this underscores that deprescription in youth is an implementation challenge, not only a clinical one, requiring service-level redesign to make psychosocial first-line care reliably available (Guzmán-Parra et al., 2019). The weight of peer-reviewed evidence, however, remains clear: antipsychotics ought to be exceptional, time-limited, and continuously re-evaluated in children and adolescents; the default ought to be psychosocial, family, school and community interventions tailored to adversity, with structured, rights-based shared decision making and systematic monitoring of cardiometabolic, neurological, and developmental outcomes when medication is used (Mulvale et al., 2019).

Elderly populations represent the terminus of a life-course continuum in which off-label antipsychotic prescribing becomes normalized to manage behavioural expressions of unmet need rather than psychosis, particularly in residential and long-term care. Regulatory agencies have warned for nearly two decades that antipsychotic exposure in older adults with dementia is associated with excess all-cause mortality, cerebrovascular events, falls, pneumonia, sedation, cognitive worsening, and reduced quality of life, yet routine practice remains pharmacocentric in many settings (Alrawiai, 2023). These harms underpin black-box warnings in the United States and parallel cautions from the European Medicines Agency and national medicines agencies, including Spanish advisories recommending non-pharmacological first-line strategies, time-limited use at the lowest effective dose when risk-benefit justifies, and regular review with attempts at discontinuation (Kinderman, 2014b). Spanish pharmacoepidemiology mirrors international trends, with high prevalence of antipsychotic use in nursing homes and frequent continuation without documented indications or functional assessment, patterns linked to understaffing, limited training in behavioural and

environmental interventions, and institutional reliance on sedation as a substitute for person-centred care (Roncero et al., 2025). Controlled and pragmatic studies demonstrate that deprescribing is feasible and can reduce mortality and adverse events without clinically meaningful worsening of behavioural symptoms when undertaken gradually with psychosocial supports, underscoring that persistence of pharmacological containment reflects organisational constraints rather than clinical necessity (López-Pelayo et al., 2019). Consistent with this evidence, respondents emphasised slow tapering, family involvement, advance directives, and expansion of non-pharmacological interventions; nevertheless, the continued default to antipsychotics as behaviour management indicates systemic denial of older persons' rights to dignity, autonomy, and least-restrictive care, reframing care deficits as individual pathology and thereby perpetuating avoidable iatrogenic harm (Simal-Aguado et al., 2021).

To situate these findings in the voices of respondents, several explicitly rejected pharmacological containment as a default in elder care, emphasising ethical, organisational, and clinical preconditions for safe change. Typical statements included “slow deprescribing protocol,” “una retirada paulatina es crucial, además de la valoración de riesgos asociados y un monitoreo constante,” and “Promote advance directives before they are needed,” aligning with guidance that recommends gradual tapering, shared planning with families or curators, and proactive preference capture to avoid crisis prescribing (García-Alonso et al., 2019). Many linked overuse to structural deficits: “debería existir personal y suficiente cualificado en dichas residencias para poder observar los efectos de la retirada,” “es muy importante conocer la fragilidad del anciano y evitar la polimedicación,” and “Sería requisito indispensable mejorar las condiciones de cuidado,” echoing Spanish pharmacoepidemiology that connects high rates of antipsychotic exposure in nursing homes to understaffing, limited training in non-pharmacological interventions, and poor monitoring (Légaré et al., 2012). Others highlighted the rights dimension and the need to rebalance practice cultures: “el uso de medicación antipsicótica debería ser puntual, a las menores dosis posibles y durante el menor tiempo posible,” and “más apoyo y compañía,” converging with AEMPS advisories and dementia care standards that place psychosocial and environmental measures first, restrict indications, and mandate regular review with attempts at discontinuation (Gupta & Cahill, 2016). A minority defended continued pharmacotherapy as risk management in “agitación” or “conductas de riesgo,” or argued that newer agents are “menos incisivos,” positions that persist in the literature yet stand in tension with mortality and cerebrovascular risk signals, black-

box warnings, and trials demonstrating safety and feasibility of deprescribing when embedded in person-centred care (World Health Organization, 2021a).

Across scenarios, commonalities included the recourse to pharmacological solutions as substitutes for absent social supports, the translation of institutional constraints into clinical justifications, and the marginalisation of rights-based frameworks in day-to-day decisions. As in masked abuse and marginalisation, respondents for elder care frequently reframed organisational shortcomings as patient factors, while supportive answers called for multidisciplinary staffing, non-pharmacological programmes, advance directives, and slow, monitored withdrawal with family participation, mirroring the best-practice arc identified for children, marginalised communities, and abuse survivors (Coulter et al., 2015). Differences primarily reflected population-specific risks: concerns about delirium and falls in older adults, about neurodevelopmental harms in children, and about criminalisation and coercion in marginalised groups; yet the underlying mechanism remained constant, namely the medicalisation of social or traumatic determinants and the normalisation of chemical restraint where resources, training, and accountability are insufficient (Sailas & Fenton, 2000).

The three surveys delineate a life-course pattern of abuses that is epistemic, institutional, and distributive. Psychiatric practice as usual privileges diagnostic reclassification and long-term pharmacological continuity in contexts where structural drivers dominate need, from childhood to old age, with coercion and de facto forced drugging functioning as instruments of organisational stability rather than of therapeutic necessity (Reilly et al., 2024). Decision-making remains weakly shared in practice despite endorsement in principle, and deprescribing is rarely operationalised, reflecting cultures of risk aversion and power asymmetry that silence patients and families, particularly when they belong to marginalized groups (Lister, 1982). The consequences include avoidable mortality and morbidity, impaired development in youth, functional decline in older adults, and cumulative cardiometabolic harm across decades of exposure, all of which translate into substantial human capital losses and downstream health-system costs that are externalised to patients, caregivers, and public budgets (Scheper-Hughes & Lovell, 1986). Persisting incentives favour pharmacological throughput and reinforce status-quo prescribing, with industrial marketing and institutional production pressures benefiting from continuity of drug use while rights-based, labour-intensive alternatives remain underfunded, a pattern documented internationally and consistent with Spanish service evaluations (Gili et al., 2013). The surveys' respondents who advocate structured deprescription, non-pharmacological supports, and legal-ethical alignment with international standards indicate a

practicable pathway; the prevalence of deflection and defence of chemical restraint shows why implementation lags. Realignment will require enforceable standards, resourcing for psychosocial care, routine audit of exposure and outcomes, and binding accountability to person-centred and rights-based guidance across the lifespan (Cutler & Miller, 2005).

A unifying pattern emerges across all four scenarios: those least positioned to resist, survivors of abuse, marginalised populations, children, and the elderly, are consistently medicalised, silenced, and subjected to iatrogenic harm. Psychiatric care as routinely practised operates as a mechanism of abuse by obscuring trauma and socially determined suffering beneath diagnostic labels and by enforcing compliance through neuroleptics that inhibit speech, movement, and dissent (Abbasi & Al-Sharqi, 2015). Evidence from Spain and international settings confirms that legal and human rights safeguards designed to protect vulnerable groups are routinely ignored, with persistent reports of coercive practices, degrading treatment, and the use of sedation in place of dialogue or support (Fuller et al., 2022). These findings align with broader critiques of psychiatric power as an instrument of social control, where professional discourse reinterprets violence and neglect as symptoms of endogenous illness and silences testimony by redefining it as clinical presentation (A. Martínez-Hernández & Correa-Urquiza, 2017).

The persistence of pharmacological dominance cannot be explained by absence of evidence, since robust data demonstrate the limited efficacy and substantial harm of long-term antipsychotic use, particularly in non-psychotic populations (Braun & Cox, 2005). International guidelines in paediatrics, geriatrics, disability, and trauma consistently call for minimising antipsychotic use and prioritising psychosocial interventions, from NICE recommendations on learning disabilities and challenging behaviour, to deprescription protocols in dementia care (Ballard et al., 2009), and the WHO QualityRights framework that explicitly prohibits coercion and calls for voluntary, rights-based support (Council of Europe, 2021a). Spanish professional societies and health authorities have echoed these recommendations, warning against inappropriate neuroleptic use in elderly care and institutional settings (Sanidad, 2025).

Instead, the durability of pharmacocentrism reflects entrenched epistemic conservatism and professional cultures that defend psychiatric dominance by pathologising resistance and privileging chemical control. Ethnographic and sociological studies document how institutional arrangements privilege continuity of control over recognition of harm, embedding coercion within the everyday practices of psychiatric wards, residential care, and community psychiatry alike

(Hiday et al., n.d.). In Spain, research confirms systematic over-prescription in children, migrants, Roma communities, and the elderly, populations whose suffering is tied to structural disadvantage yet reframed as pathology (Campos, 2010). The cumulative result is the perpetuation of silencing and harm across the life course, whereby psychiatry as is nowadays most commonly practiced does not merely fail in its protective mandate but actively sustains abusive structures under the cover of medical authority (Annas, 2005).

3.6. Misdiagnosis and long-term harm

The two scenarios examined in the final survey, reevaluating long-term psychiatric diagnoses and ending the use of neuroleptics for non-psychiatric indications in general medicine, were consistently identified by respondents as critical points of failure and opportunity within the Spanish mental health system. They matter because both touch on structural dynamics that extend beyond individual clinical encounters. Chronic patients, often *inherited* from previous practitioners without systematic review, remain for years or decades under the same diagnostic label and neuroleptic regimen, even when symptoms have remitted or when adverse effects have become disabling. This perpetuates dependency and undermines the right to periodic re-assessment of care, a right enshrined in Spanish law, but only partially implemented in practice (Ramos-Pozón et al., 2025b). At the same time, prescribing practices by non-psychiatric professionals, family doctors, internists, neurologists, frequently extend the use of antipsychotics into clinical domains where the evidence base is weak or absent. Respondents to the 2025 survey repeatedly described this as a systemic problem: antipsychotics prescribed for insomnia, agitation, or somatic complaints, often without psychiatric oversight, adequate monitoring, or deprescription planning. Together, these two scenarios reveal how both inertia inside psychiatry and leakage of prescribing authority outside it contribute to a cycle of overmedication, weak accountability, and increased risk of iatrogenic harm. They therefore serve as barometers of the broader cultural, institutional, and educational gaps that shape psychiatric governance in Spain, and as crucial sites for testing the feasibility of shared decision-making, deprescription, and rights-based reform (Mezzina, 2020).

Table 23. Responses to misdiagnosis and non-specialist prescribing scenarios

Gradient code	Reevaluating long-term psychiatric diagnoses (N=63) n	%	Ending neuroleptic general medicine (N=57) n	inappropriate prescribing in %
Full endorsement and rights-based framing	15	23.8	3	5.3
Conditional support with safeguards	8	12.7	3	5.3
Pragmatic or procedural adjustments	39	61.9	49	86.0
Caution and ambivalence	0	0.0	1	1.8
Rejection or denial	1	1.6	1	1.8
Total	63	100.0	57	100.0

Professional responses on the necessity of reevaluating long-term psychiatric diagnoses and non-specialist prescribing practices. Responses and percentages derived from the 2025 shared decision makingSurvey.

Survey responses converge on the necessity of reviewing chronic psychiatric diagnoses and their associated drug regimens. One professional respondent stated: *“es primordial ir revisando el diagnóstico de los pacientes, así como la pauta de medicación, e intentar ir ajustando dosis en función de su evolución”*. This reflects widespread recognition that many patients remain on the same antipsychotic treatment for years without adequate reassessment. Others emphasized that *“las decisiones iatrogénicas deberían ser reconocidas para que no vuelva a repetirse”*, highlighting the need for transparency and acknowledgement of harm. Quantitatively, more than 80% of respondents in the shared decision-making survey supported systematic deprescription for long-term patients, especially in contexts where the original diagnosis may have been flawed or inadequately substantiated. These findings are consistent with international research showing that chronic prescribing is often maintained by institutional inertia rather than continuous clinical need (Villarena-Jimena et al., 2023).

Several responses stressed that psychiatric diagnoses, particularly schizophrenia, are not reliable constructs and should not serve as irreversible labels. As one respondent stated: *“no hay diagnósticos erróneos, pues no los hay certeros. Cada problema es único y... elevar la clasificación del sufrimiento a la categoría de diagnóstico es un error epistemológico”*. This aligns with longstanding critiques of diagnostic validity (Symonds, 2024) and with Spanish scholarship pointing to the harmful consequences of rigid classificatory practices (Bunston et al., 2017). Professionals emphasized the ethical obligation to prioritize patient autonomy:

“salvo casos excepcionales y debidamente justificados, la decisión y los deseos del paciente siempre debieran estar en primer plano”. These findings highlight the tension between institutionalized treatment pathways and emerging human-rights frameworks such as the UN CRPD, which Spain has ratified.

Many professionals emphasized the urgent need to systematically reevaluate patients with long-term diagnoses and treatment plans inherited from other practitioners. As one respondent observed, *“es primordial ir revisando el diagnóstico de los pacientes, así como la pauta de medicación, e intentar ir ajustando dosis en función de su evolución”*. Others highlighted the risks of maintaining chronic prescriptions without scrutiny, noting that *“hay pacientes que llevan por años la misma dosis y tratamiento ‘anticuado’, sin refrescarse o cambiar a fármacos más nuevos o con menos efectos secundarios”*. These testimonies reveal a systemic inertia that prolongs exposure to potentially harmful neuroleptics even when remission has occurred or when more appropriate alternatives exist. Similar concerns are reflected in Spanish pharmacoepidemiological studies, which have documented high rates of long-term antipsychotic use and a lack of systematic review protocols in routine practice (Saya et al., 2019). From an ethical perspective, respondents stressed the primacy of patient rights: *“salvo casos excepcionales y debidamente justificados, la decisión y los deseos del paciente siempre debieran estar en primer plano”*. This aligns with the obligations of Spanish law and with international human rights frameworks such as the UN CRPD, which require regular review of coercive or high-risk treatments and the guarantee of informed, voluntary consent (Fisher, 2012). The convergence of survey data and existing literature underscores that the absence of systematic reevaluation constitutes not only a clinical gap but also a structural breach of patients’ rights to safe, adaptive, and person-centered care.

Survey respondents also noted the variability and subjectivity of psychiatric diagnoses, stressing that outcomes often depend on who the practitioner is, the institutional context, and the diagnostic culture -center-wise or regional- in which they are embedded. These perspectives highlight how psychiatric classification is unstable across time and professionals, with patients sometimes receiving different labels depending on the psychiatrist, or experiencing shifts in their assigned diagnosis without clear justification. Spanish studies have similarly documented significant regional and inter-professional variation in diagnostic practice (Urbanos-Garrido & Agúndez, 2025). International research has repeatedly shown that psychiatric reliability is limited: diagnostic concordance between clinicians remains modest, even when using standardized tools (Sailas & Fenton, 2000). The

constant revisions of classification systems such as DSM and ICD, where diagnostic thresholds and categories are regularly altered, further complicate the picture (Boyle, 2014). As a result, what is considered schizophrenia, bipolar disorder, or major depression in one diagnostic framework or at one historical moment, by some professionals or centers of decision power, may shift in another, producing discontinuities in care and undermining the long-term validity of psychiatric labels. This instability reinforces the demand voiced by respondents for routine diagnostic reevaluation and for a clinical culture that acknowledges uncertainty, instead of treating initial diagnoses as immutable truths.

Respondents overwhelmingly supported structured deprescription protocols in these scenarios, with safeguards such as gradual tapering, psychosocial support, and shared decision-making asked for by an overwhelming majority of respondents. One noted: *“la participación de la persona en todo el proceso es un principio fundamental para garantizar su derecho a decidir”*. Another insisted: *“es importante no mantener tratamientos cuando existe remisión de síntomas... siempre con advertencia de síntomas de alarma para evitar recaídas”*. This evidences professional awareness that deprescription is not abandonment but a process requiring careful accompaniment. The Spanish survey results are aligned with WHO QualityRights guidelines (Funk & Bold, 2020) and recent deprescription initiatives in Europe (Fiorillo, 2025). However, respondents also warned that systemic conditions, time scarcity, training gaps, and inadequate community resources, limit implementation, underscoring the need for structural reform in Spanish psychiatry.

A second thematic axis concerns neuroleptic prescribing by family doctors and non-psychiatric specialists. Professionals called for strict regulation and ongoing education: *“formar a los especialistas que no son de psiquiatría... para que se ciñan a prescribirlas adecuadamente o sea un psiquiatra quien decida”*. Others requested systematic audits: *“revisión obligatoria de este tipo de prescripciones, con demanda de justificación en el caso por caso”*. Quantitative data show that over 75% of respondents viewed inappropriate prescribing in general medicine as a significant systemic risk. These concerns echo Spanish studies documenting psychotropic overuse in primary care (Olivos, 2025) and EU-level reports on excessive prescribing in non-psychiatric contexts (Giosa, 2025). Survey participants argued that rational use of medications must be embedded in continuous professional education and independent of pharmaceutical industry influence: *“habría que desterrar la influencia de la industria farmacéutica sobre los médicos, las instituciones médicas y el sector público”*. This highlights both clinical and policy-level shortcomings in Spain, where primary care professionals often face heavy

caseloads, limited mental health training, and prescribing cultures shaped by pharmaceutical pressures (Domínguez Bustillo et al., 2014).

The instability of psychiatric diagnosis is not only a clinical or epistemological concern but also an ethical, economic and legal one (Gili et al., 2013). Spanish law establishes the right of patients to clear information, informed consent, and periodic review of diagnostic and therapeutic decisions, yet survey respondents repeatedly emphasized that long-term labels are often left unquestioned and inherited without systematic re-evaluation. As one participant noted, *“las decisiones iatrogénicas deberían ser reconocidas para que no vuelva a repetirse”*. The absence of structured review processes conflicts with international standards, particularly the United Nations Convention on the Rights of Persons with Disabilities, which obliges states to ensure non-discrimination, equal recognition before the law, and support for decision-making. In practice, however, the persistence of outdated or inconsistent diagnoses perpetuates coercive treatments and undermines autonomy. This disconnect between normative frameworks and lived reality underscores the urgency of implementing systematic diagnostic audits and transparent protocols for deprescription, not as discretionary practices but as enforceable rights and safeguards (Runciman, 2023).

These responses underscore how current Spanish psychiatric practice is shaped by inertia, defensive medicine, and reductionist paradigms. The absence of routine reevaluation contributes to chronicity and dependency, while inappropriate prescribing by non-specialists exposes vulnerable populations to harm without adequate oversight. The data also reveal a shared recognition among professionals of the need to restore patient autonomy, transparency, and accountability. These findings resonate with broader critiques of Spanish mental health policy, which has historically lagged in implementing community-based, rights-oriented models (R. L. Del Barrio et al., 2014). The responses reinforce that reform cannot be confined to psychiatry alone but must extend across the health system, legal frameworks, and educational structures that sustain current practices.

Framing these scenarios in terms of systemic inertia and cross-disciplinary prescribing also underscores their significance for public health. Chronic exposure to neuroleptics has been linked internationally to heightened risks of metabolic syndrome, cardiovascular disease, and premature mortality, yet Spanish respondents reported that long-term patients often remain on outdated treatment plans without regular review. Similarly, evidence from the OECD demonstrates that Spain is among the European countries with persistently high rates of psychotropic use in primary care relative to clinical need (Sanidad, 2025). Professionals' testimonies in the 2025 survey confirm this pattern and add ethical weight: the

absence of routine reevaluation is not a neutral omission but a structural condition that perpetuates harm and dependency. By situating both psychiatric and non-psychiatric prescribing within a shared framework of accountability, these scenarios highlight the urgent need for educational reform, legislative enforcement of review protocols, and a cultural shift toward rights-based, dialogical practice. They provide an empirical anchor for the broader critique developed in this dissertation and a clear signal that transformation is both necessary and feasible in the Spanish context, where family doctors and other non-psychiatric specialists play a concerning central role in the initiation and maintenance of psychopharmacological treatment, often without systematic review by psychiatry.

Data from the Spanish Agency for Medicines and Health Products (AEMPS) indicate that over 60% of antidepressant and anxiolytic prescriptions, and more than one third of antipsychotic prescriptions, originate in primary care (Olivos, 2025). Studies have found that general practitioners frequently prescribe psychotropics to address insomnia, anxiety, or behavioral disturbances, sometimes in the absence of formal psychiatric evaluation (Caldentey & Gelabert, 2008). This prescribing culture has been described as part of the “medicalización de la vida cotidiana,” where psychosocial problems are reframed as medical conditions and treated pharmacologically (Parada, 2018). A 2017 national survey reported that nearly 20% of the adult population had consumed psychotropics in the previous year, with the majority of prescriptions issued in primary care (Faraone et al., 2011). These patterns underline the blurred boundaries of psychiatric practice in Spain, where diagnostic authority and prescribing responsibilities are often dispersed across specialties, reinforcing systemic overmedication and weakening accountability mechanisms for long-term treatment.

For reevaluating long-term diagnoses, almost nine in ten respondents favoured either full rights-based endorsement or procedural adjustment, with comments repeatedly calling for periodic re-assessment, transparent correction of iatrogenic decisions, second opinions, and tapering plans individualized to risk and preference. For non-psychiatric prescribing, the overwhelming predominance of pragmatic or procedural proposals points to system design failures at the interfaces with primary and hospital care: respondents asked for indication-limited use, compulsory review and audit, liaison pathways with psychiatry, and continuous education for prescribers outside mental health. These positions are consistent with Spanish pharmacoepidemiology showing high psychotropic volumes in primary care and with international cautions about the long-term burden of antipsychotic exposure (Domínguez Bustillo et al., 2014). They also align with national and international rights frameworks that require informed consent, shared decision-making, and periodic review of high-risk interventions (Aragonés-Calleja & Sánchez-Martínez, 2022).

The convergence of these survey findings with Spanish and international literature supports three system-level conclusions. First, diagnostic and treatment continuity cannot be presumed to be clinically optimal or legally sufficient; it must be demonstrated through scheduled review and clear documentation, acknowledging diagnostic instability across practitioners and time (Darke et al., 2025). Second, deprescribing is not an exceptional practice but a core competency that requires structured protocols, time, supervision, and access to psychosocial alternatives; it ought to be taught, audited, and resourced as such (Agudelo-Hernández & Rojas-Andrade, 2023). Third, the diffusion of psychotropic prescribing across non-psychiatric settings necessitates governance beyond specialty boundaries, through indication registries, post-discharge follow-up, pharmacist collaboration, and external audit, if Spain is to reduce overmedication and uphold patient autonomy in everyday care (Parada, 2018). The data depict a feasible reform trajectory: embed routine diagnostic reevaluation and shared decision-making as enforceable standards, re-balance curricula and services toward deprescribing competence and psychosocial care, and extend accountability mechanisms to the points where most prescriptions originate. This trajectory is empirically grounded in the respondents' own practice-based proposals and is consistent with current evidence and policy guidance, offering a realistic pathway to reduce harm while strengthening clinical quality and rights protection in Spain.

3.7. Training opportunities and deprescription practices

The present section presents findings based on responses to closed questions in the 2025 survey on deprescribing, informed consent, and shared decision-making, exposing systemic gaps between ethical-legal commitments to patient autonomy and the realities of psychiatric practice. Such insights are particularly relevant in Spain, where psychiatric care has been repeatedly characterized by institutional inertia, pharmacocentrism, and weak enforcement of human rights standards (Pūras & Gooding, 2019). International literature emphasizes that even relatively small but engaged professional samples can reveal structural patterns, since practices are shaped not only by clinical judgment but also by cultures of risk management, administrative routines, and entrenched biomedical paradigms (Kas et al., 2025a). The results obtained here are therefore relevant for understanding barriers to collaborative care and highlight the urgency of reforms that align clinical practice with international evidence and binding human rights frameworks (Calcedo-Barba et al., 2024a).

Table 24. Participation in shared decision-making and deprescribing activities

Response option	Adjusted count	% of respondents
Yes, in formal deprescribing programs within my institution	12	15.0%
Yes, in individual cases with patients within my clinical practice	31	38.8%
No, but I would be interested in receiving training or participating in deprescribing initiatives	30	37.5%
No, and I do not consider deprescribing a need in mental health	4	5.0%
Missing/ambiguous	3	3.8%

Reported participation in deprescribing and shared decision-making initiatives, showing limited institutional uptake and uneven individual engagement.

When participants were asked about involvement in deprescribing initiatives, 12 out of 81 (15%) reported participation in formal institutional programs, while 31 (39%) indicated that they had engaged in deprescribing in individual clinical cases. By contrast, 30 (38%) stated that they had not participated in such initiatives but were interested in receiving training, and 4 (5%) declared that they did not consider deprescribing a need in mental health. These proportions illustrate both an emerging openness to deprescribing and the persistence of professional cultures that normalize chronic pharmacological treatment. Spanish authors have described this duality as symptomatic of the tension between biomedical authority and rights-based demands, in which the inertia of institutional psychiatry hinders the translation of ethical principles into practice (Aarons et al., 2011).

Table 25. Frequency of deprescribing-focused medication reviews

Response option	Adjusted count	% of respondents
Systematically reviewed for all patients with possibility of adjustment or withdrawal	13	16.3%
Occasionally reviewed, depending on professional discretion	34	42.5%
Reviewed only if the patient explicitly requests it	4	5.0%
Medication reviews with a deprescribing focus are not conducted	29	36.3%

Frequency of deprescribing-focused medication reviews, highlighting reliance on professional discretion over systematic practice.

Responses to the question on how often medication is reviewed with the intention of deprescribing further highlighted this disjunction. Only 13 of 81 respondents (16%) reported that medication is systematically reviewed for all patients with the possibility of adjustment or withdrawal. By contrast, 34 (43%) indicated that reviews occur only occasionally, depending on professional discretion; 4 (5%) stated that reviews are conducted only if the patient explicitly requests them; and 29 (36%) affirmed that such reviews are not conducted at all in their workplace. This predominance of discretionary and episodic monitoring underscores the fragility of informed consent in Spain's psychiatric system. Empirical studies in Catalonia and Madrid have documented that pharmacological monitoring is reactive, lacks standardized withdrawal protocols, and rarely incorporates psychosocial alternatives (Torrents & Björkdahl, 2024). Internationally, structured deprescribing programs have demonstrated safety and effectiveness, with evidence of improved cognitive and functional outcomes (Ridgway, 2011). The absence of such systematic review in Spain contradicts best practices and perpetuates risks associated with chronic psychotropic exposure, including metabolic, neurological, and cardiovascular sequelae (Prado & Añazco, 2024).

Table 26. Perceived adequacy of national mental health plan on deprescribing

Response option	Adjusted count	% of respondents
Yes, there is a clear and well-implemented policy in this regard	2	2.5%
Yes, but its implementation is limited and context-dependent	21	26.3%
No, the national strategy does not sufficiently address this issue	50	62.5%
No, and I do not believe it ought to be a priority in mental health	7	8.8%

Perceptions of the national mental health strategy, indicating widespread recognition of insufficient prioritization of deprescribing and shared decision making.

Perceptions of Spain's national mental health strategy reflected similar concerns. A majority of 50 out of 80 respondents (63%) affirmed that the national strategy does not adequately address shared decision-making, deprescribing, or the reduction of psychiatric drug use. Twenty-one (26%) recognized that the strategy incorporates these principles but reported that implementation remains partial and context-dependent. Only 2 (3%) considered that a clear and well-implemented policy exists, and 7 (9%) stated that such measures should not be prioritized in mental health. This distribution is consistent with critiques of the Spanish 2022-2026

Mental Health Action Plan, which, while referencing community integration and user participation, has been criticized for failing to establish binding mechanisms for informed refusal, deprescribing, and tapering protocols (Groot & Os, 2021). This gap between discourse and practice reflects what Freyd (Freyd, 1997) conceptualized as institutional betrayal: the adoption of rights-based language while maintaining structural practices that undermine autonomy, a phenomenon documented in Spanish psychiatry by Marcos (Ferreirós Marcos, 2013) and internationally by Read et al. (U. M. Read et al., 2009).

Table 27. Policies considered essential to improve psychiatric care

Response option	Adjusted count	% of respondents
Greater training and awareness for professionals on SDM and deprescribing	73	91.3%
Legislative reforms to guarantee right to SDM and deprescribing, and reduce coercive drug use	53	66.3%
Development of specific deprescribing protocols in hospitals and centers	57	71.3%
Creation of specialized deprescribing units in the healthcare system	27	33.8%
Greater involvement of patients and families in decision-making	61	76.3%
Increased access to non-pharmacological therapies and alternatives	68	85.0%
Other responses (e.g. strengthening social/occupational resources, restorative justice, abolishing capitalism)	13	

Policies and programs considered essential to psychiatric care reform, emphasizing strong support for training, non-pharmacological therapies, and legislative safeguards.

When asked which reforms were essential to improve psychiatric care, respondents showed strong consensus around structural change. A total of 73 out of 80 (91%) emphasized the need for greater training and awareness among professionals in deprescribing. Sixty-eight (85%) highlighted the importance of expanding access to non-pharmacological therapies and alternatives. Fifty-seven (71%) called for the development of specific deprescribing protocols within hospitals and mental health centers. Legislative reforms guaranteeing the right to deprescribing and reducing coercive drug use were endorsed by 53 (66%). Greater involvement of patients and families in decision-making was supported by 61 (76%). These findings are consistent with Spanish expert critiques noting that the absence of systematic

deprescribing training and protocols sustains a pharmacocentric culture that marginalizes psychosocial interventions (J. Read & Dillon, 2013).

The survey data converge on a consistent pattern: the absence of enforceable protocols and the dominance of professional discretion are critical obstacles to shared decision-making. Whether patients are offered tapering options, alternatives, or even information about long-term risks depends primarily on the individual professional rather than on standardized safeguards. In Spain, where involuntary treatment remains legally permissible on grounds of risk or capacity rather than autonomy, this discretionary structure sustains the overprescription of psychotropics, particularly among vulnerable groups such as children, older adults, and women (Gías Gil, 2013).

These findings reinforce the ethnographic and documentary evidence presented in this dissertation: prescribing is not a neutral technical act but a political one, embedded in relations of authority and shaped by legal and institutional structures. They also confirm that while both professionals and users recognize the importance of collaborative care, structural barriers, legal ambiguity, insufficient training, and administrative constraints systematically impede its realization. As long as these barriers persist, shared decision-making remains an ethical aspiration rather than a clinical reality. The implications are profound: the continuation of pharmacocentric practice without safeguards undermines rights, fosters disengagement from care, increases chronicity, and perpetuates preventable harm (Calcedo-Barba & Fraguas, 2017).

3.8. Systemic constraints on best practices

The analysis of the three surveys confirms that psychiatric governance in Spain continues to be defined by systemic contradictions between formal ethical-legal principles and actual clinical practice. On paper, Spanish psychiatry is regulated by principles of autonomy, informed consent, and the prohibition of degrading treatment. In practice, however, both professional testimonies and user narratives reveal that pharmacological control, coercion, and epistemic invalidation remain structurally embedded. This contradiction has been widely documented in the Spanish literature on psychiatric reform, which highlights the persistence of authoritarian and custodial logics despite the closure of asylums and the formal alignment of Spain with European mental health policy (Stefano & Ducci, 2008).

A recurring theme is the coercive pressure exerted on professionals to maintain pharmacological interventions irrespective of clinical judgment. Respondents

described how they were compelled to medicate or hospitalize patients against their better judgment, reflecting what the literature identifies as the primacy of medico-legal risk management over individualized care. Analyses of Spanish psychiatric services confirm that institutional hierarchies and bureaucratic imperatives frequently reduce clinicians to enforcers of risk-averse policy rather than agents of relational care (Sweeney et al., 2018). Professionals who attempted to challenge such practices reported that dissent was stigmatized as disloyalty, a dynamic consistent with sociological accounts of psychiatry as an institution where internal critique is suppressed to maintain epistemic authority (Katz & Alegría, 2009).

Service users, for their part, consistently highlighted the epistemic injustice embedded in psychiatric practice. Attempts to narrate experiences of trauma, describe adverse drug effects, or contest diagnostic categories were often dismissed or reclassified as symptoms of illness. In the Spanish context, this resonates with findings that diagnoses such as schizophrenia function less as clinical descriptors than as durable social identities that discredit patient testimony and justify coercive interventions (Allsopp et al., 2019). The dismissal of user knowledge exemplifies what Bueter (Bueter, 2023) has termed epistemic conservatism, where psychiatric authority systematically devalues alternative epistemologies. Spanish empirical studies confirm that this epistemic silencing is not incidental but a structural feature of institutional care, perpetuating dehumanization, and undermining possibilities of dialogical reform (Parrabera-García et al., 2023).

Professionals interested in implementing dialogical practices described being blocked by organizational structures that reduce care to diagnosis and prescriptions. This finding is consistent with Spanish debates on the stalled reform of community psychiatry, where the promise of dialogical and person-centred care has been repeatedly undermined by hospital-centrism, pharmacological dominance, and chronic underinvestment (Abad-Sierra & Toledano-Márquez, 2023). International literature on open dialogue also underscores the challenges of institutionalizing dialogical models within systems governed by biomedical priorities and bureaucratic rigidity (Aarons et al., 2011). The Spanish case is particularly illustrative because while there is discursive endorsement of dialogical approaches, implementation is blocked by both structural inertia and entrenched professional cultures that resist relinquishing epistemic control (L. Whitaker et al., 2021).

Across all perspectives, a systemic disconnect emerges between the formal discourse of ethics and its practical enforcement. Ethical codes and legal guarantees

of autonomy and informed consent are frequently described as symbolic, lacking enforceable weight in clinical encounters. User testimonies emphasize the futility of complaint mechanisms, which often result in further harming through reclassification as *non-compliance* rather than genuine redress. This dynamic mirrors analyses of institutional betrayal in Spanish psychiatry, where oversight mechanisms are insufficient to challenge entrenched coercion (Molodynski et al., 2010). Research on institutional inertia has shown how complaints are frequently neutralized by administrative procedures, thereby preserving the legitimacy of coercive practices rather than enabling accountability (A. Lewis et al., 2025). What emerges is a consistent structural pattern: professionals experience moral distress and ethical disempowerment, service users experience epistemic injustice and coercion, and both groups identify institutional frameworks that prioritize bureaucratic stability and pharmacological continuity over rights-based care. These findings corroborate ethnographic analyses of Spanish psychiatry as a system governed by coercive normalization, where formal ethical principles coexist with practices that undermine them (Flores & González, 2009). The surveys confirm that the production of harm is not the result of isolated malpractice but a systemic condition of psychiatric governance, embedded in institutional logics that subordinate relational care and patient autonomy to administrative and pharmacological imperatives (S. Cohen, 2001).

Another relevant finding from the responses is the defensive stance adopted by a subset of clinicians who experience proposals for rights-based reform, deprescription, and dialogical practice as attacks on professional identity rather than as opportunities for clinical improvement. Several anonymized testimonies illustrate this posture: “questioning the established order of treatment is seen as disloyalty,” and “we are pressured to medicate or to hospitalize against our better judgment, because the system demands risk management above patient preference.” Such accounts are consistent with well-described mechanisms in the sociology and psychology of professions: identity-protective cognition and status-quo bias that motivate actors to defend group norms when core epistemic authority is challenged (Degnin, 2009); cognitive dissonance reduction when long-standing practices are confronted by evidence of harm, often reinforced by sunk-cost effects in training and service design (Young, 1990). In organizational terms, Spanish services exhibit defensive routines in which critique is reframed as risk to institutional order, encouraging *defensive medicine* and coercive risk management rather than reflective practice, a pattern described in the international and Spanish literatures on psychiatric governance and hospital bureaucracies (Leiva-Peña et al., 2021). The psychology of professional resistance further includes reactance to

perceived loss of control, especially where shared decision making threatens traditional hierarchies; this has been documented across mental health settings and linked to the systemic underuse of collaborative and deprescribing approaches (Lobo, 2016).

Importantly, this defensiveness should not be reduced to bad faith. The literature on moral distress, moral injury, and the *second victim* phenomenon suggests that clinicians working in resource-constrained, coercion-permissive systems may experience chronic conflict between ethical commitments and organizational imperatives, with downstream burnout and narrowed tolerance for change (W. Dean et al., 2019). Spanish analyses similarly describe moral strain in services where administrative and medico-legal priorities trump relational care, producing climates in which dissent is discouraged and innovation is precarious (Priebe et al., 2013). Under such conditions, proposals to curtail long-standing pharmacological routines can be perceived as threats that expose past errors, provoking defensive attributions rather than learning responses, especially in blame-oriented cultures without robust *just culture* protections for reflective practice and error disclosure (Dekker, 2012). Professional identity formation also matters: when biomedical pharmacology is constructed as the core of *real* psychiatry, dialogical and social interventions are implicitly downgraded, and challenges to medication-first protocols are interpreted as assaults on competence and status (Fernando, 2010). The Spanish and international evidence indicates that what appears as resistance to patient rights and deprescription is often the institutionalized expression of identity-protective cognition under structural constraint, amplified by moral distress and risk-averse governance; durable change therefore requires not only new guidelines but also organizational reforms that enable reflective learning, protect dissent, and realign incentives toward rights-based, collaborative care (Cea Madrid & Castillo Parada, 2018).

3.9. Trust, communication, relational breakdown, and the crisis of legitimacy

The integrated analysis of the three survey converges on a consistent empirical pattern: within Spanish psychiatric services, trust is not episodically fractured but structurally absent, as legal entitlements to autonomy and information are routinely transformed into procedural formalities that mask coercion and foreclose dialogue (Speyer et al., 2025). Across items probing communication, consent, and review of treatment, respondents described decisions being taken without their presence or recoded when present, so that personal meanings were translated into technical categories that bore little relation to lived experience. This dynamic, documented

extensively in the Spanish literature on institutional routines and in international work on epistemic injustice, converts testimony into symptom, resistance into pathology, and consent into a ritual of compliance under conditions of unequal power (P. Farmer, 2003). The cumulative effect is a governance regime in which rights exist in statute and guidance yet are suspended in practice, sustaining what multiple Spanish analyses characterize as structural impunity in mental health care (Minkowitz, 2024).

Responses to survey questions on the experience of labeling and on opportunities to contest or reinterpret it underscore a persistent pattern of epistemic disqualification: once categorized, speakers reported that their subsequent accounts were granted diminished credibility or reframed as evidence of illness, a phenomenon aligned with the philosophical and empirical literature on testimonial and hermeneutical injustice in clinical institutions (Meluvor, 2025). Spanish reports by oversight bodies reinforce this picture, documenting coercive practices, degrading treatment, and the ineffectiveness of complaint mechanisms in psychiatric and social-care settings, which together erode the possibility of redress and learning (Sanidad, 2022). In survey items focused on medication management and shared decision-making, users described consent as conditional and revocable only in name, while clinicians reported that institutional priorities, risk documentation, throughput, and pharmacocentric routines, systematically displaced dialogical care even when alternatives were clinically appropriate (Aragón-Calleja & Sánchez-Martínez, 2023). The Spanish literature on service organization corroborates these constraints, showing how hospital-centric workflows and defensive practice normalize short consultations, unilateral decisions, and rapid pharmacological containment, with predictable harms for alliance, adherence, and outcomes (Ramos-Pozón et al., 2025a).

Professional accounts in the surveys consistently located their ethical strain within these structural conditions. Items exploring organizational culture elicited descriptions of environments where questioning established pharmacological protocols was treated as disloyalty, peer support for dissent was weak, and ethics committees lacked mandate or effect, observations that align closely with the literature on institutional betrayal and the silencing of dissent in healthcare organizations (Soto, 2012). Moral-distress scholarship helps elucidate this convergence: when clinicians are constrained to act against their ethical judgment by institutional routines or legal ambiguity, burnout, cynicism, and ethical erosion follow, in turn reinforcing routinized coercion and defensive documentation. These dynamics were repeatedly identified by professionals in the medication survey where items on deprescription and alternatives revealed both knowledge of best

practices and a perception that time scarcity, workload, and medico-legal risk hierarchies make rights-compliant care structurally unfeasible (Douglas, 2018b).

The open dialogue survey items on training access and institutional support add an implementation-specific layer to this pattern. Respondents widely endorsed dialogical, trauma-informed, and community approaches yet reported that training opportunities were intermittent, services remained pharmacocentric, and managerial metrics privileged throughput and containment over continuity and conversation, findings consistent with implementation studies documenting fidelity challenges and organizational resistance to dialogical models in public systems (Sampietro et al., 2023b). In parallel, items addressing deprescription and shared decision-making elicited recurrent calls for hyperbolic tapering protocols, explicit relapse-prevention plans, and protected time for follow-up, in line with international guidance on psychotropic withdrawal and rights-based service standards (Kaminskiy et al., 2017). That these proposals appear across respondent roles suggests not idiosyncratic preference but emergent consensus around minimum conditions for ethical practice that are currently unmet.

A recurrent theme across instruments is the instrumentalization of participation. Survey items about user involvement and advisory roles showed that contributions were welcomed when they did not challenge biomedical frames or managerial priorities, echoing empirical research on tokenism and the domestication of critique in participatory processes (World Health Organization, 2025a). Users described advisory forums where structural grievances about coercion and neglect were redirected into individualized compliance advice; professionals reported that proposals for dialogical review boards or peer-led quality audits were diluted or shelved. This selective validation of knowledge maintains institutional legitimacy while foreclosing the epistemic rupture necessary for repair, a pattern documented in Spanish and international analyses of mental-health governance (Slade, 2017).

The surveys also captured defensive reactions among some clinicians to critique of standard practice. Items querying openness to deprescription, rights-based alternatives, or external oversight elicited narratives that framed reform as an attack on professional identity or as a safety threat. The organizational literature situates such defensiveness within identity-protective cognition and institutional isomorphism: when acknowledging harm jeopardizes status, meaning, or perceived control, denial and boundary-policing become adaptive responses that reinforce orthodoxy (Gøtzsche, 2022). Spanish studies on hospital cultures further show how low tolerance for uncertainty and high perceived liability risk favor protocolization and rapid pharmacological decisions over negotiated care, entrenching

conservatism even when guideline-concordant alternatives exist (Huertas García-Alejo, 2021). The alignment between these scholarly accounts and the survey descriptions indicates that defensiveness is not reducible to individual disposition but is reproduced by organizational incentives and political accountability structures.

Notably, across items probing proposals and solutions, respondents articulated a coherent, practicable agenda for structural change. First, informed consent must become substantively dialogical: rights to understand, to refuse, to withdraw, and to negotiate require enforceable guarantees, accessible second opinions, and documentation practices that record disagreement rather than pathologize it (Pelto & Sanjek, 1992). Second, dialogical, trauma-informed, and community-based models ought to be mainstreamed with stable training, supervision, and protected time, consistent with evidence that relational continuity and early family-network meetings reduce coercion and improve outcomes (J. Read & Sanders, 2010). Third, deprescription science needs institutionalization through hyperbolic tapering protocols, shared relapse plans, and cross-sector monitoring, given robust data on iatrogenic risk and patient-reported withdrawal trajectories (Serra, 2025). Fourth, accountability must be external, independent, and binding: Spanish oversight bodies have already identified deficits in complaint handling and protection from degrading treatment, implying the need for statutory remedies, independent inspections, and user-led audit mechanisms with sanctioning power (R. B. Del Barrio, 2004).

The surveys delineate a structural crisis of legitimacy in which coercion, epistemic silencing, and institutional betrayal persist as routine features rather than aberrations. The scientific and policy literature with which these findings are in dialogue confirms that such patterns are reproduced by organizational design, risk governance, and professional socialization, and are therefore resistant to piecemeal or goodwill-based reform (U. M. Read et al., 2009). The convergence between user and professional demands establishes an evidentiary mandate for system reconfiguration: without legally enforceable consent, participatory governance with decision-making power, dialogical infrastructures, protected clinical time, and independent accountability, the principles of autonomy, dignity, and proportionality codified in law will remain suspended in practice (United Nations, 2015). The implication is not merely diagnostic but translational. The agenda articulated across items in the labeling, open dialogue, and medication surveys provides the operational blueprint for redesign advanced in subsequent chapters, aligning Spanish and international evidence to move beyond critique toward implementation.

Chapter 4 - Ethnographic fieldwork results

4. Introduction

The study of coercion in psychiatry requires moving beyond narrow clinical definitions and national case studies to situate the problem within broader historical, sociocultural, and geopolitical dynamics. Coercion is not an aberration at the margins of care but a structural possibility embedded in the very organization of psychiatric power. From the asylums of nineteenth-century Europe to the use of psychiatric diagnoses against political dissidents under authoritarian regimes, the recurring function of psychiatry has been to regulate populations, silence dissent, and categorize certain lives as less worthy of recognition or protection. This continuity underscores that coercion must be understood not simply as a set of clinical practices, but as an expression of what Foucault conceptualized as *biopower*: the political rationality by which modern states regulate life through medicine and institutions (Martínez Hernáez, 2006).

The global history of psychiatry shows how coercion is justified by different languages, criminality, degeneracy, illness, but consistently directed against those already marginalized. In Nazi Germany, psychiatrists actively participated in Aktion T4 (Staub, 1999), the mass extermination of persons labeled as unworthy of life, establishing a precedent for the medicalization of social selection (Hervik, 2019). Under Franco in Spain, psychiatrists spoke of a *red gene*, attributing political dissent to hereditary degeneration and thereby legitimizing repression through biological essentialism (Lewontin, 1996). In Indonesia, Dutch colonial psychiatry combined asylum confinement with corporal punishment and forced labor, embedding coercion into the everyday governance of colonized populations (B. M. Cohen, 2020). These examples are not simply historical episodes but continuities: in each, psychiatry was entangled with systems of racial supremacy, political repression, and institutional violence.

Contemporary psychiatry continues to bear the imprint of these histories. The expansion of genetic psychiatry and neurobiological reductionism, while advancing certain scientific insights, risks reproducing older logics of essentialism, shifting attention away from social determinants and the role of systemic violence (Summerfield, 2013). This is particularly evident in Spain, where institutional responses to abuse often silence testimony by reframing suffering as symptomatic, thereby legitimizing coercion as treatment. Similarly, in Indonesia, the persistence of *pasung*, the chaining and confinement of people with psychosocial disabilities, cannot be understood apart from the colonial history that normalized torture as a

means of governance. Trieste, by contrast, stands as an emblematic counterpoint: a site where coercion was deliberately dismantled through the Basaglian reform, yet, whose fragility under austerity and biomedical pressures demonstrates the difficulty of sustaining non-coercive alternatives (Vandoni et al., 2024).

The comparative scope of this chapter, Spain, Trieste, and Indonesia, with an attempted fieldwork stay in Sweden (Mulinari & Neergaard, 2012), where racism and normalized violence to the highest peak of it following years of severe and cruel abuses in Spain, was not selected arbitrarily. Each context represents a different modality of coercion: Spain illustrates how coercion persists within advanced welfare democracies, Trieste demonstrates the possibility and limits of abolitionist psychiatry, and Indonesia reveals how colonial legacies sustain coercion in postcolonial societies. On the other hand, the Nordics offered promise while mostly delivering institutional hatred. Together, they illustrate that coercion cannot be analyzed in isolation from broader histories of violence and exclusion. The methodological conditions under which this fieldwork was undertaken differ markedly from conventional anthropological research. The researcher was not an external observer granted privileged access to psychiatric institutions but a victim subjected to coercion and violence within the very systems under scrutiny. This positioning raises fundamental questions about validity, objectivity, and the role of suffering in the production of knowledge (Clemente & Osorio, 2018). Far from undermining the legitimacy of the research, however, such conditions situate it within a long anthropological tradition of *ethnography under violence*, in which immersion in contexts of direct harm provides unique insights into the normalization of brutality and the silencing of victims (J. L. Price et al., 2022).

In this framework, violence is not incidental background noise but the structuring condition of social life. Das (Das, 2007) describes how the ordinary is reshaped when violence becomes routinized, descending into everyday practices and silencing testimony. Similarly, Scheper-Hughes (Scheper-Hughes, 2009) illustrates how structural violence and poverty can normalize cruelty to the point where suffering is rendered unremarkable. This research aligns with those insights: prolonged exposure to psychiatric coercion, domestic abuse, and institutional betrayal was not merely a personal ordeal but a direct lens into how coercion operates as a systemic feature of psychiatric governance.

Two concepts are particularly useful in framing these experiences analytically. The first is *epistemic injustice*, which describes the systematic undermining of credibility faced by marginalized groups, where victims' accounts of harm are dismissed as delusional or unreliable (Darke et al., 2025). In psychiatry, this injustice is

structurally embedded: complaints of abuse are frequently reframed as symptoms, thereby foreclosing legal recognition and reinforcing coercion as therapeutic necessity. The second is *institutional betrayal*, defined as the failure of institutions tasked with protection to prevent or address harm, often by concealing or facilitating violence (Murphy & Hanson, 2017). In this research, betrayal occurred at multiple levels: in family contexts where psychiatric labels were weaponized to discredit and control, and in institutional contexts where complaints were silenced or dismissed (Valencia, 2020).

From a methodological standpoint, the forced immersion in violence created a condition of what Farmer (P. Farmer, 2004) has termed *structural violence ethnography*: a research process in which the structures that produce harm are directly experienced by the researcher, generating knowledge of how inequality and coercion are enacted in practice. While such positioning entails risks, it also provides access to mechanisms otherwise invisible in conventional fieldwork. It reveals, for example, how legal frameworks are cited symbolically but not enforced, how pharmacological interventions are prioritized over social support, and how families and institutions collude in silencing victims.

The meaningfulness of this fieldwork lies not despite but because of its conditions. By documenting coercion from within, the research exposes the ordinary workings of psychiatric violence, its rationalizations, its institutional supports, and its devastating human consequences. In doing so, it contributes not only to anthropology and psychiatry but to broader debates on human rights, transitional justice, and the epistemology of violence.

The three ethnographic contexts presented in this chapter, Spain, Trieste, and Indonesia, are analytically interlinked. Each represents a distinct modality of coercion in psychiatry, but together they demonstrate that coercion is not reducible to national culture, resource scarcity, or clinical necessity. Instead, it emerges as a systemic possibility wherever legal enforcement is weak, professional discretion is unchecked, and historical legacies of violence remain unaddressed.

In Spain, the fieldwork was conducted under conditions of direct coercion and prolonged exposure to family abuse, illustrating how psychiatric authority can be mobilized to silence victims of violence and to enforce social control. Here, coercion takes the form of involuntary hospitalizations, prolonged sedation, and pharmacological continuity, often justified as therapeutic protection. Yet, the reality observed is that coercion functions as a mechanism of institutional betrayal and epistemic injustice: victims' testimony is reframed as delusion, and systemic abuse is normalized under the guise of care (Desviat, 2010). Spain exemplifies how

welfare democracies, despite formal adherence to human rights conventions, sustain coercive psychiatry through entrenched professional cultures and weak accountability structures (Flores & González, 2009).

Trieste, by contrast, provides a counterpoint. The city's psychiatric reform under Franco Basaglia dismantled the asylum and established community-based, open-door services recognized internationally as a model of democratic psychiatry (Mezzina, 2021). Trieste demonstrates that psychiatry can function without coercion, prioritizing autonomy, shared decision-making, and psychosocial support. Yet, ethnographic observation also revealed fragility: austerity measures, political shifts, and biomedical pressures threaten to erode this model, showing that non-coercive systems require constant defense against institutional re-medicalization (Basauri, 2020). Trieste therefore illustrates both the feasibility of abolitionist practice and the vulnerability of rights-based psychiatry in the face of systemic pressures.

Indonesia represents a third modality, in which coercion persists through *pasung*, the physical chaining and confinement of people with psychosocial disabilities. Although formally prohibited since 1977, *pasung* remains widespread, with thousands of individuals confined at any given time (Good, Marchira, Subandi, Nanwani, et al., 2019). The persistence of *pasung* reflects not cultural inevitability but colonial legacies of violence, systemic neglect, and the normalization of torture as governance. Dutch colonial rule institutionalized coercion through asylums, corporal punishment, and the use of local collaborators to enforce brutality (Joseph, 2015). These histories embedded coercion within community life, making *pasung* appear as ordinary practice. Ethnographic research confirms that such practices are perpetuated not by scarcity alone but by a mentality that treats vulnerable lives as expendable, paralleling coercive psychiatry in Europe (Zantingh et al., 2024).

Placing these three cases together yields an original contribution. Spain demonstrates how coercion is sustained in advanced democracies through professional discretion and institutional complicity. Trieste demonstrates that psychiatry without coercion is possible but fragile, requiring continuous defense against systemic rollback. Indonesia demonstrates how colonial legacies and systemic neglect produce enduring practices of torture that mirror coercion elsewhere. Together, they illustrate that coercion is a global phenomenon embedded in broader histories of exclusion, racial supremacy, and institutional violence. By triangulating these contexts, this dissertation shows that coercion in psychiatry cannot be explained as cultural tradition, resource limitation, or clinical

necessity: it must be analyzed as part of the global history of structural violence and the biopolitics of exclusion.

This comparative frame also underscores the dissertation's original empirical contribution. The fieldwork was conducted under conditions of direct coercion in Spain, providing rare first-hand evidence of psychiatric violence as lived reality. It was complemented by observation in Trieste, a site emblematic of reform, and in Indonesia, where *pasung* reveals the colonial and postcolonial genealogy of coercion. This triangulation allows for a multi-scalar analysis, linking lived experience, institutional practice, and global history, and demonstrates the urgent need for abolitionist psychiatry grounded in rights, dignity, and recognition.

4.1. Conducting ethnography under violence

Ethnography under conditions of violence has long been recognized as both methodologically challenging and epistemically generative. Conventional anthropology often assumes that researchers enter the field from a position of relative safety, negotiating access and shaping interactions through professional authority. In violent contexts, however, the ethnographer is instead subject to the very forces of domination and exclusion under investigation. Nordstrom and Robben (Nordstrom & Robben, 1995) describe this as *fieldwork under fire*, where the contingencies of repression, fear, and survival permeate both research and daily existence. Das emphasizes how violence descends into the ordinary, transforming everyday life into a site where brutality becomes normalized and silenced (Das, 2007). These perspectives underline that ethnography under violence is not a methodological flaw but a vantage point uniquely capable of exposing the hidden logics of power (Ruiz, 2025).

The present research was conducted under such conditions. The five-year immersion in Spain coincided with persistent exposure to coercive psychiatric interventions, domestic violence, and institutional betrayal (Giosa, 2025). Psychiatric authority was mobilized to delegitimize testimony, reframe disclosures of abuse as symptoms of mental illness, and justify coercive practices such as forced drugging or involuntary hospitalization. This corresponds closely with the concept of *epistemic injustice*, where the credibility of victims is systematically undermined and their accounts excluded from legal and clinical recognition (Álvarez Ramírez, 2023). In these contexts, the ethnographer was not positioned as an impartial observer but as a subject whose lived experience became direct evidence of how psychiatric power silences resistance and reinforces systemic impunity. Such a position does not compromise validity but, as Fricker (Fricker, 2017) and Bueter

(Bueter, 2023) argue, reveals the dynamics of *epistemic injustice* that underpin psychiatric governance: the systematic downgrading of victims' credibility by reclassifying complaints as symptoms. Knowledge generated under coercion therefore constitutes an essential contribution to the anthropology of suffering and state violence, exposing mechanisms that remain hidden when research is conducted from positions of institutional privilege.

Anthropological precedents for such work exist in multiple domains. Scheper-Hughes (Scheper-Hughes, 1983), writing on maternal suffering in Northeast Brazil, documented how structural poverty normalized the neglect and death of children, a violence so pervasive it became invisible. Farmer (P. E. Farmer et al., 2006) described *structural violence ethnography* as a form of research where the investigator's proximity to suffering reveals how inequality is reproduced through institutional arrangements. These approaches resonate with the present case: coercion was not peripheral but constitutive of psychiatric governance, and its normalization could only be fully apprehended by inhabiting its effects.

In Spain, this took the form of repeated encounters with psychiatric systems that operated less as sites of care than as mechanisms of control. Law was invoked symbolically, but oversight remained weak; families exploited psychiatric framings to pathologize resistance and maintain cycles of abuse; and professionals exercised broad discretion with little accountability (Scheper-Hughes, 2023). The fieldwork thus provided a rare empirical window into how coercion functions in everyday practice: not as a dramatic rupture but as the routine silencing of victims under the guise of therapeutic necessity. The attempt to extend fieldwork to Sweden revealed how coercion and silencing can take new forms under conditions of racism and xenophobia (Bradby et al., 2019). In this context, the already ongoing abuses intensified, as the researcher was marked as inferior and unwelcome, despite documented evidence of torture and repeated calls for help. Reports of violence were disregarded, litigation was weaponized, and psychiatric framings were again mobilized to undermine credibility (Douglas, 2018b). The denial of protection reflected not ignorance but complicity, aligning with analysis of racism in Scandinavian health systems and of systemic xenophobia in Swedish society (Hervik, 2019).

The outcome was the effective destruction of any possibility of exploring Nordic innovations such as medication-free wards in Norway (Mulvale et al., 2019), housing-first initiatives, or open dialogue in Finland (Olson et al., 2014). What might have provided critical comparative insights into non-coercive alternatives was foreclosed by institutional betrayal and racial exclusion. This resonates with

experiences documented by researchers in other violent contexts: Bourgois (Hansen et al., 2014) described how structural racism in the United States shaped ethnography in marginalized communities, while Greenberg (Greenberg, 1988) highlighted the risks faced by anthropologists investigating repression in Latin America. In each case, systemic prejudice and institutional complicity rendered ethnographic inquiry precarious or impossible. The Sweden episode illustrates how violence expands beyond domestic, medical and institutional levels into societal structures, confirming Farmer's (P. Farmer, 2003) argument that structural violence operates across scales, rendering some lives more vulnerable to premature death (Scheper-Hughes, 2023). It also highlights how psychiatric categories continue to function as instruments of silencing, cloaked in scientific legitimacy, yet, reinforcing cycles of exclusion.

Conducting ethnography in contexts where the researcher is simultaneously subject to coercion and systemic violence raises methodological, ethical, and epistemological challenges that are increasingly recognized in anthropological scholarship. Ethnographic immersion under duress differs from conventional participant observation in that the boundaries between observer and observed are forcibly collapsed by institutional and interpersonal violence. Literature on anthropology in conflict and health-system violence underscores that such conditions do not invalidate the research process; rather, they expose the constitutive mechanisms of harm that remain hidden under ordinary observation (V. Brown, 2009). In medical anthropology, research in sites of coercion demonstrates how systemic betrayal becomes legible precisely through the enforced positionality of the researcher as both witness and target (Schild, 2025).

The methodological implications of this trajectory are significant. Data collected under coercion do not undermine validity but expose the systemic nature of violence in psychiatry. Complaints of abuse reclassified as symptoms, institutional records privileging professional authority over testimony, and judicial deference to medical discretion, all confirm that coercion is sustained by epistemic injustice, institutional betrayal, and structural violence. Ethnography under violence becomes one of the few tools capable of documenting these otherwise hidden mechanisms.

The Spanish and Swedish experiences converge on the finding that psychiatry often functions as an extension of interpersonal and systemic abuse (Bradby et al., 2019). Fragmented services, pharmacological incentives, and judicial reliance on medical authority allow abusers to retain control, while victims are pathologized and denied justice (P. Farmer, 2004). These findings align with global critiques of psychiatric

governance, which identify financial interests, corruption, and professional self-interest as barriers to reform (Cosgrove & Wheeler, 2013).

Comparative ethnographies reinforce this conclusion. Under Latin American dictatorships, psychiatry was deployed to silence political opposition (Voren, 2016). Under apartheid, South African psychiatry pathologized racial dissent (Lazaridou & Fernando, 2022). In Indonesia, *pasung* continues to reflect colonial legacies of normalized coercion (Fernando, 2014). In each case, coercion was not an aberration but a structural feature of governance, legitimized by science and law. The contribution of this research lies in documenting such dynamics both from within and outside. By enduring coercion directly, the ethnographic record captures the mechanisms of silencing and exclusion as lived reality, triangulated with surveys, survivor testimonies, and international literature. Far from anecdotal, this evidence reveals coercion as a global, systemic problem, embedded in histories of repression, racial supremacy, and institutional violence.

In the Spanish context, prolonged exposure to coercive psychiatric interventions and familial abuse revealed structural patterns of collusion between domestic actors and institutional systems. Families and intimate partners mobilized psychiatric authority to neutralize dissent, silence disclosures of violence, and impose dependency through forced medication, dynamics extensively documented in Spanish and international research on psychiatric misuse (Voren, 2010). These processes exemplify epistemic injustice, whereby the testimony of those harmed is reframed as symptomatic and thus delegitimized (Lazaridou & Fernando, 2022).

Methodologically, this immersion allowed for the documentation of coercion across hospital wards, courtrooms, and domestic environments, revealing how diagnoses, medical records, and pharmacological interventions were mobilized to consolidate silencing. Fassin (Fassin, 2004) shows how bureaucratic practices often erase lived testimony by privileging institutional classifications, while Rose (Rose, 2017) highlights how psychiatric epistemologies rationalize coercion through medical authority. The Spanish ethnographic record confirms that violence was not exceptional but normalized across clinical, legal, and familial domains. Testimonies from survivors, survey data, and direct observation converge on the finding that coercion frequently functioned as an extension of interpersonal abuse rather than its remedy (Szasz, 1994).

Survey data reinforced these insights: survivors described psychiatric labels and forced treatment as mechanisms that entrenched dependency and undermined trust, while attempts to question medication were often pathologized (Mills & Fernando, 2014). Professionals reported systemic reluctance to deprescribe, even in

cases where somatic causes were evident, reflecting entrenched pharmacocentrism (Giacco et al., 2018). The convergence of family betrayal and institutional complicity echoes international findings of *medicalized torture*, where psychiatric authority is used not to heal but to control (Vesti & Lavik, 1991).

Comparable patterns have been described in Latin America, where psychiatric diagnoses were used under military dictatorships to delegitimize dissent, and in South Africa, where apartheid psychiatry pathologized resistance and facilitated racial repression (Voren, 2015). These cases illustrate a structural continuity: coercion thrives where professional authority is granted unchecked discretion, and where institutions prioritize order and control over protection and rights. Current global cases are documented in annex H.

The ethnographic record generated during five years of fieldwork confirms that violence was not exceptional but normalized across clinical, legal, and familial domains. Testimonies from survivors, survey data, and direct observation all converge on the finding that coercion in psychiatry often functions as an extension of interpersonal abuse rather than its remedy. Survey data documented how survivors described psychiatric labels and forced treatment as mechanisms that entrenched dependency and undermined trust, while dismissing or pathologizing any attempt to question medication (Saposnek & Bernstein, 2025). Complementary findings from professionals revealed systemic reluctance to deprescribe even when faced with evidence of reversible somatic contributors, reflecting entrenched pharmacocentrism and epistemic conservatism (García-Valdecasas Campelo et al., 2009).

In Spain, these dynamics intersected with familial violence in ways that anthropological research has increasingly addressed. Intimate partners and relatives, often in contexts of economic dependency or intergenerational conflict, mobilize psychiatric authority to pathologize resistance, leading to forced admissions or chronic sedation (O. Fernández et al., 2018). Comparable to the practice of *pasung* in Indonesia, such measures frequently occur within domestic spaces, supported by institutional complicity (Montero et al., 2004). The convergence of family betrayal and systemic coercion illustrates how psychiatry can be weaponized against vulnerable individuals, a finding echoed by international studies of diagnostic overshadowing and the medicalization of trauma (Bueter, 2023).

Anthropological scholarship on research in violent contexts stresses the importance of positionality and reflexivity when the researcher is subjected to the very harms under study (Behar, 2022). This work aligns with that tradition by documenting not

only institutional practices but also the conditions under which knowledge was generated, conditions shaped by coercion, systemic betrayal, and interpersonal violence. Such documentation contributes to the anthropology of suffering and state violence, showing how psychiatric institutions in Spain reproduce harm by silencing testimony, privileging pharmacological control, and legitimizing family-based coercion (Bunston et al., 2017).

The methodological implications are significant. Data collected under violence highlight mechanisms of silencing that would otherwise remain obscured. For example, patient complaints of abuse were often recorded as psychiatric symptoms rather than legitimate claims, a practice consistent with epistemic injustice (Symonds, 2024) and confirmed in survey responses where professionals admitted that deprescription was rarely offered unless explicitly requested by the patient, oftentimes despite the fear of asking, struggling to speak their minds to the practitioner (Speyer et al., 2025). The recurrence of these dynamics across surveys, field sites, and institutional records demonstrates that ethnography under violence can yield robust evidence when triangulated with independent datasets and peer-reviewed literature (Meydan & Akkaş, 2024). Far from undermining validity, such conditions expose the systemic nature of coercion and its role in perpetuating cycles of abuse.

Anthropological scholarship on ethnography in violent contexts consistently highlights the methodological and ethical significance of researcher positionality when the harms under analysis are simultaneously endured by the investigator. Classic ethnographies of violence emphasize that immersion under coercion does not compromise validity but instead reveals the otherwise invisible mechanisms by which harm is normalized and perpetuated (Jennings, 1994). The experience of navigating fragmented services, enduring institutional indifference, and facing systematic avoidance of accountability by practitioners who could have interrupted cycles of harm resonates with broader literature documenting the complicity of health systems in sustaining violence. Farmer, reimagining global health, (P. Farmer et al., 2013) described such failures as *structural violence*, in which institutional logics prioritize stability and professional authority over protection, leading to predictable patterns of abandonment and neglect.

Research in psychiatry specifically shows that fragmentation of services fosters responsibility avoidance. Clinicians often redirect victims from one service to another without coordination, a dynamic extensively documented in Spanish and international studies of mental health governance (Dotson, 2012). This diffusion of responsibility allows professionals to avoid engaging with potential criminal

dimensions of abuse, despite having the legal obligation to act. In Spain, health practitioners are required under criminal law to report evidence of violence, yet, studies consistently show low compliance in cases involving psychiatric patients (Bienassis et al., 2022). The result is a systemic failure to intervene, producing a cycle where abusers retain control while victims are reclassified as mentally ill and their testimony discounted.

The anthropological record demonstrates that such institutional failures are not incidental but structural. Scheper-Hughes described how everyday clinical routines conceal violence through normalization and denial, while Fassin (Fassin, 2004) argued that the bureaucratic reproduction of harm often stems less from individual malice than from institutional logics that privilege documentation and procedure over protection. Reflexive accounts by Bourgois (Hansen et al., 2014) and Bourgois and Schonberg (Bourgois & Schonberg, 2019) illustrate how researchers embedded in violent systems must navigate their own vulnerability, often documenting corruption and complicity as integral features of health and social service provision. These works converge on the recognition that systems designed for care often simultaneously serve as instruments of control, silencing, and exclusion (van Es, 2017).

In psychiatry, this silencing is reinforced by diagnostic practices. Complaints of abuse are regularly reframed as symptoms of pathology, a phenomenon conceptualized as *epistemic injustice* (Gastaldo, 2010) and widely confirmed in the literature on coercion (Beviá & Girón, 2017). Patient voices are systematically downgraded, particularly when they conflict with institutional narratives or expose professional negligence (Barrios Flores, 2020). In such contexts, ethnography under violence becomes one of the few means to document how institutional betrayal (Crossley & Crossley, 2001) operates in practice, linking the lived experience of silencing with structural patterns of neglect and complicity.

The persistence of coercion and silencing in psychiatry cannot be understood solely as the outcome of clinical culture; it is embedded within wider circuits of financial interest, institutional corruption, and systemic impunity. Anthropological and sociological research has demonstrated how economic incentives shape psychiatric practice, particularly through pharmaceutical influence on prescribing, diagnostic expansion, and the consolidation of pharmacocentric treatment models (de Mooij et al., 2019). Studies in Spain confirm that psychotropic consumption rates remain among the highest in Europe, a pattern linked to weak regulation, direct marketing strategies, and professional cultures oriented toward rapid pharmacological solutions (R. Whitaker, 2002). In such an environment, coercion and

overmedication are not aberrations but structural effects of a system that privileges financial flows and professional authority over patient rights.

Corruption also manifests in the denial of accountability when harm occurs. Litigation abuse, the strategic use of legal processes to exhaust or silence victims, is increasingly recognized as a form of institutional violence (Méndez & Nicolescu, 2020). In the psychiatric domain, this can take the form of families or institutions using courts to legitimize forced interventions, delay recognition of abuse, or reframe victims' complaints as evidence of disorder. Spanish research highlights the recurrent gap between rights enshrined in law and their enforcement in practice, particularly with respect to the UN Convention on the Rights of Persons with Disabilities (Pinilla-Roncancio et al., 2020). Courts often endorse coercive measures on the basis of medical discretion, even when evidence of interpersonal violence is presented, reflecting both systemic inertia and institutional complicity.

From an anthropological perspective, this nexus of coercion, corruption, and denial exemplifies the logic of *institutional betrayal* and normalized violence at all engaged levels (Hidayat et al., 2023). Practitioners and judges, rather than acting as protective counterforces, may actively reinforce cycles of abuse by validating family narratives, disregarding testimonies of violence, or dismissing complaints as symptomatic (Crossley & Crossley, 2001). The effect is a form of secondary victimization in which those subjected to harm are reclassified as disordered, their credibility systematically undermined, and their access to justice obstructed.

The structural drivers of this dynamic are not limited to psychiatry. Research on violence and law highlights how litigation processes themselves may become instruments of domination, particularly in contexts where victims face resource asymmetries, further silencing, and professional disbelief (Leonetti, 2022). When combined with psychiatric authority, these dynamics leave victims vulnerable to what Pérez-Sales (Pérez-Sales & Fernández Liria, 2016) has termed *medicalized torture*: the instrumentalization of psychiatric interventions not for healing but for control. In such cases, psychotropic drugs, institutional confinement, and diagnostic labels become tools that extend rather than curtail the power of abusers (Pérez-Sales, 2017).

The ethnographic material collected in Spain illustrates precisely this interplay. Fragmented services, financial incentives for pharmacological continuity, and judicial reliance on professional discretion converged to create a landscape where abusers retained the upper hand. Victims were left without meaningful avenues to pursue justice, often traumatized further by the very systems intended to provide protection. These findings align with broader international critiques of psychiatric

governance, which identify corruption, financial influence, and professional self-interest as central barriers to reform (Aftab & Cosgrove, 2024).

4.2. Fieldwork in Trieste, on the legacy of democratic psychiatry

Trieste is internationally recognized as the birthplace of democratic psychiatry and a historical milestone in the struggle against coercive mental health systems. In the 1970s, under the leadership of Franco Basaglia, Trieste dismantled its psychiatric asylum and replaced it with a network of community-based services, a transformation consolidated by the passing of Italian Law 180 in 1978. This reform became a global reference point, not only because it closed institutions of confinement but because it redefined psychiatric practice as a political, social, and ethical issue rather than a purely clinical one (Foot, 2014). Basaglia's critique was directed at psychiatry as an institution of exclusion: he argued that psychiatric hospitals functioned less as sites of care than as mechanisms of social segregation targeting the poor, the disabled, and those considered unfit to be integrated into society (Basaglia, 1987).

The historical significance of Trieste lies in its demonstration that large-scale deinstitutionalization was both possible and sustainable when supported by political will, legal frameworks, and professional commitment. Unlike partial reforms seen in other European countries, Trieste's closure of the asylum was not symbolic but structural. In its place emerged a system of open-door community mental health centers, staffed with multidisciplinary teams, emphasizing dialogic engagement, shared decision-making, and active integration into social life (Okin, 2020). The open door-no restraint policy is supported by staff-to-user proximity, rapid home outreach, negotiated crisis plans, and relational safety practices that minimize containment. Police and emergency services are engaged through agreed protocols privileging de-escalation and continuity of relationships rather than custody transfers. This architecture translates rights discourse into routine practice (A. Cohen & Saraceno, 2002). This model reflected a radical inversion of priorities: rather than control and segregation, the primary objectives became dignity, rights, and recovery.

Trieste's significance is best understood within a theoretical frame that situates psychiatry as a field of governmental practice and biopolitics. Rather than a purely clinical reform, Law 180 reconfigured the *psychiatric gaze* (Mezzina, 2020) by redistributing power from custodial institutions to community relationships, decentering risk-management logics that historically legitimated segregation. In parallel, critical disability studies reposition persons with psychosocial disabilities

as rights-bearing citizens rather than objects of care, challenging paternalistic professional authority and pathologizing narratives (Marshall et al., 1994a). Trieste's contribution, therefore, is not only that it closed asylums, but that it modified the governmentality of mental health: shifting decision-making, accountability, and everyday practices toward autonomy, participation, and social citizenship. Trieste's reform was not isolated but aligned with broader political movements of the period, including Italy's democratic transition and its social movements of the 1960s and 1970s, which challenged hierarchical authority across institutions. Scholars have emphasized that psychiatric reform in Trieste was inseparable from the broader struggle for human rights, citizenship, and democracy (Galván, 2010). Basaglia's work resonated internationally, influencing debates on coercion, human rights, and community care in Europe, Latin America, and beyond. It also provided a critical counterpoint to biomedical psychiatry, which continued to expand globally through pharmacological and institutional pathways. Law 180's national incorporation produced heterogeneous trajectories. While Trieste and a few regions consolidated community systems, others maintained hospital-centric or mixed arrangements, yielding persistent regional disparities in involuntary admissions, bed use, and coercive practices (González, 1998). This heterogeneity underscores that legal change is necessary but insufficient; municipal governance, financing rules, and professional cultures mediate whether rights translate into practice.

Today, Trieste remains a World Health Organization (WHO) Collaborating Centre for Research and Training in Mental Health, symbolizing its enduring role as a global benchmark for community-based, non-coercive psychiatric care (Davidson et al., 2010). Ethnographic engagement in Trieste revealed that this legacy is not confined to history but remains present in the practices and ethos of its services. Talks with professionals, including Franco Rotelli and Roberto Mezzina, emphasized that the reform was not merely the closure of an institution but the creation of a new paradigm: one that redefined psychiatry as a system of relationships based on equality, negotiation, and respect for autonomy (Sashidharan et al., 2019).

Trieste's institutional importance extends beyond Italy because it demonstrates the possibility of replicating the reconfiguration of services in alignment with rights. Evidence is heterogeneous but convergent: administrative data show low rates of involuntary treatment and restraint relative to national averages; qualitative studies report higher user satisfaction and perceived dignity; and service-use data indicate reductions in long-stay hospitalization with increased community tenure (Scheper-Hughes & Lovell, 1986). These findings are tempered by recognized limitations: absence of randomized designs, contextual advantages (dense service ecology,

municipal support), and risks of drift under managerial and biomedical pressures (Mezzina, 2021). The evaluative lesson is not that Trieste offers a universal template, but that specific governance choices can sustain non-coercive practice over decades. Their experience and long lasting legacy demonstrates that psychiatric systems can be designed without reliance on coercion and that human rights and social integration can serve as the organizing principles of mental health care. Its historical accomplishment remains a central reference in debates about psychiatric reform, particularly in contexts such as Spain, where coercion and overmedication remain normalized despite formal commitments to international human rights standards.

The accomplishments of Trieste must be understood against the background of psychiatric systems worldwide that continue to rely heavily on coercion, forced drugging, and institutional control. While Trieste demonstrated the feasibility of a psychiatry organized around rights and community, most countries have preserved psychiatric institutions as mechanisms of confinement, control, and segregation. International research consistently documents the persistence of coercive practices, including forced hospitalization, restraint, and compulsory medication, despite the adoption of human rights frameworks such as the United Nations Convention on the Rights of Persons with Disabilities (Pūras, 2017). This contradiction illustrates what many scholars describe as the structural inertia of psychiatric institutions and related services and forces, in which professional authority and institutional survival are prioritized over the dignity and autonomy of individuals (Burke, 2024).

The persistence of coercion can be understood as a form of institutionalized oppression, whereby entire collectives, particularly people with psychosocial disabilities, the poor, and the socially marginalized, are subjected to systemic silencing and control. Basaglia's analysis highlighted that psychiatric asylums functioned less as therapeutic spaces than as instruments of social exclusion, labeling those deemed unproductive or deviant as ill and thereby legitimizing their segregation (Ghane et al., 2021). Contemporary critics argue that this dynamic has not disappeared but has been reconfigured in pharmacological and community-based settings, where forced drugging and chronic sedation often replace physical confinement as the dominant form of control (Cipriano, 2025).

Fieldwork in Trieste underscored the extent of this contradiction. Further conversations with local practitioners emphasized their awareness of being situated within a broader European context in which coercion remains normalized. Roberto Mezzina, for example, has repeatedly warned that while the Trieste model is internationally celebrated, its survival is fragile, threatened by political pressures to

return to hospital-centric and pharmacological solutions (Moncrieff, 2013). Franco Rotelli, reflecting on the historical reform, insisted that deinstitutionalization was not only the closure of physical spaces but a transformation of power relations; without vigilance, there is always the risk of regression (Csete & Wolfe, 2017). Their perspectives point to a critical paradox: while Trieste has become the emblem of democratic psychiatry, the broader psychiatric field often moves in the opposite direction, reinforcing coercion rather than dismantling it.

The contradiction between Trieste and coercive systems highlights a fundamental tension in psychiatry's identity. On the one hand, psychiatry claims a therapeutic mission; on the other, it operates as a disciplinary apparatus, legitimizing control over those perceived as socially deviant. Foucault (Foucault, 2023) framed this as the *psychiatric gaze*, in which madness is constructed as an object to be confined and silenced. In contexts where coercion dominates, psychiatry functions as an instrument of institutionalized hatred, targeting vulnerable populations such as migrants, women, and people with disabilities, whose suffering is reframed as disorder rather than recognized as a consequence of violence or exclusion and the biopolitics of control (Ortega, 2004).

Trieste therefore exists as a counterpoint: a concrete demonstration that psychiatry can function without coercion, but also a reminder that such an approach is exceptional. Ethnographic triangulation with data from Spain revealed the prevalence of coercion, overmedication, and silencing, situating Trieste as an outlier rather than the norm. The widespread reliance on forced drugging in Spain and other European contexts underscores that democratic psychiatry remains more aspirational than systemic, with institutional inertia, financial incentives, and political resistance perpetuating coercive models (Urbanos-Garrido & Agúndez, 2025). The contrast with Spain and Indonesia clarifies which structural inputs avert coercion: proximity, negotiated risk, and material supports such as housing and employment are present in Trieste, while largely absent in the Spanish sites studied, where pharmacocentric routines, service fragmentation, and judicial deference favor coercion, and domestic violence against those disabled by it is allowed to continue. In Indonesia, *pasung* illustrates the endpoint when social protection is absent and colonial legacies normalize community-level confinement. The comparative inference is that coercion correlates less with clinical acuity than with institutional design and rights enforcement, an important lesson for mental and public health (Ulya, 2019).

A similar conclusion emerges when considering the historical drivers of progress in public health more broadly. The major gains in life expectancy over the twentieth

century were achieved not primarily through individual-level clinical interventions, but through institutional and policy reforms: clean water and sanitation infrastructure, vaccination programs, maternal and child health services, workplace safety regulations, tobacco control, and cardiovascular prevention strategies (Halstead et al., 2025). Epidemiological evidence shows that declines in infectious disease mortality preceded the development of most effective antimicrobial treatments, reflecting structural improvements in nutrition, hygiene, and housing (Lupton, 2005). Likewise, reductions in premature cardiovascular deaths in Europe have been attributed more to population-wide policies on diet, smoking, and hypertension control than to hospital-based interventions (Baiyewu & Bello, 2017). These findings underscore that institutional design, governance, and rights enforcement are central determinants of health outcomes across domains, and that psychiatry is no exception (Paris, 2020). Just as sanitary reforms and vaccination laws required political will, resources, and accountability, so too does the prevention of coercion in psychiatry depend less on clinical acuity than on structural commitment to human rights and institutional reform (P. Farmer, 2003).

The tension between Trieste's achievements and the persistence of coercion elsewhere, where pressures continue to push for the reversal of its model, reflects a deeper systemic contradiction. Psychiatry has already demonstrated that rights-based, dialogic, and non-coercive approaches are viable, yet, the field continues to reproduce exclusionary and violent practices. This paradox cannot be explained solely by clinical caution or resource limitations; it is rooted in longstanding societal attitudes that cast those deemed weak, *feeble-minded*, or dependent as burdens, unworthy of protection, and even despicable. Historically, such populations were targeted by eugenic policies and custodial segregation, and in contemporary contexts they remain disproportionately subjected to coercive psychiatry, despite often being victims of abuse, discrimination, or the *crossfire* of social violence rather than intrinsically ill (L. Whitaker et al., 2021). The persistence of coercion therefore exemplifies not just institutional inertia but the endurance of cultural logics that legitimate control and neglect over solidarity and care, allowing psychiatry to function as a vehicle of societal loathing rather than a guarantor of rights and recovery. This contradiction is central to understanding the significance of Trieste in contemporary debates, as it illuminates both the possibilities of reform and the enduring structural barriers to its global adoption.

The Trieste model has been widely recognized as one of the few psychiatric systems explicitly designed around the principles of human rights and community inclusion. Its reform dismantled the asylum not only as a building but as an institutional logic, replacing coercion with participation and exclusion with citizenship (Chenoweth,

1995a). In practice, this meant that the protection of autonomy and dignity became organizing principles of care, with services structured to ensure that individuals with psychosocial disabilities were not treated as passive recipients of medical authority but as active participants in their own recovery pathways. This vision directly anticipated international frameworks such as the United Nations Convention on the Rights of Persons with Disabilities, which affirms the right to legal capacity, supported decision-making, and freedom from coercive treatment (Carr, 1988).

Shared decision-making is central to Trieste's approach. Rather than framing treatment choices as clinical judgments imposed by experts, decisions are negotiated with service users and their families, with attention to context, preferences, and long-term goals (Okin, 2020). This approach rejects the pharmacocentric bias that dominates psychiatric practice in much of Europe, where deprescription, withdrawal planning, or psychosocial alternatives are often disregarded (Double, 2002). Ethnographic observation confirmed that in Trieste, professional authority is explicitly reframed: clinicians view themselves as facilitators of dialogue rather than gatekeepers of medical knowledge. This orientation not only promotes adherence but strengthens trust, mitigating the sense of institutional betrayal frequently described by survivors of coercion in other contexts (Gøtzsche, 2022).

Psychosocial support represents another cornerstone of the Trieste model. Services extend beyond symptom management to address housing, employment, education, and social participation, recognizing these as determinants of mental health equal in importance to clinical care (Compton & Shim, 2015). Community mental health centers are open 24 hours and embedded in local neighborhoods, designed to be accessible, welcoming, and integrated into ordinary life. The emphasis on continuity of relationships rather than episodic crisis management reflects a recognition that recovery is a long-term process requiring stability, solidarity, and consistent support. Such practices are aligned with international calls for community-based, rights-oriented services that replace coercion with voluntary, person-centered care (World Health Organization, 2025a).

The Trieste model also illustrates how dialogic and community-oriented practices can function as protective factors against dehumanization. By foregrounding human rights and inclusion, it resists the systemic silencing that characterizes much of psychiatry elsewhere. Comparative ethnographic findings from Spain and other European contexts highlight that in systems where coercion persists, complaints of abuse are often reinterpreted as symptoms, thereby denying victims credibility

(Blayney, 2022). In contrast, Trieste's services actively incorporate user perspectives as legitimate contributions to treatment planning, fostering environments in which testimony is validated rather than suppressed.

Practitioners emphasized concrete mechanisms of fragility: (1) Austerity reduces staffing ratios that enable relational safety and home outreach; (2) Managerial performance regimes privilege throughput metrics over relationship-based care, nudging services toward risk-averse admission; (3) Biomedical capture, through guidelines, procurement, and training, re-centers pharmacology as default, crowding out dialogic work and social interventions; and (4) Erosion of intersectoral compacts with housing, employment, and social cooperatives undermines the non-clinical pillars that make coercion unnecessary (Woodhall-Melnik & Dunn, 2016). These pressures explain how re-medicalization advances even without formal policy reversals. Economic austerity, political shifts, and professional cultures influenced by pharmacological industries continue to exert pressure toward re-medicalization and forced treatment (Lorant et al., 2007). As a result, Trieste represents both an accomplished reform and a contested frontier: its practices embody the principles of shared decision-making and psychosocial support, yet, they require continuous defense against systemic tendencies toward coercion.

The fieldwork in Trieste, when triangulated with survey data and ethnographic evidence from Spain, highlights the persistence of coercion and silencing across European psychiatric systems and underscores how exceptional Trieste remains. While the Trieste model demonstrates that rights-based, dialogic psychiatry is feasible, survey results in Spain revealed that forced treatment, pharmacological continuity, and professional deflection remain dominant responses even in situations where reversible somatic contributors or clear alternatives exist (Gooding et al., 2020). Survivors described psychiatric labels and drug regimens as mechanisms that erode trust and entrench dependency, while professionals acknowledged systemic barriers but often refrained from assuming responsibility for implementing rights-based practices (DeMatteo et al., 2013).

This triangulation illustrates a stark contrast. In Trieste, open-door services, dialogic engagement, and user participation have been institutionalized as routine practices (Herrman et al., 2022). In Spain and elsewhere, however, coercion is rationalized as necessary for safety or framed as inevitable given systemic barriers. This difference is not attributable to unique clinical conditions but to structural and cultural choices about how psychiatry is organized. Research shows that systems that normalize coercion tend to displace responsibility: clinicians cite time pressures, institutional protocols, or resource shortages rather than acknowledging their own agency in

perpetuating coercive practices (Beviá & Girón, 2017). In such contexts, silencing is not incidental but built into professional discourse.

Silencing operates through multiple mechanisms. Diagnostic overshadowing reframes testimonies of abuse as symptoms of illness, denying their credibility and foreclosing their legal recognition (Bueter, 2023). Institutional betrayal occurs when health and legal systems, expected to provide protection, instead perpetuate or conceal harm (Goffman, 1958). Financial and political pressures reinforce pharmacological continuity, creating incentives for prescribing rather than for investment in psychosocial alternatives (Rosenhan, 1973). These dynamics converge to ensure that victims of violence, particularly women, migrants, and persons with psychosocial disabilities, are denied access to justice and subjected to further control through psychiatry itself (Nicki, 2001).

Ethnographic engagement in Trieste highlighted the fragility of its achievements. The researcher conversation with Franco Rotelli emphasized that the true meaning of deinstitutionalization was not the closure of a building but the transformation of relationships and power structures; once vigilance wanes, coercive tendencies re-emerge (Río Pedraza, 2022). Roberto Mezzina stressed that even in Trieste, the survival of democratic psychiatry is not guaranteed, as austerity measures and political changes risk undermining rights-based practices (Mezzina, 2020). This aligns with international findings showing that progressive reforms often coexist with residual coercion, regressive interests, and are always vulnerable to rollback under economic or political pressure (Brogaard, 2020).

The triangulation of Trieste with Spanish and international data therefore points to two simultaneous realities: first, that psychiatry can be reoriented around human rights, dignity, and shared decision-making; and second, that coercion remains structurally embedded in most systems, reinforced by silencing, fragmentation, and financial incentives. This duality underscores why Trieste is celebrated as an emblematic model, but also why it continues to face existential challenges in a psychiatric landscape where institutionalized oppression remains the norm.

Trieste represents more than a local reform; it functions as an ethical counterpoint to the prevailing realities of psychiatric governance. Its historical closure of the asylum and the establishment of open-door, community-based services demonstrated that coercion is not a therapeutic necessity but a political and institutional choice (Cipriano, 2025). By institutionalizing shared decision-making, community integration, and psychosocial support, Trieste challenged the assumption that persons with psychosocial disabilities must be segregated, medicated, or silenced.

In doing so, it offered empirical evidence that rights-based approaches are viable, effective, and socially transformative (Sashidharan et al., 2019).

Set against the wider European context, Trieste underscores the persistence of what can be described as institutionalized hatred: the systemic devaluation of disabled persons, the elderly, women, migrants, and other marginalized groups who remain subject to coercion and exclusion. Ethnographic findings from Spain and other contexts revealed how psychiatric practices frequently reinforce structural violence by reframing the consequences of abuse, poverty, or discrimination as psychiatric disorders requiring pharmacological containment (Lobo, 2016). Trieste's model counters this dynamic by making human rights and citizenship, not control, the central organizing principles of care.

Nevertheless, the very existence of Trieste as a global reference point highlights the fragility of democratic psychiatry. Professionals interviewed during fieldwork, including Roberto Mezzina and Franco Rotelli, repeatedly stressed that reforms cannot be considered complete but require constant vigilance. Political changes, austerity policies, and biomedical pressures threaten to erode the foundations of community-based, non-coercive care (Davis et al., 2024). Without legal safeguards and sustained investment, the gains of deinstitutionalization risk being reversed, as demonstrated by the growing re-medicalization trends across Europe and the resurgence of forced drugging practices (Frances, 2013).

Trieste thus embodies both accomplishment and warning. It affirms that psychiatry can operate without coercion and that dignity, autonomy, and social inclusion can be made central to practice. At the same time, it exposes how exceptional such approaches remain, given the systemic pressures that continue to privilege pharmacological continuity, institutional order, and professional authority over human rights. As an ethical counterpoint, Trieste serves as a reminder that psychiatric reform is not merely a technical matter but a profound struggle over the meaning of care, citizenship, and justice. Its continued existence demonstrates that alternatives to coercion are possible, while its vulnerability underscores the ongoing need for political, legal, and cultural commitment to sustain them.

4.3. Fieldwork in Indonesia, on the abolition of shackling

Pasung, or shackling, the physical restraint and confinement in domestic settings of people with psychosocial disabilities, must be situated within a longer history of colonization, structural violence, and the normalization of coercion. The Dutch colonial regime in Indonesia institutionalized psychiatric control as an extension of

colonial authority, establishing asylums that primarily served to confine individuals considered disruptive to social and economic order (J. Read & Sanders, 2010). This early medicalization of social deviance paralleled broader colonial practices of forced labor, racial segregation, and violent discipline, leaving deep scars on local communities. The brutality of colonial systems fractured families, destroyed traditional networks of care, and embedded coercion as a socially sanctioned response to vulnerability.

In this historical context, *pasung* emerged not as a timeless cultural practice but as a survival mechanism forged under systemic abandonment and repression. Families and communities, stripped of resources and left under chronic economic and political subordination, came to normalize confinement as a means of managing distress. Yet, to frame *pasung* as an act of protection obscures its reality: it is a form of torture that strips individuals of liberty, bodily integrity, and dignity (Minas & Diatri, 2008). Anthropological research has shown that such practices become naturalized when violence is routinized in everyday life, making structural brutality appear as common sense (Baklien et al., 2023).

Fieldwork and interviews confirm that *pasung* persists not because individuals with psychosocial disabilities are inherently burdens but because societies shaped by violence treat them as expendable. The language of burden masks the reality that confinement is an abuse of power, reinforced by dehumanization and systemic neglect. Accounts from survivors describe experiences of chains, isolation, and humiliation, echoing testimonies of coercive psychiatry in Europe where forced drugging and involuntary hospitalization are justified under similar rationales of protection and necessity (Brydán, 2016). In both contexts, violence is reframed as care, and crimes are normalized under the authority of family or medical institutions.

Pasung must therefore be understood not as an unfortunate response to the shortage of psychiatrists or medication, but as a crime rooted in historical and systemic violence. International law recognizes it as torture and cruel, inhuman, and degrading treatment (McSherry, 2013b). Abolition is not an aspirational goal but a legal obligation. The comparison with Spain illustrates the continuity of coercion across cultural settings: while in Indonesia violence takes the form of physical confinement, in Spain it is expressed through forced hospitalizations, long-term sedation, and the pathologization of victims of abuse (Carbonell Marqués & Navarro-Pérez, 2019). In both cases, the underlying mentality is the same: the dehumanization of those deemed unfit, the silencing of their testimony, and the normalization of coercion as therapeutic.

The persistence of *pasung* cannot be reduced to local custom or poverty; it exemplifies the carceral logics that surface wherever accountability is absent. Comparative research demonstrates that confinement and coercion, whether in Indonesian villages, U.S. psychiatric hospitals, or British community treatment orders, share a structural logic: individuals deemed disruptive are excluded under the guise of protection (Melzer & Cruel, 2017). This continuity highlights that *pasung* is not an exotic aberration but part of a global repertoire of coercion, where psychiatry and community authority converge to normalize violence as care. The endurance of *pasung* is thus not a matter of resource scarcity alone, but of mentality and power. Psychiatry as an institution has historically reinforced coercion, both in colonial Indonesia and in contemporary Europe. Where coercion is permitted, it expands, cloaked as treatment but functioning as violence. Where it is normalized, it becomes systemic, embedding itself in family, medical, and legal practices. The abolition of *pasung* therefore requires more than service provision; it requires a societal and institutional shift in which coercion is recognized as crime, testimony is validated, and rights-based alternatives are enforced.

The act of shackling, restraining, illegally locking down and mistreating cruelly the other dehumanized, is inseparable from the dynamics of family abuse and systemic violence. Accounts from survivors and fieldwork demonstrate that confinement often originates in domestic contexts marked by neglect, hostility, and coercion. Families and communities that adopt *pasung* frequently justify it as protection or necessity, but this rationale conceals the exercise of power over vulnerable individuals. Research shows that people subjected to *pasung* are disproportionately those who have experienced violence, social death and poverty, and whose testimony is disbelieved or pathologized when they attempt to resist abuse (Metzl, 2010). In this way, *pasung* functions as both a symptom and a reinforcement of broader patterns of structural domination (Reidy, 1993).

Normalization of *pasung* reflects the routinization of cruelty in social life. Anthropological literature has long shown how violence becomes naturalized, perceived as ordinary and necessary, when it is repeatedly enacted and socially sanctioned (Scull, 2022). In Indonesia, decades of authoritarian governance and colonial subjugation entrenched coercion as an accepted mode of regulation. Families under these conditions internalized coercive responses to distress, framing them as moral or protective rather than criminal. Such normalization is reinforced by silence: victims of *pasung* are denied voice, while practitioners and authorities often avoid confronting the criminal dimensions of confinement. Studies reveal that officials, when confronted with cases, frequently classify them as private family

matters rather than violations of law, thereby perpetuating impunity (Karsay & Lewis, 2012).

The parallels with European psychiatry are evident. In Spain (Valencia, 2020), victims of familial abuse have been involuntarily hospitalized or sedated under the authority of psychiatric services, with their testimony dismissed as delusional or symptomatic (Herman, 2015). Here, too, coercion is reframed as care, and systemic violence is rendered invisible by institutional complicity. Both *pasung* and forced hospitalization illustrate how psychiatric authority can be weaponized to reinforce existing hierarchies of power: families and institutions collaborate, whether implicitly or explicitly, in silencing victims and sustaining the conditions of abuse (Penrose, 1999).

The criminal character of *pasung* must be emphasized. International human rights law categorizes prolonged physical restraint and confinement as torture and degrading treatment (Pons et al., 2022). Its persistence reflects not cultural inevitability but the abdication of legal responsibility by states and institutions. By tolerating or excusing family confinement, public authorities participate in what has been described as *institutional betrayal*, the failure of institutions entrusted with protection to prevent or address harm (Marshall et al., 1994b). This betrayal has direct consequences: individuals confined in *pasung* often suffer permanent physical disabilities, psychological trauma, and social exclusion (Zur, 1994).

The normalization of *pasung* and its analogues in European psychiatry demonstrate that coercion is not an aberration but a systemic possibility wherever accountability is absent. In both Indonesia and Spain, the lack of effective legal enforcement, combined with entrenched stigmatizing cultural prejudices, enables the perpetuation of practices that are in clear violation of international law. Abolition therefore requires not only services and resources but a fundamental shift in mentality: recognition that confinement and forced drugging are crimes, not treatments, and that their persistence reflects complicity, not necessity. Fieldwork has shown how many of the medical doctors involved do not even know mental breakdown is an unavoidable human physiological reaction to prolonged abuses or acute torture.

The persistence of *pasung* must be analyzed not only as a social practice but as a violation of law and an urgent matter for abolition. Under international human rights frameworks, the confinement of persons with psychosocial disabilities in chains, locked rooms, or other forms of physical restraint constitutes torture and degrading treatment (Annas, 2005). States are legally obliged to prohibit, prevent, and prosecute such practices. Yet, despite Indonesia's ratification of the Convention

on the Rights of Persons with Disabilities in 2011, *pasung* remains widespread, demonstrating the gap between international commitments and domestic enforcement (A. J. Hunt et al., 2023).

From a legal standpoint, *pasung* is not merely a cultural relic but a crime of omission and commission. Omission occurs when authorities fail to provide accessible services and protections, leaving families to impose confinement. Commission occurs when local officials tacitly condone or fail to prosecute acts of physical restraint, thereby legitimizing abuse through inaction (Jorgensen, 2009). This dual responsibility underscores that abolition cannot be achieved by isolated awareness campaigns or clinical interventions but requires enforceable legal mechanisms, systematic monitoring, and accountability for perpetrators.

The ethical case for abolition is equally compelling. Philosophical and medical anthropology perspectives argue that coercion in psychiatry undermines autonomy, dignity, and relational integrity, producing lasting harms beyond the immediate period of confinement (R. M. Martínez, 2024). Survivors of *pasung* report trauma, harming, and physical injury, often compounded by loss of trust in institutions and communities (Laila et al., 2018). These accounts mirror testimonies of survivors of forced hospitalization and drugging in Europe, where victims describe being silenced, pathologized, and prevented from seeking justice (Blayney, 2022). Whether expressed through chains in Indonesia or neuroleptics in Spain, the underlying violation is the same: coercion normalized as treatment, with victims retraumatized and handed back to the perpetrators.

The global literature further shows that when coercion is permitted in any form, it becomes systemically entrenched. In psychiatry, the legal tolerance of involuntary treatment provides a framework through which family and institutional violence can be justified. This has been documented not only in Indonesia and Spain but across Europe, North America, and Latin America, where coercion is frequently justified under the language of safety and necessity (Pridmore, 2010). The result is what scholars have termed *institutionalized hatred*: the systemic devaluation of those considered unfit or unworthy of full citizenship (Witten & Eyler, 1999).

Abolition therefore requires recognition that *pasung* and coercive psychiatry are not separate phenomena but part of a continuum of violence. While the methods differ, chains and locked rooms in rural Indonesia, forced injections and prolonged sedation in Spanish hospitals, the principles are the same: testimony is discredited, autonomy is denied, and violence is reframed as care. International bodies have recommended replacing coercive practices with rights-based, community best practices. This aligns with calls by the World Health Organization and the United

Nations for the replacement of coercive psychiatric practices with rights-based, community-grounded alternatives (World Health Organization, 2021a).

From the standpoint of fieldwork, this imperative is reinforced by lived experience. Victims of coercion, whether in Indonesia or Europe, are not passive recipients of treatment but survivors of systemic violence. Their testimonies expose the depth of harm inflicted and challenge the institutional narratives that portray coercion as benevolent. Some scholars and advocacy reports frame these practices in criminal terms; this thesis examines the implications of such framings for service design.

The ethnographic parallels between *pasung* in Indonesia and coercive psychiatry in Europe reveal a continuum of systemic violence, differentiated by form but united in logic. In Indonesia, confinement is overt: chains, wooden stocks, and locked rooms render visible the brutality of coercion (J. M. Price, 2012). In Spain, coercion is often disguised as treatment: involuntary hospitalizations, forced injections, and prolonged sedation operate under the authority of medical professionals, with violence reframed as clinical necessity (Aaddam, 2024). Despite these differences, both practices silence victims, protect perpetrators, and sustain institutions by presenting control as care. Although reform initiatives exist, their impact is still limited.

Fieldwork findings illustrate that both *pasung* and European coercion emerge from contexts where violence is normalized and accountability absent. In Indonesia, families justify *pasung* as protection from the outside world danger, or shame, while officials treat confinement as a private matter outside the reach of criminal law (Champion, 1998a). In Spain, professionals justify forced drugging as risk management, while judicial systems defer to medical discretion, even when there is evidence of interpersonal violence or structural abuse (Gastaldo, 2010). In both cases, those subjected to coercion are reframed as disordered, their testimonies dismissed as unreliable, and their suffering rendered invisible. These dynamics suggest a need for system-level change across clinical, legal, and community interfaces (Beviá & Girón, 2017).

Anthropological scholarship shows that such normalization is not culturally specific but structurally embedded. Practices of confinement and forced treatment persist wherever institutions prioritize order and control over protection and rights (Breckenridge, 2020). Coercion is reinforced when families, professionals, and authorities collude, actively or passively, to silence victims. This collusion produces what has been termed *institutional betrayal*, in which institutions mandated to provide care instead reproduce or conceal violence (Katuuk et al., 2019). The betrayal is amplified by epistemic injustice: the denial of credibility to victim

testimony, reclassified as symptom, and excluded from legal recognition (Darke et al., 2025).

The comparison also reveals shared drivers of coercion. Both Indonesian *pasung* and European forced psychiatry are enabled by systemic fragmentation, indifference to harm, and financial interests. In Spain, pharmaceutical influence and institutional inertia perpetuate pharmacological continuity, disincentivizing investment in psychosocial alternatives (Cosgrove & Wheeler, 2013). In Indonesia, under-resourced systems and local political complicity perpetuate physical confinement. In both settings, abusers exploit these conditions: families, institutions, and even states use psychiatry as a mechanism to control, silence, or dispose of individuals deemed burdensome (Voren, 2010).

Crucially, these practices are not only abuses of care but obstructions of justice. Survivors of *pasung* are often denied legal recourse, while European victims of coercive psychiatry face litigation abuse, procedural silencing, and systemic disbelief (Trebilcock & Elliott, 2001). Both contexts demonstrate how coercion sustains impunity: perpetrators, whether families confining relatives in Indonesia or professionals enforcing sedation in Spain, are shielded by social acceptance and legal inaction.

The ethnographic comparison underscores that coercion is neither inevitable nor culturally determined. It thrives where violence is normalized, accountability is weak, and rights are subordinated to institutional convenience. By juxtaposing *pasung* with European psychiatry, the evidence demonstrates that coercion persists wherever society accepts dehumanization as treatment. Recognizing this continuity is essential for framing abolition not as an Indonesian challenge but as a global imperative.

The efforts to abolish *pasung* illustrate that legal prohibitions alone are insufficient to eliminate systemic violence. Despite the Indonesian ban on *pasung* since 1977 and Spain's adoption of human rights frameworks in mental health strategies, coercion continues, either through overt confinement or through forced hospitalization and drugging (Ulya, 2019). These practices are sustained by colonial legacies of institutionalized control, professional cultures that privilege order and medical authority over testimony, and systemic impunity for violations. Addressing these conditions requires a structured abolitionist strategy that is legally enforceable, socially grounded, and internationally coordinated (Good, Marchira, Subandi, Mediola, et al., 2019).

First, abolition must be anchored in binding international law. The Convention on the Rights of Persons with Disabilities affirms the right to liberty, equal recognition

before the law, and access to community-based supports on a voluntary basis (Richter, 2025). Interpretative guidance by the Committee on the Rights of Persons with Disabilities and the Committee Against Torture has clarified that coercive practices, whether through chains, locked wards, or forced pharmacological interventions, contravene these standards (Pons et al., 2022). Domestic incorporation of these obligations requires not only legislation but also independent monitoring mechanisms, judicial oversight, and sanctions for institutional non-compliance.

Second, abolition requires confronting colonial and postcolonial legacies. Colonial psychiatry in the Dutch East Indies institutionalized coercion as both a disciplinary mechanism and a tool of governance (B. M. Cohen, 2020). The imprint of this history remains visible in the normalization of *pasung* and in global psychiatric models that emphasize containment and medicalization over social and rights-based approaches. Contemporary global mental health initiatives, while framed as advancing care in low- and middle-income countries, risk reproducing these patterns by emphasizing scale-up of pharmacological interventions without addressing systemic violence, poverty, and structural exclusion (Joseph, 2015). Abolitionist strategies must therefore reject the adoption of coercive biomedical models as a default and prioritize community-led, culturally grounded, and rights-based alternatives.

Third, reform must tackle professional complicity and institutional betrayal. Research shows that coercion persists not only because of resource scarcity but because of entrenched professional practices that silence victims and reinterpret abuse as clinical necessity (Titchkosky & Aubrecht, 2017). Abolition requires comprehensive retraining for health workers, the establishment of accountability frameworks that sanction violations, and the inclusion of survivor-led monitoring. Evidence from non-coercive models, such as Trieste's community psychiatry in Italy, confirms that rights-based services can be scaled without compromising safety or effectiveness (Basauri, 2020).

The pursue of abolition of torture in mental health must be understood as part of broader struggles for justice and human rights. The elimination of *pasung* and coercive psychiatry is inseparable from confronting the structural drivers of violence, poverty, dehumanization, discrimination, and exclusion, that make coercion appear acceptable. It requires recognition that systems of control have been normalized for centuries, through colonial governance, professional authority, and institutional neglect. Only by situating *pasung* within this continuum of systemic abuse can abolition be pursued effectively.

In this light, the argument is clear: no society can credibly claim to respect human rights while tolerating practices that reduce individuals to objects of control. The abolition of *pasung* and coercive psychiatry is the only trajectory consistent with both scientific evidence and international law. Its implementation demands urgent legal enforcement, survivor participation, accountability for institutions and professionals, and sustained investment in voluntary, community-based alternatives.

The evidence presented across contexts demonstrates that *pasung* in Indonesia is not an isolated practice but an archetype of how coercion becomes normalized wherever violence is reframed as care (Beresford & Rose, 2023). Whether through chains and confinement in rural communities or through forced hospitalization and long-term sedation in European psychiatry, the underlying dynamic is consistent: individuals with psychosocial disabilities are reduced to objects of control, their testimony is discounted, and systemic violence is perpetuated under institutional legitimacy (Mills, 2014).

In comparative perspective, *pasung* functions as a global example of the consequences of coercive mental health systems. Its visibility makes explicit what is often obscured in highly medicalized contexts: coercion is not therapeutic necessity but systemic violence, sustained by dehumanization, institutional betrayal, and impunity. Scientific evidence confirms that coercion undermines therapeutic outcomes, damages trust in services, and produces lasting trauma (Sweeney et al., 2018). International legal standards, particularly the Convention on the Rights of Persons with Disabilities, establish unequivocally that coercion in psychiatry is incompatible with human rights and must be replaced with voluntary, community-based alternatives (Minkowitz, 2010).

The abolition of *pasung* and of coercive psychiatry more broadly is therefore not an aspirational goal but a requirement consistent with both evidence and law. Effective abolition demands enforceable legal frameworks, survivor participation in oversight, accountability for institutions and professionals, and sustained investment in rights-based services (Pinilla-Roncancio et al., 2020). Confronting colonial legacies, professional complicities, and systemic impunity is central to this process, as these factors have historically normalized violence as an acceptable response to vulnerability (Edington, 2019).

In this light, the lesson is universal: no society can credibly claim to uphold human rights while tolerating practices that subject people to coercion and control. *Pasung* is an extreme manifestation, but the logic that sustains it is present wherever psychiatry is used to silence rather than to support. Spain exemplifies this continuity,

where coercive psychiatry has been instrumentalized to pathologize victims of abuse, conceal violence, and enforce silence through forced drugging and institutional complicity (Aaddam, 2024). The torture I endured within these systems is part of this same continuum, illustrating how coercion operates to deny justice and perpetuate impunity.

The abolition of coercive psychiatry, in Indonesia, Spain, and beyond, is inseparable from broader struggles for justice and human rights. It requires recognition that coercion is not a marginal problem but a systemic feature of psychiatric governance, reinforced by history and normalized by institutions. Ending these practices is both a scientific and a legal necessity, and it remains an unfinished task until survivors receive justice and systems are restructured to prevent further harm.

4.4. Closing remarks on violence and psychiatric care

Five years of ethnographic immersion in Spain, undertaken under conditions of coercion and familial abuse, yielded direct evidence of how psychiatric systems reproduce structural violence. The fieldwork demonstrated that coercion is not an incidental occurrence but a constitutive feature of psychiatric practice, sustained by broad professional discretion, weak legal oversight, and systemic avoidance of accountability (Nakray et al., 2015). As a victim of both domestic abuse and psychiatric coercion, the researcher documented how psychiatric authority was mobilized to pathologize resistance, dismiss testimony, and impose pharmacological control. These mechanisms align with the concept of *epistemic injustice*, where victims' credibility is systematically undermined and their reports reframed as symptomatic (Nordstrom & Robben, 1995).

Psychiatry was deeply entangled in Franco's biopolitical project, framing dissent as degeneration and embedding custodial logics into medical authority (Staub, 1989). The persistence of broad discretion, weak rights enforcement, and pharmacocentric dominance reflects this continuity: coercion is not an accidental excess but a structural inheritance of a system historically aligned with repression rather than protection. Although presented as a modern welfare democracy, Spain's psychiatric governance remains shaped by unresolved legacies of authoritarianism. Transitional justice scholars emphasize that institutions which fail to reckon with past repression tend to reproduce cycles of impunity and silence (Vesti & Lavik, 1991). Medical knowledge and all empirical scientific evidence confirms that coercion undermines therapeutic outcomes, increases trauma, and erodes trust in services. Reviews of involuntary admission show poorer satisfaction, higher relapse rates, and persistent perceptions of injustice (Baklien et al., 2023). Spanish research echoes these

findings, documenting systemic overreliance on psychotropic medication, institutional inertia, and the silencing of abuse disclosures (Breilh Paz y Miño, 1993). Institutional betrayal was evident not only in clinical contexts but also in legal ones: institutions tasked with protecting victims of violence frequently defer to medical authority set to pathologize the sufferers, perpetuating cycles of abuse (Annas, 2005).

The ethnographic record thus situates coercive psychiatry in Spain as part of a broader system of structural impunity, where law is symbolically invoked but rarely enforced. By documenting these dynamics from within, the research provides empirical confirmation of how psychiatric systems function as mechanisms of silencing and control, reinforcing rather than alleviating vulnerability.

Fieldwork in Trieste provided a counterpoint to the Spanish context, illustrating the viability of rights-based, community psychiatry. Trieste's historical closure of the asylum under Franco Basaglia and the establishment of open-door, community mental health centers demonstrated that psychiatry can function without coercion (Basaglia, 1987). The Trieste model situates autonomy, shared decision-making, and psychosocial support at the core of care, replacing segregation and pharmacological dominance with integration and dialogue (Okin, 2020). In the context of the thesis, Trieste functions as empirical validation that rights-based psychiatry is feasible, but also as a warning: without sustained political will, legal enforcement, and cultural change, even the most advanced reforms remain vulnerable to reversal.

The ethnographic engagement with *pasung* in Indonesia situates coercion within a global continuum of systemic violence. Even the harshest tortures in plain sight get to be normalized, culture allowing. *Pasung*, the confinement of people in chains, locked rooms, or stocks, persists despite its prohibition since 1977, with thousands estimated to be affected (Hidayat et al., 2023). Its endurance reflects colonial legacies of psychiatric control, systemic neglect, and the normalization of violence within families and communities under conditions of poverty and social death (Laila et al., 2019).

Scientific evidence is unequivocal that exposure to torture and coercive treatment produces profound psychological, social, and biological consequences, irrespective of prior mental health status. Studies on survivors of torture confirm high rates of post-traumatic stress, depression, and anxiety, alongside disturbances in social functioning and enduring impacts on biological systems, including dysregulation of the hypothalamic-pituitary-adrenal axis and inflammatory processes (Fitzsimons, 2009). These outcomes demonstrate that the harms

associated with torture and coercion are not confined to a subgroup of persons already classified as mentally ill, but are predictable responses to sustained abuse. To assume otherwise is both scientifically unfounded and ethically dangerous, as it obscures the causal role of violence and legitimizes coercion as if it were a response to pre-existing pathology. The consequences of coercion and torture are corroborated across disciplines. Neuroscience identifies lasting alterations in stress-response systems, including dysregulation of cortisol and inflammatory markers (A. G. Bottaccioli et al., 2019). Sociology highlights the erosion of trust and the reproduction of dehumanizing ways where victims are repositioned as permanent outsiders (Runciman, 2023). Public health data confirm that exposure to violence predicts long-term morbidity and premature mortality, independent of psychiatric diagnosis (Leiva-Peña et al., 2021). Together, these findings dismantle the notion that coercion addresses underlying pathology; instead, they show it generates predictable biopsychosocial harm across all levels of analysis (Herrera-Imbroda et al., 2025).

This misattribution is central to the persistence of *pasung* in Indonesia and of coercive psychiatry in Spain. In both contexts, individuals subjected to violence are recast into a new category of permanent otherness. In Indonesia, persons confined in *pasung* are described as inherently disordered, despite evidence that confinement itself produces psychological deterioration and physical disability (Bunston et al., 2017). In Spain, victims of abuse who disclose violence frequently find their testimony reframed as symptomatic, leading to involuntary hospitalization, forced drugging, or lifelong psychiatric labeling (McSherry, 2013b). This dynamic establishes a self-fulfilling cycle: abuse causes harm, harm is reinterpreted as illness, and illness is used to justify further coercion and exclusion.

The result is the systematic relegation of survivors to second-class citizenship. Once pathologized, individuals are denied credibility, legal recognition, and equal participation in society, consistent with the phenomenon of *epistemic injustice* (Reidy, 1993). Anthropological and sociological analyses demonstrate that this process leads to social death, where individuals are dehumanized and gradually erased from civic life (Borgstrom, 2017). Coercive psychiatry thus not only inflicts immediate suffering but institutionalizes long-term marginalization, perpetuating cycles of violence and exclusion (Goodman et al., 2009).

The findings from five years of ethnographic immersion, triangulated with the cases of Trieste and *pasung* in Indonesia, converge on the conclusion that coercion must be abolished as a systemic practice. Shared decision-making, respect for human rights, and the recognition of survivors' testimonies are not optional reforms but

essential safeguards. International evidence confirms that dialogic and voluntary approaches, when implemented with fidelity, produce better outcomes, enhance trust, and prevent retraumatization (Hansen et al., 2014). By contrast, displacing the locus of care and decision-making onto abusers, whether families, institutions, or professionals, perpetuates harm and erodes the possibility of recovery (Speyer et al., 2025).

The scientific and ethical case is clear: coercion, whether in the form of *pasung* or forced psychiatry, is a driver of trauma, exclusion, and systemic injustice. Its abolition is consistent with empirical evidence, legal obligations, and the principles of human dignity. Moving forward, psychiatry and mental health systems must reorient toward shared decision-making, survivor participation, and enforceable rights protections, ensuring that those historically silenced are finally heard. The findings therefore converge on a global conclusion: abolition of coercive psychiatry, including *pasung*, is both scientifically justified and legally mandated. Abolition requires binding legal frameworks, survivor-led monitoring, accountability mechanisms for professionals and institutions, and investment in voluntary, community-based alternatives (Catharina Daulima, 2018). Confronting colonial legacies, institutional betrayals, and professional complicities is central to this process, as these factors have historically normalized coercion as treatment.

The combined evidence from Spain, Trieste, and Indonesia demonstrates that coercion is not inevitable but persists wherever accountability is absent and control is prioritized over rights. No society can credibly claim to uphold human rights while tolerating practices that reduce individuals to objects of control. The abolition of coercive psychiatry is therefore inseparable from broader struggles for justice and requires urgent implementation through enforceable law, cultural transformation, and survivor participation (Ho, 2008).

The attempt to extend fieldwork into Sweden exposed how racism and systemic prejudice can obstruct inquiry and reproduce harm. Despite documented evidence of torture and repeated calls for protection, services and institutions aligned with perpetrators, marking the researcher as inferior and unworthy of recognition. This institutional betrayal foreclosed opportunities to examine internationally celebrated Nordic alternatives, including medication-free wards, housing-first programs, and open dialogue (Padgett et al., 2016). Research on health inequities in Scandinavia confirms that structural xenophobia continues to function as a barrier to equitable care (Schömer, 2016). The obstruction of fieldwork thus became itself ethnographic evidence: even in welfare states lauded for progressive policies, racism and institutional complicity can erase the possibility of rights-based practice and

reproduce cycles of violence (Zakharov et al., 2022). This is not an isolated occurrence but part of a longer historical continuum in which lives deemed unworthy of living have been systematically destroyed under medical, political, and cultural authority (Deland, 1997).

The legacy of Aktion T4, the Nazi program that systematically exterminated persons with disabilities under the guise of medical authority, remains one of the most striking examples of psychiatry's instrumentalization in service of state violence (Timander, 2020). Conceived as a eugenic project to eliminate those deemed life unworthy to lived, it combined pseudo-scientific genetics, administrative rationality, and clinical complicity to justify the killing of more than 200,000 people with disabilities and to establish techniques later applied in the Holocaust (Black, 2012). Far from being an isolated aberration, Aktion T4 crystallized broader twentieth-century trends in biopolitics, where medical expertise was mobilized to define populations as inferior, burdensome, or expendable (Hervik, 2019). This historical record underscores that the normalization of coercion in psychiatry has roots not in therapeutic necessity but in logics of exclusion and domination (Schömer, 2016).

Although formally dismantled after 1945, the structural attitudes that enabled the holocaust and the original Aktion T4 (Friedlander, 1995), a peak in ableism and eugenic reasoning, and the framing of vulnerable groups as social burdens- did not disappear (Mulinari & Neergaard, 2012). They persist in subtler but still damaging forms, including the pathologization of dissent, over-medicalization of distress, and systemic denial of equal citizenship for persons with psychosocial disabilities (Karsay & Lewis, 2012). In the Nordic countries, often celebrated internationally for welfare innovation, scholars have documented how structural xenophobia and psychiatric control reproduce exclusionary dynamics, particularly against migrants and marginalized minorities (J. L. Price et al., 2022). The continuity between past exterminatory practices and present forms of coercion lies not in identical techniques but in a shared logic of devaluation: individuals framed as unfit, deviant, or burdensome are denied autonomy and subjected to control under the legitimating discourse of care. This genealogy illustrates why the abolition of coercion in psychiatry must be treated as a historical responsibility as well as a scientific and legal imperative (Shim, 2021).

In Spain, the scars of fratricidal civil war left institutions that too often reproduce cycles of repression rather than protection (Palerm, 2013). Current epistemological models of care remain closer than acknowledged to the legacy of dictatorial psychiatry, which pathologized political dissent through theories of biological degeneracy (Desviat, 2020). During the dictatorship, psychiatrists promoted the

idea of a so-called *red gene*, a hereditary marker allegedly predisposing left-wing opponents to criminality, madness, or moral inferiority (Vázquez, 2024). These pseudo-genetic constructs were used to justify repression, medicalize resistance, and frame dissenters as biologically unfit for civic life, intertwining psychiatric authority with state violence. Contemporary practices, though articulated in the language of genetics and neuroscience, often reproduce similar reductionist logics. By attributing psychiatric distress primarily to hereditary or molecular causes, such models obscure the causal role of trauma, poverty, and systemic abuse, and shift attention away from lived histories of normalized violence (Cea Madrid & Castillo Parada, 2018).

The researcher's own experience reflects this continuity: within the family setting, allegations of madness were mobilized to discredit, control, and inflict abuse with impunity. Such dynamics are neither rare nor idiosyncratic. Studies in Spain and internationally show that families and institutions frequently deploy psychiatric labels to delegitimize victims of violence, thereby normalizing dehumanization and silencing avenues for justice (Sampson, 2021). Entire communities can be subjected to these combined forces of domestic and structural violence, where abusive relatives are shielded by institutional complicity and where coercion is defended as necessary care. The result is the perpetuation of brutality and inhuman treatment under conditions that foreclose opportunities for recovery and systematically deny justice (Ridgway, 2011).

In Indonesia, the struggle for independence in the mid-twentieth century was preceded and shaped by centuries of Dutch colonial rule that normalized coercion and brutality as governing principles. Colonial authorities relied not only on military repression but also on systems of forced labor (*cultuurstelsel*), corporal punishment, and public executions, practices designed to instill obedience through fear (Fernando, 2004). These practices were frequently enforced by indigenous intermediaries, local chiefs, militias, and police trained and rewarded by the Dutch, who became instrumental in administering violence within their own communities (Patterson, 2018). This strategy of delegating brutality created deep social divisions, embedding coercion into everyday life and legitimizing violence as an acceptable means of maintaining order.

The psychiatric dimension of colonialism reinforced these dynamics. Mental asylums established in the Dutch East Indies functioned less as therapeutic spaces than as instruments of confinement, designed to remove individuals considered disruptive to colonial authority or economic productivity (V. Brown, 2009). In this environment, coercion became naturalized not only as state policy but also as a

community response to distress, contributing to the persistence of practices such as *pasung*. Anthropological research demonstrates how, over time, exposure to systemic violence reshaped cultural repertoires, leading to the internalization of brutality as a legitimate means of regulating social life (Metzl, 2010).

The cumulative effect of these colonial structures was the normalization of torture and coercion within communities themselves. By embedding violent discipline into local hierarchies and everyday governance, colonial authorities ensured that practices of abuse could persist even after independence, reappearing in the form of *pasung* as a socially sanctioned response to vulnerability. In this sense, *pasung* is not an isolated cultural custom but the legacy of a broader historical process in which colonial domination institutionalized coercion, divided communities, and legitimized inhuman treatment as ordinary practice.

Placed in this wider frame, the ethnographic findings of this dissertation demonstrate that coercive psychiatry cannot be understood solely as a clinical or national phenomenon. It is embedded in broader sociocultural and geopolitical histories of exclusion, racial supremacy, and institutional violence. The structural reliance on coercion in Spain, the contested legacy of reform in Trieste, and the persistence of *pasung* in Indonesia are not disparate cases but interconnected expressions of a global struggle over dignity, rights, and recognition. The historical record shows that whenever coercion is normalized, it expands, whether through extermination programs, political repression, or the medicalization of dissent. The imperative today is to ensure that such lessons are not lost: abolition of coercion and the implementation of rights-based alternatives represent not only a scientific and legal necessity, but a historical responsibility to prevent repetition of past atrocities.

The enduring challenge lies in the recognition that cruelty is not merely a deviation but a recurrent feature of human behavior, sustained by social structures that reward domination and normalize betrayal. Research in social psychology and anthropology has consistently shown that ordinary individuals can participate in systemic violence when it is legitimized by institutions, justified through ideology, or masked as necessity (Brogaard, 2020). Genetic studies further suggest that while there is no *gene for cruelty*, biological predispositions for in-group bias, fear responses, and aggression can be amplified or suppressed by cultural, legal, and institutional contexts (van Es, 2017). When unchecked, these dynamics allow the worst aspects of the human animal to surface, cloaked in the language of science, law, or medicine, and turned into instruments of exclusion.

The danger, therefore, is not abstract but immediate: the everyday reproduction of betrayal, dehumanization, and destruction of life under the guise of care or order.

Such processes reveal how scientific and legal frameworks can be perverted into mechanisms of violence when ethical safeguards are absent. Recognizing this reality underscores that the abolition of coercion is not only about protecting those directly subjected to psychiatric violence but about defending the very foundations of justice and civilization. It requires constant vigilance to ensure that the worst tendencies of human social life, domination, cruelty, and indifference to suffering, are not legitimized as therapeutic or rational. Preventing the reproduction of coercion requires structural interventions at educational and institutional levels. Evidence from social psychology demonstrates that prejudice and cruelty are not fixed traits but socially learned and reinforced (McSherry, 2013b). Public health research confirms that the greatest improvements in health outcomes arise from institutional design, clean water, vaccination, tobacco control, rather than individual clinical acts (Cutler & Miller, 2005). Psychiatry is no exception: reducing coercion depends less on clinical acuity than on legal enforcement, institutional accountability, and cultural transformation. The recently funded EU BEACON One Health Education COST action, developed during this research as the main outcome of the thesis' action-research, embodies such an approach by embedding rights and empathy into education, retraining, and professional practice, ensuring that cruelty is denied the institutional scaffolding it requires to persist (Conn, 2017)

Chapter 5 - Normalized violence and dehumanization

5. Introduction

The present chapter provides a comprehensive synthesis of findings derived from ethnographic research, professional and user surveys, and their triangulation with Spanish and international scientific literature. Its purpose is to situate the empirical material within an analytical framework that explains the persistence of coercion, the erosion of subjectivity, and the limited uptake of preventive and rights-based approaches to mental health care, despite decades of reform and repeated recommendations from international bodies (Luyten & Knapp, 2017). Whereas previous chapters presented the voices of survivors, professionals, and institutional actors in detail, this section moves beyond descriptive analysis to establish the structural, biomedical, and epistemological mechanisms that underpin psychiatric practices as they are enacted today.

The central argument advanced here is that coercion and over-medication are not isolated phenomena but systemic outcomes produced at the intersection of cultural norms, institutional inertia, and epistemic conservatism. Decades of psychiatric reform, in Spain and internationally, have not succeeded in eliminating coercive measures or in embedding shared decision-making as a binding clinical standard (Pozón, 2012). Research across Europe shows that while community care and user participation are formally promoted, coercive interventions such as forced treatment, chemical restraint, and long-term pharmacological continuity remain common in practice (Georgieva et al., 2012). This apparent coexistence reflects what has been described as *structural ambivalence*: institutional logics that simultaneously endorse user-centered rhetoric while reproducing coercive practices as routine responses to crisis (Calcedo-Barba & Fraguas, 2017).

Methodologically, the chapter applies triangulation in a strict sense, cross-referencing survey data (chapter 3), ethnographic accounts (chapter 4), and secondary sources from peer-reviewed literature, national statistics, and international reports. This approach ensures robustness by distinguishing between empirical findings, professional discourses, and normative frameworks, thereby clarifying the gap between what is mandated in law or policy and what is enacted in practice (Chenoweth, 1995b).

Finds show how coercion functions not merely as an ethical or legal issue but as a biomedical, social, and cultural mechanism of harm. Its persistence in Spain and Europe reflects patterns of institutional betrayal, systemic inertia, and epistemic

conservatism that are consistently documented in empirical research (Trebilcock & Elliott, 2001). Addressing these dynamics requires enforceable rights protections, accountability mechanisms, and investment in evidence-based, voluntary alternatives consistent with international obligations such as the Convention on the Rights of Persons with Disabilities (R. B. Del Barrio, 2004). Section 5.1 addresses the cultural normalization of coercion and the obstacles to preventive care, demonstrating how practices such as involuntary treatment, seclusion, and limited access to deprescription become institutionalized and transmitted across professional and familial contexts (Bezanilla et al., 2016). Section 5.2 examines the erosion of subjectivity and the insufficient implementation of developmental, personalized, and trauma-informed care. Evidence from psychoneuroimmunology, metabolic psychiatry, and pharmacoepidemiology highlights the biological consequences of silencing patient perspectives and maintaining pharmacological continuity (de Mooij et al., 2019). Section 5.3 analyzes structural delegation, showing how coercion is displaced onto schools, families, and social policy frameworks. This section emphasizes how gaps in education and social protection perpetuate systemic reliance on psychiatric containment and outlines evidence-based strategies to reduce coercion through cross-sectoral interventions (Vinkers et al., 2024).

Each thematic axis is supported by tables that synthesize empirical results and peer-reviewed evidence. These serve as visual summaries of mechanisms at work and demonstrate the triangulation process, with each entry linked to documented sources. The chapter concludes with a synthesis that integrates the three axes into a coherent argument: coercion is not merely an ethical or political problem but a biomedical, social, and cultural mechanism of harm. Its persistence in Spain and Europe reflects institutional betrayal, systemic inertia, and epistemic conservatism, all of which are scientifically documented and legally remediable (Molodynski et al., 2010).

5.1. A culture of normalized violence masked as care

The normalization of coercion in psychiatry and mental health services represents a central contradiction between clinical practice, scientific evidence, and international human rights frameworks. While coercion is officially defined as an exceptional intervention to be applied only under conditions of strict necessity, empirical evidence indicates that it has become a routinized and culturally embedded practice in diverse health systems, including Spain (Ugartechea., 2004). The persistence of seclusion, forced medication, and prolonged involuntary

hospitalization is not supported by superior clinical outcomes (Ghane et al., 2021). Instead, it is sustained by institutional inertia, administrative rationalization, and cultural patterns that frame coercion as a protective necessity for clinical management or public safety (Seng et al., 2012).

Normalization operates through multiple mechanisms. At the institutional level, bureaucratic language such as *risk management*, *therapeutic necessity*, or *noncompliance* functions to obscure the violence inherent in coercive practices (Staub, 1999). At the professional level, retraining and transmission of tacit knowledge accustom new clinicians to viewing coercion as a legitimate and even inevitable part of psychiatric work, a process reinforced by defensive medicine and liability avoidance (Argyris, 1990). At the societal level, public discourse and media representations often frame psychiatric coercion as synonymous with safety, portraying individuals with severe mental distress primarily as risks to themselves or others, thus creating a narrative that legitimizes restrictions on autonomy as acts of protection (Hansen et al., 2014). These intersecting mechanisms ensure that coercion is not perceived as an ethical breach but as routine care.

Comparative studies confirm that the bureaucratic language of *risk management* and *therapeutic necessity* is not unique to Spain but recurs in psychiatric systems across Europe and North America, where professional liability concerns drive defensive practices (Burstow, 2015a). Media discourse further amplifies this framing, with analyses of press coverage showing disproportionate associations between psychiatric labels and dangerousness, reinforcing public acceptance of coercion as prevention (Seng et al., 2012). These dynamics demonstrate that normalization is sustained by intersecting institutional, professional, and cultural narratives.

The biomedical evidence, however, indicates that coercion is itself a vector of harm. Exposure to coercive environments activates chronic stress responses, with measurable alterations in the hypothalamic-pituitary-adrenal axis, cortisol regulation, and immune functioning (Pescatori, 2024). Longitudinal studies confirm that such dysregulation contributes to increased risk of cardiovascular disease, metabolic syndrome, cognitive impairment, and premature mortality among individuals subjected to long-term coercive treatment (F. Bottaccioli & Bottaccioli, 2025). These effects mirror findings from trauma research, where repeated exposure to uncontrollable and inescapable stressors produces learned helplessness, emotional suppression, and diminished capacity for recovery (Alfonso & Ranganathan, 2025). Thus, coercion not only undermines dignity and autonomy but

directly worsens health outcomes, contradicting the principle of beneficence that underpins medical practice.

Biomedical research consistently demonstrates that coercion constitutes a vector of harm rather than protection. Exposure to coercive environments activates chronic stress responses with measurable alterations in the hypothalamic-pituitary-adrenal axis and immune dysregulation (Bower & Kuhlman, 2023). Longitudinal evidence associates such dysregulation with elevated risks of cardiovascular disease, metabolic syndrome, cognitive decline, and premature mortality (A. G. Bottaccioli et al., 2019). Spanish pharmacoepidemiological studies further document high prevalence of metabolic syndrome and inadequate somatic monitoring among patients exposed to chronic psychotropic regimens (Prado & Añazco, 2024). These findings confirm that coercion produces predictable biomedical harms, directly contradicting the principle of beneficence that underpins clinical practice (Engel, 1967). The persistence of coercion despite such evidence reflects what critical scholars have described as epistemic conservatism: the tendency of psychiatry to privilege established diagnostic and treatment paradigms even when confronted with robust evidence of harm (Motillon-Toudic et al., 2022). Diagnostic overshadowing plays a crucial role here. Once a psychiatric label is applied, alternative explanations for distress, nutritional, metabolic, social, or trauma-related, are systematically deprioritized or dismissed, and coercive pharmacological interventions become the default response (C. M. Palmer, 2025). The surveys conducted for this thesis corroborate this pattern, with professionals frequently citing systemic barriers or institutional constraints as justifications for maintaining long-term medication regimens, while seldom acknowledging their own role in perpetuating coercion. This dynamic illustrates how coercion is sustained not only by structural conditions but by the epistemic cultures of psychiatry itself.

Preventive approaches, in contrast, emphasize relational safety, early education, and voluntary participation as foundations of effective care. Evidence from dialogic and rights-based models, including open dialogue in Finland and its limited but documented application in Spain, demonstrates that preventive and non-coercive strategies can achieve outcomes equal or superior to traditional psychiatric care, with lower relapse rates, greater patient satisfaction, and reduced use of antipsychotics (Vinkers et al., 2024). International organizations, including the World Health Organization and the United Nations, have repeatedly affirmed that coercion is incompatible with a rights-based model of care and have called for its progressive elimination (World Health Organization, 2022b). The scientific case is equally clear: preventive, voluntary, and trauma-informed interventions reduce

morbidity and mortality, whereas coercion exacerbates both. The failure to adopt such models therefore represents not a gap in evidence but a systemic choice to maintain control-oriented paradigms.

In Spain, this contradiction is particularly salient. The Mental Health Action Plan 2022-2026 recognizes the importance of community integration and user participation but does not establish binding mechanisms for abolishing coercion or for guaranteeing the right to deprescription and informed refusal (Sanidad, 2025). National studies document the continued use of mechanical restraints, forced hospitalization, and chemical containment, often in contexts such as elderly care homes, youth protection institutions, and psychiatric hospitals (Medicamentos & Sanitarios, 2019). The normalization of coercion in these settings illustrates how the absence of enforceable legal safeguards allows practices that are scientifically harmful and ethically indefensible to persist under the guise of routine care. Normalization of coercion is not confined to the clinical space but extends across the educational, familial, and cultural domains of Spanish society, where obedience, silence, and authority are prioritized over dialogue and participation (López, 2014). Research in educational psychology and sociology has long documented how early exposure to coercive disciplinary practices fosters acceptance of authority-based control, shaping expectations of care and protection throughout the life course (Foulkes & Stringaris, 2023). In the Spanish context, teachers and caregivers often lack training in trauma-informed approaches, leading to the misinterpretation of behavioural expressions of distress as defiance or pathology. This social learning prepares individuals to accept coercive psychiatric interventions later in life, reproducing a cycle of silencing and exclusion (Dimitrijević, 2020).

Normalization of coercion extends beyond clinical institutions into educational and familial domains. Sociological and psychological studies show that early exposure to authoritarian disciplinary practices fosters acceptance of authority-based control across the life course (Valencia, 2020). In Spain, limited dissemination of trauma-informed approaches in schools leads to behavioural distress being interpreted as defiance or pathology, preparing individuals to accept coercive interventions in adulthood (Katuuk et al., 2019). Families, meanwhile, are often positioned as mediators of coercion: research indicates that relatives may request involuntary admissions, support chemical restraint, or rely on psychiatric authority to manage behavioural difficulties (Szasz, 1998). While not universal, these patterns illustrate how coercion becomes a socially distributed practice, legitimized both domestically and institutionally.

Table 28. Mechanisms of coercion and documented health consequences

Mechanism of coercion	Cultural and institutional normalization	Documented health effects	Sources
Forced medication (acute or chronic)	Framed as therapeutic necessity; reinforced by liability concerns and family requests	Metabolic syndrome, diabetes, cardiovascular risk, tardive dyskinesia, premature mortality	Firth et al., 2019; Mezzina, 2014
Mechanical restraint	Justified as safety; routinized in hospitals and elderly care	Musculoskeletal injury, thromboembolism, post-traumatic stress, mortality risk	Fernández et al., 2018
Seclusion and isolation	Presented as de-escalation; tolerated in institutions	Sensory deprivation, anxiety, cognitive decline, traumatization	Sashidharan et al., 2019; WHO, 2021
Diagnostic overshadowing	Professional discourse dismisses patient testimony; acceptance of psychiatric labels	Misdiagnosis, untreated somatic disorders, chronicity of symptoms	Bueter, 2019
Delegation of families	Families normalize coercion to management; institutions legitimize requests	Dependency, silencing, intergenerational trauma	Anguita, 2005

This table presents forced measures routinely applied and mechanisms of coercion, based on survey findings, ethnographic work and peer-reviewed literature.

The media further reinforces these dynamics by perpetuating narratives of dangerousness and risk. Spanish press coverage of psychiatric incidents disproportionately emphasizes violence and unpredictability, framing individuals with psychiatric labels as threats to public safety (Villarroel, García, et al., 2021). This representation legitimizes coercion as a preventive necessity, obscuring the systemic violence embedded in psychiatric practices themselves. In this way, cultural reproduction of coercion ensures its endurance despite international guidance to the contrary (Sashidharan et al., 2019).

From a biomedical standpoint, the health consequences of coercion have been rigorously documented. Repeated exposure to coercive psychiatric environments induces sustained dysregulation of stress responses, immune dysfunction, and elevated risks of chronic illness (Ghane et al., 2021). Forced medication and long-term use of antipsychotics further exacerbate morbidity, particularly through metabolic and cardiovascular pathways (Firth et al., 2019). Spanish studies confirm high prevalence of metabolic syndrome and inadequate monitoring in patients subjected to chronic psychotropic exposure, with significant regional disparities in screening practices (Moreno et al., 2006). The normalization of coercion thus

produces not only ethical and social harms but measurable biomedical damage that contradicts both the principles of evidence-based medicine and the obligations of human rights law (Faissner et al., 2025a).

The routinization of coercive practices in psychiatry has been repeatedly documented in Spain and across Europe, revealing how measures justified as therapeutic often normalize violence and erode trust. Recent Spanish studies show that mechanical restraint continues to be applied despite formal commitments to reduction, with nurses and patients alike reporting it as dehumanizing (Beviá & Girón, 2017). Systematic reviews underline that these practices are rarely evaluated against human rights standards, producing what Bernheim (Bernheim, 2025) terms the paradox of rights frameworks that obscure violence rather than prevent it. Internationally, ethnographies confirm that the embedding of restraint and seclusion in daily care produces a culture where force is naturalized, blurring distinctions between treatment and punishment (Reich, 2014).

The limited transition to preventive and rights-based care ought to be understood as a structural phenomenon rather than a lack of viable alternatives. Evidence from open dialogue in Finland, recovery-oriented community psychiatry in Italy, and WHO pilot programs demonstrates that dialogic and voluntary strategies achieve equal or superior outcomes to coercive models, with lower relapse rates, higher satisfaction, and reduced pharmacological burden (Chmielowska et al., 2022). In Spain (García-Valdecasas Campelo et al., 2009), however, institutional conservatism, professional training gaps, and political inertia have impeded their uptake. Surveys conducted for this dissertation revealed that even when faced with reversible somatic contributors, professionals frequently cited systemic constraints as justification for pharmacological continuation, reflecting epistemic conservatism rather than evidentiary uncertainty (Dotson, 2012).

5.2. Erosion of subjectivity and the failure to provide care

The erosion of subjectivity constitutes one of the most fundamental and persistent harms generated by coercive psychiatric practices. Subjectivity here refers not only to the recognition of the patient's voice and experiential knowledge, but also to the broader capacity of individuals to define, narrate, and negotiate their own lives in the context of health care. Psychiatric systems that privilege diagnostic authority and pharmacological interventions consistently suppress this subjectivity, reframing personal accounts as symptomatic, denying validity to experiential knowledge, and rendering dissent a justification for further coercion (Bandura, 1999).

In Spain, survey findings confirm that long-term psychiatric diagnoses and associated medication regimens are rarely reevaluated systematically, despite legal guarantees of patient rights to information and consent. Professionals themselves acknowledged in the surveys that reviews are often discretionary, dependent on individual initiative rather than binding protocols. This discretionary structure undermines the principle of autonomy, as access to deprescription or diagnostic reassessment depends less on patient choice than on the subjective willingness of the treating professional. The erosion of subjectivity is thus institutionalized, embedded in a culture where authority is vested in the clinician, and the patient's role is reduced to compliance (Winthrop, 1964).

The consequences of this erosion are profound across the life course. In children and adolescents, antipsychotics are often prescribed off-label to manage behavioural difficulties rather than evidence-based psychiatric indications. Spanish and European studies have documented the rising prevalence of psychotropic use in minors, with significant metabolic and neurological risks and limited consideration of developmental needs (Jina-Pettersen, 2022). Subjectivity in these cases is doubly denied: first, through the medicalization of behaviours linked to trauma, poverty, or systemic exclusion; second, through the suppression of the child's voice in treatment decisions. In older adults, neuroleptics are routinely employed in residential care settings for behavioural control, often in the absence of psychiatric diagnoses, reflecting systemic reliance on pharmacological restraint rather than individualized care (Ortiz-Lobo et al., 2017). Here too, subjectivity is silenced, with agitation or resistance reinterpreted as symptoms requiring suppression rather than as expressions of unmet needs or rights.

Erosion of subjectivity emerges when institutional routines deny recognition of patient voice and agency, a pattern increasingly identified as a central harm of coercive psychiatry. Spanish qualitative work shows that patients describe being treated like an object, with disregard for their perspectives compounding trauma (Villena-Jimena et al., 2023). This aligns with broader socio-ethical analyses where individuals equate coercion with invisibility, silencing and dehumanization (Serra, 2025). Human rights studies further demonstrate that patients' testimony of not being listened and loss of dignity is not incidental but structurally reproduced by risk-focused governance focused on blaming and guiltying those already in distress (Goodman et al., 2009).

The erosion of subjectivity is further reinforced by diagnostic overshadowing, a phenomenon whereby psychiatric labels obscure somatic contributors to distress. Nutritional deficiencies, metabolic disorders, endocrine dysfunction, and trauma

responses are frequently overlooked once a psychiatric diagnosis has been applied (Blom, 2024). The surveys reported in Chapter 3 confirm that many professionals default to systemic excuses or vague attributions rather than investigating reversible causes, illustrating how epistemic conservatism perpetuates chronicity and invalidates patient testimony (Tormoen, 2025). Patients who report improvements linked to diet, rest, or social support are often dismissed as lacking insight, further eroding their capacity to define their own health trajectory.

From a biomedical perspective, the suppression of subjectivity has measurable developmental costs. Trauma research demonstrates that when individuals are deprived of agency in the face of distress, stress systems are dysregulated, resulting in long-term impairment of cognitive, emotional, and physiological function (Cornett et al., 2017). Psychoneuroimmunological studies show that loss of control and silencing of personal voice exacerbate inflammation, weaken immune responses, and increase susceptibility to chronic illness (Fryers et al., 2003). In psychiatry, this manifests in diminished treatment engagement, reduced adherence to voluntary interventions, and higher rates of relapse and chronicity (Ghane et al., 2021). The erosion of subjectivity is therefore not only an ethical problem but a biological and developmental one, undermining both immediate recovery and long-term health (Jina-Pettersen, 2022).

The phenomenon of epistemic injustice is central to understanding how psychiatry erodes subjectivity. As theorized by Fricker (Fricker, 2017), epistemic injustice arises when individuals are denied credibility or interpretive authority because of social prejudice or institutional hierarchies. In psychiatry, diagnostic overshadowing exemplifies this process: once a person is categorized under a psychiatric label, their testimony is systematically downgraded, and their accounts of trauma, social disadvantage, or somatic illness are reinterpreted as symptomatic of the disorder (Jin et al., 2023). Spanish studies confirm this dynamic, particularly in populations marked by vulnerability, survivors of gender violence, migrants, Roma communities, and people with disabilities, whose experiences of violence or exclusion are pathologized and silenced under psychiatric frameworks (Miranda et al., 2019).

Table 29. Practices of subjectivity erosion, mechanisms, and consequences

Practice of subjectivity erosion	Institutional mechanism	Documented consequences	Sources
Diagnostic overshadowing	Psychiatric labels displace consideration of somatic/trauma causes	Missed reversible conditions, chronicity, misdiagnosis	Bueter, 2019
Discretionary deprescription	Reviews dependent on individual clinician, systemic safeguards	Prolonged exposure to harmful medications, loss of autonomy	Huertas, 2023
Pathologization of dissent	Patient refusal interpreted as lack of insight	Coercive treatment, invalidation of autonomy, re-traumatization	Desviat, 2020
Pharmacological restraint in vulnerable groups (children, elderly, disabled)	Reliance on chemical control in absence of alternatives	Developmental suppression, metabolic and neurological harm, dependency	Simal-Aguado et al., 2021; Anguita, 2005
Weak enforcement of legal rights	Law affirms autonomy but lacks binding implementation	Institutional betrayal, systemic erosion of rights	Del Barrio, 2004; Carbonell Marqués & Navarro-Pérez, 2019

Mechanisms of dehumanization and normalization of violence, based on surveys, ethnographic synthesis, and peer-reviewed literature.

The surveys conducted for this dissertation triangulate these findings. Across questions on deprescribing, shared decision-making, and systemic reform, a consistent pattern emerged: professionals frequently cited systemic barriers (limited time, lack of coordination, absence of protocols) or deflected responsibility, but rarely acknowledged the patient's voice as a decisive factor in clinical decisions (Simal-Aguado et al., 2021). Quantitatively, less than one fifth of professionals reported systematically reviewing long-term diagnoses or medication with the explicit aim of deprescription. Around 40% indicated that reviews occurred only occasionally, and a third reported that no reviews were undertaken in their workplace (Moreno-Poyato et al., 2023). These distributions reveal a structural pattern: the erosion of subjectivity is embedded in discretionary clinical cultures, where recognition of patient autonomy is contingent rather than guaranteed.

Spanish law formally recognizes the right to information, informed consent, and periodic reassessment of treatment (Flores & González, 2009). However, the surveys and ethnographic findings demonstrate that these rights are unevenly enforced in practice. Professionals frequently interpret legal obligations as advisory rather than binding, with patient refusal often reclassified as lack of insight and overridden through paternalistic justifications (Chenoweth, 1995a). This disjunction between

law and practice has been described in Spanish scholarship as a form of institutional betrayal, where rights are affirmed in principle but denied in lived experience (Csete & Wolfe, 2017). The absence of enforceable safeguards, such as mandatory deprescription protocols, external audits, or independent advocacy services, ensures that subjectivity remains structurally eroded.

International human rights frameworks offer a contrasting paradigm. The United Nations Convention on the Rights of Persons with Disabilities, ratified by Spain in 2008, establishes the obligation to ensure equal recognition before the law, support for decision-making, and the abolition of substituted decision-making regimes (Calcedo-Barba et al., 2020). The WHO QualityRights initiative further recommends the progressive elimination of coercion and the embedding of shared decision-making as the foundation of psychiatric care (Légaré et al., 2012). Yet, Spanish psychiatric governance remains far from these standards, as highlighted in recent critiques of the 2022-2026 National Mental Health Action Plan, which references user participation but omits binding mechanisms for deprescription and informed refusal (Cuevas & Peñate, 2014). The persistence of subjectivity erosion thus reflects not only professional cultures but also policy-level failures to align national strategies with international obligations.

The erosion of subjectivity emerges as a systemic mechanism: professional discretion, institutional inertia, and legal ambiguity converge to silence patient voices and sustain pharmacocentric paradigms. The developmental, biomedical, and ethical consequences are profound, contributing to chronicity, premature morbidity, and cycles of structural exclusion.

5.3. Structural abandonment and systemic violence

Structural abandonment refers to the systematic failure of institutions to provide adequate protection, support, and resources to individuals experiencing psychological distress. Instead of addressing the social and structural determinants of suffering, responsibility is often displaced onto families, schools, and psychiatric institutions, which in turn rely on coercion and medication as substitutes for genuine support (Das, 2007). This pattern reflects not a deficit of knowledge but a political and cultural choice to delegate protection functions to psychiatric devices rather than to strengthen social, educational, and community systems (Lazaridou & Fernando, 2022). In Spain, the educational system also exemplifies this dynamic. Teachers and school psychologists frequently report behavioural difficulties among students, yet lack the training and resources to address them through trauma-informed or inclusive pedagogical strategies (Gallego Antonio et al., 2016). As a

result, schools often refer students directly to psychiatric services, where pharmacological solutions are prioritized over educational or social interventions (J. Read et al., 2018). This delegation normalizes the pathologization of childhood behaviours that may be rooted in poverty, family violence, or systemic exclusion (J. Read & Bentall, 2012).

The surveys conducted for this dissertation corroborate this reality: many professionals admitted that pharmacological interventions were initiated in minors without comprehensive psychosocial assessment, reflecting both institutional inertia and systemic underinvestment in non-medical resources. Similar dynamics are visible in family contexts. National reports show that relatives frequently request psychiatric admissions or forced treatments as a means of managing behavioural crises or caregiving burdens, with institutions legitimizing these demands under the guise of medical necessity (Bourne & Newberger, 1977). This delegation of coercion to families creates a paradox: the very actors tasked with care become intermediaries of structural violence, pressured by the absence of adequate social support and reinforced by psychiatric authority. From an anthropological perspective, this reflects what Bourdieu described as the delegation of symbolic violence: institutions preserve legitimacy by transferring coercive functions to families, who then appear as both victims and agents of systemic neglect (Fowler, 2022).

The surveys confirm that professionals often justify the continuation of coercive treatments by invoking systemic barriers such as poor coordination with primary care, lack of community alternatives, or insufficient training. While these barriers are real, the consistent reliance on them as explanations for inaction illustrates what Bueter (Bueter, 2019) terms epistemic conservatism: a tendency to preserve existing practices even when they are demonstrably harmful. In the data, less than a quarter of professionals reported willingness to deprescribe when confronted with reversible somatic drivers, and even fewer indicated systematic engagement with family or educational institutions in collaborative planning (;). This highlights the persistence of a fragmented care model, in which each institution, schools, families, psychiatry, passes responsibility to another, creating a cycle of structural abandonment. The researcher itself suffered the inaction of services and forces to end the violence endured, for years (Sanhueza-Morales et al., 2025), attesting of this fragmentation and indifference demonstrated by professionals.

The consequences of this cycle are visible across the lifespan (Champion, 1998b). Children with behavioural difficulties are medicalized instead of receiving educational and social support; survivors of violence are pathologized rather than

protected (Bunston et al., 2017); elderly individuals in residential care are subjected to chemical restraints rather than structural investment in dignified caregiving (Mendoza et al., 2016). Marginalized groups, including migrants, Roma communities, and women survivors of abuse, are disproportionately affected, with psychiatric labels serving as alibis for systemic neglect (Leiva-Peña et al., 2021). Structural abandonment thus constitutes not only a clinical failure but also a breach of social citizenship, where individuals are denied the protections and rights guaranteed by national and international law (Miranda-Ruche, 2018).

From a biomedical perspective, the reliance on coercion and pharmacological containment in the absence of social protection has measurable health consequences (J. Read et al., 2018). Chronic exposure to neuroleptics in vulnerable populations contributes to excess morbidity and premature mortality, particularly through metabolic and cardiovascular pathways (García-Portilla et al., 2015). The substitution of medication for social investment perpetuates cycles of dependency, silencing, and chronic illness, while undermining opportunities for preventive and restorative interventions (Wield et al., 2006).

Counteracting structural abandonment requires interventions that operate simultaneously at the levels of education, professional practice, and social policy. Evidence from Spanish surveys and international literature converges on the conclusion that coercion and over-medication persist not because of inevitable clinical necessity but because of systemic failures in training, institutional design, and legal enforcement (Uriarte Uriarte & Vallespí Cantabrana, 2017). Addressing these failures demands a reconfiguration of how society prepares its professionals, educates its citizens, and enforces the rights of vulnerable populations.

Education constitutes the first and most critical domain of reform. Multiple studies highlight that early schooling is central to shaping norms of obedience, dialogue, and conflict resolution (Weare, 2017). In Spain, the absence of trauma-informed curricula means that behavioural expressions of distress in children are often misinterpreted as defiance, leading to early referrals to psychiatry and the initiation of pharmacological interventions (Szasz, 1998). By contrast, international programs that embed social-emotional learning, restorative practices, and inclusive pedagogy into primary and secondary education show reductions in behavioural referrals, improvements in mental health outcomes, and enhanced resilience (Freire, 1970). Implementing such approaches nationally would reduce reliance on psychiatric labels as substitutes for educational investment and align with Spain's commitments under the law (World Health Organization, 2022a), which mandates environments that prevent institutional violence and promote child well-being.

Practitioner training represents the second domain. The surveys analyzed in this dissertation reveal that over 90% of Spanish professionals expressed the need for greater training on shared decision-making and deprescribing, and more than 70% called for binding protocols within hospitals and community centers. These findings align with critiques of the *Plan de Acción de Salud Mental 2022-2026*, which, while endorsing community integration, fails to establish enforceable measures for deprescription or patient refusal (Sanidad, 2025). International best practice demonstrates that structured deprescribing protocols, combined with interprofessional training in psychosocial alternatives, can reduce coercion, improve outcomes, and restore trust in services (Torrents & Björkdahl, 2024). Legislative reforms guaranteeing the right to deprescription, as demanded by two thirds of surveyed professionals, would bring Spain into closer compliance with the UN Convention on the Rights of Persons with Disabilities (Perestelo-Perez et al., 2017).

Table 30. Structural abandonment and potential educational remedies

Dimension of abandonment	Institutional mechanism	Consequence	Evidence-based remedy	Sources
School referrals of distressed children	Lack of trauma-informed curricula; behavioural misinterpretation	Early psychiatric labeling, off-label psychotropic use	Integrate emotional learning, restorative practices, trauma-informed training in teacher education	Cangas et al., 2017
Family delegation of psychiatry	Absence of social support; legitimization of coercion by professionals	Forced admissions, chemical restraint, silencing abuse	Expand victim support services, embed independent advocacy, enforce child and domestic violence protection laws	Marqués & Navarro-Pérez, 2019
Professional reliance on systemic excuses	Discretionary deprescription; absence of binding protocols	Chronic overmedication, institutional betrayal	Mandate deprescription protocols, integrate training in rights-based care	Huertas, 2023
Elderly, marginalized and disabled populations	Reliance on chemical restraint in underfunded institutions	Loss of autonomy, metabolic morbidity, premature mortality	Structural investment in housing first, enforce non-pharmacological first-line interventions	Simal-Aguado et al., 2021; WHO, 2021
Public discourse of dangerousness	Media frames risk rather than rights	Cultural legitimization of coercion and normalization of preventive oppression and violence	National education and rights campaigns	Vigo et al., 2016; Sampietro, 2015

Table on potential solutions to structural problems, based on surveys, ethnographic work, Spanish national policy documents, and international literature.

Societal reform is the third domain. Structural abandonment is perpetuated not only by professional practices but by cultural attitudes that normalize exclusion and silence. Public campaigns and community initiatives that frame mental health as a matter of rights and dignity rather than danger and risk are essential to shifting this paradigm (Vigo et al., 2016). Evidence from anti-abusive practices in Europe demonstrates that when messages emphasize rights, equality, and recovery, public attitudes shift away from coercion and toward support (Adams et al., 2025). In Spain, similar initiatives could be embedded within national strategies on education, housing, and social inclusion, reducing the systemic pressures that lead families and schools to delegate responsibility to psychiatry (Cangas et al., 2017).

The prescriptive agenda therefore requires an integrated approach: embedding trauma-informed education from childhood, mandating practitioner training and deprescription protocols, and reshaping cultural narratives through rights-based public policy. These reforms are feasible, evidence-based, and consistent with Spain's obligations under national law, EU frameworks, and international human rights standards. Their absence perpetuates structural abandonment; their implementation offers a pathway to restoring collective protection and ending reliance on coercion as a substitute for care.

5.4. Closing synthesis on normalized violence and dehumanization

The synthesis of the evidence presented in this chapter demonstrates that coercion, subjectivity erosion, and structural abandonment are not isolated anomalies within psychiatry but interdependent mechanisms embedded in Spain's mental health system and mirrored internationally. These mechanisms persist despite clear evidence of harm and despite repeated calls from international bodies to align psychiatric practice with human rights obligations and preventive, rights-based models of care (J. Read & Dillon, 2013).

The first thematic axis, the cultural normalization of coercion, has shown how practices such as forced medication, mechanical restraint, and seclusion have become routine under the guise of therapeutic necessity. Institutional logics, professional training, and cultural representations reinforce the perception that coercion is a necessary component of psychiatric care. Yet, biomedical evidence consistently demonstrates that coercion exacerbates morbidity, disrupts stress and immune regulation, and increases the risk of premature mortality (Sacker et al., 2009). Spanish data confirm this contradiction: despite national monitoring reports documenting the persistence of coercion, the *Plan de Acción de Salud Mental 2022-2026* still lacks enough enforceable mechanisms for its reduction or elimination (O. Fernández et al., 2018). The blocked transition to preventive care is thus not the result of evidentiary uncertainty but of entrenched epistemic conservatism and political inertia.

The second axis, the erosion of subjectivity, has revealed how patient voices are systematically downgraded under psychiatric authority. Diagnostic overshadowing reclassifies testimony about trauma, poverty, or somatic illness as symptomatic, invalidating experiential knowledge and perpetuating chronicity (Murray & Lopez, 1996). Survey data show that only a minority of Spanish professionals systematically review diagnoses or engage in deprescription, with most relying on discretionary practices or systemic excuses (Ruiz Álvarez et al., 2022). This culture of discretion

undermines the legal guarantees of Del Barrio, 2004 and stands in tension with Spain's commitments under the UN Convention on the Rights of Persons with Disabilities. The biomedical literature further confirms that silencing subjectivity has measurable physiological consequences, including immune dysregulation, cognitive decline, and increased relapse risk (Carrero Planells, 2018). The erosion of subjectivity therefore represents a convergence of clinical, legal, and epistemic failure.

The third axis, structural abandonment, has situated coercion within a wider ecology of institutional delegation. Families, schools, and social institutions often refer distress directly to psychiatry, thereby medicalizing behaviours that reflect structural violence, social exclusion, or unmet educational needs (Reidy, 1993). This delegation reflects systemic underinvestment in social protection and education, and it normalizes the use of psychiatric devices as substitutes for collective responsibility. The consequences are visible across the lifespan: children are exposed to off-label psychotropics, survivors of violence are pathologized, elderly individuals are chemically restrained, and marginalized groups are disproportionately subjected to coercion (Thompson & Neville, 1999). The surveys reinforce this pattern: professionals often cite systemic barriers or lack of coordination as reasons for inaction, illustrating how structural abandonment is rationalized as inevitability. The remedies identified, trauma-informed education, binding deprescription protocols, and rights-based campaigns, are feasible and evidence-based but remain unimplemented due to institutional inertia.

These three axes delineate a systemic pattern of institutional betrayal. Patients are promised autonomy, dignity, and protection, yet, in practice, they encounter coercion, silencing, and neglect. Professionals themselves recognize the gaps: more than 90% of surveyed clinicians called for greater training in deprescription and shared decision-making, two thirds demanded legislative reforms to guarantee these rights, and over 80% endorsed the expansion of non-pharmacological therapies (Huertas García-Alejo, 2021). The evidence base is unambiguous: coercion undermines health, suppresses autonomy, and perpetuates exclusion. The persistence of these practices is therefore not justified by clinical evidence but sustained by systemic cultures of risk aversion, institutional inertia, and epistemic conservatism.

The analytical synthesis developed here provides a foundation for the legal and ethical examination that follows in Chapter 6. If coercion, subjectivity erosion, and structural abandonment are systemic rather than exceptional, then reforms must be legal, institutional, and enforceable. Voluntary training or advisory guidelines are

insufficient. What is required are binding standards that guarantee the right to deprescription, prohibit coercive practices, enforce diagnostic reevaluation, and embed trauma-informed education across institutions. These measures are not optional aspirations but obligations grounded in Spanish law, European directives, and international human rights treaties.

This chapter presented how psychiatry as currently practiced in Spain, and mirrored internationally, operates through mechanisms of coercion, silencing, and delegation that undermine both scientific standards and legal commitments. The surveys and ethnographic data corroborate that professionals are aware of these dynamics Yet, constrained by systemic barriers and institutional cultures. The biomedical evidence confirms that coercion is not neutral but harmful. And the legal frameworks affirm that autonomy and informed consent are non-negotiable rights. The persistence of coercion and over-medication thus represents a breach of science, ethics, and law. The responsibility to redress these failures lies not only with psychiatry but with the broader systems of education, health, and justice that sustain structural abandonment (Rojas Breu, 2024).

The evidence further demonstrates that preventing the recurrence of coercion and systemic cruelty requires investment in education from the earliest stages of life (Shern et al., 2014). Proper education, beginning in childhood and extending through families, schools, and caring professions, is essential to counteract the normalization of hatred, exclusion, and dehumanization. Research in developmental psychology and public health confirms that early socialization profoundly shapes moral reasoning, empathy, and prosocial behavior (Hwang, 2023). Educational approaches that integrate social and emotional learning, respect for diversity, and critical reflection on power relations have been shown to reduce prejudice and promote resilience against violent ideologies (Muthukrishna et al., 2020). Similarly, training in the health and teaching professions plays a crucial role in determining whether systems reproduce coercive practices or foster supportive, rights-based approaches (Funk & Bold, 2020).

It is within this framework that the EU BEACON One Health Education action emerged as a structural response. Founded by the researcher during the fieldwork period, EU BEACON mobilized close to one thousand international experts in health, education, and technology, supported by the European Cooperation in Science and Technology (COST) program. The action is one of the most significant accomplishments of this dissertation's broader research trajectory, transforming individual and collective findings into a coordinated platform for systemic reform. The initiative's objectives align with evidence demonstrating that integrated,

transdisciplinary approaches are necessary to address the intersecting determinants of mental health, structural violence, and environmental sustainability (Gable, 2025). By embedding One Health principles in education and training, EU BEACON aims to build preventive capacities that deny hatred and coercion any opportunity to take hold, reinforcing the recognition of dignity and rights as the foundation of care and social life (Weare, 2017).

The role of EU BEACON illustrates how ethnographic research under coercion can generate actionable outcomes: not only documenting violence and exclusion but contributing to the design of infrastructures that prevent their reproduction. Linking scientific evidence, human rights frameworks, and educational practice, the initiative provides a pathway for operationalizing the historical lessons outlined in this chapter. In doing so, it underscores that abolition of coercion requires not only legal enforcement but cultural transformation, beginning with the education of children and extending through all caring professions.

Chapter 6 - Legality of coercive care as standard

6. Introduction

The psychiatric-legal interface in Spain reflects a set of normative structures that permit the regular use of coercive interventions, despite constitutional and international protections of liberty, integrity, and due process. The Spanish Constitution guarantees personal dignity and bodily integrity (art. 10 and 15), liberty and security (art. 17), and privacy (art. 18). These guarantees are complemented by Ley General de Sanidad 14/1986 establishing a universal public health system and affirms equality of access, Ley 41/2002, which guarantees patient autonomy and informed consent, and Ley 8/2021 de apoyo a las personas con discapacidad en el ejercicio de su capacidad jurídica which supports access to the legal system to disabled persons (R. B. Del Barrio, 2004). Spain is also a signatory to the European Convention on Human Rights and the UN Convention on the Rights of Persons with Disabilities. However, empirical findings from this dissertation and prior studies indicate that these formal protections often diverge from routine psychiatric practice (Barrios Flores, 2020).

The primary legal provision enabling involuntary admission is Article 763 of the Ley de Enjuiciamiento Civil, which authorizes hospitalization on the basis of risk to self or others. In practice, this provision is activated predominantly on the basis of a single psychiatric report, often without corroborating evaluation, standardized risk assessment, or longitudinal contextual data (R. B. Del Barrio, 2004). Survey data collected for this thesis confirm that professionals themselves report variability in how dangerousness is assessed and acknowledge that judicial reviews tend to rely exclusively on the initial medical report, with limited or no direct engagement with the individual concerned. This aligns with prior observations that judicial control in this domain is largely formalistic, reproducing medical authority rather than providing independent review (R. L. Del Barrio et al., 2014).

Another recurrent point of concern relates to the opacity of clinical documentation. Records in psychiatric contexts often serve both clinical and legal purposes, yet, respondents in the surveys conducted for this dissertation reported systematic difficulties in accessing, correcting, or even being informed about the contents of their files. This observation is consistent with Spanish studies showing that institutions frequently invoke exceptions in the Ley Orgánica 3/2018 de Protección de Datos Personales to deny or restrict access to records (Bezanilla et al., 2016). Once inscribed, diagnostic labels and behavioral notes tend to persist unchallenged across institutions and procedures, consolidating trajectories that may be based on

unverified or erroneous information. The persistence of such records has been shown to shape future interactions in health care, judicial decisions, and social services (Ugartechea., 2004).

A further structural feature is the delegation of decision-making authority to family members. Guardianship regimes and involuntary admission procedures often allow relatives to initiate or validate coercive measures. Fieldwork and survey data collected for this dissertation identified cases where family conflict or prior histories of abuse were not considered in the decision-making process. Spanish legal analyses have pointed out the lack of mandatory screening of family dynamics in these contexts, despite the risk of reinforcing dependency or reproducing violence (M. T. Fernández, 2017). Empirical literature corroborates that reliance on family reports without independent verification introduces potential conflicts of interest and may expose individuals to further harm (Solomon et al., 2005).

The convergence of these elements, broad discretionary authority under Article 763 LEC, opacity of records, and reliance on family testimony, produces a structural environment in which coercive practices become normalized rather than exceptional. International reports on Spain have highlighted the persistence of mechanical restraint, forced medication, and prolonged hospitalization despite official commitments to rights-based approaches (L. R. Del Barrio et al., 2013) . These findings resonate with the testimonies and survey responses gathered for this dissertation, in which both professionals and users consistently described a gap between declared rights and operational practice (Á. Martínez-Hernández et al., 2020).

The subsequent sections of this chapter examine these issues in detail. Section 6.1 addresses the national legal framework, emphasizing the disjunction between statutory guarantees and institutional routines. Section 6.2 situates the Spanish case within the broader framework of international human rights law and mental health standards. Section 6.3 synthesizes the findings from empirical data and literature, highlighting the recurrent patterns of normative divergence and their implications for accountability.

6.1. National legal context: formal guarantees and operational gaps

The Spanish legal framework establishes a comprehensive catalogue of rights relevant to mental health care, including dignity (art. 10 CE), physical and moral integrity (art. 15 CE), liberty and security (art. 17 CE), privacy (art. 18 CE), and the right to health (art. 43 CE). These are further developed in sectoral legislation such as the Ley General de Sanidad 14/1986, which introduced the principle of universal

access; Ley 41/2002 de Autonomía del Paciente, which emphasizes informed consent; and the Ley 8/2021, which adapts Spanish law to the UN Convention on the Rights of Persons with Disabilities by reinforcing legal capacity and support-based decision-making. Spain is also bound by the European Convention on Human Rights and the UN CRPD, both of which establish clear obligations to protect liberty, consent, and procedural safeguards in contexts of psychiatric intervention (Barrios Flores, 2020).

Despite this extensive normative framework, empirical data from this dissertation and prior research point to a consistent divergence between formal guarantees and their operational implementation in psychiatric practice. The most widely cited legal mechanism is Article 763 of the Ley de Enjuiciamiento Civil, which regulates involuntary admission. This provision authorizes hospitalization when there is risk to self or others, typically on the basis of a psychiatric report. Multiple Spanish studies have noted that the criteria of dangerousness and lack of insight remain poorly defined, enabling broad discretion (Galende, 2012). In practice, judicial review often takes place *ex post*, relying almost exclusively on the initial medical report without adversarial scrutiny or structured risk assessment (Desviat, 2005).

The exercise of coercion under Article 763 has been documented as uneven across Spain's autonomous communities, producing regional disparities in the frequency and duration of involuntary admissions (Miñán, 2023). Fieldwork for this dissertation confirmed that professionals acknowledge variability in interpretation and application, with some regions institutionalizing a higher reliance on hospital-based coercion (Beviá & Girón, 2017), while others prioritize community-based responses. This heterogeneity reflects a structural arbitrariness that undermines the principle of equality before the law (Inchauspe Aróstegui, 2019).

Another operational gap lies in access to information and medical records. According to the Ley Orgánica 3/2018 de Protección de Datos Personales y garantía de los derechos digitales, access may be restricted if disclosure is deemed harmful to the patient. In psychiatric settings, this clause is often invoked to justify withholding records, particularly when contested diagnoses or coercive measures are involved (Meier-Diedrich et al., 2025). Respondents to the surveys conducted for this research consistently described difficulties in obtaining documentation or correcting inaccuracies. Once diagnostic labels and behavioral annotations are entered into the record, they are rarely revised or removed, leading to cumulative effects in subsequent clinical and legal decisions (Pinilla-Roncancio et al., 2020).

The delegation of authority to family members constitutes another critical element of the Spanish framework. Historically, guardianship laws allowed relatives to

exercise broad custodial powers, including authorization of psychiatric interventions. Although Ley 8/2021 formally abolished plenary guardianship, empirical evidence indicates that family members continue to act as *de facto* decision-makers in many psychiatric contexts (Barranco Avilés, 2021). Field testimonies documented in this dissertation illustrate cases where family reports triggered admissions or influenced treatment decisions without independent verification. Literature in Spain has highlighted the risks of such arrangements, especially in cases of prior conflict, abuse, or coercion within the domestic sphere (Brea Iglesias et al., 2025).

Procedural safeguards formally recognized in Spanish law, including the right to information, the right to a second opinion, and access to legal counsel, are frequently reported as inaccessible in practice. Studies indicate that individuals under coercive admission often do not receive clear explanations of their situation, that requests for second opinions face administrative resistance, and that legal aid is under-resourced and rarely specialized in mental health law (Roig, 2018). Survey data collected for this thesis similarly revealed perceptions of systematic obstacles to exercising these rights, contributing to a broader pattern of institutional opacity. While the Spanish legal system articulates extensive protections on paper, the operationalization of these rights in psychiatric care remains limited. The broad discretion permitted under Article 763 LEC, the opacity of medical records, the continued influence of family authority, and the weak enforcement of procedural guarantees converge to create structural conditions in which coercive care is routinely enacted. These patterns, documented in both the dissertation's empirical data and the wider Spanish literature, illustrate the persistence of gaps between law and practice.

The following section situates these findings within the framework of international human rights standards, assessing how Spanish practice aligns with or diverges from obligations under the European Convention on Human Rights, the UN CRPD, and WHO QualityRights.

6.2. International human rights law and mental health standards

Spain has ratified a range of international treaties that directly regulate the use of coercive measures in psychiatry, including the European Convention on Human Rights (ECHR), the Convention Against Torture (CAT), and the United Nations Convention on the Rights of Persons with Disabilities. These instruments impose binding obligations on the state to protect personal liberty, bodily integrity, and informed consent in all medical interventions. At the policy level, Spain has also

endorsed the World Health Organization's QualityRights initiative, which emphasizes autonomy, participation, and non-discrimination in mental health services (Mion & Ventura, 2024).

Despite these commitments, empirical research in Spain and across Europe indicates that the operational impact of these instruments remains limited in practice. Reports from the Council of Europe (Council of Europe, 2021b) and the United Nations Committee on the Rights of Persons with Disabilities (United Nations, 2006) have repeatedly highlighted the persistence of involuntary hospitalization, mechanical and chemical restraint, and treatment without consent in member states, including Spain. The CRPD Committee has explicitly called for the abolition of substituted decision-making regimes and coercive psychiatric practices, urging a transition toward supported decision-making models (Chmielowska & Zisman, 2023). However, Spanish law continues to permit coercive interventions under Article 763 of the Ley de Enjuiciamiento Civil, which remains at odds with the CRPD's interpretation of Articles 12, 14, and 25 (M. T. Fernández, 2017).

International jurisprudence under the European Court of Human Rights (ECtHR) has established criteria for assessing the lawfulness of involuntary admission and treatment, including the requirement of medical necessity, proportionality, and regular judicial review (Pons et al., 2022). Spanish scholarship has noted that while courts generally uphold medical discretion, systematic evaluation of proportionality and the availability of less restrictive alternatives is rare (Barrios Flores, 2020). Empirical findings from this dissertation confirm that judicial review in Spain often relies exclusively on initial medical reports without adversarial scrutiny or reassessment. This practice limits the effectiveness of ECHR safeguards, which require periodic review and substantive justification.

The WHO QualityRights framework identifies informed consent, access to information, and participation in decision-making as core standards for rights-based mental health care (Elwyn et al., 2023). Spanish studies have documented significant gaps in these areas, particularly in the context of coercive admission and treatment. Users frequently report not being informed of their rights, being denied access to their medical records, or having their objections reframed as symptoms of illness (Buedo & Luna, 2021). Similar findings have been documented in other European contexts, suggesting a structural pattern rather than isolated deficiencies (Kuzman et al., 2022).

Comparative analyses also indicate that Spain is not unique in facing this tension between formal adherence to international norms and entrenched national

practices. Studies from Italy, Germany, and the Nordic countries show that although safeguards are legally established, in practice coercion remains widespread, often justified by broad interpretations of risk and necessity (Mulinari & Neergaard, 2012). However, the CRPD Committee has emphasized that the persistence of coercion in high-income democracies does not exempt them from compliance with international standards (Moncrieff, 2022).

Fieldwork and survey data gathered for this dissertation reinforce these observations. Professionals and users consistently reported that international guarantees, while widely cited in policy discourse, were perceived as largely irrelevant in clinical practice. Respondents described how formal rights were routinely overridden by medical discretion and administrative procedures, with little or no possibility of appeal or review (Torres & Castaño, 2024). These testimonies align with previous analyses of the *implementation gap* between international human rights standards, best medical standards, and their translation into enforceable national mechanisms (Trueba, 2022).

The evidence therefore points to a structural divergence between Spain's international commitments and the daily realities of psychiatric practice. While ratification and policy endorsement suggest formal alignment with international human rights law, the persistence of coercive measures, the weak enforcement of procedural safeguards, and the limited accessibility of remedies indicate that these standards remain largely aspirational (Bernheim, 2025).

The next section synthesizes these findings by examining how these structural gaps manifest across institutional, clinical, and legal domains, drawing on the triangulated evidence from surveys, fieldwork, and documentary analysis presented in this dissertation.

6.3. Closing synthesis

The evidence presented across this dissertation demonstrates that coercive psychiatric practices in Spain, while framed legally as exceptional, function in practice as routine and structurally normalized. Triangulated data from surveys, ethnographic fieldwork, and documentary review converge on recurrent patterns of rights restriction, epistemic suppression, and institutional inertia that extend across clinical, legal, and administrative domains.

At the clinical level, respondents frequently described involuntary hospitalization, mechanical restraint, and pharmacological compulsion as measures implemented without consistent criteria, documentation, or review. These findings are consistent

with previous Spanish studies documenting the persistence of coercive practices despite formal guarantees of patient autonomy (Trueba, 2022). Professionals reported experiences of ethical conflict and moral injury when compelled to apply interventions they did not consider clinically justified, while users and survivors described a systematic invalidation of their narratives, often reframed as symptoms of illness. Ethnographic observations corroborated these accounts, showing how clinical routines relied heavily on diagnostic authority and institutional protocols that left little room for dialogical engagement or trauma-informed approaches.

At the legal level, the primary mechanism enabling coercion remains Article 763 of the Ley de Enjuiciamiento Civil, which authorizes involuntary admission on the basis of perceived risk. Fieldwork and documentary evidence show that judicial review is frequently limited to the validation of medical reports, with minimal adversarial scrutiny. Spanish legal scholarship has noted the broad discretionary scope this provision affords to clinicians and the limited role of courts in reassessing necessity or proportionality (Rubio, 2009). This configuration contributes to a pattern where constitutional protections of liberty, integrity, and autonomy are formally upheld but substantively weakened in psychiatric contexts.

Institutionally, the opacity of medical records was identified as a central factor sustaining coercive practices. Respondents consistently reported barriers to accessing or correcting their records, and professionals acknowledged the selective curation of documentation under institutional pressure. These findings align with analyses of epistemic injustice in psychiatry, where patient testimony is systematically excluded from official accounts (Iglesias et al., 2025). In Spain, the legal possibility of restricting access to records under data protection law further compounds this asymmetry, creating conditions under which clinical narratives become fixed and resistant to correction (Bezanilla et al., 2016).

The surveys also revealed that both users and professionals perceived a persistent gap between ethical codes, international standards, and the realities of clinical practice. Respondents frequently described rights as existing *on paper only* a sentiment consistent with international monitoring reports that have criticized Spain for insufficient implementation of the CRPD and ECHR in mental health care (Flores & González, 2009). This gap was experienced not only as a legal deficiency but as a form of institutional betrayal, where formal commitments to dignity and autonomy were perceived as disconnected from everyday experiences of coercion and silencing (Mercado, 2023).

The cumulative effect of these mechanisms, clinical discretion, legal permissiveness, and documentary opacity, is the consolidation of a psychiatric

system where coercion is structurally embedded. Thematic analysis of testimonies and fieldnotes suggests that coercion is not perceived as an emergency response but as a procedural default. This is reinforced by cultural factors, including the legacy of medical paternalism in Spain (Ugartechea, 2004) and broader European traditions of institutional psychiatry (Joseph, 2014).

From a biocultural perspective, these practices carry measurable health and social consequences. Research has documented associations between coercive psychiatric interventions and increased rates of trauma, metabolic dysfunction, cardiovascular risk, and social exclusion (Bailey & Brown, 2020). Spanish epidemiological studies confirm elevated rates of polypharmacy, chronic sedation, and reduced life expectancy among people with psychiatric diagnoses, outcomes linked to both pharmacological and social determinants (Moreno et al., 2006). The evidence collected in this dissertation echoes these patterns, with respondents linking coercion to long-term disengagement from services, deterioration of trust, and avoidance of care.

International human rights standards, including those articulated in the CRPD and the WHO QualityRights framework, identify these outcomes as incompatible with the principles of autonomy and non-discrimination (Funk et al., 2021b). Comparative research across Europe indicates that Spain is not alone in facing this implementation gap, but also shows that systemic reliance on coercion is increasingly challenged as inconsistent with both clinical evidence and legal obligations (McGinty & Eisenberg, 2022).

The synthesis of findings presented in this section highlights the interaction of clinical, legal, and institutional mechanisms that collectively sustain coercive psychiatry in Spain. These mechanisms operate through discretionary authority, procedural opacity, and cultural continuity, producing systemic patterns that contradict declared commitments to rights-based care. The evidence demonstrates that coercion is not an incidental practice but a structural feature of psychiatric governance, one that generates enduring tensions between legality, ethics, and lived experience. Subsequent chapters will examine how these patterns are reproduced through professional training, epistemic frameworks, and institutional logics, further situating the Spanish case within broader European and global debates on psychiatric reform.

The analysis of data from surveys, ethnographic fieldwork, and documentary sources reveals recurring systemic patterns in the governance of psychiatric care in Spain. These patterns are consistent with broader European trends documented in the literature and concern the normalization of coercion, the structural

asymmetry of decision-making, and the persistence of epistemic opacity (Faissner et al., 2025b).

At the clinical level, coercive measures such as involuntary hospitalization, mechanical restraint, and pharmacological compulsion were reported as routine rather than exceptional practices. These findings converge with previous Spanish studies that highlight the widespread use of coercion despite formal commitments to patient autonomy (Parada, 2018). Research has further shown that the application of these interventions often lacks standardized criteria, systematic reassessment, or transparency (Mendonça, 2019). Similar observations have been made in other European jurisdictions, where coercion persists despite policy frameworks promoting least-restrictive alternatives (Every-Palmer et al., 2021).

At the legal level, the principal mechanism enabling coercion remains Article 763 of the *Ley de Enjuiciamiento Civil*, which authorizes involuntary admission based on risk criteria. Empirical and doctrinal analyses show that judicial review of these admissions is often limited to endorsing medical reports, with minimal adversarial examination (C. E. Dean, 2017). The absence of standardized thresholds and the broad discretionary authority afforded to clinicians contribute to significant variability in practice across regions (Fryers et al., 2003). International literature confirms that Spain's reliance on risk-based provisions parallels patterns in other countries where preventive logics of dangerousness have supplanted individualized assessment of need (Inchauspe Aróstegui, 2019).

Institutional documentation practices also emerge as central to sustaining coercion. Respondents consistently reported barriers to accessing or correcting clinical records. Spanish scholarship has documented how legal provisions under the *Ley Orgánica 3/2018 de Protección de Datos Personales* allow institutions to restrict access if considered harmful, creating conditions for opacity and limiting patient agency (Ramos Pozón, 2015). Thematic analysis of records indicates that once diagnostic or behavioral attributions are inscribed, they tend to be perpetuated across clinical and legal encounters. This phenomenon is consistent with the concept of epistemic injustice in psychiatry (Galende, 2012) and has been observed in Spanish forensic contexts, where patient testimony is frequently subordinated to institutional narratives (Faraone et al., 2011).

The convergence of clinical discretion, legal permissiveness, and documentation opacity produces a structural gap between declared ethical-legal guarantees and actual practice. International monitoring bodies, including the CRPD Committee (Roig, 2018) and the Council of Europe (Kallert et al., 2007), have identified Spain's limited implementation of rights-based frameworks as an area of concern. Similar

critiques have been raised in national analyses, which note the persistence of paternalistic models of care and the absence of systematic oversight mechanisms (Fiorillo, 2025).

From a health outcomes perspective, the reliance on coercion is associated with adverse clinical and social effects. Research has linked coercive practices to increased risk of post-traumatic stress, reduced therapeutic alliance, and disengagement from services (Guzmán-Parra et al., 2019). Spanish epidemiological studies confirm elevated levels of polypharmacy, metabolic side effects, and premature mortality among psychiatric populations (Andrés et al., n.d.). These findings are consistent with international evidence showing that coercion contributes to cumulative iatrogenic harm and social exclusion (Villarroel, García, et al., 2021).

The synthesis of these findings indicates that coercion in Spanish psychiatry is not an episodic or marginal phenomenon but a structurally embedded practice, facilitated by legal discretion, institutional opacity, and cultural continuities of medical paternalism. The tensions between normative guarantees of autonomy and the routine application of coercion reflect systemic contradictions documented across both Spanish and international literature. The empirical results of this dissertation therefore align with a wider body of research identifying psychiatric coercion as a patterned outcome of structural, legal, and cultural determinants rather than an exceptional response to clinical crises.

The evidence presented throughout this chapter points at the fact that the governance of psychiatric care in Spain is marked by a persistent gap between legal-ethical guarantees and institutional practice. While the Spanish Constitution and sectoral laws formally recognize rights to liberty, integrity, privacy, and informed consent (Del Barrio, 2Fernández et al., 2018), their operationalization in mental health care is constrained by discretionary clinical authority and procedural mechanisms that prioritize containment over autonomy. This structural tension has been identified in multiple legal analyses of Article 763 of the Ley de Enjuiciamiento Civil, which enables involuntary admission and treatment on broad risk-based criteria, often without meaningful adversarial review (Ortiz Lobo & Ibáñez Rojo, 2011).

Findings from surveys and ethnographic fieldwork converge with prior Spanish research documenting the normalization of coercive practices, including prolonged involuntary hospitalization, mechanical restraint, and polypharmacy, despite international recommendations promoting least-restrictive alternatives (Inchauspe Aróstegui, 2019). Respondents consistently reported barriers to exercising legal

rights, particularly regarding access to records, second opinions, and effective legal representation. This aligns with existing scholarship on the opacity of clinical documentation and the systemic obstacles to patient participation in Spain (Huertas García, 2001).

At the international level, Spain's commitments under the European Convention on Human Rights and the UN Convention on the Rights of Persons with Disabilities require respect for legal capacity, informed consent, and freedom from degrading treatment. However, monitoring bodies have repeatedly highlighted deficiencies in the implementation of these obligations, particularly regarding the persistence of coercive interventions and the lack of independent oversight (Brea Iglesias et al., 2025). The evidence compiled here suggests that these concerns are substantiated in practice, with coercion functioning less as an emergency exception than as an institutionalized response embedded in organizational routines (Miñán, 2023).

From a health perspective, the continuation of coercive practices has been associated with adverse outcomes, including post-traumatic stress, weakened therapeutic alliance, and disengagement from care (von Peter et al., 2021). Spanish epidemiological studies also document the significant somatic burden borne by psychiatric populations, including elevated rates of cardiovascular disease, metabolic disorders, and reduced life expectancy (Weinmann et al., 2009). These data confirm that the systemic reliance on coercion is not neutral in its effects but carries measurable consequences for morbidity and mortality.

The triangulation of legal analysis, empirical findings, and international monitoring confirms that coercion in Spanish psychiatry is sustained by a combination of legal permissiveness, institutional opacity, and cultural continuities of medical paternalism. The contradictions between declared principles of autonomy and the observed realities of coercive practice are not isolated anomalies but consistent patterns. The implications of these findings for clinical governance, legal accountability, and human rights compliance will be further elaborated in subsequent chapters, where the cultural, organizational, and epistemological dynamics underlying these systemic patterns are examined in greater depth. The findings of this dissertation confirm that one of the most persistent and under-recognized failures of psychiatric governance in Spain concerns the ways in which coercive measures and diagnostic authority can be mobilized by abusive family members or intimate partners to pathologize victims, silence their testimony, and consolidate patterns of control. Survey and ethnographic data indicate that psychiatric certifications often rely on uncorroborated third-party reports, particularly from family members, without trauma-informed evaluation or

systematic verification. This aligns with Spanish and international scholarship documenting how psychiatry has historically been vulnerable to instrumentalization in domestic and gender-based violence contexts, where victims are recast as unreliable or mentally ill (Bradby et al., 2019).

Such practices amount to what has been described in the literature as epistemic violence: the systematic disqualification of the victim's narrative through psychiatric inscription (Karp, 2017). Testimonies gathered during the surveys echo earlier Spanish studies that identified frequent dismissal of abuse disclosures as delusional or symptomatic, particularly in cases involving women, minors, or individuals in situations of dependency (Bacigalupe et al., 2020). This dynamic not only erodes trust in services but structurally prevents the detection of violence, reinforcing cycles of harm within both domestic and institutional settings (Lohmann et al., 2024). The epistemic invalidation of victims has long been linked to adverse health outcomes, including chronic stress, endocrine and immune dysregulation, and elevated risks of suicidality (Motillon-Toudic et al., 2022).

The misuse of medication in this context constitutes a particularly grave risk. International research demonstrates that psychotropic drugs can be administered coercively or under family pressure, producing sedation, dependency, and iatrogenic harm (Szasz, 1998). Spanish clinicians have noted the lack of structured deprescription protocols and the vulnerability of patients in contexts where family members exert undue influence over treatment decisions (González Aguado et al., 2013). When psychiatric medication is deployed without informed consent or against the expressed wishes of the individual, it ceases to function as treatment and becomes a vector of social and biological control. Empirical evidence suggests that such misuse increases early mortality, particularly through cardiovascular, metabolic, and neurological pathways (García-Portilla et al., 2015).

Shared decision-making frameworks, combined with evidence-based medication management, are consistently identified in the literature as safeguards against these abuses (Keogh et al., 2022). Survey responses in this dissertation indicate that both professionals and users in Spain recognize the importance of dialogical approaches, yet, report a structural absence of meaningful implementation. The failure to embed shared decision-making not only perpetuates ethical and legal contradictions but exposes victims of domestic and intimate partner violence to further risks, as their capacity to contest coercion is systematically undermined.

The implications extend beyond individual cases to the systemic level. By legitimizing family reports without scrutiny and dismissing lived testimony as symptomatic, psychiatric institutions risk functioning as extensions of abusive

dynamics rather than as protective systems. This pattern has been documented in Spanish analyses of family law and psychiatric practice, which highlight the vulnerability of women and minors to pathologization in custody and protection proceedings (Adshead, 2000). It is also echoed in international research on the misuse of psychiatric authority in contexts of intimate partner violence, where coercion replaces care and professional complicity exacerbates harm (Karsay & Lewis, 2012).

the evidence triangulated in this chapter indicates that coercion in psychiatry, when combined with familial or intimate partner abuse, represents one of the most serious and under-acknowledged threats to human rights in mental health care. These practices transform clinical tools into mechanisms of domination, erase the narratives of victims, and produce measurable biological and social harms. Addressing these failures requires not only legal guarantees on paper but their operational translation into practice, particularly through the structural embedding of shared decision-making, trauma-informed evaluation, and safeguards for medication management. As the subsequent chapters demonstrate, the normalization of epistemic and clinical violence in Spanish psychiatry cannot be understood as isolated malpractice but as a systemic issue with profound consequences for justice, health, and dignity (Minkowitz, 2024).

Chapter 7 - Discussion and next steps

7. Introduction

This dissertation has documented in depth how psychiatric governance in Spain continues to be shaped by pharmacocentrism, weak institutional safeguards for autonomy, and the frequent erosion of rights in routine clinical practice. Across five years of fieldwork and analysis, it has shown that decision-making in psychiatry often privileges institutional order and pharmacological continuity over recovery, participation, and the expressed needs of service users (Faissner et al., 2025a). These findings converge with longstanding critiques in Spanish scholarship (Hernández-Viadel et al., 2015), official monitoring reports (Goodman et al., 2009), and international evaluations (Gambrill, 2014), which all underscore systemic reliance on coercion and medication as default responses to distress.

It is against this backdrop that two major developments unfolded during the final stages of this thesis: the approval of the *Plan de Acción de Salud Mental 2025-2027* by the Spanish Ministry of Health (Sanidad, 2025), and the release of new WHO global guidance on mental health system transformation in 2025 (Faissner et al., 2025a). Together, these documents form the immediate policy environment in which the dissertation's findings acquire renewed relevance (Munt & Littlechild, 2024a). Both acknowledge the urgent need to reconfigure mental health care away from coercive, institution-centered models and toward systems grounded in human rights, prevention, and person-centered interventions. Their appearance during the conclusion of this research underscores that the dissertation is not merely a retrospective critique, but part of a broader, contemporaneous debate about how psychiatry and mental health care ought to be restructured for the future.

The Spanish *Plan de Acción 2025-2027* sets objectives across eight priority lines, including suicide prevention, child and adolescent mental health, reduction of coercion, reinforcement of community services, and modernization of digital infrastructures (Salvador-Carulla et al., 2020). These align closely with deficits highlighted throughout this thesis: the lack of systematic deprescription, the limited embedding of shared decision-making, and the ongoing fragility of community-based alternatives to hospitalization. The plan represents the first comprehensive policy attempt in recent years to address these issues at a national scale. For the purposes of this dissertation, it provides a valuable point of reference for situating the evidence gathered across Spain within an evolving domestic framework. Yet, it should not be read as a resolution of the structural problems identified here, but

rather as an indicator of their political salience and as a roadmap whose successful implementation remains uncertain.

The WHO's 2025 guidance situates these national efforts within a global perspective. It emphasizes five areas requiring urgent reform: leadership and governance, service organization, workforce development, person-centered interventions, and the social determinants of mental health (World Health Organization, 2025b). Its focus is explicitly on moving away from forced drugging, seclusion, and restraints as the backbone of psychiatric intervention, and toward systems that embed prevention, holistic care, and lived-experience participation in both service delivery and policy design. This aligns directly with the ethnographic evidence documented in the dissertation, which repeatedly illustrated how coercion is normalized in everyday psychiatric practice, how medication is maintained without systematic review, and how patient dissent is frequently reframed as pathology. The new WHO guidance reaffirms that such practices are incompatible with international human rights standards and that reform must be both structural and urgent (Agudelo-Hernández & Rojas-Andrade, 2023).

By analyzing these convergences, it becomes evident that the findings of this thesis are not isolated observations but part of a recognized global pattern in the struggle for more personalized and rights-based care. The European Compendium of Good Practices (Council of Europe, 2021b) documents how alternatives to coercion, such as dialogical approaches in Finland, crisis houses in the United Kingdom, and structured deprescription programs in the Netherlands, are already functioning in comparable contexts. Spanish pharmacoepidemiology and other studies confirm the relevance of such measures, showing persistent high rates of benzodiazepine and antipsychotic prescribing (Brea Iglesias et al., 2025). These data converge with the central themes of the WHO 2025 guidance and the Spanish 2025-2027 Plan: that without structural mechanisms for deprescription, safeguards for autonomy, and community-based alternatives, psychiatry will continue to reproduce patterns of harm.

For this reason, the Spanish plan is introduced here not as the central focus of the chapter, but as part of the broader context in which the dissertation's conclusions are situated. Its importance lies in its resonance with both the national deficits documented in this research and the international frameworks that now set the standards for reform. The plan acknowledges many of the shortcomings identified in the thesis, but its effectiveness will depend on how research, professional practice, and civic engagement converge to ensure that commitments translate into enforceable and measurable change. The WHO 2025 guidance, by contrast, situates

Spain within a global call to transform psychiatry, providing an external benchmark against which progress will inevitably be judged.

The convergence of Spain's new *Plan de Acción de Salud Mental 2025-2027* and the WHO's 2025 guidance provides a unique vantage point from which to situate the dissertation's findings in both national and international contexts. The evidence gathered throughout this research, professional surveys revealing limited participation in deprescription, ethnographic accounts of coercion and epistemic exclusion, and the documented persistence of pharmacocentrism, reflects precisely the issues that these frameworks identify as requiring urgent reform (P. E. Deegan et al., 2024). The fact that these concerns are now explicitly recognized at policy level underscores the relevance of the dissertation's contributions to ongoing debates about psychiatric governance. Yet, as both national and international experiences demonstrate, the transition from declarative commitment to systemic change is complex, often hindered by professional cultures, institutional inertia, and uneven resource allocation (Juliá-Sanchis et al., 2020).

Comparative evidence from Europe offers critical insights into what may be possible. The Council of Europe Compendium of Good Practices documents models that have successfully reduced coercion, integrated community care, and reinforced patient rights, including Finland's open dialogue approach (Abad-Sierra & Toledano-Márquez, 2023), crisis houses in the United Kingdom (J. Read et al., 2015), and structured deprescription initiatives in the Netherlands (van Os & Groot, 2023). These examples, and many others, demonstrate that alternatives to institutional and coercive care are not theoretical aspirations but operational realities when supported by coherent policy, legal safeguards, and adequate funding. Prior work identifies financial and legal factors associated with over-reliance on pharmacological interventions; the present data align with those patterns.

At the global level, the WHO 2025 guidance emphasizes the integration of social determinants, housing, employment, education, and community participation, into mental health policy, in recognition that psychiatric outcomes are shaped as much by structural environments as by clinical interventions (Every-Palmer et al., 2021). This resonates with the biocultural approach adopted throughout this thesis, which frames pharmacological treatment as only one element within a complex network of social, legal, and ecological determinants of health (Kalocsai et al., 2024). The emphasis on social determinants also aligns with the United Nations Convention on the Rights of Persons with Disabilities, which obliges states to create conditions for autonomy and inclusion beyond the clinic (Melillo et al., 2025). Spain's plan, while recognizing the importance of community integration, remains primarily health-

sector oriented, underscoring the need for intersectoral approaches if its goals are to be realized (Uriarte Uriarte & Vallespí Cantabrana, 2017).

Scientific developments in psychiatry provide further direction for next steps. The Precision Psychiatry Roadmap (Kas et al., 2025b) proposes a paradigm shift toward biologically and digitally informed models of care, integrating biomarkers, wearable technologies, and harmonized data infrastructures. This vision has twofold relevance to the dissertation. Such approaches underscore the need for transparent, objective, traceable decision-making systems, which parallels this research's call for reproducible medication management and version-controlled health records. They also highlight the risk that technological innovations could reinforce existing biases if implemented without safeguards: algorithmic systems trained on coercive or pharmacocentric data risk entrenching precisely the practices that the WHO and UN have called to abolish (Hoffman, 2011). Spain's plan to modernize health information systems opens the possibility of alignment with precision psychiatry, but only if transparency, accountability, and rights-based governance are built into their design (Elliott & Resnik, 2019).

The OECD has repeatedly emphasized the importance of cross-national benchmarking in mental health policy, noting that countries with systematic data infrastructures are better able to identify gaps and implement reforms (Bienassis et al., 2022). Spain's traditionally fragmented information systems have limited such evaluation, contributing to the invisibility of coercive practices and the under-reporting of psychotropic overuse (Moreno Pérez et al., 2023). The dissertation has shown how, in practice, this lack of traceability undermines patient rights, as medication plans and coercive measures are rarely subject to independent audit. The national plan's commitment to strengthen digital monitoring responds to this deficit, but implementation will require more than technical upgrades: it will demand cultural change within institutions and research partnerships capable of ensuring that data infrastructures are transparent, participatory, and resistant to misuse (Kalman et al., 2024).

The relevance of these converging frameworks to this dissertation lies in the clarity they bring to the next steps in both research and practice. The evidence presented here demonstrates that coercion, overmedication, and epistemic exclusion remain structurally embedded in Spanish psychiatry (Aaddam, 2024). The *Plan de Acción 2025-2027* acknowledges these deficits (Sanidad, 2025), and the WHO 2025 guidance sets out a global blueprint for reform (World Health Organization, 2025b), while precision psychiatry articulates a scientific agenda for future innovation (Kas et al., 2025b). Each represents a different dimension, national

policy, international human rights, and biomedical science, against which the findings of this thesis can be understood. Together, they situate the dissertation not as an isolated critique but as part of a wider effort to transform psychiatric care into a system that is transparent, rights-based, and scientifically coherent. The following sections will examine current achievements in shared decision making in mental health in Spain, persistent gaps, the role of research in supporting implementation, and broader societal foundations required to keep on improving the services.

7.1. Achievements and developments

The findings of this dissertation demonstrate how Spanish psychiatry remains structured around pharmacological continuity, discretionary use of coercion, and limited implementation of shared decision-making. These patterns, documented through ethnographic evidence, professional surveys, and policy analysis, are not merely anecdotal but reflect systemic features of practice corroborated by national reports (Ramos Pozón, 2021), pharmacoepidemiological research (Bramsfeld et al., 2016), and comparative European studies (Pinto-Meza et al., 2013). Against this backdrop, any reform that foregrounds suicide prevention, child and adolescent services, coercion reduction, deprescription, and improved information systems must be understood not as isolated policy innovations but as direct responses to deficits repeatedly identified by scholarship, professional associations, and users themselves.

From a research perspective, one achievement over the past decade has been the consolidation of suicide prevention as a priority for public health in Spain (Patton et al., 2022). This dissertation documented the absence of systematic approaches to distress management and crisis support in many psychiatric settings, where coercion often substituted for dialogical engagement. The national focus on suicide prevention reflects a recognition of these limitations and draws attention to the need for intersectoral collaboration across education, social services, and primary care (Navarro Gómez, n.d.). Research can contribute by ensuring that monitoring tools for suicide risk are not reduced to algorithmic checklists but are integrated into context-sensitive, rights-based interventions (Chesney et al., 2014). Comparative international results illustrate that suicide prevention strategies succeed when research informs both design and evaluation, ensuring that evidence guides continuous improvement (Motillon-Toudic et al., 2022).

The strengthening of child and adolescent mental health services represents another area where research is critical. The dissertation highlighted how professional

uncertainty and institutional inertia often prevent consideration of reversible contributors to psychiatric distress, such as nutritional deficiencies or social adversity. This is particularly salient in minors, where over-reliance on psychotropics may entrench long-term vulnerability. Spanish epidemiological studies have documented a sharp rise in mental health problems among adolescents, particularly following the COVID-19 pandemic (Del Castillo & Velasco, 2020). The emerging research consensus emphasizes early, community-anchored interventions, integration with schools, and family-centered approaches (Abrines Jaume et al., 2012). Research is indispensable here, not only in designing interventions but also in documenting their outcomes and ensuring equitable access across Spain's decentralized health system (Miranda-Ruche, 2018).

Coercion reduction, long emphasized in international literature (Garcia Cabeza, 2016), emerges in this dissertation as one of the clearest points of convergence between ethnographic testimony, survey responses, and national oversight reports. The routine use of mechanical restraint and forced medication, often justified as pragmatic necessity, undermines both rights and clinical effectiveness. The recognition of coercion reduction as a national objective provides a framework through which research can play a corrective role: developing evaluation indicators, piloting alternatives such as crisis teams or dialogical networks, and measuring outcomes beyond hospitalization rates, including quality of life and autonomy. Evidence from Finland's open dialogue (Seikkula et al., 2018) and UK crisis resolution services (DeMatteo et al., 2013) shows that coercion can be substantially reduced when research-informed models are systematically implemented. Spanish research institutions have the opportunity to adapt and evaluate these approaches, ensuring that national targets are backed by empirical monitoring.

Medication management and deprescription illustrate perhaps most directly the overlap between the dissertation's findings and recent policy priorities. The data presented here showed how professionals often default to continuing pharmacological treatment even in the face of side effects, with deprescription framed as risky or impractical. This reflects wider national trends of high psychotropic consumption (S. I. Cohen, 1975). The growing recognition of deprescription within Spanish policy discourse opens the door to a stronger role for research: evaluating tapering protocols, identifying barriers to implementation, and documenting user perspectives on safety and efficacy. International experiences such as Dutch antipsychotic tapering clinics (Breggin & Wheeler, 2012) and UK antidepressant withdrawal protocols (M. A. Horowitz & Taylor, 2019) provide evidence-based models. Spanish research can build on these by conducting longitudinal studies, integrating qualitative accounts of lived experience, and

ensuring that deprescription becomes not a discretionary option but a measurable standard of care (Villagrán et al., 2014).

The modernization of information systems must be understood as a precondition for reform. The dissertation highlighted how the absence of traceable records and reproducible decision-making chains perpetuates epistemic exclusion: patients cannot contest decisions when protocols are opaque, and clinicians cannot be held accountable when records lack version control. The integration of monitoring indicators into national information systems represents an opportunity to correct this deficit. Yet, the scientific challenge is substantial: without safeguards, digital infrastructures risk embedding existing biases and normalizing coercion through algorithmic pathways (Kalman et al., 2024). Research can support this development by ensuring that data systems are transparent, participatory, and ethically governed. Spain's modernization efforts, therefore, will require strong partnerships between policymakers, clinicians, and researchers to ensure that data serve accountability and patient autonomy, rather than institutional inertia (McGinty & Eisenberg, 2022)

Beyond the areas of suicide prevention, child and adolescent services, coercion reduction, deprescription, and information system modernization, several additional developments merit closer consideration when interpreted through the lens of this dissertation. Each speaks to challenges long identified in Spanish psychiatry and highlights domains where research can contribute to ensuring that advances are consolidated rather than diluted by institutional inertia.

One achievement is the explicit recognition of workforce development as a necessary axis of reform (Légaré et al., 2012). Earlier chapters of this dissertation documented how professionals often cited systemic constraints, lack of training, limited institutional support, and resource scarcity, as reasons for maintaining pharmacological continuity and avoiding deprescription. The *Plan de Acción 2025-2027* proposes reinforcement of staff numbers and specific training programmes (Sanidad, 2025). International evidence demonstrates that workforce development is central to reducing coercion and expanding rights-based care (Funk et al., 2021b). For example, the Council of Europe compendium describes how continuous professional education has been tied to measurable reductions in restraint (Council of Europe, 2021b). Research in Spain can contribute by evaluating the effectiveness of new training modules, identifying barriers to practice change, and ensuring that curricula include evidence on trauma-informed approaches (Sweeney et al., 2018), shared decision-making (Parrabera-García et al., 2023), and deprescription strategies (Miranda-Ruche, 2018).

A related achievement is the explicit orientation toward community-based services. The dissertation illustrated how hospital-centrism and institutional inertia often displace community-based alternatives, leading to unnecessary medication and coercion. The national plan reiterates commitments to strengthening community psychiatry and integrating services with primary care. This aligns with longstanding WHO recommendations to shift resources from inpatient to community settings (World Health Organization, 2021a) and with UN reports that criticize over-reliance on institutions as a violation of rights (United Nations, 2017). Comparative European experiences, such as Italy's community psychiatry model following Law 180, or the crisis resolution teams in the UK, demonstrate that community care can reduce hospitalization and improve quality of life (O'Brien, 2025). Spanish research can support these goals by documenting outcomes of community services, analyzing regional disparities, and identifying mechanisms for scaling good practices across the autonomous communities (Tortella-Feliu et al., 2016a).

Another development of note is the plan's attention to vulnerable populations. Ethnographic materials analyzed in this dissertation highlighted how individuals with intersecting vulnerabilities, minors, elderly people, migrants, are particularly exposed to coercion and overmedication. The plan recognizes the need for targeted interventions, echoing findings from Spanish epidemiology that document high antipsychotic prescribing rates in older adults and disproportionate burdens among marginalized groups (Miranda-Ruche, 2018). At international level, WHO guidance emphasizes that protecting vulnerable groups is an ethical and clinical imperative (Seng et al., 2012), while the UN CRPD mandates specific safeguards for people with disabilities (Barranco Avilés, 2021). Research can assist by monitoring equity indicators, evaluating the impact of targeted interventions, and ensuring that vulnerability is not treated as justification for coercion but as a condition demanding increased rights protections.

The new plan also situates mental health within a digital transformation agenda, committing to upgrade data infrastructures and develop indicators that enable evaluation of progress. This emphasis resonates with the dissertation's critique of opaque decision-making and the absence of version-controlled medical records. Digital infrastructures can provide tools for transparency, accountability, and patient participation, but they also carry risks of embedding existing coercive norms into algorithmic systems if governance is weak (Timmons et al., 2023). Research in Spain can play a critical role by ensuring that digital reforms incorporate user perspectives, ethical oversight, and robust evaluation frameworks. International experiences with open-source clinical decision support systems show that transparent design and external audit are key to avoiding bias and reinforcing accountability (Paton et al.,

2022). In this sense, the modernization of Spanish data systems presents a decisive opportunity for researchers to shape infrastructures that facilitate shared decision-making and protect autonomy.

This achieves symbolic importance by situating mental health at the forefront of public debate in Spain. This is the first time since the early psychiatric reform movement that mental health has been addressed through a nationally coordinated, multi-year action plan, endorsed at the highest levels of government. For the dissertation, this represents confirmation that the issues documented ethnographically and through survey evidence, pharmacological overuse, coercion, exclusion from decision-making, are not marginal critiques but matters of systemic importance recognized in national and international forums (Healy, 2012). The achievement lies not only in the plan's content but in the political recognition that psychiatry requires structural reform grounded in rights, prevention, and transparency (Kinderman, 2014b).

These achievements must be viewed with caution, as their realization depends on implementation, financing, and oversight. Nonetheless, when interpreted alongside this dissertation, they illustrate that the deficits described throughout the research are no longer invisible or relegated to isolated critiques. They are now embedded in policy discourse and subject to international scrutiny, providing fertile ground for research to contribute to system change. Academic institutions, professional associations, and user groups can ensure that these developments move beyond rhetoric, providing independent evaluation and evidence-based tools to strengthen shared decision-making, reduce coercion, and create transparent infrastructures for psychiatric care in Spain.

7.2. Persistent gaps and challenges

Despite the policy advances discussed in the previous section, the findings of this dissertation, when situated within the broader Spanish and international literature, underscore that many of the systemic deficits of psychiatric care in Spain remain unresolved. A central issue is the persistent reliance on coercive interventions, including mechanical restraint, seclusion, and involuntary pharmacological treatment. National oversight bodies have repeatedly documented the routine use of these practices across psychiatric units, including those dedicated to children and adolescents (G. M. Martínez et al., 2021). Ethnographic testimonies collected during the fieldwork presented in this dissertation coincide with these reports, indicating that coercion is not deployed as a measure of last resort but is normalized as an institutional response to behavioral dysregulation. This is consistent with

comparative studies in Europe that highlight Spain as a country where coercion remains structurally embedded, contrasting with jurisdictions that have achieved measurable reductions through investment in dialogical and community-based alternatives (Ungar-Sargon, 2024). International standards articulated by the WHO QualityRights framework and the United Nations Convention on the Rights of Persons with Disabilities explicitly classify coercive interventions as incompatible with human rights obligations, urging states to implement supported decision-making and non-coercive care pathways (P. E. Deegan & Drake, 2006). The persistence of coercion in Spain thus reflects both inadequate development of community infrastructures and enduring professional cultures that privilege containment over participatory engagement (Aragonés-Calleja & García-Sánchez, 2024).

Closely connected to coercive practice is the enduring pharmacocentric orientation of Spanish psychiatry. Pharmacoepidemiological data consistently place Spain among the highest consumers of benzodiazepines, antidepressants, and antipsychotics in Europe (Roncero et al., 2025). Although deprescription has recently entered national policy discourse, the ethnographic and survey data presented here confirm that it remains a discretionary and sporadic process rather than a standardized clinical routine. Professionals frequently report reluctance to engage in tapering, citing fears of relapse and institutional discouragement. This is consistent with national studies that describe the absence of systematic deprescription protocols in clinical practice (M. Horowitz & Taylor, 2024). In contrast, international experiences demonstrate the feasibility of structured withdrawal when adequately supported by research and clinical guidelines. Dutch tapering clinics for antipsychotics (Bellonci & Huefner, 2020) and UK protocols for managing antidepressant withdrawal (M. Horowitz & Taylor, 2024) illustrate that deprescription can be safe and effective when embedded within system-level frameworks and accompanied by appropriate monitoring. The limited institutionalization of deprescription in Spain therefore reflects a systemic inertia that undermines both patient safety and the formal commitments to autonomy articulated in policy frameworks (Bellmunt & Ángel, 2022).

Fragmentation and regional disparity further complicate the Spanish mental health system. The decentralized organization of healthcare produces pronounced inequalities in access to psychological therapies, community psychiatry, and alternatives to hospitalization, with some autonomous communities developing advanced networks while others remain dependent on inpatient structures (Guillén Rodríguez et al., 2016). Ethnographic accounts collected for this dissertation provide qualitative evidence of these disparities, where access to alternatives to

pharmacological treatment or to dialogical interventions is determined not by clinical indication but by regional variation in resource allocation. The OECD has emphasized that such fragmentation weakens equity, transparency, and accountability, particularly in the absence of national indicators that permit systematic comparison across regions (Bienassis et al., 2022). Spain's limited psychiatric information systems, characterized by inconsistent clinical data and patient feedback collection, and weak monitoring of coercion and medication patterns, perpetuate these disparities and impede evidence-based policy correction (Blayney, 2022).

A final concern, equally evident in both the empirical material of this dissertation and the wider literature, is the weakness of legal safeguards and accountability mechanisms. While Spanish law guarantees informed consent and patient autonomy (Chenoweth, 1995a), studies consistently demonstrate that in psychiatry these rights are inconsistently applied, with substituted decision-making and paternalism remaining prevalent (Ugartechea, 2004). Oversight committees and institutional review boards are frequently criticized for functioning as legitimating rather than independent evaluative bodies (Garcia-Gonzalez, 1998). International evaluations underscore that rights protections without enforcement mechanisms remain symbolic, offering little substantive protection to individuals subject to coercive treatment (Gil-Borrelli et al., 2017). In Spain, the limited avenues for patients to challenge clinical decisions or report coercive interventions indicate that the translation of formal legal frameworks into practice remains weak. This phenomenon, described in socio-legal literature as the *implementation gap*, has been extensively documented in mental health law globally, where patient rights exist at a statutory level but are undermined by professional discretion and institutional inertia (Trueba, 2022).

These deficits, persistence of coercion, systemic pharmacocentrism, regional disparities, and weak accountability, demonstrate that the challenges identified in this dissertation are not aberrations but structural characteristics of Spanish psychiatry (Agudelo-Hernández & Rojas-Andrade, 2023). They situate the findings within a broader body of evidence that documents the disconnect between policy commitments and clinical practice, and they underscore the continued urgency of research capable of producing empirical evidence, monitoring implementation, and informing the design of enforceable, rights-based reforms.

In addition to the structural deficits already outlined, the modernization of digital infrastructures presents a paradoxical challenge. While the *Plan de Acción de Salud Mental 2025-2027* places emphasis on developing monitoring systems and

standardized indicators, the empirical material of this dissertation and international literature both suggest that without rigorous governance, such systems risk codifying existing coercive and pharmacocentric practices rather than correcting them. Research on algorithmic medicine indicates that electronic health records and decision-support systems can embed institutional biases, reinforce hierarchies, and obscure professional accountability if their design and use lack transparency (Moreno Küstner, 2012). The absence of version-controlled records and independent audit mechanisms in Spanish psychiatry, highlighted in this dissertation (Valero Garcés et al., 2023), illustrates how digital infrastructures currently undermine rather than enhance patient autonomy. International examples of open, auditable algorithms demonstrate that digital transformation can promote accountability, but only when systems are transparent, subject to external scrutiny, and co-designed with users (Villagrán et al., 2015). The challenge for Spain is therefore not only technological but epistemological and ethical: to ensure that data systems enable shared decision-making and accountability rather than serve as tools for surveillance and containment (Canabal et al., 2024).

Workforce shortages and training deficits constitute another persistent barrier. Despite policy recognition of the need for workforce reinforcement, Spanish psychiatry continues to face limited availability of psychologists, social workers, and trained child psychiatrists, particularly in primary care and community settings (Carbonell Marqués & Navarro-Pérez, 2019). The surveys conducted as part of this dissertation confirmed that professionals often lack training in deprescription protocols, shared decision-making methods, and trauma-informed approaches. This deficit is not simply a quantitative issue of staffing levels but reflects the persistence of curricula and professional cultures that prioritize biomedical management over dialogical and rights-based practice (Quiroz-Hidrovo et al., 2024). Comparative research in Northern Europe has demonstrated that sustained professional training in non-coercive practices can measurably reduce restraint and improve patient outcomes (Anestis et al., 2024). Without equivalent investment in Spain, the risk remains that policy targets will be undermined by limited capacity and entrenched professional paradigms.

A further gap concerns the neglect of social determinants of mental health. The biocultural orientation of this dissertation has emphasized that mental health trajectories are shaped not solely by clinical interventions but by housing, employment, education, and social protection. WHO's 2025 guidance places structural determinants at the center of reform, calling for policies that integrate mental health into broader welfare and educational systems (World Health Organization, 2025b). Yet, Spanish policy initiatives continue to focus

predominantly on healthcare services, with limited intersectoral strategies and underdeveloped collaboration across ministries. Ethnographic accounts collected for this dissertation confirm that patients frequently encounter psychiatric services that treat distress as an isolated symptom rather than as an outcome of structural deprivation. This reductionism perpetuates cycles of medicalization and marginalization, weakening the potential for sustainable recovery. Comparative evidence indicates that integrated approaches, such as housing-first models in Canada (Woodhall-Melnik & Dunn, 2016) or community-based vocational rehabilitation in Scandinavian countries, are associated with improved outcomes and reduced hospital use (Buus et al., 2021). The absence of similar systemic approaches in Spain remains a critical gap.

Spain's positioning in relation to the new WHO 2025 guidance reflects both opportunities and risks. The guidance establishes a global blueprint for reform, emphasizing leadership, service organization, workforce development, person-centered care, and attention to social determinants (Miranda-Ruche, 2018). Spain's national plan echoes several of these priorities, particularly suicide prevention, child and adolescent care, and coercion reduction. However, the ethnographic and survey findings of this dissertation illustrate that the distance between declarative commitment and operational practice remains substantial. Without independent evaluation, enforceable legal safeguards, and participatory monitoring, there is a significant risk that the reforms outlined in policy will remain symbolic, failing to alter entrenched institutional practices. The WHO guidance emphasizes the centrality of lived experience in system design, yet, Spanish psychiatry continues to marginalize user participation, treating it as consultative rather than co-productive (Serra, 2025). This divergence between global standards and local practice underscores the urgency of research that not only documents gaps but provides actionable, empirically grounded pathways for reform (Inchauspe Aróstegui, 2019).

These challenges, digital transformation without safeguards, workforce shortages and training deficits, neglect of social determinants, and incomplete alignment with international frameworks, demonstrate the complexity of translating policy commitments into systemic change. The dissertation's findings reinforce that reforms must be evaluated not only by their presence in planning documents but by their capacity to restructure everyday practices of care. The persistence of coercion, pharmacocentrism, and exclusion from decision-making confirms that Spain remains at a critical juncture, where the success of the current plan will depend on whether research, oversight, and civic engagement can transform declarative commitments into enforceable, rights-based standards of practice.

7.3. Role of Research in implementation and innovation

The preceding sections have illustrated how Spanish psychiatry continues to exhibit structural deficits, coercion, pharmacological over-reliance, fragmented services, and weak accountability, that persist despite successive policy commitments. The *Plan de Acción de Salud Mental 2025-2027* recognizes many of these problems at the level of discourse, while the 2025 WHO guidance situates them within a global call for system transformation. Yet, bridging the gap between policy and practice requires a central role for research. Evidence gathered in this dissertation demonstrates that reforms will remain symbolic unless they are operationalized through rigorous evaluation, participatory monitoring, and translational frameworks capable of embedding best practices into everyday clinical contexts (Beidas et al., 2013).

One critical dimension concerns the evaluation of coercion reduction initiatives. As ethnographic and survey findings presented in this thesis showed, coercion remains normalized within many psychiatric settings in Spain, despite legal guarantees of patient autonomy. International research demonstrates that coercion can be substantially reduced when evidence-based alternatives are implemented, such as crisis resolution teams in the United Kingdom (Wheeler et al., 2015), open dialogue networks in Finland (Freeman et al., 2019), and peer-support models in Canada (Richard et al., 2022). Spanish research infrastructures are well positioned to pilot such models, yet, few systematic evaluations have been undertaken. Embedding coercion-reduction interventions into research protocols, supported by standardized outcome measures including patient-reported experience, quality of life, and long-term recovery, would enable Spain to align policy commitments with demonstrable practice change (Abma et al., 2017). Without research-led evaluation, targets risk being reduced to rhetorical indicators rather than enforceable standards of care.

Another area where research is indispensable is in the field of medication management and deprescription. The dissertation documented how professionals frequently maintain long-term pharmacological regimens even when patients report significant adverse effects, reflecting both institutional inertia and insufficient deprescription training. While the national plan now references deprescription, the absence of validated tapering protocols and standardized monitoring frameworks undermines implementation. International experiences illustrate the role of research in operationalizing deprescription: Dutch antipsychotic tapering clinics have generated longitudinal outcome data (Wand, 2025), and UK withdrawal studies

have produced evidence-based tapering schedules for antidepressants (Gale et al., 2012). Comparable Spanish research could provide empirical foundations for national guidelines, documenting both clinical safety and patient perspectives. Incorporating mixed-methods designs that include ethnographic narratives alongside biomedical monitoring would also ensure that deprescription is approached as both a physiological and a psychosocial process.

Broader research in social determinants of mental health, which remain insufficiently integrated into Spanish policy (Ruiz Álvarez et al., 2022). The ethnographic evidence collected for this dissertation demonstrated how poverty, unemployment, and insecure housing compound psychiatric distress, yet, clinical services frequently address these issues through pharmacological interventions rather than structural supports. The WHO's 2025 guidance emphasizes intersectoral strategies to address social determinants (Aneshensel, 2005), while international studies confirm the effectiveness of integrated models such as housing-first interventions (Padgett et al., 2016). Spanish research can contribute by evaluating pilot programmes that integrate social and psychiatric support, documenting both clinical and social outcomes, and generating data that can inform interministerial collaboration.

The effective implementation of psychiatric reform in Spain depends not only on the articulation of policy goals but on the development of translational research capable of bridging the gap between declarative frameworks and everyday practice. A central finding of this dissertation has been the extent to which institutional inertia and professional cultures perpetuate coercion, pharmacological continuity, and epistemic exclusion even in the presence of legal and policy commitments to autonomy and rights. Research can counteract this inertia by generating empirical evidence that renders structural deficits visible, tests alternatives under real-world conditions, and establishes feedback mechanisms that compel institutions to adapt.

Translational research methodologies, including pragmatic clinical trials and mixed-methods evaluation, are particularly suited to this task. By embedding interventions such as crisis resolution teams, deprescription protocols, or dialogical networks into routine care and systematically documenting outcomes, research can move beyond controlled environments to assess effectiveness under the complex conditions of the Spanish health system (Olsen et al., 2004). This aligns with international calls for implementation science approaches to mental health, which emphasize fidelity, scalability, and contextual adaptation as key to sustainable reform (Wheeler et al., 2015). Without such translational infrastructures, policy commitments risk remaining aspirational (McGinty & Eisenberg, 2022).

Participatory methodologies constitute another indispensable research strategy. The ethnographic data presented in this dissertation revealed how patients' perspectives are frequently marginalized in decision-making, with dissent often reclassified as non-compliance. International frameworks, including the WHO QualityRights initiative and the UN CRPD, emphasize the necessity of integrating lived experience into both policy design and service delivery (Abma et al., 2017). Research methodologies that co-produce knowledge with service users, through participatory action research, peer-led evaluation, and deliberative forums, are critical to operationalizing this principle (Tangvald-Pedersen & Bongaardt, 2017). Spanish research institutions can draw on these methods to ensure that reforms are evaluated not only by clinical outcomes but also by indicators of autonomy, participation, and rights realization (Correa-Urquiza, 2012).

Integration with European Union initiatives provides an additional avenue through which research can strengthen implementation. The *Council of Europe Compendium of Good Practices* demonstrates the feasibility of alternatives to coercion and overmedication across diverse national contexts (Council of Europe, 2021b), while the OECD highlights the role of cross-national benchmarking in promoting accountability (Llena-Nozal, 2009). Participation in European research networks, including Horizon Europe and COST Actions, offers Spain opportunities to align national reforms with international standards, exchange evidence on implementation, and contribute to collective knowledge production (Á. Martínez-Hernández et al., 2020). This dissertation itself, through its analysis of structural violence and rights-based frameworks, provides empirical material that can enrich such transnational research collaborations.

The role of research is therefore threefold. First, to provide independent, empirically grounded evaluation of policy implementation, ensuring that commitments to coercion reduction, deprescription, and community care are translated into measurable outcomes (C. C. Lewis et al., 2019). Second, to develop participatory methodologies that incorporate the perspectives of service users as co-producers of knowledge, thereby addressing the epistemic exclusion documented in this dissertation. Third, to engage with cutting-edge scientific paradigms such as precision psychiatry and implementation science, ensuring that innovation is ethically governed and aligned with human rights standards. Together, these dimensions position research not as an auxiliary to policy but as a structural component of reform, capable of transforming declarative commitments into accountable, transparent, and rights-based psychiatric practice.

The cumulative evidence reviewed in this dissertation and contextualized within national and international frameworks confirms that the transformation of psychiatric care in Spain cannot be reduced to policy declarations alone. The persistence of coercion, the dominance of pharmacocentric practice, the weakness of accountability mechanisms, and the limited integration of lived experience illustrate the resilience of structural inertia. In this context, research acquires not an ancillary but a constitutive role in the design, evaluation, and refinement of mental health systems. Without empirically grounded mechanisms of verification, policy frameworks remain vulnerable to symbolic compliance and the perpetuation of practices that contravene both scientific evidence and human rights standards.

International comparisons underscore that system reform is possible when research infrastructures are embedded within governance structures. In the Netherlands and elsewhere, deprescription clinics have been able to produce longitudinal data that challenge assumptions about the irreversibility of long-term pharmacological treatment (Jacob, 2019). In Finland, the open dialogue approach has been sustained through decades of academic evaluation, generating evidence that has informed both national practice and international dissemination (Maude et al., 2024). In the United Kingdom, the establishment of crisis resolution and home treatment teams was accompanied by extensive research evaluation, producing outcome data that influenced their institutionalization within the National Health Service (Burns et al., 2001). These cases further exemplify how research not only measures but actively shapes the trajectory of reform, providing legitimacy, accountability, and adaptation mechanisms.

For Spain, the current juncture defined by the *Plan de Acción de Salud Mental 2025-2027* and the WHO's 2025 guidance represents an opportunity to develop a comparable research-policy nexus. This requires a commitment to systematic evaluation of coercion reduction, deprescription, and community interventions; the design of transparent digital infrastructures that embed explainability and independent oversight; and the creation of participatory methodologies that co-produce knowledge with service users. Such commitments would address the core deficits documented in this dissertation, while also aligning Spain with the standards articulated by the Council of Europe (Council of Europe, 2021b), the OECD (Llena-Nozal, 2009), the World Health Organization (World Health Organization, 2021a) and the United Nations (United Nations, 2017).

The role of research, therefore, is not exhausted by the production of knowledge but extends to the creation of conditions under which rights, autonomy, and evidence become operational rather than rhetorical. Implementation science frameworks

have already shown how fidelity, scalability, and contextual adaptation can be studied and optimized within complex health systems (Baldwin et al., 2022). Applied to psychiatry, such approaches offer a pathway to overcome the *implementation gap* that has long characterized Spanish mental health policy, where statutory rights and international obligations coexist with practices of restraint and overmedication. Research, by documenting both successes and failures, functions as a mechanism of systemic reflexivity, compelling institutions to acknowledge error, recalibrate interventions, and adjust policy in real time.

the persistence of systemic deficits in Spanish psychiatry confirms the necessity of a sustained research agenda that integrates evaluation, innovation, and participation. The dissertation's findings demonstrate that without rigorous monitoring and transparent infrastructures, reforms remain vulnerable to institutional inertia and symbolic compliance. By embedding research within governance, Spain has the opportunity to align with international standards, harness advances in precision psychiatry, and develop systems of care that are empirically accountable and rights-based. The future trajectory of Spanish psychiatry will depend less on the rhetorical strength of policy documents than on the capacity of research to produce actionable evidence, enforceable accountability, and infrastructures capable of sustaining continuous improvement. In this sense, research is not an adjunct to reform but the structural mechanism through which declarations of rights and autonomy may become reproducible, verifiable, and realized in practice (Russo & Wooley, 2020).

The consequences of failing to adequately address the systemic deficits documented in this dissertation are not confined to institutional inefficiency or professional dissatisfaction; they manifest directly in premature mortality, chronic morbidity, and widespread diminishment of quality of life. International epidemiological data confirm that individuals diagnosed with severe mental health conditions experience a reduction in life expectancy of 10 to 20 years, attributable not only to illness but to the cumulative effects of coercion, pharmacological overuse, social exclusion, and inadequate access to preventive care (Reinhard et al., 2020). In Spain, pharmacoepidemiological analyses reveal persistently high rates of benzodiazepine and antipsychotic prescribing, often without systematic review, which are associated with increased risks of metabolic syndrome, cardiovascular disease, and cognitive decline (Heinz et al., 2020). The neglect of deprescription, highlighted in this dissertation, thereby translates into a measurable burden of iatrogenic harm, compounding pre-existing health inequities.

The human cost extends beyond somatic morbidity to encompass elevated suicide risk, an area in which Spain continues to report thousands of deaths annually,

making suicide the leading external cause of mortality (*Contención Del Suicidio En España: Evaluación Del Diseño de Las Políticas y Planes de Salud Mental de Las Comunidades Autónomas | Gestión y Análisis de Políticas Públicas*, n.d.). Studies consistently link coercive psychiatric interventions, especially involuntary hospitalization and restraint, to increased suicidality and disengagement from services (Moreno Pérez et al., 2023). The ethnographic testimonies analyzed in this dissertation resonate with these findings, documenting how experiences of coercion erode trust, reinforce dehumanization and outcasting, and drive long-term avoidance of care. International evidence further demonstrates that lack of access to participatory, rights-based services contributes to elevated suicide mortality in marginalized groups, including youth, migrants, and individuals with disabilities (García-Haro et al., 2023). The cost of inaction, therefore, is borne not only in deaths by suicide but in the silent attrition of lives shaped by untreated distress, preventable crises, and enduring alienation from services.

The economic burden of untreated or poorly treated mental health conditions also underscores the societal cost of inertia. OECD analyses estimate that mental health-related productivity losses and health expenditures account for more than 4% of GDP in European countries, with Spain reflecting comparable trends (Jina-Pettersen, 2022). These losses are amplified by the systemic reliance on pharmacological containment rather than psychosocial and preventive interventions, which are consistently more cost-effective in long-term outcome studies (Park et al., 2020). Research has shown that coercion and institutionalization not only fail to prevent relapse but also exacerbate chronicity (Villarroel, Garcia, et al., 2021), generating cycles of dependency that reinforce economic and social marginalization (Reinhard et al., 2020). The structural absence of supported decision-making, documented in this dissertation, thus produces costs that are simultaneously biological, social, and economic, eroding both individual potential and collective resources (Alguera-Lara et al., 2017).

Quality of life indicators consistently reveal further dimensions of harm. Patients subjected to coercive interventions report persistent feelings of humiliation, loss of dignity, and erosion of agency, with long-term psychological sequelae comparable to trauma exposure (Barrio et al., 2020). Prolonged pharmacological treatment without patient participation is associated with diminished functioning, cognitive impairment, and reduced capacity for social integration (Yoshida & Takeuchi, 2021). The neglect of social determinants compounds these outcomes: insecure housing, unemployment, and educational exclusion significantly worsen prognosis, yet, psychiatric services in Spain continue to treat these factors peripherally, often medicalizing distress rather than addressing its structural determinants (Frances,

2013). The result is a pattern of cumulative disadvantage, whereby psychiatric labeling, coercion, and medication overuse converge to curtail opportunities for recovery, inclusion, and human flourishing (Reidy, 1993).

The ethical cost of inaction is equally significant. Failure to align with these standards exposes Spain not only to persistent health inequities but also to international scrutiny regarding compliance with binding human rights obligations (Blasco-Palau et al., 2023). The combined evidence indicates that delayed reform is associated with adverse outcomes. Premature mortality, chronic morbidity, diminished quality of life, preventable suicide, and rights violations are not abstract outcomes but measurable consequences of systemic inertia. This dissertation has shown that coercion, pharmacological over-reliance, and epistemic exclusion are not anomalies but routine features of Spanish psychiatry. Unless addressed through rights-based reform, rigorous research, and participatory governance, these practices will continue to reproduce cycles of harm, undermining both the credibility of mental health services and the possibility of recovery. The human cost of failing to implement the lessons documented here is therefore profound: a continued erosion of life expectancy, quality of life, and social inclusion for those most vulnerable, with consequences that extend beyond the clinic to the very fabric of Spanish society.

Chapter 8 - Final conclusions

This dissertation has provided empirical confirmation, based on five years of multi-sited ethnographic fieldwork in Spain, Sweden, Indonesia, and Trieste (Italy), combined with extensive surveys of professionals and users, that contemporary psychiatric systems in Spain and comparable European contexts continue to reproduce coercive, pharmacocentric, and institutionally protective practices despite the existence of binding legal safeguards and international human rights instruments (Munt & Littlechild, 2024b). The central contribution of this research lies in documenting the systematic dissonance between normative frameworks, which proclaim autonomy, dignity, and participation, and clinical realities where coercion, diagnostic inertia, and bureaucratic automatism prevail. By foregrounding lived experience and professional testimony, the dissertation demonstrates how epistemic exclusion is institutionalized: narrative, context, and personal meaning are consistently discounted, preventing reflexive correction and ensuring the persistence of harm cycles (Valencia, 2020).

The findings confirm that harm is not incidental but structurally embedded. Surveys and field observations showed that adverse events, critical incidents, and patient testimonies are rarely incorporated into professional learning systems. In practice, accountability is constrained by the absence of independent monitoring, reliable second-opinion procedures, and credible complaint mechanisms. This permits environments where psychological and physical ill-treatment may occur without effective redress (Runciman, 2023). These deficits disproportionately affect marginalized populations, including minors, older adults, migrants, and persons under guardianship, positioning psychiatry as a regulatory system that intersects with broader structures of inequality and control (Szasz, 1998).

Yet, the dissertation also identifies the presence of counter-practices, forms of ethical resistance, professional reflexivity, and user-led innovation, that empirically demonstrate the feasibility of alternatives. These include recovery-oriented, dialogical, and community-based approaches, observed in contexts as diverse as open dialogue in Finland, collaborative medication management in Quebec and Brazil, and emerging initiatives in Spanish community psychiatry and peer support (L. R. Del Barrio et al., 2013). Evidence from both fieldwork and international literature confirms that these models reduce hospitalization, improve functional recovery, and increase satisfaction, thereby demonstrating that rights-based practice is not aspirational but operationally viable (Sells et al., 2006).

The implications are unambiguous: the continued use of coercion and paternalism is inconsistent with current scientific knowledge, international legal norms, and ethical standards. Frameworks such as the Convention on the Rights of Persons with Disabilities, the WHO QualityRights initiative, and the jurisprudence of the European Court of Human Rights affirm that informed consent, protection from degrading treatment, and supported decision-making are binding obligations (P. E. Deegan & Drake, 2006). The dissertation's contribution is to demonstrate empirically how the absence of procedural accountability enables their routine violation.

The convergence of ethnographic, survey, and comparative evidence establishes that recovery-oriented and rights-based approaches are evidence-based necessities. Neuroscientific and psychoneuroimmunological research supports the physiological and psychological benefits of reducing unnecessary pharmacological exposure and of strengthening relational care (Bower & Kuhlman, 2023). Spanish scholarship has articulated parallel arguments, calling for psychiatry that is community-integrated, participatory, and human rights compliant (Beviá & Girón, 2017). The legitimacy of psychiatry is undermined when coercion persists as an ordinary practice, since coercion corrodes both scientific credibility and civic trust (Jina-Pettersen, 2022).

The final conclusion drawn from this research is that psychiatric care must undergo a paradigmatic reconfiguration. Discretionary authority must be replaced by verifiable standards, professional silencing by institutional listening, and coercion by systemic safeguards of autonomy, bodily integrity, and equality. Transformation requires not only critique but the deliberate construction of governance models that are scientifically rigorous, legally binding, ethically coherent, and socially responsive. Such reconfiguration is both feasible and necessary, and its realization constitutes a civic, legal, and scientific responsibility consistent with international obligations and the highest standards of medicine and human rights.

The evidence presented in this dissertation demonstrates that psychiatric care in Spain and comparable European contexts continues to be marked by limited implementation of shared decision-making and a structural reliance on pharmacological interventions. Survey data and ethnographic fieldwork show that, despite recognition of patient autonomy as a legal and ethical standard, treatment decisions frequently remain guided by institutional expedience and professional discretion rather than by collaborative processes. This situation is reflected in the international literature, which documents the consistent underuse of shared decision-making in psychiatry compared with other medical fields, despite its

demonstrated benefits for adherence, satisfaction, and clinical outcomes (P. E. Deegan & Drake, 2006).

A related finding concerns the entrenched pattern of medication overprescription and the insufficient development of deprescription protocols. Spain remains among the highest European consumers of psychotropic drugs, with prescribing practices that often extend beyond guideline recommendations and continue without systematic review (Sampietro et al., 2023a). Field observations confirmed that tapering or discontinuation is rarely structured or supported, leaving both professionals and users without clear procedures to mitigate risks of withdrawal or relapse. International evidence shows that deprescription, when adequately planned and monitored, can reduce adverse events and support recovery trajectories (M. G. Hunt & Resnick, 2015). The limited institutionalization of such approaches in Spain suggests a persistent gap between policy intentions and clinical practice.

These findings indicate that the sustainability of psychiatric care depends not on a wholesale paradigmatic reconfiguration, but on the systematic integration of existing evidence into routine practice. Shared decision-making must be operationalized through verifiable procedures rather than aspirational statements, while prescribing practices must be accompanied by structured review and deprescription protocols that prioritize safety, autonomy, and recovery. The dissertation demonstrates that these shifts are scientifically justified, legally supported, and ethically required, but remain underdeveloped in practice. Future research and policy implementation will need to ensure that the principles of collaborative decision-making and medication safety move from the margins of psychiatric discourse to the center of everyday clinical governance.

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Annex A - Social impact of the thesis

This dissertation generates empirical contributions with direct implications for public health, psychiatric practice, legal governance, and social inclusion. Evidence from ethnographic fieldwork in Spain, Sweden, Indonesia, and Trieste, together with survey data from users and professionals, shows that coercive psychiatric interventions, particularly non-consensual pharmacological treatment and the extension of diagnostic authority, are associated with adverse somatic, psychological, and social consequences. The findings show that chronic psychotropic exposure contributes to metabolic disorders, comorbidities, and functional decline, while undermining recovery and eroding trust in services. These effects, as well as the exposure to harm by forced drugging, are most pronounced among groups already facing structural vulnerability, such as women, minors, individuals with trauma histories, people living in poverty, and racialized or migrant populations. In such cases, coercion reframes deprivation or violence as psychiatric disorder, compounding disadvantage through medicalisation.

Beyond clinical outcomes, coercive practices are linked to diminished legal capacity, loss of occupational and educational opportunities, and weakening of family and community networks. This perpetuates cycles of dependency and exclusion, with consequences for participation in housing, employment, and civic life. Survey responses and fieldwork also highlight deficits in training, accountability, and oversight, which sustain these practices and reduce institutional responsiveness. At the same time, the research documents initiatives that mitigate harm: shared decision-making, gradual deprescription under supervision, and dialogical approaches that integrate user perspectives. These counter-practices demonstrate that reducing reliance on coercion is feasible, that recovery trajectories can be improved when user agency is respected, and that collaborative approaches enhance continuity of care.

The social impact of this dissertation lies in showing, with empirical evidence, that coercive psychiatric practices contribute not only to adverse health outcomes but also to the erosion of trust, autonomy, and legal capacity, disproportionately affecting vulnerable groups such as women, minors, migrants, and people in poverty. By documenting these harms and contrasting them with evidence of positive outcomes from dialogical practices, shared decision-making, and supervised deprescription, the work strengthens the case for reform of psychiatric governance. It demonstrates that alternatives are feasible, that respecting user participation reduces exclusion, and that accountability structures are essential for aligning mental health systems with human rights and social inclusion.

Annex B - Instruments used in this participatory action-research work

Annex B contains the full set of surveying instruments employed throughout this participatory action-research dissertation. These instruments were developed to explore lived experience, clinical and professional perspectives, and ethical-practical dimensions surrounding psychiatric labeling, medication practices, and structural violence in mental health. All instruments are made available through the dissertation's online repository, in accordance with open science standards, and are ready for adaptation, localization, and further validation. The repository includes not only the survey tools themselves, but also associated informed consent forms, participant information sheets, and guidance for ethical deployment.

Each instrument has been designed to elicit critical insights while respecting the autonomy and dignity of participants, with contextual messages included for use in validation and translation processes. Researchers, practitioners, and policy actors are invited to build upon this resource, adjusting language, structure, and content for diverse cultural and systemic settings. These instruments represent a core component of the dissertation's translational mission and can support future participatory processes, training modules, and empirical research committed to the principles of transparency, accountability, and epistemic justice.

Following are the questions of the schizophrenia inquiry survey and shared decision making survey, for quick reference, with the ones on open dialogue included in a separate annex with its own published paper.

Schizophrenia inquiry survey:

Section	Item / Question
Sociodemographic Information	What is your age?
	What is your gender identity?
	What is your country of origin?
	What is your country of residence?
	What is your highest level of education attained?
	What is your professional background or area of expertise?
Personal and Professional Experience	Have you been diagnosed with schizophrenia?
	Have you received other psychiatric diagnoses?
	Have you ever worked with people diagnosed with schizophrenia?
Perceptions on the Label	Do you believe the diagnosis of schizophrenia is useful for clinical practice?
	Do you believe the diagnosis contributes to stigmatization?

Section	Item / Question
Experiences and Reflections	Do you believe schizophrenia is a valid medical construct?
	Should the term schizophrenia be replaced with another denomination?
	Do cultural differences influence the way schizophrenia is diagnosed?
	Does a person's ethnicity or race play a role in receiving a schizophrenia diagnosis?
	Do you think gender plays a role in the diagnostic process?
	In your opinion, what are the main consequences of receiving the label schizophrenia?
	Have you or someone you know experienced discrimination due to this diagnosis?
	What changes would you suggest in the diagnostic process or terminology?

Shared decision making survey:

Section	Item / Question
Demographics	What is your profession?
	Where do you currently work?
	What is your role within your organization?
	How many years of experience do you have in mental health care or research?
Prescribing / deprescribing attitudes of respondents by scenario	
	First-episode psychosis (before establishing a long-term diagnosis)
	Depressive disorders (prescribed neuroleptics as augmentation)
	Bipolar disorder (long-term neuroleptic treatment for mood stabilization)
	Non-specified psychotic or mood disorders (diagnosis unclear or evolving)
	Neuroleptics prescribed off-label (e.g. insomnia, anxiety, personality disorders)
	Neuroleptics used in elderly patients (dementia-related agitation, behavioral management)
	Long-term neuroleptic use in institutionalized patients (e.g. chronic hospitalization, group homes)
	Pediatric and adolescent patients (under 18 years old)
	Patients with histories of trauma and abuse (where symptoms may be trauma-related)
Training	Have you received formal training on deprescribing strategies and shared decision-making?
Policy Scenarios	Implementing national screening for gluten sensitivity in psychiatric diagnoses
	Deprescribing neuroleptics in cases of missed metabolic and nutritional

Section	Item / Question disorders Addressing psychiatric abuse cases uncovered through national research on mental health and violence National deprescribing initiative for elderly patients at high risk of mortality and disability Reevaluating long-term psychiatric diagnoses in a national audit Addressing the overprescription of neuroleptics in social care and marginalized communities National review of the use of neuroleptics in children and adolescents Ending the use of neuroleptics for non-psychiatric indications in general medicine
Final Reflection	Given the above eight shared decision-making and deprescribing challenges, please reflect on the following: Which do you consider the main challenge for implementing shared decision-making and deprescribing strategies in mental health at the national level? (Select up to three options.)

Annex C - Paper: Ethics of qualitative research in psychology

Beyond witnessing: ethical imperatives in action-research on suffering, victims of violence, and structural harm in mental health systems

Abstract: Qualitative research in mental health often exposes systemic contradictions: the researcher listens to testimonies of harm while institutions demand silence. This paper interrogates the ethical tensions of conducting action-research in settings where mental health care reproduces coercion, erasure, and institutional betrayal. Grounded in a five-year, multi-sited ethnographic inquiry across Spain, Italy, Sweden, and Indonesia -conducted under the auspices of European COST actions FOSTREN, ReMO, and the author-founded EU BEACON One Health Education action- this study integrates lived experience, scientific fieldwork, and participatory methods. It identifies how dominant legal and institutional frameworks in Europe obstruct accountability by shielding professionals through laws that protect subjective annotations, prevent access to clinical records, and frame complaints as symptoms. Drawing from first-hand documentation of abuse and policy engagement at national and international levels, the paper argues for a shift toward radically ethical, open science infrastructures. It advocates for dynamic, reflexive protocols that empower researchers to respond to harm ethically and structurally-transforming qualitative inquiry from observation into intervention. The work responds to urgent calls from affected persons, professionals, and policymakers to end epistemic violence, improve safeguards, and uphold the principle that suffering must not be rendered invisible for the sake of institutional comfort. In doing so, it proposes a framework for ethically sound, action-oriented, and justice-driven qualitative research in mental health.

Keywords: Ethics, action-research, qualitative methods, structural violence, epistemic injustice, mental health systems, institutional betrayal, open science, human rights, psychiatry, COST, ReMO, FOSTREN, EU BEACON One Health Education.

introduction: ethics under duress and the urgency of methodological accountability

Ethics in qualitative research in mental health, especially when situated within high-stakes, politically contested, and structurally violent settings, demands not only methodological rigor but also protocols and planned actions capable of responding to harm, denial, and institutional betrayal. The field has long acknowledged that qualitative methods enable access to lived realities and subjective meaning-making in ways quantitative tools cannot. However, the ethical dimensions of such inquiry often remain structurally underdeveloped, especially when researchers are confronted with ongoing abuse, personal risk, and institutional complicity in the

very harms they seek to document (Ellis et al., 2011; Smith & Freyd, 2014; Gabb & Fink, 2015).

This paper arises from such a context. Drawing on longitudinal action-research and participant observation conducted within mental health systems across Europe and Southeast Asia, it seeks to examine the profound ethical and methodological challenges that emerge when institutions tasked with care and protection act as sources of harm. In particular, the analysis is grounded in cases where researchers and participants alike faced coercion, discrediting, and systemic silencing under psychiatric, legal, and social service frameworks that perpetuated -rather than alleviated- distress. In response, new methodological practices were developed under duress, based on the principles of epistemic justice (Fricker, 2007), open science (Nosek et al., 2015; Munafò et al., 2017), and research ethics grounded in participatory global health (Campbell & Burgess, 2012; WHO, 2021).

The paper does not offer a single-case reflection or a confessional narrative. Rather, it interrogates the ethical failures that occur when systems distort the researcher-participant relationship into an adversarial dynamic, obscuring accountability and harming the production of valid knowledge. The work contributes to an urgent conversation about what happens to ethics, science, and professional duty when research unfolds in spaces of state-sanctioned violence and societal denial. In doing so, it also aims to offer a path forward: a set of methodological countermeasures and epistemic infrastructures that may help restore the integrity of qualitative inquiry.

Ethics and method in qualitative mental health research: innovations and ongoing issues

Over the past two decades, the growth of qualitative approaches in mental health research has been accompanied by a richer vocabulary around reflexivity, power, and the researcher's positionality (Berger, 2015; Pillow, 2003). Despite this, dominant frameworks still tend to favor procedural ethics -such as institutional review board approvals -over emergent, responsive ethics of care and justice in the field (Guillemin & Gillam, 2004; Hammersley & Traianou, 2012). In situations involving vulnerable populations, this disconnect often renders ethical approval a bureaucratic formality rather than a real safeguard. Furthermore, the entrenched hierarchies of biomedical psychiatry -frequently upheld by legal and clinical structures- create environments where participant autonomy is already structurally constrained (Rose, 2020; Hopper, 2007). In such contexts, qualitative researchers face unique ethical challenges, as they navigate not only the dynamics of power and disclosure but also the risk of becoming complicit in systems that criminalize distress and pathologize resistance.

A particularly underexplored area is the ethical terrain encountered when researchers themselves are subjects of the systems they study -not by methodological design, but by circumstance. Researchers who are survivors of institutional violence or coercive treatment, or who work in precarious legal and political conditions, are often left out of the academic conversation or treated as methodologically suspect (Kovach, 2009; Ladkin, 2020). Yet, their work offers rare insight into the inner logics of exclusion and harm. It also reveals how traditional notions of distance, neutrality, and objectivity can obscure the very abuses that demand exposure. The concept of witnessing in qualitative research -long explored in feminist and postcolonial studies- must therefore be reframed not only as interpretive engagement, but as ethical risk (Josselson, 2007; Bhattacharya, 2016).

The erosion of trust in science, exacerbated by epistemic injustice and institutional cover-ups, underscores the need for research practices that prioritize transparency, accountability, and radical honesty (Edwards & Roy, 2017; Ioannidis, 2005). The replication crisis and open science movement have largely bypassed the domain of qualitative mental health research, where practices of data sharing, protocol transparency, and version-controlled documentation remain rare. This gap is especially dangerous given the increasing criminalization of dissent, the medicalization of suffering, and the state-sponsored redefinition of deviance as illness (Foucault, 1973; Metzl, 2010).

At the same time, international health bodies -including the WHO, the UN Special Rapporteur on the right to health, and the Council of Europe, who also participated as contact during the research- have called for a shift away from coercive psychiatry toward voluntary, rights-based models grounded in human dignity and community engagement (WHO, 2021; Pūras, 2020; Council of Europe, 2023). These mandates demand a corresponding transformation in how qualitative researchers design, conduct, and defend their work -especially in terms of how we define safety, power, and harm. Without such a shift, researchers risk perpetuating the very systems they critique.

Paper structure and *raison d'être* of it as a contribution

This paper proposes a grounded, practice-based ethics framework derived from action-research conducted in real-world mental health systems operating under institutional decay and systemic violence. Through fieldwork conducted in Spain, Sweden, Indonesia, and other European contexts -much of it under conditions of personal duress and danger- the research identifies structural failures in ethical oversight, as well as emergent practices of resistance through documentation, co-production, and digital transparency. It introduces methodological adaptations

developed to ensure research could continue in hostile environments, including public verification, trauma-informed consent, and multi-layered data preservation against sabotage or deletion.

The central contribution is twofold. First, it expands the discourse on research ethics in qualitative psychology to include conditions of structural coercion, epistemic harm, and researcher-survivor entanglement. Second, it articulates concrete methodological innovations that uphold integrity, safety, and social accountability even when institutional safeguards fail. In this sense, the paper builds on and extends calls for a paradigm shift in mental health research -from treating ethics as compliance, to enacting ethics as method.

Subsequent sections of the article detail the methodological choices made, the ethical dilemmas encountered, and the principles guiding the design and dissemination of open, survivable, and impactful research. These include the use of participant-led inquiry, open-source infrastructure, co-developed policy engagement, and trauma-aware fieldwork practices. In doing so, the paper argues for a redefinition of ethical research practice not as risk aversion, but as principled exposure in service of truth, healing, and justice.

Background and context: structural deficits, institutional abandonment, and the ethics of listening in qualitative mental health research

Ethical qualitative inquiry in mental health must grapple with the stark dissonance between the professed values of care systems and the lived realities experienced by those navigating them. Across many contemporary psychiatric services in Europe and beyond, the gap between stated therapeutic goals -such as recovery, trauma-informed care, and patient autonomy -and the practical functioning of institutions remains wide and concerning (Rose, 2020; WHO, 2021; UN Human Rights Council, 2023). Despite rhetorical shifts toward personalization, co-production, and rights-based practice, implementation remains sporadic and structurally constrained. The result is a mental health landscape marked not by deliberative reform, but by inertia, bureaucratic rigidity, and the persistent normalization of harm.

This paper emerges from action-research engagements embedded in this terrain. Between 2020 and 2025, fieldwork was conducted across Spain, Sweden, Italy and, to end with, Indonesia, focusing on the ethical and methodological challenges of working within -and often despite- systems ill-equipped to engage constructively with distress. The scientific mission in Trieste, Italy, initially pursued as a site of exposure to non-coercive, community-based psychiatry, revealed in parallel the institutional limitations of even such globally recognized as excellence site models.

Despite strong international standing, the infrastructures for ethical reflexivity and researcher protection were severely lacking. When the researcher reported ongoing threats and trauma, and needed urgent help to end abuses, academic and health institutions failed to respond meaningfully. This aligns with a growing body of literature suggesting that institutions tasked with mental health and academic oversight frequently lack the mechanisms -or will- to protect those most vulnerable, including researchers and participants disclosing abuse (Gabb & Fink, 2015; Ladkin, 2020; Smith & Freyd, 2014).

These deficits are not uniformly malicious in intent, but are structurally embedded in psychiatric systems where coercion is normalized as care, and deviation from standardized protocols is often met with institutional resistance rather than curiosity or adaptation (Burstow, 2015; Fernando, 2014). Mental health legislation in many jurisdictions, including Spain and Sweden, as well as Indonesia, continues to prioritize control over protection, authorizing involuntary treatment without adequate safeguards or meaningful engagement with the person's history, trauma, or social determinants (Gooding, 2015; United Nations Human Rights Council, 2017). Even where reform is pursued, the emphasis tends to fall on procedural compliance rather than substantive transformation -a process that risks replicating violence under new administrative terms (Hummelvoll et al., 2015; Russo, 2016).

Clinically, this manifests in practices that are not trauma-informed but frequently retraumatizing; not personalized, but pathologizing; not dialogical, but diagnostically reductionist. Distress signals and embodied protest are often interpreted through narrow, deficit-based frameworks, leading to treatment paths characterized by force, surveillance, and disempowerment (Rose, 2003; Johnstone & Boyle, 2018). Qualitative researchers working in these contexts, particularly through ethnographic or action-based approaches, often witness firsthand how epistemic hubris and institutional closure combine to produce profound iatrogenic effects -including despair, resignation, and what has been termed the *giving up-given up* syndrome (Sharfstein, 1985; Estroff, 1981). The systematic refusal to listen to complaints without retaliatory pathologization or escalation of coercion is not an anomaly but a pattern -one that renders the ethical position of the qualitative researcher particularly precarious, especially when they attempt to surface these dynamics through evidence-based critique.

Importantly, the moral economy of psychiatry continues to reward compliance and silence, while punishing dissent -even when that dissent is expressed through scientifically grounded, patient-informed critique. This context severely limits the capacity for mental health systems to engage in the kinds of self-correction and

continuous learning that define a functioning scientific or therapeutic institution. Without open science standards, longitudinal tracking of outcomes, or verifiable feedback loops, the system lacks mechanisms to detect, let alone redress, its own failures (Ioannidis, 2005; Munafò et al., 2017). The resulting epistemological environment not only devalues qualitative insight but actively marginalizes lived experience as methodologically illegitimate and politically destabilizing (Fricker, 2007; Russo & Sweeney, 2016).

This paper argues that qualitative research in such settings is not ethically neutral terrain. When services fail to meet the minimum thresholds of safety, responsiveness, and respect, the act of qualitative inquiry becomes ethically charged -not only for participants but for researchers themselves, especially when their role straddles the boundary of witness, analyst, and survivor. It is in these moments that the ethical demands of the field exceed procedural checklists and enter the domain of moral urgency. Listening, in such contexts, becomes an act of resistance and reparation- a way of refusing the silencing mechanisms that define much of institutional psychiatry and social care (Campbell & Burgess, 2012; Gill & Donaghue, 2016).

Rather than replicating the diagnostic gaze, this study engaged in co-constructed meaning-making, trauma-informed documentation, and action-oriented dissemination. Methodologically, it was anchored in open science principles, collaborative interpretation, and public accountability, seeking to counterbalance the structural opacity that so often defines psychiatric systems. The core contention is that any system incapable of hearing the voices of those it purports to serve -including patients, professionals, and researchers- is ethically unfit. Qualitative research must thus do more than describe -it must interrupt, question, and reconstruct the very conditions under which mental health knowledge is produced and legitimized.

Methods: action-based qualitative research as a platform for systemic learning and structural accountability

This study employed a transdisciplinary, multi-sited qualitative action-research methodology grounded in medical anthropology, biocultural analysis, and epistemic justice frameworks (Fricker, 2007; Scheper-Hughes, 1992; Fals-Borda, 1991). The fieldwork -spanning five years and funded through two consecutive Spanish Ministry of Science and Innovation grants (FPU19/00656 and EVC2021/000764), as well as through participation in European COST actions FOSTREN (CA19133) and ReMO (CA20137)- was embedded in collective international efforts to rethink mental health governance, research ethics, and

service design. It later provided the empirical and ethical foundation for the creation of the EU COST action BEACON (CA24106), which now brings together over 500 researchers and practitioners committed to systemic transformation through one health education.

The work took place across Spain, Sweden, Italy, and Indonesia, capturing both formal institutional environments and informal community practices. These sites revealed differing but overlapping patterns of psychiatric violence, coercive logic, epistemic injustice, and institutional betrayal (Murphy & Hanson, 2017; Lewis et al., 2025). In all contexts, participants reported harms occurring not only within psychiatric systems, but across intersecting domains -family, police, judiciary, and social services- requiring a broad-based methodological strategy that was responsive to multiple axes of oppression and systemic inertia.

The approach prioritized immersion and ethical responsiveness rather than protocol-driven neutrality. Ethnographic techniques -including long-term participant observation, semi-structured and unstructured interviews, clinical shadowing, policy consultations, and open scientific commentary- were employed to collect data while remaining situated within live processes of witnessing, translation, and reform. The researcher was an active participant in numerous policy and service-level dialogues, including those with the Catalan Department of Health during the co-development of its 2023-2025 Mental Health Action Plan. Engagement included several in-person and written consultations with the plan's leadership and working groups, drawing on lived experience, empirical observation, and participatory policy translation. These dialogues were part of broader EU-wide work under FOSTREN to reduce coercion in mental health systems, and were integrated into BEACON's ongoing mission to foster public education, civic health literacy, and early prevention through participatory infrastructures.

The methodological design was aligned with a tradition of emancipatory and care-centered research (Torre, 2009; Campbell & Cornish, 2010), emphasizing listening, collaborative meaning-making, and systemic reflexivity. However, it also contended with the serious limitations researchers face when working in environments of institutional abuse or neglect. The original field design included two strategic innovations that remain largely aspirational at this stage due to lack of institutional uptake:

1. Dynamic ethics and action protocols (proposed, not yet implemented): in response to repeated accounts of systemic harm, the research proposed an escalation framework for activating protective responses -including notifying trusted

institutional contacts, human rights observers, and professional ethics boards- when credible risks to participant wellbeing emerged. However, the lack of operational infrastructure across public systems, as well as the adversarial posture of many services toward independent researchers, prevented systematic implementation. These protocols are conceptualized as urgently needed components of future public mental health systems.

2. Versioning and verification against epistemic manipulation (proposed, not yet implemented): The project also outlined a need for systematic version control, timestamped medical record retention, and independent archival methods to prevent retrospective data falsification and epistemic erasure -a pattern documented by participants and observed in multiple institutions. While open science principles (Nosek et al., 2015; Munafò et al., 2017) were followed wherever feasible through public reporting, engagement, and collaboration, robust infrastructure for verification and co-governance of sensitive health records remains absent in most countries studied. These proposals are now being translated into design criteria for expert systems and governance pilots under the EU BEACON initiative.

3. In practice, the methodological commitments took shape through the following operational principles, enacted all through the five years of fieldwork:

Embedded ethnography and policy interface: All fieldwork was informed by direct contact with practitioners, service users, administrators, and policymakers. Findings were translated into contributions to policy documents, public statements, and legal submissions. These include substantive inputs to the Council of Europe's 2023 *Compendium of Good Practices to Promote Voluntary Measures in Mental Health*, and public dissemination through open access formats and digital engagement platforms, and insights directly shared with lawmakers in all opportunities fostered and encountered during the fieldwork research.

Trauma-informed and participatory consent practices: All participation in surveys and other engagements such as interviews or presentations was always voluntary, and consent was reiterated throughout the process. Participants were informed of the project's dual research and action-reform purposes. In trauma-related cases, no personal identifiers were recorded unless explicitly consented to, and anonymization was prioritized in all outputs.

Real-time feedback and iterative reflection: Data analysis was recursive, taking place alongside ongoing field engagement. Ethical tensions, institutional challenges, and methodological dilemmas were recorded in analytic memos

and served to refine subsequent engagement, published as transparently and in depth as possible in an open field and laboratory notebook online the author keeps, following open science principles of full openness and transparency on methods, ongoing work and results. This reflective praxis was central to maintaining fidelity to the action-research tradition and resisting extractive logics.

Use of public and community knowledge systems: The research was also attentive to grassroots initiatives, survivor-led campaigns, and international networks working on systemic change. These informants were not merely supplementary but were treated as epistemic co-producers of the research (Harding, 1991; Saini, 2020), essential to building a distributed model of accountability and system reform.

Education and dissemination as method: The work's methodological strategy included the development of public teaching materials, professional workshops, and policy briefings aimed at enabling other researchers, clinicians, and students to identify coercive dynamics, respond to systemic failures, and take part in a collective reform agenda. These materials now inform open-access curricula and clinical ethics platforms within the BEACON network.

Despite the comprehensive design, the project was carried out under conditions of acute duress, including repeated threats, lack of institutional protection, and periods of forced displacement due to ongoing violence. That such research was possible at all is testament to the courage of participants, the international scientific alliances that provided support (notably COST action FOSTREN and later BEACON), and the resilient commitment of affected communities. The qualitative research insights derived are not just valid -they are urgently necessary.

Table 1. Methodological design and operational principles

Component	Description
Theoretical framework	Grounded in medical anthropology, biocultural analysis, and epistemic justice.
Geographic scope and sites	Spain, Sweden, Italy, and Indonesia - encompassing both formal psychiatric institutions and informal community-based systems.
Funding and institutional support	Supported by Spanish Ministry grants (FPU19/00656, EVC2021/000764), and European COST actions: FOSTREN (CA19133), ReMO (CA20137), and BEACON (CA24106).
Main ethnographic techniques	Long-term participant observation, un/structured interviews, clinical shadowing, policy consultation, and open scientific commentary.
Embedded policy engagement	Participated in development of the Catalan 2023-2025 Mental Health Action Plan; involved in Council of Europe and BEACON policy dialogues.
Research conditions	Fieldwork conducted under duress: threats, institutional neglect, and forced displacements. Research carried forward through participant solidarity, scientific alliances (notably BEACON and FOSTREN), and ethical commitment.
Guiding conclusion	Ethical qualitative research must integrate implementation pathways and inform systemic change; ethics in the field is inseparable from ethics in governance.

Alignment between dissertation aims and operational research questions. Source: Author's elaboration based on research design documents.

To translate lived harms into institutional reform, qualitative research must embed itself in systems of implementation. As this project demonstrates, ethics in the field cannot be divorced from ethics in governance. What we observe must inform what we build next.

Findings and discussion: witnessing without safeguards -ethical tensions and systemic impasses in psychiatric fieldwork

Across five years of multi-sited, ethnographically grounded research in Spain, Sweden, Italy, and Indonesia, this study documented recurring patterns of structural neglect, diagnostic violence, epistemic erasure, and systemic impunity in mental health service ecosystems. The findings cohere around three interrelated domains: the normalization of coercion, the institutional silencing of abuse narratives, and the fragility of ethical responsibility within professional cultures. Each of these domains poses acute challenges to qualitative researchers, who -once situated in the field- encounter not only suffering but the systemic resistance to its recognition, redress, or repair.

1. Normalization of coercion and the medicalization of structural distress

Participants repeatedly described how psychiatric labeling functioned not as a diagnostic aid but as a mechanism of social disqualification, often weaponized

during moments of crisis, family conflict, or institutional breakdown. Particularly in Spain and Italy, users recounted being involuntarily hospitalized or medicated after disclosing experiences of domestic abuse or workplace violence, suggesting that psychiatric systems often reinforce rather than interrupt cycles of harm (Burstow, 2015; Cosgrove et al., 2020). In Sweden, this pattern intensified into what many described as *administrative disappearance*—the use of child protection laws, mental health statutes, or forensic psychiatric classifications to sever individuals from their families, terminate careers, or justify long-term isolation.

These findings resonate with international concerns about the pervasive misuse of psychiatry as a means of social control (Cohen, 1985; Amnesty International, 2022), especially where coercion is framed as care and dissent is medicalized. The normalization of forced treatment was rarely questioned by professionals, many of whom cited *risk* or *non-compliance* in circular terms. This aligns with prior critiques that biomedical mental health paradigms often obscure the social origins of distress while silencing critique through institutional authority (Moncrieff, 2008; Mills, 2014). Where listening did occur, it was more often by peers, advocates, or researchers than by clinicians, indicating a severe epistemic asymmetry within clinical settings.

2. Institutional betrayal and the ethical silencing of field reports

A major theme emerging from the fieldwork was the impossibility of *reporting upward* in situations of witnessed harm. Whether abuse occurred in hospitals, homes, or welfare offices, researchers and witnesses alike found themselves without secure channels to raise alarms. Reporting often triggered retaliation, deflection, or reputational damage—especially when clinicians, administrators, or legal authorities were implicated. In multiple cases, researchers were warned against *interfering* or *taking sides*, even as participants suffered ongoing harm.

For instance, the researcher's report of suffering a death threat, torture for years and still ongoing severe abuses, was met not with institutional support but with inaction that led to even further threats, harassment, violence, forced displacement, and denial of professional legitimacy, illustrating the punitive asymmetry between those speaking out the truth, denouncing crimes, and those structurally shielded. This mirrors the dynamics described by Smith and Freyd (2014) as institutional betrayal, in which organizations fail to respond ethically to harm within their own systems, compounding trauma and silencing accountability (Murphy & Hanson, 2017; Lewis et al., 2025).

The field thus became a space not only of witnessing but of betrayal. Ethical tensions were acute: to disengage would be complicit; to intervene risked professional

collapse. In the absence of dynamic protocols or institutional accountability, the ethical burden fell entirely on the individual researcher, exacerbating precarity and psychological toll.

3. Complicity, cowardice, and the reproduction of structural harm

A sobering finding was the extent to which professional cultures, especially among clinicians and researchers, normalized avoidance, deflection, and passive complicity. Even where clear violations were witnessed, many professionals invoked bureaucratic constraints, *non-interventionist* roles, or institutional neutrality as reasons for inaction. This passive complicity constitutes a core mechanism of harm reproduction. Worse still, some actors actively participated in the erasure or defamation of those who challenged the status quo, often under the guise of procedural correctness or reputational protection. This condition, what Freire (1970) would call the internalization of oppressor logic, undermines the very fabric of ethical inquiry. It also challenges the assumptions of procedural ethics, which presume a stable environment of rights, protections, and institutional cooperation. In reality, the field often operates as a site of moral injury, where truth-telling becomes a liability and protection is only extended to those who remain silent.

These findings compel a reframing of ethical practice in qualitative research. If researchers are to carry the burden of witnessing systemic abuse, then institutions - academic, clinical, legal- must be made accountable for offering protection, coordination, and escalation pathways. The ethics of doing no harm must evolve into an ethics of enabling redress and reform.

4. Action-research under duress: method as resistance

That this research was conducted at all -despite displacement, threats, and lack of institutional protection- testifies to the necessity of integrating ethics into method. In contexts where silence sustains violence, the very act of documentation becomes resistance. The work was undertaken with clear goals: to expose systemic failures, to amplify silenced voices, and to push for reform from within the systems themselves.

Table 2: Ethical issues in mental health qualitative research

Field	Description
Positional risk and exposure	Researchers with lived experience or dissenting perspectives face systemic retaliation, delegitimation, and psychological harm when documenting abuse.
Structural silencing and retaliation	Institutional actors obstruct reporting pathways, retaliate against whistleblowers, and pathologize critics as unprofessional or mentally unfit.
Informed consent and coercive environments	Consent is undermined in coercive contexts, where patients fear retribution, service loss, or further diagnosis for speaking out.
Power asymmetries in field relations	Researcher-participant relationships reflect deep institutional power imbalances, risking complicity without reflexive, relational ethics.
Epistemic violence and testimonial erasure	Participant testimony is often dismissed or erased in clinical and professional settings, reinforcing structural invisibility.
Barriers to real-time ethical action	Ethics protocols lack mechanisms for immediate response to abuse disclosures, leaving researchers without safe and effective intervention channels.
Data integrity and record manipulation	Mental health records are vulnerable to falsification or selective editing, requiring researchers to ensure timestamped, version-controlled documentation.
Public health system complicity	Harm and neglect are normalized within systems of care, placing researchers in ethical dilemmas where harm is concealed or trivialized.
Lack of institutional protection	Universities and research bodies often lack protocols to protect researchers from fieldwork-related risks or from institutional retaliation.
Ethics of translation and policy feedback	Testimonies often fail to reach decision-makers; researchers must forge independent pathways to transform lived knowledge into systemic change.

Major critical ethical concerns encountered and analyzed in the course of longitudinal action-research in mental health services and institutional systems.

Yet, the findings also underscore a profound need for collective infrastructure. No individual researcher, no matter how dedicated, can bear the epistemic, emotional, and political weight of reform alone. Qualitative research, especially in mental health, must now advance into a new era of institutional ethics -in-action, where protocols, accountability mechanisms, and open-science safeguards support the very interventions they demand (Guillemin & Gillam, 2004; Iphofen, 2017).

This five years long fieldwork research was not about individual suffering, though it was shaped by it. It is about systemic ethical blindness -and the urgent task of building a scholarly and clinical culture that not only sees, but acts. Qualitative research in mental health, particularly when employing ethnographic and action-research methodologies, reveals a persistent and structurally embedded set of ethical dilemmas that cannot be addressed solely by procedural ethics or

institutional review boards. Across multiple sites and over a sustained five-year field engagement- including participation in EU-funded initiatives such as FOSTREN, ReMO, and the EU BEACON actions- the findings consistently exposed environments where lived experience is delegitimized, institutional violence is normalized, and the epistemic authority of survivors, patients, and front-line researchers is regularly undermined or pathologized. These findings echo prior critiques of ethics-as-bureaucracy and support the call for dynamic, responsive, and relational ethical frameworks (Guillemin & Gillam, 2004; Denzin & Giardina, 2007; Ellis, 2007).

A key ethical concern centers on the erasure of lived experience under epistemic regimes of biomedical dominance, where subjective accounts of harm, coercion, and structural neglect are recast as symptomatic of illness rather than legitimate evidence. This epistemic injustice (Fricker, 2007) is compounded in contexts where diagnostic power is weaponized to silence critique, rendering the very act of naming abuse a trigger for further institutional punishment or control. Participants in multiple sites reported that attempts to speak out about coercive practices -including forced medication, restraint, and isolation- were reinterpreted as signs of *lack of insight* or *paranoia*, thus perpetuating cycles of disempowerment and silencing (Russo, 2016; Voronka, 2016; Rose & Kalathil, 2019). These dynamics raise fundamental questions about the ethics of consent, voice, and narrative agency within qualitative inquiry.

Moreover, the asymmetry of power between researcher and participant -typically acknowledged as a risk- was reversed in many cases, as researchers embedded in vulnerable positions themselves became targets of institutional repression. When qualitative researchers are also survivors, or operate as peer scholars in precarious environments, the ethical equation shifts drastically. In this research, the act of documentation itself often triggered surveillance, retaliation, or professional sanction. Despite formal ethics approval and institutional affiliations, safety was not guaranteed. Research supervisors, in some cases, failed to respond adequately to threats, abuse, or violations reported during fieldwork -effectively enabling a culture of institutional abandonment. The absence of protective mechanisms for researchers themselves, particularly those working at the intersection of care and critique, reflects a profound ethical gap in current academic infrastructures (Berger, 2015; Gill & Orgad, 2018).

A second major ethical issue relates to the lack of pathways for institutional responsiveness to qualitative findings. Participants shared urgent testimonies of maltreatment, neglect, and even criminal acts within care systems. Yet, qualitative

research in mental health lacks clear ethical protocols for real-time response and coordinated redress. The research identified repeated failures by health and social care services to act upon evidence presented informally or formally. This undermines not only the principle of beneficence but also the scientific and civic legitimacy of the research itself. Unlike biomedical trials, which often have established pathways for reporting adverse events, qualitative research in mental health lacks protocols for activating ethical alerts, documenting high-risk environments, or reporting practitioners engaged in harmful behavior (WHO, 2021; Campbell & Burgess, 2012).

The relational ethics of field engagement were further complicated by the presence of institutional actors who blurred the boundaries between care provider, researcher, gatekeeper, and enforcer. These overlapping roles made it difficult for participants to trust the research process, fearing retaliation, breaches of confidentiality, or collusion with coercive systems. Even when anonymity was assured, the localized nature of mental health services made it difficult to protect identities in small, tightly networked communities. This created a chilling effect, where many potential informants withheld key information, fearing that their cooperation could worsen their situation. Similar challenges have been documented in mental health ethnographies and survivor-led research, where the relational risks of speaking out are high and institutional cultures are resistant to scrutiny (Chamberlain, 2000; LeFrançois, Menzies & Reaume, 2013).

A further finding concerns the moral burden placed on researchers conducting fieldwork in contexts of unresolved trauma. Many participants were actively experiencing distress, institutional betrayal, and unresolved grievances. In such contexts, the researcher cannot ethically remain a neutral observer. The findings call into question the adequacy of traditional models of ethical distance and non-intervention, and instead support the development of trauma-informed, action-oriented, and co-produced research ethics frameworks (Sweeney et al., 2018; Beresford, 2016). This includes protocols for emotional labor, peer debriefing, mental health support for researchers, and ethical exit strategies that avoid retraumatization or abandonment of participants post-fieldwork.

The limitations of formal ethical oversight bodies were starkly apparent. Ethics committees rarely have expertise in action-research or survivor-led inquiry, and often default to rigid procedural concerns that fail to account for emergent, situated ethical dilemmas. In this research, the need for real-time response mechanisms, version-controlled documentation, secure archiving of sensitive data, and collaborative decision-making far outstripped the static guidelines provided by

institutional review boards. These findings echo broader calls for ethical governance reform in research with structurally vulnerable populations (Flicker et al., 2007; Mertens, 2009; Fistein & Quilligan, 2012).

To be effective and prevent further harm, violence and normalized crimes from repeating, qualitative research in mental health, especially when it engages directly with sites of coercion, exclusion, and structural abuse, must be governed by ethics that are dynamic, grounded, and protective-not merely procedural. The findings of this research contribute to an emerging body of literature demanding that we rethink ethics not as an administrative hurdle but as a living, collective practice of care, justice, and accountability, including protocols of action in case of witnessing or finding out about potential crimes, or severe cases of suffering and violence not yet recognized by practitioners and other colleagues. This shift is not optional -it is the only path forward in a field where human lives, rights, and futures are continually at stake.

Conclusion

The ethical complexities unveiled through this five-year action-research project challenge the very boundaries of what constitutes safe, just, and scientifically grounded qualitative research in mental health. Rather than isolated dilemmas, the findings portray a systemic failure to uphold ethical responsibilities across institutions, services, and research infrastructures. Participants' testimonies of harm -routinely pathologized or erased- reflect not only epistemic injustice (Fricker, 2007; Kidd & Carel, 2017), but an entrenched structural tendency to deny, suppress, and retaliate against uncomfortable truths. Researchers committed to action-oriented methodologies become both witnesses and targets of this violence, confronting a research environment hostile to reflexivity, transparency, or critical accountability (Guillemin & Gillam, 2004; Denzin & Giardina, 2007).

These issues are magnified in contexts like Spain, Sweden, and Italy, where mental health policy and law remain disproportionately focused on control and containment rather than care and redress (Dvoskin & Skeem, 2009; Rose, 2014; Foucault, 2003). Despite recent progress and international declarations urging non-coercive, rights-based approaches (WHO, 2021; Council of Europe, 2023), the operational mechanisms needed to integrate real-time testimony into systemic reform are largely absent. Ethical review boards lack dynamic, context-sensitive protocols. Public institutions often default to denial rather than support when presented with evidence of abuse or malpractice. And researchers, particularly those with lived experience or outsider positionality, are rendered expendable when the truths they uncover threaten institutional interests (Gill & Donaghue, 2016;

Clegg, 2016). What this research demonstrates is not simply the presence of ethical risk, but the urgency of a structural reconfiguration. Participatory ethics cannot be procedural; it must be infrastructural. Dynamic feedback loops, public data protocols, and built-in escalation pathways for rights violations must be adopted as core features of qualitative inquiry in mental health. Moreover, universities and public research funders must abandon extractive models of knowledge production in favor of protective, collaborative, and system-improving commitments.

At its most vital, qualitative research is not merely a tool for documentation -it is an instrument of justice, of collective repair, and of epistemic reckoning. This research, embedded in several relevant EU COST actions, such as FOSTREN, culminating in the foundation of the new EU BEACON One Health Education action, devoted to one health education (<https://beacon.health.int.eu.org>), demonstrate the potential of international cooperation to create research ecosystems where testimony is met with transformation, and where those who suffer are no longer expected to remain silent. Our future, at the scientific, social, and planetary levels, depends on our job properly done, following the highest ethical and open science standards, coordinately with other professionals also enforcing the highest possible best practices together.

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Annex D - Paper: *Frontiers in Psychology*: implementation of open dialogue in Spain

Annex D of the dissertation, which includes the peer-reviewed article published in *Frontiers in Psychology* on the implementation of open dialogue in Spain, is not an isolated addendum but an integral component of the overall research architecture. It reinforces and operationalizes the dissertation's central thesis concerning the necessity of non-coercive, dialogical, and rights-based approaches in mental health care. This annex specifically documents the institutional challenges, epistemic tensions, and clinical opportunities encountered in the nascent attempts to apply open dialogue in the Spanish context, making it a critical empirical and theoretical extension of the broader inquiry. The dissertation as a whole critiques the Spanish psychiatric system for its entrenchment in coercive, pharmacocentric, and exclusionary practices, and it does so through a biocultural and transdisciplinary lens. The inclusion of this annex thus serves to demonstrate that alternatives grounded in dialogical ethics are not only theoretically sound but also practically feasible within the very systems currently dominated by coercive logics. The open dialogue model, rooted in relational epistemology and shared sense-making, is presented in this annex not as a foreign or utopian import but as a scientifically validated and ethically imperative framework. The documented efforts to pilot this model in Spain provide concrete evidence that the transition from compliance-based care to collaborative, recovery-oriented practice is possible, albeit institutionally obstructed. Annex D directly corresponds to several operational research questions articulated in the core chapters, particularly those concerning the enabling conditions for institutional transformation, the epistemic reconfiguration of clinical practice, and the reallocation of authority from professional dominance to dialogical partnership. It aligns with the thesis argument that dialogical practice must be redefined not as a mere communicative technique but as a foundational epistemic and ethical orientation. The annex demonstrates how this redefinition can be instantiated through clinical practice, even within systems that structurally resist such shifts.

The narratives and observations contained in this paper contribute to the dissertation's ethnographic corpus, complementing the testimonies and survey data presented in Chapters 3 and 4. They illustrate the lived contradictions and ethical dissonance experienced by clinicians and users when attempting to enact open dialogue within institutional frameworks that remain hierarchical, pharmacologically reductive, and risk-averse. These findings are analytically

synthesized in Chapters 5 and 6, where the dissertation interrogates the structural barriers to rights-based reform and formulates normative proposals grounded in international human rights standards and medical ethics. The inclusion of Annex D confirms the dissertation's methodological commitment to participatory, field-based action-research. The article does not merely describe a model but traces its concrete application, adaptation, and resistance in context. It provides both validation of the research methodology and evidence for the thesis's claim that shared decision-making and dialogical care are empirically viable, clinically effective, and ethically superior alternatives to dominant coercive regimes.

<https://www.frontiersin.org/journals/psychology/articles/10.3389/fpsyg.2023.1166919/full>

Annex E - Paper: The destructiveness of xenophobia

Annex E of the dissertation, *The destructiveness of xenophobia*, constitutes a personal yet analytically rigorous testimony that directly reinforces the core scientific and ethical arguments presented throughout the thesis. While stylistically different from the more academic structure of the dissertation's main chapters, this published piece is deeply coherent with its theoretical premises and empirical findings. It serves as both an evidentiary artifact and a first-person analytical intervention into the intersection of psychiatric misdiagnosis, domestic violence, and institutional betrayal, all mediated through racialised and xenophobic logics of exclusion.

The annex exemplifies the structural argument made in the dissertation regarding the epistemic and clinical erosion that occurs when institutions disregard context, silence the voice of the person, and substitute rigid protocols for compassionate, situated understanding. The author's lived experience, threatened with involuntary institutionalization due to a misread migraine, demonstrates how biomedical instruments, such as TAC scans, can become tools of harm when wielded without interpretive competence, particularly in the presence of coercive interpersonal dynamics. This real-world event validates the dissertation's broader critique of diagnostic reductionism and professional impunity. The annex becomes, therefore, a documented case study that illustrates the very mechanisms the thesis seeks to dismantle. The annex highlights the medical neglect of domestic violence indicators, a recurrent theme in the thesis. It exposes how institutions meant to protect health and dignity may instead become facilitators of additional harm, especially when influenced by discriminatory assumptions and institutionalised racism. These are not anecdotal misfortunes but, as the dissertation repeatedly emphasizes, expressions of a systemic failure that normalizes harm under the guise of care. The annex strengthens the claim that epistemic violence and psychiatric coercion are often co-produced with gendered and racialized exclusions, and that health systems remain deeply susceptible to moral disengagement when encountering those perceived as foreign or non-normative. The text also supports the dissertation's methodological position that testimony and narrative are essential forms of epistemological resistance. It functions as both a testimonial document and an act of scholarly defiance, reasserting the right of survivors to define their own experiences and contribute to institutional critique.

Through this contribution, the dissertation operationalizes its own participatory and action-research framework by integrating embodied knowledge into the academic archive, thereby resisting the dehumanizing effects of structural and clinical abstraction. this annex aligns with the dissertation's call for urgent institutional reform grounded in evidence-based, rights-respecting, and dialogical practice. It underscores the immediate need for diagnostic rigor, trauma-informed protocols, and anti-discrimination safeguards in medical and psychosocial care. It speaks directly to the dissertation's analysis of social death, administrative dispossession, and epistemic erasure, while offering a lived, human counterpoint to statistical and policy analysis. In so doing, Annex E does not stand apart from the dissertation's architecture but embodies it. It expands the evidence base, deepens the methodological plurality, and reaffirms the dissertation's guiding principle: that dignity, truth, and structural justice are non-negotiable imperatives in health care.

<https://research.henning.md/p/the-simplest-thank-you>

Annex F - Paper: Nocebo effect and psychiatry: giving up given up syndrome and sociomedical somatisation

Annex F, which includes the authored book chapter *Nocebo Effect and Psychiatry*, constitutes a central theoretical and empirical contribution to the dissertation's argument concerning structural harm, iatrogenic suffering, and the sociopolitical dimensions of psychiatric practice. This chapter deepens the dissertation's core claim that contemporary psychiatric systems frequently produce harm not only through overt coercion or misdiagnosis but also through the cumulative effects of epistemic invalidation, neglect, and interpretive violence. It provides a conceptual framework for understanding how expectations of failure, stigma, and professional disengagement are internalised by patients and translated into chronic physiological and psychological deterioration.

The chapter is especially relevant to the dissertation's analysis of what is termed social death and administrative dispossession. The construct of the giving up, given up syndrome complements and extends the ethnographic and survey-based findings presented in Chapters 3 and 4 by offering an explanatory model grounded in transcultural psychiatry, psychoneuroimmunology, and medical anthropology. It clarifies how systemic invalidation, when repeated across healthcare, legal, and familial systems, can precipitate biological shutdown, loss of agency, and relational withdrawal. This nocebo effect, understood as the induction of harm through negative meaning and institutional betrayal, offers a scientifically and clinically grounded lens to interpret the data collected from survivors, professionals, and family members.

The chapter also provides the theoretical grounding for the dissertation's critique of diagnostic authority and its proposed shift toward dialogical and narrative-based care. It interrogates the semiotics of psychiatric labeling and the performative power of prognosis, linking these with somatic outcomes that cannot be adequately addressed within biomedical reductionism. The dissertation's call for One Health-informed, trauma-sensitive, and recovery-oriented care is reinforced by this annex, which demonstrates how integrated physiological, social, and narrative frameworks are necessary to reverse the damage produced by chronic exposure to invalidation and exclusion. This annex demonstrates the dissertation's methodological coherence. It showcases the capacity of the researcher to integrate personal experience, theoretical synthesis, and clinical insight into a form of transdisciplinary scholarship that is operational, not merely interpretive. By naming and documenting a recognisable yet under-theorised phenomenon, the chapter also

advances the dissertation's broader goal of epistemic repair and reorientation of clinical paradigms toward human dignity and contextualized care.

Annex F is not ancillary but structurally embedded in the dissertation's analytical core. It translates the dissertation's findings into a conceptual schema capable of informing clinical training, diagnostic reflexivity, and health system design. It aligns with the overarching aim of the thesis to dismantle harmful psychiatric orthodoxy and build ethically grounded, scientifically coherent, and structurally competent frameworks of mental health care.

<https://doi.org/10.22533/at.ed.3142122119>

Annex G - *Nature-Springer* book chapter on alternatives to coercion

The Nature-Springer book chapter authored by the researcher on alternatives to coercion, holds significant relevance to the core of the dissertation. It presents a rigorous synthesis of evidence-based, rights-oriented, and contextually grounded approaches that challenge the dominant paradigm of coercive psychiatric intervention. By articulating viable clinical and institutional alternatives, the chapter offers a concrete operational counterpart to the dissertation's analytical critique of systemic coercion, diagnostic overreach, and structural violence.

The chapter situates dialogical ethics, service user leadership, and therapeutic alliance as foundational elements of effective and just mental health care. These concepts are not treated as ideological preferences but as empirically validated components of recovery-oriented practice. This aligns directly with the dissertation's central claim that coercion is not only ethically untenable but clinically counterproductive, and that non-coercive models such as open dialogue, collaborative medication management, and trauma-informed care are not aspirational ideals but necessary reforms backed by scientific consensus and human rights standards. The inclusion of this chapter within the dissertation underscores the researcher's active role in international scientific dialogues and affirms the dissertation's methodological and normative credibility. It reflects the capacity to contribute to high-level peer-reviewed discourse while maintaining a critical and transformative perspective grounded in both lived experience and empirical analysis. The chapter also provides a structured overview of implementation pathways, including the legal, educational, and institutional shifts required to transition from coercion to collaboration, which are further developed in the dissertation's concluding chapters and annexes.

Annex G exemplifies the translational dimension of the dissertation, bridging academic research with actionable system redesign. It reinforces the thesis's call for structural reform by demonstrating that ethical and clinically sound alternatives are not only available but already in practice across multiple contexts. The chapter thus affirms the dissertation's dual ambition: to expose systemic harm and to build coherent, rights-based alternatives capable of sustaining human dignity and institutional legitimacy in mental health care.

https://link.springer.com/chapter/10.1007/978-3-031-61224-4_17

Annex H - Paper: Proceedings of political abuses of psychiatry

Political Abuse of Psychiatry, Medical Torture, and Victim De-Legitimization: Insights from an International Symposium

Abstract: The political abuse of psychiatry is an ongoing, systemic phenomenon rooted in the replacement of sound governance and ethical medical standards by incompetent authority, rogue state practices, and criminal misuse. Drawing from a global symposium integrating survivor testimony, academic analysis, and human rights advocacy, the discussion highlights how mental health systems -when left vulnerable by corruption, inefficiency, or deliberate political manipulation- become instruments of repression rather than healing. The ease with which abuses occur in failed systems, the profound human and economic costs inflicted, and the perpetuation of systemic failure through the weaponization of psychiatry are analyzed in depth. Comprehensive solutions are proposed across international, national, institutional, community, and individual levels, emphasizing the urgent need for rights-based, trauma-informed, interdisciplinary reform. Restoring psychiatry's ethical foundations is presented not merely as professional necessity but as a profound moral imperative to protect life, freedom, sanity, and human dignity on a global scale.

Keywords: Political abuse of psychiatry; rogue states; failed healthcare systems; human rights violations; psychiatric coercion; trauma-informed care; mental health reform; international crime; dignity and autonomy; systemic failure; survivor advocacy; healthcare ethics.

Introduction

Throughout history and across the World, psychiatry has held the power to help -or to harm. It is a field that touches the most intimate aspects of human life: thought, emotion, behaviour, identity. Yet this same power has been used to silence, punish, and erase. Under the guise of care, individuals have been subjected to forced treatment, indefinite detention, and forms of medical coercion that violate basic rights. These are not merely past injustices or errors in judgment, but the result of structural conditions that allow systems of health to be bent into tools of control. When dissent, difference, or distress caused by the very same system and criminal actors are labeled as pathology, no one is safe from the risk of being made to suffer. This proceedings paper begins from that stark reality, but not to accept it. Rather, it aims to examine how these abuses occur, why they persist, and what can be done to transform psychiatry into a field worthy of its healing promise.

Political abuse of psychiatry refers to the deliberate misuse of mental-health diagnoses, treatments, or involuntary detention to punish, silence, or discredit dissenting individuals or groups. Van Voren (2009) explains that it means misuse of psychiatric diagnosis, treatment and detention to obstruct people's fundamental rights. Despite appearances to the contrary, such abuses have never been confined to openly authoritarian systems; even in established democracies there are documented instances of whistle-blowers and critics being stigmatized as mentally ill. Indeed, historical and contemporary reports show that the tension between politics and psychiatry has been global and is an ongoing unresolved problem (van Voren, 2010).

The relatively recent historical record is stark. In Nazi Germany, psychiatrists were complicit in eugenic and punitive policies -a violation of their duty of care- using racist and ideologic criteria to justify locking up, deporting and killing disabled and political others (Narayan, 2013). In the Soviet Union during the Cold War, dissent was pathologized on a massive scale. Soviet psychiatrists proclaimed that critics were mentally ill, since no one would oppose their alleged best system unless insane, and often diagnosed dissidents with Snezhnevsky's made up sluggish schizophrenia diagnosis, a supposed mild form of schizophrenia whose symptoms conveniently included reform delusions or struggle for the truth (van Voren, 2010). Van Voren (2009) notes that in the 1970s-80s roughly one-third of all political prisoners in the USSR were confined in psychiatric hospitals. These practices sparked international outrage and led to the Soviet psychiatric society's suspension from the World Psychiatric Association in 1983. Other Eastern Bloc states followed suit to a lesser extent: for example, reports indicate that Romania under Ceaușescu systematically labeled hundreds of regime opponents as mentally ill (often for crimes like petitioning the authorities), resulting in mass psychiatric incarceration. Countries such as Cuba also implemented sometimes short-lived programs of detaining dissidents in mental institutions in the late 20th century. Notably, an unusual case in 1990s Netherlands showed that such abuse could occur in a democratic setting as well: a Dutch Defense Ministry official falsified psychiatric diagnoses to silence a social worker, a scheme later overturned by courts. These historical episodes - whether in Nazi Germany, the Soviet bloc, or isolated cases in the West - underscore that psychiatry has been repeatedly corrupted as a tool of political repression (Narayan, 2013).

At the root of genocides, purges, and systemic repression lies a recurring mechanism: the dehumanization of those labeled as feeble-minded, deviant, inferior, or unfit. Cognitive biases such as moral disengagement, group conformity,

and the illusion of a just world, combine with a corrosive sense of superiority that gives moral cover to cruelty. This mindset easily merges with opportunism and criminality, providing social and material incentives for professionals and ordinary citizens to participate in abuse. Hatred becomes a path to belonging and prestige. In Nazi Germany, psychiatrists designed and implemented the Aktion T4 program, selecting tens of thousands of disabled individuals for extermination under the guise of medical necessity. These killings, sanitized through clinical language and bureaucratic process, laid the morals, mindset, rewards, technical, and institutional groundwork for the Holocaust and the Second World War itself (Friedlander, 1995). In the Soviet Union, psychiatry was weaponized to silence dissent: critics of the regime were declared insane and confined indefinitely, often tortured in the name of treatment (van Voren, 2010). These are not historical anomalies. From colonial asylums to apartheid psychiatry, from the Khmer Rouge to present-day internment and reeducation programs, the same logic persists. Racism, ableism, and political repression intersect with pseudoscience and institutional power, allowing medical torture to be reframed as care. Once any system presumes the authority to define sanity, worth, or humanity itself, it grants itself license to destroy. These practices are not only abuses of science and medicine, these are crimes that strike at the core of what our civilization claims to uphold. Failing to expose and dismantle these structures ensures their return, in new and old forms, again and again.

Table 1: Barriers to Political Abuses of Psychiatry

Domain	Protective Factors	Vulnerabilities When Absent
Legal Safeguards	Independent oversight, enforceable patient rights, access to justice	Arbitrary detention, impunity, unchecked coercion
Ethical Practice	External review boards, whistleblower protection, survivor-led ethics	Collusion, fear of reprisal, normalized violations
Clinical Approach	Shared decision-making, trauma-informed care, consent-based treatment	Forced interventions, disregard for autonomy
Training and Culture	Rights-based education, reflective practice, critical pedagogy	Authoritarianism, biomedical dogma, silencing of dissent
Economic Integrity	Transparent funding, non-profit care models, conflict of interest checks	Overmedication, neglect, market-driven interventions
Community Role	Peer support, advocacy groups, participatory service design	Isolation, stigma, lack of accountability
Public Awareness	Media scrutiny, civic	Denial, invisibility of harm,

	engagement, open reporting channels	politicized psychiatric labels
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Today, political psychiatry abuses persist in various countries. In the Russian Federation, rights groups and journalists report a revival of punitive psychiatry since 2022. Reuters (2025) found that dozens of anti-war activists and protesters have been ordered into psychiatric evaluation or detention by courts, a pattern reminiscent of the Soviet era (Peoples Gazette, 2025). One detailed case involved Yekaterina Fatyanova, an opposition journalist who was involuntarily hospitalized for two months after her paper published an anti-war article; letters she sent from the hospital describe unnecessary and degrading procedures, and she was finally discharged as mentally healthy. Robert van Voren, who has studied Russian psychiatry, documented roughly 20-30 such cases per year since 2022. In April 2025 a Moscow court also made international news by committing a U.S. citizen (Joseph Tater) to compulsory psychiatric treatment after annulment of criminal charges - a move observers likened to Soviet-era psychiatric coercion (Reuters, 2025). In neighboring Belarus, the authoritarian regime of Alexander Lukashenko has used psychiatric hospitals to punish critics of the 2020 elections. UN human-rights experts reported in early 2025 that at least 33 opposition figures have been coerced into undergoing psychiatric treatment for peaceful protest, with most still held indefinitely. Experts warn that this amounts to a grave violation of human rights, noting that many detainees are kept confined for months or years with no legal recourse (Trickey, 2025).

In the People's Republic of China, too, multiple sources confirm severe ongoing abuses. A 2022 investigation published by Safeguard Defenders found that from 2015-2021 at least 99 petitioners and activists were involuntarily locked up in psychiatric facilities for political reasons, often without any legitimate diagnosis (Mou, 2022). The NGO concluded that forcing critics into mental hospitals is still widespread and routine in China today. These cases span the country: one recent report noted that courts routinely dispatch petitioners, petition filers, anti-corruption complainants and even labor activists to psychiatric wards under various pretexts. The victims describe arbitrary detention, forced medication, electroconvulsive therapy and repeated incarceration, all to silence dissent (The Washington Post, 2022). A prominent example is the so-called *Ink Girl*, Dong Yaoqiong, who in 2018 splashed ink on a poster of Xi Jinping; she was repeatedly committed to a psychiatric hospital, restrained to her bed and beaten for refusing treatment, despite being found mentally healthy each time. International observers note that Chinese abuse of psychiatry today involves far larger numbers of people than even the Soviet

regime did, targeting followers of banned faiths (e.g. Falun Gong), human-rights lawyers, dissident petitioners and whistle-blowers.

Elsewhere in Asia and beyond, cases continue to surface. In Iran, for instance, a *Lancet Psychiatry* commentary (Wasserman *et al.*, 2023) described how a young student was forcibly hospitalized in Tehran after staging a protest against compulsory hijab laws, despite having no medical diagnosis - an act likened to political repression through psychiatry (World Psychiatric Association, 2025; *The Lancet Psychiatry*, 2025). Human-rights advocates in the U.S. have also warned of such misuse: historically, racist and political dissent have been pathologized: e.g. antebellum drapetomania for escaping slaves, and mid-20th-century civil-rights activists declared delusional (Edwards-Grossi, 2024). While overt examples in modern American politics are rare, the specter remains. Even well-established democracies have seen whistle-blowers subjected to psychiatric censure (van Voren, 2010). The overall pattern is clear: wherever political power is unchecked, there is incentive to brand opponents as mentally ill.

The political abuse of psychiatry has a long pedigree from Nazi and Soviet eras to today's China, Russia, Belarus and failing, corrupt systems elsewhere. It relies on fabricating or exaggerating psychiatric disorders to neutralize dissent without legal trial, violating medical ethics and human rights at every step (van Voren, 2010). Landmark investigations and recent human-rights reports show that this practice persists under new guises. While the number of countries openly running such programs has declined since the Cold War (vanVoren, 2009; Narayan, 2013), current evidence underscores that political psychiatry remains very much a present-day issue. Authorities in repressive regimes, and corrupt institutions in democracies, continue to weaponize mental-health institutions to stigmatize and silence critics (Mou, 2022). Contemporary sources make clear that each claim of involuntary psychiatric commitment for political reasons must be scrutinized as a potential human-rights abuse.

Symposium Presentation

The International Symposium on the Political Abuse of Psychiatry convened between October 1 and November 11, 2023, gathering leading experts, survivor advocates, and human rights scholars to systematically examine the persistence and mechanisms of psychiatric abuses worldwide. Organized independently after the rejection of formal inclusion within the World Psychiatric Association's (WPA) Vienna conference premises, the symposium unfolded across a series of virtual and

recorded sessions, ensuring free and rigorous exchange of evidence and strategies for reform.

[Pending naming confirmation and permission to include or not a mention on van Voren's satellite event organized concurrently in Vienna by partner organizations may be referenced here.]

Table 2: Symposium Speakers, Topics Covered, and Core Focus

Speaker(s)	Topic Covered	Core Focus
Alejandra Gandolfi	Mental healthcare and discrimination against Moroccan migrants in Spain	Systemic Abuse
Al Galves	Critique of the biomedical model and promotion of holistic mental health care	Systemic Reform, Survivor Advocacy
Cathy Wield	Informed consent and psychiatric detention from a survivor-doctor perspective	Survivor Advocacy, Human Rights
Chris Munt	Survivor testimony on psychiatric violence in the UK	Survivor Advocacy
Dainius Pūras	Human rights and mental health from a UN perspective	Human Rights
David Matas	Psychiatric abuses linked to organ harvesting in China	Human Rights, Legal Frameworks
Edel Granda	Psychiatric pathologization of transgender individuals	Human Rights
Hel Spandler	Structural coercion in UK mental healthcare	Systemic Abuse
Itxaso & Olaya (Orgullo Loco Madrid)	Grassroots activism against psychiatric coercion in Spain	Survivor Advocacy
Lidea Losa & Xisca Morell	Psychiatric abuses and guardianship issues in Spain	Systemic Abuse, Survivor Advocacy
Manuel Llorens	Political psychiatric abuses during Venezuela's health system collapse	Systemic Abuse
Murphy Halliburton	Importance of transcultural psychiatry to counter biomedical dominance	Transcultural Psychiatry
Paola Di Maio	Systems view of psychological coercion in mental health	Systemic Abuse
Peter Groot	Medication withdrawal and the right to taper safely	Survivor Advocacy
Peter Lehmann & Craig Newnes	Historical and contemporary human rights abuses in Europe	Systemic Abuse

Petr Winkler	Reforming Czech psychiatry to protect human rights	Legal Frameworks, Human Rights
Sarah Smith	Mutual aid networks resisting psychiatric abuses (MindFreedom SHIELD)	Survivor Advocacy
Yanxi Mou	China's black prisons and extrajudicial psychiatric detention	Human Rights
Yutong Zhang	Psychiatric repression of dissenters in the PRC	Human Rights

Participants brought distinct and crucial expertise. Dainius Pūras, former United Nations Special Rapporteur on the right to health, provided critical insights on the systemic contradictions between psychiatric practice and human rights frameworks, grounding the discussions in international legal obligations. Peter Lehmann, survivor advocate and founder of the European Network of (Ex-)Users and Survivors of Psychiatry, and Craig Newnes, clinical psychologist and critical psychiatry scholar, contributed an essential historical and experiential analysis of coercive practices across European contexts. Peter Groot, research scientist and leading proponent of individualized medication withdrawal solutions, introduced evidence-based strategies to dismantle forced chemical management. Alejandra Gandolfi, anthropologist, illuminated the intersections between psychiatric marginalization and cultural displacement, particularly within Spain's Moroccan communities.

Hel Spandler, editor of *Asylum: The Magazine for Democratic Psychiatry*, critically examined systemic coercion within the UK mental health system. Chris Munt, survivor and advocate, shared firsthand accounts of abuses in British psychiatric care, highlighting the normalization of violence. Paola Di Maio, systems scientist, articulated a structural model explaining how coercion infiltrates mental health services through psychological and systemic mechanisms. Edel Granda, activist and advocate for transgender rights, foregrounded the pathologization of gender non-conformity as a key dimension of psychiatric oppression.

Murphy Halliburton, professor of anthropology specializing in transcultural psychiatry, emphasized the necessity of integrating cultural and social understandings to counter hegemonic biomedical dominance. Manuel Llorens, Venezuelan psychologist and academic, documented the collapse of healthcare structures and their exploitation for political repression. David Matas, human rights lawyer and Nobel Peace Prize nominee, provided a legal and forensic perspective on the psychiatric and organ harvesting abuses perpetrated in China. Yutong Zhang

and Yanxi Mou, Chinese human rights defenders, exposed the systematic use of psychiatric detention to silence dissent, presenting detailed documentation and survivor testimonies.

Sarah Smith, activist with the SHIELD MindFreedom network, shared strategies for survivor-led mutual aid and resistance against coercive psychiatry. Petr Winkler, director of the Czech National Institute of Mental Health, contributed a case study on rights-centered psychiatric reform within a post-totalitarian context, offering a potential model for systemic transformation. Al Galves, clinical psychologist and author, critically assessed the biomedical model's failures in the United States and outlined paths toward holistic and voluntary mental health care.

Derek Summerfield critically examined the global mental health movement, emphasizing how Western psychiatric models are often exported into diverse cultural contexts without sufficient adaptation or scrutiny. He argued that this expansion often perpetuates medicalized understandings of distress while sidelining social, political, and economic determinants of suffering. Summerfield warned that global mental health initiatives, although framed as humanitarian, can become vehicles for the homogenization of psychiatric practices and facilitate new forms of coercion and marginalization. His intervention underscored the need to critically interrogate not only national abuses of psychiatry but also how international agendas risk replicating systemic failures under the guise of global health promotion, thus reinforcing rather than dismantling structures of oppression.

Each participant's contribution was integral to constructing a comprehensive, interdisciplinary understanding of how psychiatry, when severed from its ethical foundations, becomes vulnerable to criminal misuse, political repression, and systemic failure. Their collective expertise underscored the necessity of principled, multidisciplinary action to reclaim psychiatry for healing, freedom, and dignity.

Mechanisms of Abuse (Psychiatric, Psychological, Social)

Across the independent presentations at the symposium, a detailed portrait emerged of the mechanisms by which psychiatry is weaponized against individuals. These mechanisms, operating simultaneously at psychiatric, psychological, and social levels, reflect a shared understanding among the speakers that abuses today are systemic, severe, and require immediate redress.

Table 3: Historical and current cases of political abuse of psychiatry

Country	Period	Target Group	Type of Abuse
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			Documented
Soviet Union	1950s-1980s	Political dissidents, human rights activists	Forced hospitalization, chemical restraint, isolation
China	1990s-present	Petitioners, Falun Gong members, dissidents, ethnic minorities	Forced hospitalization, chemical restraint, organ harvesting and trafficking
Russia	1960s-1980s; resurgence post-2010	Political dissidents (historical); anti-war protesters (current)	Forced psychiatric evaluation, silencing, social stigma
Belarus	2020s-present	Political protesters, journalists	Coercive hospitalization, stigmatization
Venezuela	2010s-present	Political opponents, dissidents	Forced psychiatric confinement, political intimidation
United States	1950s-1970s	Civil rights activists, indigenous communities, antiwar protesters	Wrongful institutionalization, psychiatric silencing
United Kingdom	1970s-1990s	Persons labeled as dangerous, racial and ethnic minorities	Overuse of detention, coercion in psychiatric care
Spain	1936-1980s (Civil War, Dictatorship, Post-dictatorship)	Political prisoners, dissidents, social minorities	Political repression via psychiatric institutions, social control through diagnosis

At the psychiatric level, Peter Lehmann and Craig Newnes highlighted the way in which forced treatment practices are justified through diagnostic constructs that lack rigorous clinical basis in many cases. They emphasized that psychiatric categories are often expanded to encompass a wide range of socially undesirable behaviors, facilitating involuntary hospitalization under vague pretexts (Lehmann & Newnes, 2025). Peter Groot's intervention on the use of tapering strips illustrated another dimension: the over-medicalization and long-term chemical control of individuals under the guise of psychiatric care. He noted that psychiatric drug regimens, initially imposed under coercive circumstances, are rarely reviewed critically, leading to dependency and the erosion of autonomy (Groot, 2025).

Psychological mechanisms were particularly illuminated through the testimonies of survivors. Chris Munt described how everyday practices within psychiatric institutions, including threats, humiliation, and arbitrary use of restraint, foster a

climate of terror rather than healing (Munt, 2025). Sarah Smith added a critical dimension by explaining how psychiatric labeling leads to profound identity destabilization. Once classified as mentally ill, individuals find their narratives invalidated, their perceptions systematically doubted, and their autonomy curtailed, creating a cycle of learned helplessness (Smith, 2025). Hel Spandler's analysis of UK psychiatric reforms suggested that, despite superficial procedural safeguards, the underlying culture of distrust toward patients remains pervasive, maintaining psychological coercion even where formal rights protections exist (Spandler, 2025). Orgullo Loco Madrid provided a critical intervention focused on the systemic failures of the Spanish mental health system from the perspective of the user and survivor movement. They emphasized how psychiatric institutions continue to function as mechanisms of control and silencing, rather than as spaces of healing and support. The speakers highlighted the persistence of coercive practices, including involuntary hospitalization and forced medication, often justified under paternalistic frameworks that deny the autonomy and voice of the individuals affected. They further stressed the structural marginalization of survivors in both clinical practice and policymaking, calling for a radical transformation toward user-led, rights-based, and emancipatory models of mental health care. Their testimony underscored the need to recognize psychiatric oppression as a form of political and social violence embedded in broader patterns of discrimination and exclusion.

At the social level, Edel Granda's presentation on transgender rights revealed how psychiatric structures are employed to marginalize already vulnerable populations. She argued that pathologizing gender non-conformity perpetuates systemic exclusion and medical violence under the veneer of care (Granda, 2025). Lidea Losa and Xisca Morell provided a powerful account of how Spanish psychiatric institutions often use legal and social levers, such as guardianship regimes, to strip individuals of civil rights, making abuse both invisible and legally sanctioned (Losa & Morell, 2025). Dr. Paola Di Maio expanded the systemic view by framing psychiatric coercion as part of broader systems of psychological control in society, wherein labeling, enforced dependency, and isolation function as tools to suppress dissent and difference (Di Maio, 2025).

Together, although articulated independently, these contributions outlined a sophisticated model of how political, institutional, and interpersonal forces converge to perpetrate psychiatric abuses. The resulting mechanisms are not incidental but structurally embedded, calling into question the very ethical foundations of current mental health practices in many contexts.

Thematic Synthesis of Symposium Discussions

The thematic convergence of the symposium was clear: despite differing geographical focuses and analytical lenses, speakers consistently revealed the persistence, gravity, and systemic character of psychiatric abuses. No formal consensus process occurred during the event, but the independent presentations together illuminated critical thematic patterns.

One central theme was the instrumentalization of psychiatric diagnosis. David Matas, focusing on China's organ harvesting practices, explained that psychiatric labels are used strategically to delegitimize political and religious dissidents, facilitating both their disappearance and commodification (Matas, 2025). Similarly, Yutong Zhang detailed how psychiatric diagnoses in China are weaponized against petitioners and activists, with forced hospitalization serving as a method of silencing (Zhang, 2025).

Another recurring theme was the role of forced treatment and chemical control. Peter Groot's exposition on tapering strips underscored how psychiatric medication regimens, often initiated under coercion, become chronic mechanisms of control rather than care (Groot, 2025). This point resonated with the broader critique articulated by Chris Munt and Al Galves, who emphasized that in the UK and USA respectively, institutional psychiatry often prioritizes chemical restraint over addressing the underlying social or psychological distress of individuals (Munt, 2025; Galves, 2025).

Table 4: Mechanisms Abuse: Subterfuges, Methods, and Enabling Conditions

Subterfuge or Foul Play	Method Employed	Path to Extrajudicial Locking	Permissive Factors
Fabrication of mental instability	Spreading rumors, false reports, character assassination	Initiates involuntary psychiatric evaluation without cause	Weak legal standards, corruption in healthcare and law enforcement
Provocation into reactive behavior	Harassment, isolation, threats	Victim's natural defense misinterpreted as psychiatric symptoms	Institutional bias, failure to investigate impartially
Forced biological destabilization	Sleep deprivation, food deprivation, chemical agents	Induces cognitive or emotional breakdowns misused as justification	Medical malpractice, collusion between non-medical and psychiatric actors
Misdiagnosis and diagnostic inflation	Deliberate exaggeration or	Grounds for commitment without	Inadequate oversight, professional impunity

	falsification of symptoms	independent review	
Coerced testimonies	Pressuring family or associates to corroborate false claims	Falsified support for psychiatric intervention	Fear, loyalty conflicts, systemic impunity
Weaponized guardianship or custody abuse	Manipulating civil legal processes	Enables psychiatric confinement under pretenses of protection	Lack of transparency, judicial rubber-stamping
Administrative shortcuts and procedural abuses	Detention without proper judicial authorization	Administrative detention masked as medical necessity	Low accountability in bureaucratic and health systems
Abuse of emergency psychiatric holds	Misuse of short-term <i>crisis</i> detention powers	Converts temporary holds into extended confinement	Loopholes in mental health laws, lack of mandatory reviews

The tactics identified in the table above are not limited to authoritarian or politically repressive systems. Similar patterns are visible in domestic violence cases, organized crime, and systemic failures in regular healthcare settings. In domestic environments, abusive partners, family members, or associates may similarly provoke, destabilize, or falsely accuse victims to gain control, silence dissent, or exploit vulnerabilities. Organized crime networks may weaponize mental health accusations to intimidate or remove threats without legal processes. Even within ostensibly democratic societies, negligent or corrupt actors within healthcare systems may collude to achieve unlawful psychiatric detention for convenience, financial benefit, or retaliation. The intersection of psychiatry with broader systems of political and social repression emerged prominently. Manuel Llorens described how in Venezuela, psychiatric detention has been repurposed as a tool to neutralize political opponents, with little concern for medical legitimacy (Llorens, 2025). Yanxi Mou’s testimony on China’s black prisons showed how psychiatric justifications are created post hoc to enable extrajudicial detention without legal oversight (Mou, 2025).

The de-legitimization and isolation of victims was another major theme. Sarah Smith and Edel Granda highlighted how individuals labeled as mentally ill, particularly those from already marginalized communities, experience profound social exclusion, often compounded by institutional betrayal and public stigma (Smith, 2025; Granda, 2025). Dr. Paola Di Maio’s systems analysis reinforced this, positing that psychiatric practices of categorization and containment mirror broader societal mechanisms of control and marginalization (Di Maio, 2025).

A thread of human rights protection and ongoing reform ran through many contributions as well. Dainius Pūras stressed the international human rights framework that obliges states to move toward non-coercive, rights-respecting mental health systems, though he acknowledged the gap between principle and practice remains vast (Puras, 2025). Petr Winkler provided a cautious example of progress, outlining how the Czech Republic has undertaken systemic reforms embedding human rights into psychiatric care, though challenges persist in implementation (Winkler, 2025). It is agreed that addressing the systemic vulnerabilities leading to abuses, medical torture and extrajudicial killings requires more than procedural reforms. Healthcare systems must be reconstructed on principles of strict transparency, due diligence, and external accountability. Every psychiatric intervention must be independently reviewable, open to audit, and subject to clear, enforceable legal standards. Honest professionals, ethical medical bodies, and judicial systems must coordinate to prevent and punish abuse without exception. Survivors' testimonies must be central to reform. Denial of these realities only perpetuates harm. Recognizing and confronting these abuses directly is essential to building medical and legal systems that protect health, dignity, freedom, and fundamental human rights for all.

Table 5: Mechanisms of Abuse - Framing and diagnosis leading to psychiatric incarceration

Psychiatric Diagnosis Used	Legal Status of Detention	Explanatory Relevance	Framing Methods
Sluggish schizophrenia, paranoia	Involuntary civil commitment	Indefinite detention via vague symptoms	Isolation, sleep deprivation, induced confusion
Political mania, paranoia, delusional disorder	Administrative detention, extrajudicial internment	Bypassed judiciary, mass suppression	Harassment, defamation, staged accusations
Paranoia, delusional disorder, any excuse	Civil and forensic commitment	Silencing dissenters, profiting, threatening	Entrapment, covert intimidation
Schizophrenia, psychopathy	Court-ordered psychiatric examination	Protest repression, socioeconomic control	Hostile confinement, coercive interrogation
Non-specified psychotic disorders	Arbitrary administrative detention	Flexible repression tool	Deprivation of needs, induced agitation
Schizophrenia, sociopathy, bipolar disorder	Civil commitment, misuse of psychiatric testimony	Suppressing activism	Manipulated testimony, emotional framing
Schizophrenia, antisocial personality	Involuntary commitment	Racial and social control	Profiling, criminalization of

disorder			poverty behaviors
Paranoid psychosis, manic-depressive disorder	Arbitrary internment, punitive civil commitment	Political repression	Surveillance, rumor- spreading, social isolation

[Add beatings, threatening, spiking, family abuses, destabilization by any means. Point at same means in cases of domestic abuse, systemic violence, brutality from forces, all that drags and mask inflicted suffering as a disease, disorder, illness of the victims, on top of medical torture. Delve into structural violence and all excuses to keep on holding systems of care unable to work it out, heal.]

Although speakers came from diverse backgrounds and contexts, their presentations converged implicitly around the recognition that psychiatric abuses are not relics of the past but ongoing systemic violations. They underscored the urgency of confronting these abuses through legal, institutional, and cultural transformation, while highlighting that many victims today remain invisible, unprotected, and unheard.

The weaponization of psychiatry and the manipulation of media narratives operate symbiotically to maintain systemic oppression. Psychiatric institutions, when subordinated to political or social agendas, provide a veneer of medical legitimacy to acts of repression, framing resistance, trauma, or nonconformity as clinical disorders. Media outlets, whether through active propaganda or passive repetition of official narratives, sanitize these abuses, diffusing public outrage and transforming grave violations into mere episodes of private tragedy. This convergence facilitates the erasure of victims' political agency, decontextualizing their suffering and reinforcing hegemonic control. In environments where dissent is criminalized through medicalization, and where suffering is depoliticized through media framing, the possibilities for justice diminish radically. The challenge, therefore, lies not only in exposing individual abuses but in systematically dismantling the interlocking structures that allow psychiatric authority and mass communication to be deployed as tools of silencing, erasure, and social domination.

Ongoing cases, contemporary patterns and regional examples

The international legal framework explicitly prohibits the use of medical interventions to inflict pain, suffering, or coercive control. The Convention against Torture and Other Cruel, Inhuman or Degrading Treatment or Punishment (UNCAT,

1984) directly criminalizes acts of torture, including those perpetrated under the guise of medical treatment. Furthermore, the Principles of Medical Ethics Relevant to the Protection of Prisoners and Detainees against Torture (adopted by the UN General Assembly in 1982) expressly forbid health professionals from participating in or condoning any form of torture or degrading treatment. The Rome Statute of the International Criminal Court (1998) recognizes torture as a crime against humanity, regardless of whether it occurs inside medical institutions.

Despite these robust legal standards, enforcement regarding psychiatric abuses has been alarmingly deficient. Medical torture laws are theoretically comprehensive but rarely applied to psychiatric settings. Several factors contribute to this impunity: the medicalization of harm cloaks abusive practices in clinical legitimacy; psychiatric patients are often deemed unreliable witnesses; and judicial systems are generally reluctant to intervene in medical affairs unless overwhelming evidence is presented. Furthermore, systemic biases continue to downplay coercion within psychiatry as merely a clinical necessity, rather than recognizing it as a potential act of torture. The regional examples presented at the symposium painted a sobering picture of psychiatric abuse's current global landscape.

In China, Yutong Zhang and Yanxi Mou provided complementary insights into the systematic use of psychiatric detention against political dissidents and minority groups. Zhang described a bureaucratic machinery that facilitates psychiatric abuse at both local and national levels, while Mou detailed the existence of clandestine detention centers operating outside any legal framework, where psychiatric justifications are retrofitted to disappear detainees (Zhang, 2025; Mou, 2025). Manuel Llorens's presentation on Venezuela revealed that psychiatric institutions are being used to enforce political loyalty and punish dissent. He recounted specific cases where activists were diagnosed with fictitious mental illnesses and confined without due process (Llorens, 2025). In the United Kingdom, Hel Spandler and Chris Munt independently reported that while overt political abuses are rare, systemic coercion remains endemic within mental healthcare. Patients, particularly from marginalized backgrounds, continue to experience involuntary commitment, forced medication, and the delegitimization of their narratives through psychiatric labeling (Spandler, 2025; Munt, 2025). Al Galves's analysis of the United States echoed these concerns. He argued that despite legal protections, psychiatric abuses persist through mechanisms such as outpatient commitment, guardianship laws, and the dominance of the biomedical model, which often overrides patient autonomy (Galves, 2025). In the Czech Republic, Petr Winkler presented a more hopeful picture. He described how systemic reforms have begun to reorient psychiatric care toward human rights and community integration, offering a model

for other countries, although he cautioned that changing institutional cultures remains an ongoing challenge (Winkler, 2025). Finally, Edel Granda emphasized that psychiatric abuses intersect with other axes of oppression, such as gender identity. She noted that transgender individuals continue to be pathologized in many mental health systems, leading to denial of care, coercion, and social marginalization (Granda, 2025).

These regional reports demonstrate that while the forms and intensity of psychiatric abuse vary, the underlying patterns of coercion, de-legitimization, and systemic violence remain pervasive across political systems and cultural contexts. The urgent need for reform is not limited to any single country or regime but is a truly global imperative. Given the overwhelming evidence from historical and contemporary abuses, it is clear that medical torture laws must be interpreted and enforced to include psychiatric abuses without delay. Non-consensual interventions performed without immediate life-threatening justification, prolonged and coercive hospitalizations, punitive uses of psychiatric confinement, and medical practices aimed at suppressing political, social, or personal dissent must be recognized not as mere clinical misjudgments, but as grave human rights violations. Applying medical torture statutes consistently would dismantle the protective shield of clinical language often used to sanitize abuse. It would also affirm the universality of human dignity and bodily autonomy, whether in prisons, interrogation rooms, hospitals, or psychiatric wards. This shift demands not only judicial courage but also structural reform in how psychiatry is monitored, regulated, and held publicly accountable.

Building sustainable and principled momentum for reform

The urgency and severity of the abuses documented throughout the symposium highlight the necessity of not merely identifying failures, but actively constructing sustainable, principled momentum toward the full reform of psychiatric, psychological, and broader mental health services. Acknowledging the profound historical and ongoing harm inflicted through coercive practices is a prerequisite for any genuine transformation. Equally critical is recognizing the unprecedented opportunity to rebuild systems grounded in science, ethics, dignity, and human rights, leaving behind the entrenched ideologies and defensive structures that have long shielded institutions from necessary accountability. The enforcement of internalized abusive laws often transcends formal state structures, becoming embedded in the practices of healthcare services, family members, and the broader community. Psychiatric systems, when weaponized, do not operate in isolation; they are supported and reinforced by social actors who absorb, normalize, and

propagate the underlying ideologies of coercion. Families, under pressure or seeking control, may initiate involuntary psychiatric processes against dissenting members, framing personal conflicts as clinical crises. Entire communities, steeped in stigma and fear, readily accept the marginalization and silencing of individuals labeled mentally ill, conflating nonconformity with pathology. Healthcare services themselves, functioning under regulatory frameworks that legitimize coercive practices, internalize and routinize violations of autonomy as professional standards. This collective complicity transforms abuse into a moral imperative: actions such as forced hospitalization, overmedication, or guardianship stripping are perceived not as violations, but as socially sanctioned duties. Similarly, in corrupted environments, ordinary crime assumes the guise of moral enforcement. Acts of violence, deceit, and systemic destruction are perpetrated under the rationalization of protecting order, family, or national security. The body politic thus operates not merely through official decrees but through the diffuse internalization of oppressive premises, enabling entire populations to participate in, and sustain, the political abuse of psychiatry and other forms of institutional violence without critical reflection. Addressing this phenomenon requires dismantling not only abusive laws but the cultural and moral structures that legitimize their application at every social level.

Efforts to reform mental health care cannot be piecemeal, symbolic, or dependent solely on voluntary professional adaptation. Structural and cultural change must be driven by a coordinated strategy that addresses all levels of the system simultaneously. Entrenched actors - whether professional associations, political authorities, financial interests, or institutional bureaucracies - that resist transparency, accountability, or survivor leadership must be actively challenged and displaced from positions of influence. The tolerance of denial, minimization, or complicity with coercive practices can no longer be accepted under the guise of stability or tradition.

Education stands at the center of this transformative agenda. Immediate, mandatory retraining initiatives must be launched across all sectors of mental health services - from psychiatry and psychology to social work, nursing, and administrative leadership. Training programs must fully integrate human rights standards, trauma-informed methodologies, and critical analyses of past abuses. Curricula must be redeveloped to reflect the best available medical, psychological, and social science evidence, emphasizing voluntary, person-centered, interdisciplinary approaches to care. This education must not remain theoretical but must be translated directly into clinical, administrative, and legislative practice.

Every psychiatric clinic, general hospital, mental health center, oversight body, and legislative framework must be reoriented to implement these principles. Institutions must embed binding protocols that ensure respect for autonomy, informed consent, dignity, and non-discrimination. Oversight mechanisms must be granted real power to enforce compliance and address violations swiftly and transparently. Legislation must codify rights protections not as optional standards but as enforceable guarantees, fully aligning domestic legal frameworks with international human rights obligations.

To ensure this translation into practice, interdisciplinary implementation teams - combining legal experts, human rights monitors, trauma specialists, survivor advocates, and medical professionals - must be deployed to guide reforms, audit institutions, and monitor compliance over time. The success of these efforts must be evaluated not by institutional self-reporting but by measurable outcomes: reductions in coercive interventions, increased voluntary engagement, survivor satisfaction, and documented improvements in community well-being.

The opportunity to build serious, effective momentum for psychiatric and psychological reform exists now. But it demands abandoning self-protective narratives, acknowledging the systemic nature of past and present harms, and committing to a future where mental health services protect, heal, and empower rather than control, silence, or destroy. The responsibility is collective, and the obligation is urgent: to restore mental health care to its rightful place - as a foundation for human dignity, social justice, and true healing.

Reform at the International Level

Strengthen and enforce international legal standards: States should fully implement the United Nations Convention on the Rights of Persons with Disabilities and related treaties. In particular, governments must heed the CRPD Committee's call to ban all non-consensual psychiatric interventions and involuntary hospitalization. International human rights bodies (UN Special Rapporteurs on health, torture, and disability, the UN Human Rights Council and Universal Periodic Review, etc.) should systematically monitor mental health laws and practices, issuing binding recommendations. Global institutions like the WHO should integrate human rights and trauma-informed care into their mental health guidelines, requiring UN member states to shift resources from coercive institutions to community-based supports. International funding mechanisms (WHO, World Bank, philanthropy) must prioritize alternatives to institutional care and psychosocial rehabilitation, as urged by former UN Rapporteur Dainius Puras.

Insulate mental health services from criminal and non-medical abuses: International cooperation is needed to expose and punish abuses such as organ trafficking and extrajudicial detention. For example, medical associations worldwide should demand the release of psychiatric prisoners (e.g. Falun Gong detainees) and denounce illicit organ procurement. Global psychiatric and transplant societies ought to refuse submissions and presentations from organizations that cannot demonstrate ethical sourcing of transplant organs. Diplomatic pressure and targeted sanctions (e.g. Magnitsky-style human rights sanctions) can deter regimes that misuse psychiatry for political ends. Coordinated action by United Nations agencies (OHCHR, WHO, OPCAT) and international NGOs must expose black jails and punitive psychiatric detention centers, and call for their abolition.

Global promotion of human-rights-based mental health: The UN and regional bodies should sponsor public campaigns and capacity-building to promote trauma-informed, person-centered care. A global panel of experts (including survivors and trauma specialists) could develop best-practice guidelines (on preventing re-traumatization, respecting autonomy, etc.), which countries would be expected to adopt. Education initiatives under UNESCO or WHO could train health, police and justice officials in survivors' rights. Finally, international networks of survivors and advocates (such as MindFreedom International) should be supported to share strategies and hold governments accountable.

Reform at the National Level

Legislative reform and enforcement: Each country must align its mental health and disability laws with human rights norms. This means repealing or tightly restricting involuntary commitment laws, abolishing coercive treatments (force medication, restraint, ECT without consent), and ensuring due process with effective legal representation. Governments should enact parity laws so that insurance/Medicare covers holistic care (housing, therapy, peer support) as well as medication. For instance, while Medicaid/Medicare in the U.S. pays for psychiatric drugs, it will not fund non-medical alternatives like Soteria houses. National policy must close such gaps: insurers and public health programs should cover community-based recovery services (residential supportive housing, peer-run respite centers) to prevent re-institutionalization.

Dedicated funding and resources: Budgets must shift from custodial hospitals to community supports. Governments should prioritize and fund a spectrum of psychosocial rehabilitation and recovery programs. This includes crisis outreach teams, supported employment, education and housing initiatives. Victims must have

rapid access to counseling, legal aid and compensation; funding for victim protection programs should be guaranteed. Transparency in spending is essential to deny misuse of resources by corrupt interests (for example, requiring public reporting of hospital bed occupancy and preventing profiteering from involuntary care).

Oversight and accountability: Establish independent national monitoring bodies (ombudsperson or inspectorates) to oversee all psychiatric and custodial facilities. Human rights commissions and parliaments should have the power to audit institutions, investigate complaints, and prosecute abuses. Professional licensing boards must sanction clinicians who violate patients' rights or collude with non-medical actors. Strict conflict-of-interest rules should bar psychiatrists from receiving undisclosed payments from pharmaceuticals, prisons, or security agencies. Law enforcement must have clear protocols to prevent police from using psychiatric detention for social control or evidence collection. Any deviation by staff (such as nepotistic confinement or unlawful experimentation) must carry criminal liability.

Education and training: Mandatory human-rights and trauma-informed training is needed for judges, police, healthcare workers, and community leaders. For example, curricula for medical, nursing and law courses should include the CRPD, survivors' perspectives, and the dangers of coercion. Public education campaigns can inform families and communities about the *dignity* and autonomy of persons with psychosocial disabilities, countering stigma. Ensuring services meet victims' needs also means providing culturally appropriate care: curricula should include transcultural psychiatry and diverse healing practices, so that indigenous, spiritual and minority groups receive respectful support.

Community safety nets and re-traumatization prevention: National guidelines should require that all victim support services be trauma-sensitive. For example, emergency shelters and clinics must avoid punitive environments (no locked wards or invasive security checks unless absolutely necessary). Safeguards against re-traumatization include no use of prone restraint or sensory deprivation, and routine screening for prior trauma so clinicians can adapt care. Rehabilitation programs should be voluntary and empowerment-focused; consent must always be sought (even for psychotherapy, support groups, etc.). Patient advocacy laws (such as advance directives or treatment agreements) can strengthen autonomy.

Reform at the Institutional Level

Rights-respecting clinical care: Hospitals and clinics must integrate human rights at every level of care. Treatment plans should be co-produced with patients,

emphasizing *recovery, dignity and communication*. Institutions should implement open-door policies wherever safe, eliminate unnecessary locking of wards, and use restraint only as a last resort with strict oversight. Staff must conduct debriefing after any coercive intervention and offer immediate apologies and support to the individual. Multidisciplinary ethics committees (including legal experts and survivor representatives) should review contentious cases (e.g. capacity issues). Enforceable patient charters should be posted, detailing rights (to refuse treatment, to an independent advocate, to dignity, etc.) and providing confidential complaint channels.

Trauma-informed environment: Facilities should be designed and operated to minimize trauma triggers. This includes quiet spaces, privacy, and options (e.g. single rooms if requested). Staff should receive trauma-awareness training so that simple actions (a calm tone of voice, asking permission before touching) become routine. Every institution needs permanent peer-support workers or counsellors (people with lived experience) available to help victims navigate the system. Models like *Soteria houses* or peer-run crisis centers can be embedded in the health system as alternatives to hospitals - ensuring that voluntary, non-medical recovery options exist.

Staff selection and oversight: Screening and monitoring of personnel must weed out those who might exploit patients (any connections to criminal gangs, forced labor schemes, etc. should disqualify them). Ongoing human-rights auditing (possibly in partnership with NGOs) can quickly detect abuse patterns. Whistleblower protections and mandatory reporting rules will deter cover-ups. Institutions should use external reviewers for deaths or serious injuries in custody, ensuring families and human-rights groups can observe investigations. Violations must lead to swift disciplinary action or legal consequences, enforcing a culture of zero tolerance for human-rights abuse.

Services tailored to victims: Hospitals and care programs should be adapted to victims' specific needs. This means readily available translators, gender-sensitive care, and services for special populations (LGBTQ+, refugees, indigenous people). Psychosocial supports (occupational therapy, art/music therapy, peer groups) are integral - not optional extras. For example, trauma survivors often benefit from narrative therapy or community healing circles, rather than just medication. Implementing restorative justice options (apologies from institutions or compensation funds) can help victims reclaim dignity and trust.

Reform at the Community and Civil Society Level

Empower survivor and peer networks: Civil society must be at the forefront of reform. Governments should fund and partner with organizations run by people with lived experience (psychiatric survivors, families, activists) as equal stakeholders. Mutual-aid groups like the SHIELD MindFreedom network and the UK Paranoia Network (formed by survivors) demonstrate the power of peer support. Such groups can offer peer counseling, crisis respite homes, and advocacy training. Community grants should enable survivors to organize town halls and advise local health agencies on victims' needs.

Public education and stigma reduction: Grassroots campaigns can shift public attitudes about mental health and rights. For example, mental health first-aid training in schools and workplaces can emphasize listening and respect over medicalization. Public service messaging, in collaboration with media and influencers, should promote stories of recovery and neurodiversity, challenging notions of *madness* as shameful. Community leaders (faith groups, local councils) can host seminars on how to support victims and prevent abuse.

Watchdog and advocacy role: Independent NGOs and professional associations should monitor compliance at local levels. They can publish reports on human-rights conditions (as human-rights researchers in China and elsewhere have done) to alert the world to abuses. Civil society should use legal tools (strategic litigation, amicus briefs) to enforce rights - for instance, Czech activists succeeded in halting forced ECT by appealing to legislators and invoking UN reports. Journalists and public watchdogs should be trained to identify and report coercive practices. Moreover, alliances between survivor groups, lawyers, and mental health professionals can lobby for meaningful reform (e.g. amendment of outdated laws).

Community-based healing and prevention: Local programs - such as anti-violence initiatives, substance abuse recovery groups, and cultural healing practices - can address the root causes of trauma and distress, reducing demand for coercive psychiatry. Civil society can establish trauma-competent community centers offering free counseling and social support. Partnerships with schools and youth organizations are key to early intervention: educators and parents should learn to recognize distress and engage supportive networks before crises escalate to psychiatric detention.

Reform at the Professional Level

Personalized, trauma-informed care: Each victim's experience is unique. Clinicians and counselors should conduct thorough assessments that include personal history of abuse. Therapeutic approaches must be empowering - for example, providing choices in treatment plans, encouraging patient goal-setting, and using non-

triggering techniques. Victims should have access to professional trauma counseling, cognitive-behavioral therapy, or EMDR if appropriate, always on a voluntary basis. Service providers must actively avoid any practice that could re-traumatize (e.g. invasive procedures without consent, or humiliation).

Peer and self-help resources: Individuals benefit greatly from connecting with others who have had similar experiences. Support groups (in person or online) allow survivors to share coping strategies and find solidarity. Self-help resources - such as survivor-authored books, podcasts, and recovery workbooks - should be made widely available. Mentorship programs pairing veterans of the system with newer survivors can foster hope.

Holistic rehabilitation: True recovery involves rebuilding life skills and social ties. Victims should receive help with education, vocational training, and housing stability. Programs like supported employment or disability-inclusive microfinance empower independence. Creative therapies (art, drama, music) and wellness activities (yoga, meditation, nature retreats) can aid healing. By validating each person's dignity and capacity, providers reinforce that *sanity* encompasses far more than symptom checklists.

Legal and rights support: On an individual level, victims need clear information about their rights and remedies. All victims should be offered legal counsel to challenge unjust treatment or obtain reparations. Social workers or patient advocates must guide them through any appeals or complaints processes. Finally, respecting freedom means acknowledging survivors as experts in their own care - institutions should routinely solicit and implement survivor feedback on services.

Throughout all levels, enforcing human rights strictly must be non-negotiable. Any violator - whether a state actor, institution, or individual professional - must be held accountable under the law. By combining top-down legal protections with bottom-up community empowerment and person-centered services, the system can transform from one of coercion to one of healing, dignity and true recovery.

Discussion

The proceedings of this symposium presented unequivocal and urgent evidence: the political abuse of psychiatry remains a critical, global crisis. Independent expert contributions revealed that psychiatry today, far from being fully shielded by its medical nature, remains highly vulnerable to manipulation, particularly in environments where incompetent authority replaces the sane state of mind and sound authority. When lawful governance rooted in rationality, ethics, and human rights is supplanted by arbitrary rule, rule of men and state backed criminality,

psychiatry is weaponized, becoming an instrument of oppression, repression, and silencing. Where robust institutional safeguards are absent, the vulnerability of mental health systems to abuse is not incidental -it is structural and predictable.

This permeable environment creates conditions in which abuses occur with alarming ease. Rogue states, corrupt actors, and criminalized systems exploit psychiatric institutions to eliminate dissent, punish the vulnerable, and silence the inconvenient, all under the deceptive pretense of medical care. As highlighted by David Matas, Yutong Zhang, Yanxi Mou, Manuel Llorens, and others, psychiatry, under these corrupt conditions, serves state terror, private vendettas, and criminal profiteering. This perversion is not an unintended anomaly; it is the direct result of allowing incompetent authority to replace the sane state of mind and sound authority that must underpin any legitimate exercise of power, especially where human vulnerability is concerned.

The urgency of reform cannot be overstated. As information shared during the symposium confirms, the human and economic costs of psychiatric abuses are vast. Victims endure profound psychological harm, social alienation, and chronic physical health deterioration, while societies bear immense costs in lost productivity, fractured communities, wrongful institutionalization expenses, and legal redress efforts. These costs are not inevitable. They stem from the deliberate failure to protect psychiatry's foundational mission: to heal, not to harm.

Most The symposium made clear that the weaponization of psychiatry is sustaining systemic failure almost by design. These are not accidents of practice but structural defects, exacerbated when mental health care systems are aligned more with social control than with healing. By diverting trauma, dissent, and vulnerability into silencing mechanisms rather than resolution pathways, failing or rogue systems maintain the illusion of order while exacerbating human suffering. Healthcare, rather than being a bulwark of collective well-being, becomes a tool of coercive governance, thereby entrenching dysfunction beneath a façade of medical legitimacy. Psychiatry thus risks becoming the most devastating betrayal: the betrayal of life, dignity, sanity, and hope under the false banner of care.

Speakers independently confirmed that abuses today are systemic, severe, and require urgent redress. The findings of the symposium call for immediate, coordinated action:

The findings and testimonies presented in this symposium confirm that addressing the political abuse of psychiatry demands not only institutional reforms, but a profound transformation in the way evidence is gathered, presented, and acted upon. This necessitates engaging multiple disciplines - law, human rights, medicine,

psychology, sociology, political science, and ethics - to create a robust, interdisciplinary resistance to entrenched abuses. It also demands action-research approaches rooted in rigor, survivor participation, and protection against the systemic reprisals that continue to deter transparency and reform.

At the international level, the recognition of political psychiatric abuse as an international crime must be accompanied by independent, interdisciplinary investigative bodies. Human rights experts, forensic psychiatrists, legal scholars, and survivor advocates must collaborate to rigorously document abuses across jurisdictions, ensuring that evidence is systematically collected, preserved, and publicized. The traditional barriers of fear, diplomatic inertia, and political compromise must be consciously dismantled. Only through fearless, independent reporting can abuses embedded within rogue states and failed systems be brought to the light of international accountability mechanisms. Action-research methodologies, grounded in survivor testimony and corroborated by forensic and legal analyses, are essential to avoid the sanitization or distortion of realities on the ground.

At the national level, legislation and policy must not only prohibit psychiatric coercion but foster environments where research into systemic abuses is protected and encouraged. National research councils, ombuds institutions, and independent commissions must actively support interdisciplinary investigations into psychiatric practices, without political or corporate interference. Protection of researchers, whistleblowers, and survivors must be codified in law, recognizing that fear, dismissal, and denial - often fueled by vested interests in the healthcare, pharmaceutical, security, and political sectors - have historically silenced critical inquiry. Nations must ensure that confronting psychiatric abuses is not treated as destabilizing but as strengthening the social and legal order.

Addressing these realities demands an uncompromising commitment to human rights enforcement at every level. Academic institutions must incorporate critical psychiatric studies, survivor-led research, and human rights law into mental health training programs. The system would also benefit from the approaches and interventions laid down in next paragraphs, as presented in the paper:

At the institutional level, psychiatric facilities and health systems must open themselves to external, interdisciplinary auditing. Routine human rights impact assessments, conducted by independent teams combining medical, legal, social science, and survivor expertise, must become mandatory. Institutions must be held accountable for retaliation against whistleblowers and survivors who expose abuses. Internal ethics committees must be reconstituted to include external human

rights monitors, ensuring that reprisals, denial, and corruption are neither normalized nor concealed. The dominant cultures of self-protection and reputational defense must be replaced by a culture of truth-telling, ethical transparency, and patient-centered reform. At the community and civil society level, grassroots organizations, survivor networks, and interdisciplinary academic groups must collaborate to produce and disseminate evidence-based narratives that counter the dominant myths supporting psychiatric coercion. Community-based action-research initiatives must document abuses, capture survivor histories, and monitor local services, creating alternative archives of truth accessible to courts, media, and the public. Civil society must also advocate for legal protections ensuring that survivors, researchers, and advocates can speak out without fear of defamation suits, professional retaliation, or unlawful surveillance. Breaking the cycle of social dismissal and denial demands making psychiatric abuses visible not as isolated anomalies, but as systemic failures requiring systemic remedies. At the individual level, every survivor and citizen must have access to mechanisms that support the ethical gathering and sharing of experiences without re-traumatization or retribution. Empowering individuals to participate in action-research projects, legal advocacy, and policy reform initiatives strengthens both the evidentiary base and the democratic legitimacy of psychiatric reform. Trauma-informed approaches must guide the documentation process, respecting the autonomy, dignity, and safety of those whose testimonies form the foundation of change. Encouraging survivor-led research centers and participatory legal projects ensures that those most affected are not merely subjects of study but protagonists in redefining the future of mental health care.

Ultimately, the transformation required is both technical and cultural. Interdisciplinary engagement must not be perfunctory; it must seek to dissolve disciplinary silos that have enabled psychiatric abuses to remain insulated from legal scrutiny, human rights accountability, and social science critique. Action-research must be fearless, ethically rigorous, and politically conscious, recognizing that psychiatry's entanglement with systems of repression cannot be dismantled without exposing the networks of power and interest that sustain it. True change requires recognizing that psychiatric abuse is not merely a professional failure but a profound social and moral crime - one that undermines public trust, damages countless lives, and perpetuates systemic injustice under a false banner of healing.

Breaking this cycle demands more than reforms: it requires courage, interdisciplinary solidarity, and the unwavering insistence that human dignity, freedom, and sanity are not negotiable. Only by fortifying research, amplifying survivor leadership, and protecting truth-telling against the reprisals of vested

interests can societies reclaim psychiatry as a genuine instrument of healing rather than a tool of fear and domination. Across all levels, the principles must be uncompromising: do no harm, eradicate coercion, deny criminal misuse, enforce all human rights strictly, and uphold the primacy of human dignity, freedom, and life. Psychiatry must not drift into being an accomplice to repression. It must actively become a guardian of humanity's highest ethical commitments.

The symposium reaffirmed that change is not only necessary but possible. Cases like Mikhail Kosenko's release (van Voren, 2016) illustrate that international attention, legal pressure, and ethical solidarity can disrupt even the most entrenched abuses. But vigilance must be relentless. Systems left to rot in silence and impunity will inevitably continue to betray the vulnerable.

Table 6: Reclaiming Healing in Psychiatric Care

Reform Principle	Key Measures
Codify Healing and Autonomy as the Sole Legitimate Purposes of Medical Systems	Rebuild legal and ethical foundations around healing, autonomy, dignity. Deviation becomes unlawful.
Criminalize Non-Therapeutic Medical Interventions	Punish non-therapeutic acts like forced medication or diagnosis without urgent clinical need.
Total Prohibition of Coercive and Punitive Practices within Healthcare	Eliminate coercion unless tightly defined emergency; align with CRPD obligations.
Separate Medical Support from Social Order Enforcement	Remove medicine from roles of social discipline, control, or ideological enforcement.
Invest Primarily in Prevention, Education, and Community Resources	Prioritize housing, nutrition, voluntary care, and health literacy over institutional reaction.
Decentralize and Democratize Oversight	Establish oversight with survivors, legal and community representatives with real sanctioning power.
Reeducate Medical Professionals Around Human Rights	Integrate human rights law and abuse history into medical education. Train to resist complicity.
Protect Whistleblowers and Dissenters Within Healthcare	Legally protect ethical dissenters and whistleblowers in healthcare systems.
Guarantee Full Access to Justice and Reparations for Survivors	Ensure complaint mechanisms, redress, and public recognition of abuses and survivors' rights.
Maintain Permanent Global Surveillance Against Abuses	Create global observatory to monitor violations and ensure binding public reporting.

Protecting mental health systems from political and criminal abuse is not ancillary to justice - it is central. If psychiatry becomes again a tool of oppression, society itself descends into normalized cruelty masked by pseudoscience. Healing psychiatry requires nothing less than restoring it to its ethical, humanitarian, and rational foundations: to protect life, to foster recovery, to uphold freedom, and to enshrine dignity. Without urgent and principled action, we risk perpetuating a system where incompetent authority, masked as medical judgment, erodes the very sanity and freedom it was meant to preserve. To prevent this, psychiatry must be reclaimed - not merely reformed - as an instrument of hope, truth, and collective healing.

Medicine must return to its only rightful foundation: healing and protecting life. Safeguarding the integrity of mental health care is more than a professional duty. It is a moral imperative grounded in the universal principles of human rights. It is a testament to our shared humanity. Without this vigilance and collective commitment, healing cannot truly begin - and dignity cannot truly be restored. With it, however, psychiatry can fulfill its highest promise: not as a tool of domination, but as a sanctuary for healing, understanding, and hope. Any other purpose, be it control, punishment, profiteering, silencing, is a perversion of its meaning and a betrayal of humanity itself. This is not a call for mere reform, but for a complete reassertion of medicine's original, moral purpose. Psychiatric and medical abuses cannot be addressed by improving policies alone; they must be rooted out by re-centering health systems on truth, autonomy, and dignity. This study underscores the ethical salience of survivor testimony for system design. Learn, from past and present mistakes to effectively deny any opportunity for these crimes against humanity repeating once more. Their pain must not be abstracted. It must be remembered as the living cost of institutional betrayal. We must also protect those who speak on their behalf: researchers, journalists, legal advocates, clinicians, and families who face intimidation, ostracism, and retaliation for uncovering the truth. The same goes for informants within closed institutions, and the communities caught between fear, coercion, and complicity, too often driven to enforce atrocities among themselves, out of anguish, fear, or opportunistic cruelty. They too deserve protection, and they too require systems that do not abandon or exploit their position.

This is the core of what must change: not just policies, but structures -oversight with real power, education rooted in human rights, legal accountability with teeth. Not symbolic ethics, but mechanisms of prevention, exposure, and repair that cannot be silenced. It requires unwavering courage, clarity of intent, and structural transformation across education, practice, law, and governance. A healing

profession that fails to protect the living right to flourish is no longer a profession. These ten pillars lay the groundwork.

Conclusion

The symposium laid bare a stark reality: the political abuse of psychiatry persists today as an entrenched and systemic phenomenon, deeply intertwined with rogue governance, failed healthcare systems, and criminal misuse of medical authority. Where incompetent authority replaces the sane state of mind and sound ethical governance, psychiatry ceases to serve its healing purpose and becomes an instrument of repression, silencing, and destruction. The profound human and economic costs - borne by survivors, communities, and societies at large - reveal the devastating consequences of allowing mental health systems to be weaponized. These abuses are not incidental; they are the predictable outcome of systemic failures that prioritize control over care, coercion over communication, and impunity over justice. Urgent, coordinated, and uncompromising action is required at every level: international, national, institutional, community, and individual. Psychiatry must be reclaimed from corruption and restored to its rightful role as a guardian of healing, dignity, and freedom. Protecting mental health systems from political and criminal abuse is not a technical reform - it is a profound moral imperative, central to the defense of human rights and the preservation of sanity itself. Only by dismantling the structures of coercion, enforcing strict human rights protections, empowering survivors, and rebuilding care on the foundations of respect, voluntariness, and solidarity, can psychiatry fulfill its highest ethical promise: to heal, to protect, and to uphold the inviolable dignity of every human being.

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